Feel To Heal

RELEASING TRAUMA THROUGH BODY AWARENESS AND BREATHWORK PRACTICE

BY

GITEN TONKOV

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To my son, Sage.

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Editor's Foreword

Reflections of a Ghostwriter

The usual method of working with an author is to assist him or her in collecting thoughts, and to arrange these thoughts for publication. We technicians look at what's been compiled and provide critiques about what's missing. We talk about form, style, tone, and voice. We might collect supporting information, check sources, and analyze illustrations. We think about grammar. We also have discussions. Sometimes lots of discussions. If a hired writer is lucky, he or she might get to travel to some interesting location, attend professional conferences or family reunions, and enjoy home-cooked meals. Meals cooked by someone else.

In Giten's case, the experience was somewhat different. Giten is experiential. He has lived a life of hands-on study and exposure to his subject areas. His mission is to work with people in locations all across the globe, and he speaks several languages that are not English. He is brave. He is confident and curious, interested, and welcoming of new information. He is open to change. These are a few reasons why he was able to create his own modality, the BioDynamic Breathwork & Trauma Release System. Making something new requires flexibility.

Giten is also in constant motion. If, as a researcher, you ask for the scientific cite behind a statement he relies on, he can tell you *that* he read the material, and often, who wrote it. But since his life is contained in a suitcase, and his practice rooms are sprinkled in resorts on several continents, he cannot quickly flip to a page. He often just knows things, and explains them elegantly and with calm intensity.

Giten carries old traditions into today's world. Like medicine men and layers-on of hands from generations past, he travels from village to village, sharing what he knows. He does not travel with a library, but instead uses his experience, intuition, knowledge, and common sense. In Giten's case, he works on the world stage, and the villages are in different countries. He practices what he preaches.

Still, at first I didn't fully understand the depth of what he was offering. It was tempting to fall back on my psychology training, and to stay in my mind when exploring what Giten was describing. This was inevitable, since we began by talking—through interviews that sometimes spanned blocks of several days in person, or long, intensive phone or video discussions. We logged weeks of dialog over more than a year. As I got the facts down, however, I realized that I needed to involve more than my mind. I had to experience BBTRS.

And what, exactly, did I experience? What, exactly, *is* BBTRS? It uses breathwork to stimulate the body's natural nervous-system processes, which in turn allow us to better sense what is happening within us. With coaching, a practitioner learns how to feel—and the body is allowed to move in natural, organic expression that actually releases tension. Additional elements, including meditation, bodywork, sound, and emotional expression facilitate the release.

BBTRS involves thinking differently enough to stop thinking. The body simply takes over, because it already knows what to do.

I did not have to be in charge, and after the first session, I understood what Giten had been telling me. When the week was over, BBTRS had cleaned out my insides as if I had received an internal shower.

For the first time, I also knew how to really, truly breathe.

With this new perspective, I then talked with doctors, psychologists, massage therapists, BBTRS trainees, statisticians, and trauma specialists. I spent months researching specific physiological processes, physics, energy, Eastern and Western medicine, sound waves, bodily structures, lifespan development, epigenetics, a variety of animals, anxiety, PTSD, sexuality, disease, meditation, emotion, cross-cultural implications, and happiness. In the process, I learned about my own internal terrain.

The result is this offering, a book that attempts to capture Giten's contributions to the subtle process of trauma release as it stands today. His practice assumes that the world is more than we can see; it is scientific and metaphysical, spiritual and material. And because of every butterfly that flaps its wings—and every person Giten works with—his practice will continue to grow and change. As we all do.

Just as I began my study, we begin this project with words—even though words can go only so far.

As you read, take off your shoes and empty your pockets.
Now it's your turn.

Kristin Donnan
South Dakota, USA

Preface

This book was born when I was twelve years old, when I was blindsided by a car accident that suddenly knocked me unconscious. I had not "seen it coming," and for years after, I assumed that I had fully recovered through medical treatment. Later, when practicing cathartic breathwork as an adult, I discovered remnants of the trauma that were still present in my body. In considering how my twelve-year-old self had experienced the impact, I finally realized that my fight-or-flight responses simply had not processed in the moment. Any chance for release or follow-through had been stopped dead in its tracks when I lost consciousness. Through breathwork, I dedicated myself to peeling back the layers of the trauma locked in my body. Because of the cathartic nature of the work I chose at the time, however, I was exhausted for months afterward.

Breathwork was a natural go-to because of my first love. I had played the trumpet seriously from the ages of seven to nineteen, with the intention of becoming a professional musician. I played for hours each day, using my breath to generate sound. My breath was my tool, my entertainment. Only my fellow musicians understood why we chose to practice instead of joining our friends for games.

My family moved from Ukraine to the United States when I was seventeen. Within a couple of years afterward, I realized that I had reached the limit of my ability with the trumpet. I was hindered by physiology, by the way my mouth was naturally shaped. I accepted that music would not form the foundation of my future, and I turned instead to something else I had experienced in my youth. My uncle had seen my seriousness and dedication as a child, and my commitment to a "practice." He was a talented masseur, and he had regularly rubbed the tension from my shoulders. He decided to teach me massage.

Therefore, when I considered a profession other than music, I turned to physical therapy and medical massage, and then to a wider range of bodywork. I found fascinating the process of working in different modalities, including Swedish and Thai massage, Shiatsu, lymph drainage, and the five elements in Chinese medicine. To me, it made sense to look inside the body, to consider it as a garden in need of nourishment and tending. In my private practice, I employed the basic tenet that each person I touched was in need of a tailored, individualized approach. When I laid my fingers on anyone, I could feel the person's level of stress, openness or protection, and what techniques to use.

I developed a thriving bodywork practice in New York City, and also spent a few months each year exploring other countries, experiences, and trainings in different modalities to add to my practice. I explored everything from the counseling aspect of working with people, to Stan Grof's Holotropic Breathwork, to hallucinogenic plants, all intended to expand my awareness of what might be possible. My relationship with meditation also became a favorite area of study, and, eventually, part of my daily routine. I had first meditated at the age of five, while looking out at the sea; I was naturally attuned to it, and introduced meditation into my bodywork sessions. I soon found that if I was in a meditative state, then clients would naturally fall into that same space. Over time, I would ask them to breathe, which opened them to deeper relaxation. We explored the differences between natural breath and conscious breathing, which led to even more physical and emotional release.

The idea that release occurred deep within the body's tissues made organic sense to me. During my youth, I had spent several hours each day using my entire body to create sound. The vibrations had become part of me. They had arisen through a coordinated inner symphony performed by the diaphragm, chest and rib muscles, arm and leg muscles, and facial muscles—all fueled by the breath. I had played from the inside out, and felt the music form through my inspiration. When I touched people through bodywork enhanced with breath, I saw this same sort of symphony happening within them.

The next big piece of the puzzle came after my cathartic experiences around my accident. I continued to find myself activated by certain kinds of stressors—but not just relating to the accident. I had become deeply aware of my childhood—our family's dynamic, our cultural upbringing—and even how these circumstances were evident in the development of my physical body and personality. I realized that how I related to myself and others was tied to this history. My life had not been perfect. Like everyone, some day-to-day challenges and upsets had stayed with me. My history was part of my bones.

To achieve the next level of understanding—for myself and my clients—I had only one place to turn: my body. I naturally sought out somatic, or body-based release modalities. After many sessions over the length of a year, not only had I actually released my accident-based traumas, but also I had a fuller understanding of how to walk in the world with more resilience. The practice we used did not involve breathwork, but it did provide great relief. I felt as if I was "almost somewhere." I just didn't know exactly where.

The final key that led to my current work came from ayahuasca, a traditional spiritual medicine used by indigenous peoples of the Amazon basin. I experienced a vision while participating in a ceremony in Colombia. It was a dynamic inner knowing of how to combine breath, touch, meditation, body awareness, and elements of somatic therapy to take my work to another level. I knew in that moment that I could dramatically reduce the time needed for a person to transcend something that was deeply embedded in the body. With time, I also added the crucial aspects of sound and emotional expression to the new modality, which was named the BioDynamic Breathwork & Trauma Release System®. In this book, the modality will often be referred to as BBTRS®.

In the decade since this realization, I have felt sure with every cell of my body that this modality works. And with each training of new practitioners, or each experience with a breather, my vision is reinforced. Ayahuasca visually demonstrated for me the interior of the human body and what happens when we breathe. I saw the natural movement, the support provided by touch, and how tissues respond. It was kind of like watching a movie, but much more than a beautiful visual experience. It was an understanding that this modality can help people.

This book is a new expression of the work, and a new way to share it. With every person who experiences BBTRS, it grows and shifts. Perhaps your experience will also cause it to grow into its next form.

PART 1

TAKING ON TRAUMA

“Some doors only open from the inside. Breath is a way of accessing that door.” ~ *Max Strom*

Chapter 1

A Look Inside

Trauma is real. It’s not just an idea, or a feeling of upset. Trauma is a physical reaction that becomes part of our bodies, a response to situations large and small. It builds and settles and takes up residence in our tissues over time, expressing itself in our everyday lives, sometimes for years. And years.

Since trauma can look like almost anything in almost anyone, we don’t know of a single aspirin or prayer or surgical procedure or yoga posture that erases it. What we do know is that even day-to-day stress can wreak havoc on our quality of life, our health, and our relationships. Imagine what trauma can do.

Trauma ups the ante. Trauma is what happens when our systems become overwhelmed, whether by a specific event or life circumstances. If we cannot adequately process and “clear” these experiences—consider the effects of long-term abuse, an attack, sexual violence, or wartime—then our lives can become unmanageable. Importantly, we can trigger the body’s natural “emergency” responses even when we’re not being chased by a tiger or threatened at gunpoint. Depending on about a million factors, or possibly more, any given person can experience trauma because of a perfect storm of events. If the exactly wrong thing happens at a moment when we are physically, emotionally, mentally, and/or spiritually exhausted, afraid, confused, or otherwise unable to cope, then trauma can result.

Researchers and practitioners have been in the trenches—literally and figuratively—with trauma sufferers for decades. They've studied various therapeutic approaches, with some of the most fruitful focusing on veterans suffering from Posttraumatic Stress Disorder (PTSD). Veterans and others who work and live in wartime or dangerous settings provide living examples of the effects of intense or long-term stress—and what works for them has been shown to benefit people with a variety of life experiences and challenges.

"What works" is always a challenging proposition, because no single approach helps everyone. However, researchers have found that sometimes during traditional psychotherapy, veterans could "re-activate" trauma responses. That is, right there in the therapy room, they felt like they were "re-living" past events—just because they were talking about them. Like a ticking time bomb, the trauma seemed to be hidden within their bodies. That's when researchers focused away from "what happened" and toward the body's natural functions. How does it process intense experiences? How does it hold tension? If two people share the same traumatic experience, and one does *not* have PTSD, then where does the tension go? How do people process or release tension?

With this shift of focus, the field of trauma recovery turned to "somatic" therapies. "Soma" is about biology—it's a word referring to our physical bodies, distinct from the mind, psyche, or soul. When researchers tried mindfulness or relaxation practices in PTSD treatment, the results were positive. For example, studies showed that breathing-based meditation and yoga reduced PTSD symptoms—even in Navy Seals. "Navy Seals are not new-age, cuddly people," says author and meditation teacher Max Strom. "Navy Seals only use technologies that work."[1] Meanwhile, a handful of top therapists was going to the next level. They zeroed in on triggering the body's auto-pilot, natural stress-releasing functions. Their work would revolutionize the field of trauma treatment.

In February 2015, President Barack Obama signed into United States law the Clay Hunt Suicide Prevention for American Veterans Act.[2] While its purpose is to aid veterans suffering from PTSD, of course any new attention or funding for trauma treatment benefits the broader population. Because of the legitimization of trauma research in recent years, along with the new legislation, trauma release practices have become more and more mainstream. It is now accepted that stress or upsetting events act as catalysts that leave behind…something. We can call it residue, "stuckness," energetic blockages, or some other word that is more than just a concept. We're talking actual, physical manifestations, where the body has accommodated itself to manage additional stress in the tissues. Science shows that this tension is not the only residue that can be traced to trauma stuck in our systems. Otherwise unexplained symptoms, recurrent nightmares, flashbacks, physical illnesses, and psychological expressions are also common. Thankfully, like a splinter, this trauma can be removed.

In the BioDynamic Breathwork & Trauma Release System® (BBTRS®), we combine successful somatic approaches with other elements to allow our natural physiological functions to clean out what was left behind by traumatic events. Results happen when we engage our natural, physical processes—the ones that mammals automatically activate when encountering danger, whether real or perceived.

Stress versus trauma

The terms for how we feel, by definition, are subjective. One person's idea of a headache might be different from someone else's—and words like "stress" can have limitless shades of meaning. Therefore, our task in a book about trauma is to define terms.

We've all felt stress—and we've seen it in those hunched shoulders or clenched jaws in a family member, a coworker…or in the mirror. Our best efforts to fill our lives with a healthful diet, rest, exercise, enriching relationships, and a good balance of work and play cannot prevent stress from creeping in.

And it does creep. Stress can be an invisible, moving target. For the purposes of this book, stress is often the result of thoughts and beliefs, both conscious and unconscious—especially those rooted in fear. We feel fear about what we are able or unable to do, about the manageability of our lives, about money and love and relationships, and about what is happening in the wider world. Our thoughts fuel this fear, and all manner of negative manifestations can result.

Stress can also be present from past events. Mental memories and "sense memories" can be buried deep within us, acknowledged or not, often for long periods of time. They can become unexpectedly triggered by an event, or they can cause illness or disease.

Trauma, on the other hand, goes beyond stress. Unlike stress, which often can be successfully managed by lifestyle changes and various forms of psychotherapy, trauma is the result of our system's overload. Trauma happens when we can't cope with an event, a set of circumstances, relationship dynamics, or even ongoing stress. Generally, we can't "talk ourselves out" of trauma, because our body has taken over.

Trauma results when our systems read a situation and see a threat. We are hard-wired to respond to threats in ways that bypass our conscious thoughts, and this is where trauma becomes insidious. We might think we're fine, and we might feel okay—but something's just not right. The presence of trauma can be obvious. Consider a war veteran who suffers from PTSD. A car's backfire can stimulate that person to dive under the nearest table, as if dodging gunfire. On the other hand, trauma is not as obvious in other circumstances, where a person might feel suffocated, anxious, or "frozen" when someone else feels no response, or is mildly uncomfortable.

In this volume, "stress" is considered to be a possible contributing factor to trauma, but trauma is not the same as stress. The term "tension" is used not as a synonym for stress, but instead to indicate the state of contraction in the body. Specifically, tension is how the body holds trauma, and it is what is released in the BioDynamic Breathwork & Trauma Release System.

Therefore, trauma is not an experience, but our body's response to an experience. Tension is what we attempt to find, feel, and release as a result of trauma. Meanwhile, stress leaps and dances around the process, adding its two cents at every turn. And in certain people, stress results in trauma, too.

When this cheetah chases prey, it stimulates the warthog's autonomic fight-or-flight mechanism. These built-in processes are meant to save our lives in the face of danger.

Why we react

What happens in our bodies in response to traumatic events and circumstances is hard-wired. First coined as "fight or flight" in 1915 by Harvard physiologist and authority on psychosomatic medicine Walter Cannon, an "acute stress response" occurs when animals and people perceive a harmful or threatening event. Our bodies flood with hormones and our blood flow changes, all within as little as one-twentieth of a second—shorter than the time between two heartbeats, and long before our conscious mind has processed the threat.[3] In that split-second of deliberation, our bodies automatically calculate how best to utilize the extra energy that could very likely save our lives.

Getting Out of Trouble

Cannon's work showed that when our bodies react to such an event, automatic processes take over—changes in our blood supply, brain impulses, hormone secretions, and

organs ready us for a “violent display of energy.” Cannon said our body’s first survival decision is whether to run away or engage the threat.

In the one hundred years since Cannon first described fight-or-flight, many other researchers have refined our understanding of the body’s auto-pilot responses. Another viable option is to freeze. In some animals, the freeze response is part of the fight-or-flight preparation; the brain is ready for any action, but remaining still can provide the best chance for survival. “Playing dead” or fainting is not a decision; it’s an automatic, useful response in the wild, because some predators are not interested in killing an animal unless it is running or fighting back. In these cases, a prey animal’s heart, respiratory rate, and body temperature reduce, and in some cases an animal stops breathing for several minutes. In humans, the freeze response feels impossible to overcome. “I just could not move—I could not run to my child as the car approached. My legs would not work.” We might freeze when we are totally overwhelmed, and see no opportunity for survival or escape.

For humans and some other mammals, there is one other possible response to a threat. It is called “social engagement.” Basically, it’s a process of making friends in order to diffuse the aggression. The researcher who coined this term, Stephen Porges, says, “If somebody’s really angry at us, we want to explain what happened so they don’t

strike out at us."[4] Therefore, in situations where people are able to work with others, escape a difficult situation, or take control, fight-or-flight subsides. However, when we can't escape or take control, we might remain in a state of anxiety, even when the threat has subsided.

Each of us has felt these natural processes in some way at some time. A traumatic event can be as simple as being startled by an aggressively barking dog, or as overwhelming as a personal assault. It can also be the cumulative effect of long-term but less obvious stressors, such as nagging interpersonal difficulties, ideas about what we could be doing better in our lives, or generalized anxiety. Importantly, it's also the result of suppressing our bodies' natural responses to these events and day-to-day experiences.

All these "decisions"—to fight, flee, freeze, or work with others—occur through involuntary processes in our bodies. However, just because these processes are natural doesn't mean they're perfect.

Two unfortunate possibilities are commonplace in the human experience. The first is that the protective fight-or-flight process can be stimulated as a "false alarm." Environmental, mental, or emotional circumstances can trigger the entire chain reaction in situations when we're not really in danger. In these cases, our state of heightened reaction is overkill, and might have consequences. An obvious consequence is that we're just unnecessarily triggered, anxious, and out of balance. Even worse, we might become so accustomed to this level of activation that it becomes our baseline—either a comfort zone or a launching pad. According to PTSD researcher Bessel van der Kolk, people with traumatic histories might feel more comfortable in times of stress, seek out moments of high stress, or blunt the pain of stress with alcohol and drugs. “Many traumatized people seem to seek out experiences that would repel most of us,” he writes, “and patients often complain about a vague sense of emptiness and boredom when they are not angry, under duress, or involved in some dangerous activity.”[5]

The second unfortunate possibility involves what we do with our fight-or-flight energy after it’s activated. Instead of letting it “do its thing,” which naturally is to dissipate, we hold it in. Let’s go through a scenario. Say we’re faced with real danger, or have come upon someone else’s altercation or accident. A mugger. A car wreck. A house fire. Fight-or-flight says that we first might run away, fight, hide, or attempt to help a victim. We might call the police. We even might lift a car off an accident victim or achieve some other heroic—and seemingly impossible—act.

Then, after the urgency is over, we arrive at that next moment. The immediate danger has passed, but we’re still full of adrenaline. Now what? We try to slow our breathing, and we often instinctively hug ourselves by wrapping our arms around our own upper arms and shoulders. We shut down the body’s natural responses—often never, ever experiencing what they would have done on their own. Therefore, our initial response to traumatic events is automatic, and our secondary one is protective. Both can result in traumatic residue that can stay with us for years, or for a lifetime.

Unfortunately, because we want to avoid traumatic events, we also tend to avoid the most effective method of dissipating the extra energy that results from them—shaking, or tremoring. Tremoring is the natural outlet for built-up energy that courses through the system because of the fight-or-flight response. Left to its own devices, the body simply "shakes it out." By instead attempting to "calm down" our natural responses, we inadvertently cut short our bodies' natural reaction cycles. And when those cycles are cut short, trauma remains in our tissues.

Keep in mind that we're usually as uncomfortable with somatic activation in other people as we are in ourselves. Consider what we do when a child is "freaking out" with distress, or what an emergency responder does when assisting an accident victim, assuming physical injuries are not the primary concern. What does anyone do when they see others exhibiting heightened responses, especially uncontrollable tremoring? We calm them down, we put blankets around their shoulders, and we say, "Shhhhh." On the other hand, if we come face-to-face with a person who is hyperventilating, sweating profusely, and shaking, we might even weigh whether we are in danger. In today's world, floods of adrenaline and blood sugar concern us. At best, these signs of activation are not socially understood as normal reactions; at worst, they are seen as indicators of volatility.

To be fair, being "out of control" can be a terribly worrying phenomenon, in ourselves and in others. It can signal danger. It can signal acute distress. However, if we begin to understand the normal physiology around letting trauma *leave* our bodies, then dealing with life's stressors becomes a different ballgame. We simply look at it differently. We look at ourselves differently.

Can we avoid trauma, or become more resilient?

Trauma usually results when your fight-or-flight is engaged, but there's nowhere to run. In everyday life, "nowhere to run" might mean any one of a million things. Maybe it's literally that you cannot physically or emotionally escape danger; it might mean that something difficult asks too much of your current ability to cope. Trauma also might remain if you *could have* coped, but didn't have the opportunity to, or shut yourself down.

It's easy to empathize with those who live through a devastating earthquake or terror attack. We all can imagine the aftermath of being mugged or freezing up in a television interview. Isolated incidents at any level of intensity can leave lasting impressions in the body—but chronic stressors are potentially as debilitating. As our societies have become less predator-and-prey and more industrial-and-technological, we might not be familiar with being attacked—but we are used to constant worry.

In the long term, humans are not built to sustain high energetic levels of fight-or-flight activation. Our auto-pilot fight-or-flight response was designed as a short-term survival tool. Just think for a moment about how society "manages" an emergency. Choose just about any significant public event. Begin with one that didn't even happen to you personally, and think about being glued to a screen, playing and replaying the video clips, absorbing the anxiety, alarm, fear, and devastation of those affected. As observers, our minds become infected by the social activation, the seemingly endless discussion about hypothetical outcomes. On the other hand, when we experience a personal event, such as the death of a loved one, we often launch into action. We push aside obligations that just yesterday were paramount; we buy last-minute plane tickets, organize events, and "run on empty." Because of our overwhelmed state, friends bring food for us, and take care of the cat.

Now imagine living like that for years—in the "state of emergency" that is intended to last a few days or weeks. People do it all the time. Perhaps they're experiencing a chronic medical problem, a legal difficulty, or financial strain. Perhaps they're worrying about global warming or big problems they "can't change"—such as the plight of refugees, indigenous populations, or endangered animals. In these circumstances, people become used to "putting aside" the day-to-day matters of life. They focus on distress. They feel miserable, and accommodate feeling miserable. Then they become used to it, and even build their lives around it.

The kind of chronic stress that changes lives like this can come in limitless forms, and it causes a wide variety of physical and mental ailments. The good news is that we are amazingly resilient. When in doubt, consider the countless examples of people who have survived—and thrived—after unimaginable hardship. If they can do it, then the average person who wants to "feel better" can change his or her relationship with stress.

"Stress relief" is now a household term. More and more people are aware of practices like getting enough sleep, or spending time with family. Practices like "mindfulness" have filtered into the mainstream lexicon, and meditation is no longer just for gurus and hippies. Now, with trauma release, we can literally—physically, at the cellular level—change what's happening in our bodies. The process is natural, and relatively easy. It allows us to let the misery and discomfort fall away—so that we can live our actual lives.

Resilience in motion

“Nisarga Eryk Dobosz has been a bodyworker for more than twenty years, and now conducts bodywork and breathwork trainings around the world. Despite his success with others over the years, however, until he encountered BBTRS, Nisarga had his own challenges. In his personal life, he had difficulty in connecting with his heart—and numbed his feelings when the emotional road became rocky. Nisarga had experienced abandonment in his childhood, and he held himself apart from relationship as an adult. He simply didn’t trust personal connections.

Everything changed during his first BBTRS training, in Goa, India. “We were a few days into a really intensive, ten-day training when I received a message that my father had died. He lived in France at the time, and here I was, in India, in this process,” he recalls. “Finding out about his death was tremendously painful, but something happened to me that I didn’t expect. I got a lot of overwhelmed feelings, but breathwork gave me a sensitivity that I had never known. I could really feel myself. The emptiness was so painful—but through using my breath, I had the capacity to feel the presence of emptiness. I was able to say goodbye, and to understand that our parents are meant to die, and we are meant to feel these transitions.”

Nisarga’s life changed in the next few days. With the help of the group members, he celebrated the life of his father with gatherings on the beach. He allowed himself to deeply recognize that he would never talk to his father again, and that a chapter in his life was ending.

Both the breathwork and the loving presence of the others in the group provided an unparalleled support system, a new understanding of relationship—and a new career path. “The essence of breathwork is feeling, being present with the body,” he says. “Breath is an inner resource. It allows me to go through life with the experiences of ecstasy and pain, but not being attached to either. These states don’t last forever, which gives me peace, and a new trust in life. That’s how I knew that working with the breath was my path. It changed me, and my understanding of feelings and emotions.”

The sensitivity that opened in Nisarga some eight years ago has remained his new way of looking at the world. He now has no need to escape from his feelings, he is more present to his daily life, and he takes responsibility for his pain. “I’m cherishing feelings now,” he says.[6]

It’s important to note that all people do not respond to stress in the same way. Even when two people find themselves sharing an experience, they might react very differently—because the resilience of everyone’s nervous system is unique. Any person’s level of resulting trauma is therefore completely individualized, and begins in our physiology. We will talk about the nervous system in greater detail in Chapter 2, but in general, how well we manage stress has to do with the balance within the nervous system. Laurie Leitch, PhD, Director of Threshold GlobalWorks, is a psychotherapist and clinical trainer who developed the Social Resilience Model (SRM). SRM's focus is on building self-regulation skills for individuals and groups, and it relies on a deep understanding of how to improve our nervous system's responses to stress. She explains the basics this way:

"Today, in part because of the amazing advances in neuroimaging, we can see pictures of how electrical impulses travel in the brain and what parts of the brain light up when we think, feel, or do certain things. We know how clever the brain is at learning to compensate for damage. We know that no matter how old we get, our brains are able to learn (thank goodness!)…creating new neuronal pathways. As the saying goes, 'the neurons that fire together wire together.' This can be good news and bad news. The good news is that people can be helped to have greater resilience by practicing skills that reinforce healthier pathways in the brain. The bad news is that remaining stuck in disabling beliefs, negative emotions, and hurtful practices wires dysregulation into the brain."7

Leitch says that when the various parts of the nervous system work in harmony, the body functions in "The Resilient Zone," making us capable of integrated functioning. "There is a fit between thinking, feeling, and sensing," she explains, "and you can respond to situations rather than being reactive to them. When they are in The Resilient Zone, people are better able to work in collaboration with others, exhibit principled behavior, and think creatively."8

Somatic therapist Peter Levine defines trauma as something that interrupts the regular coping mechanisms of the body—and kicks us out of our Resilient Zones. He says that if fight, flight, or freeze is successfully completed, then any situation doesn't necessarily have to be "traumatic." It might be unpleasant, or truly awful, but it doesn't have to damage us. For example, if we flee from an aggressor to safety, and the fight-or-flight response goes through its cycle—including shaking out any remaining energy—then we might not retain residual trauma. On the other hand, if the system goes into full-bore activation, but is not fully expressed to the end, then the charge can get caught, or stuck in the body.

Levine says that common results of an uncompleted stress cycle are tension and behavior change. Think of it as a new "first impression" each day. Each morning, consider if you or someone you know has shifted from aware, energized, and interactive to frozen or hyperactive or lost in daydreams. Is anyone you know plagued by a constant or intermittent cloud of worry, anxiety, and defeat?

Like the "disabling beliefs" or "hurtful practices" that Leitch describes, trauma can impede our resilience. However, there is a key difference between beliefs or practices and embedded trauma. We can "think our way" out of a belief and "decide" to change a practice. We can discuss our worries or anxieties with our counselors or our family, even if we might not be touching their underlying roots.

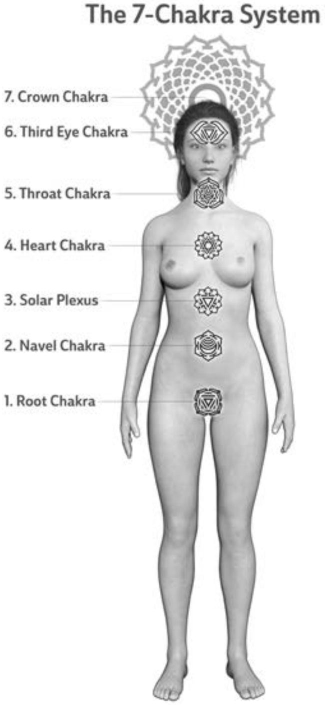
However, we can't simply think or talk or decide our way out of trauma that has lodged in our tissues.

There are two ways that BBTRS can help people to manage stress and trauma. The first is to deal with traumas already in the body. If our bodies are quite full of trauma—our inner "glass is full"—then we're just stressed out. First, we must empty the glass. Second, not only does the work release the trauma from the tissues, but also it teaches people to be aware on an intimate level of what's happening in the body. We can increase sensitivity, learn to regulate our physiological systems, and support healthier day-to-day living. In this process, we can change the neural pathways, the ways messages are sent from our brains to the rest of our bodies. We can change, at a foundational, physical level, how we function. So yes, we can become more resilient when traumatic experiences enter our lives, and we also can avoid holding the trauma when they do.

Where is trauma held—and how do we let it go?

Although trauma can be held anywhere in the body, there is a direct relationship between trauma and seven "belts of tension" that generally correspond with yogic chakras. Activation starts in the brain—where it can cause long-term changes in structure and function[9]—and then filters down the spinal cord, through the nerves, and into the rest of the body.

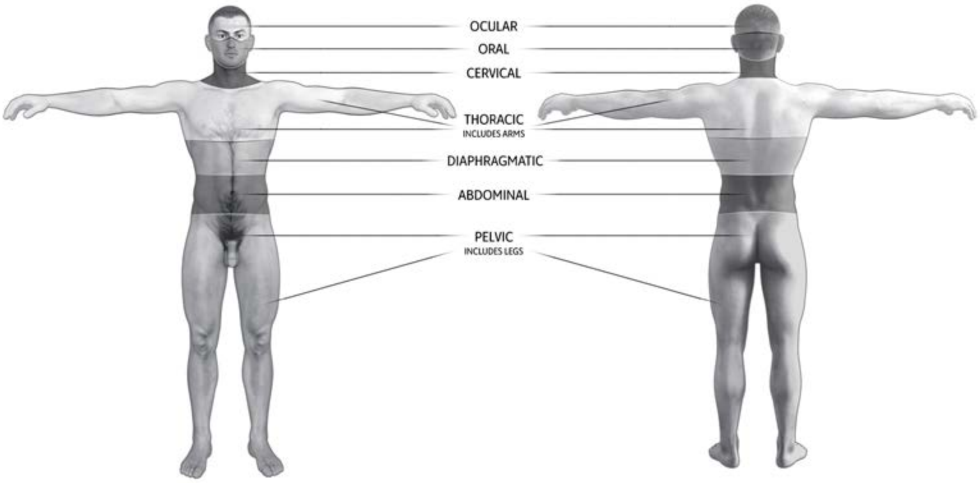
Later in this text, each belt of tension will be addressed individually. However, here we'll mention one area that is particularly sensitive to trauma: the core, a set of deep muscular structures in the lower torso. It is particularly vulnerable because the belly is the only area in the body that is not protected by a hard shell of bones. We rely on core muscles, both superficial and deep, to stay erect—and to unconsciously "armor" ourselves. The body automatically protects us by tightening muscles, whether against significant threats or simply as we interact with the people and situations in day-to-day life. Therefore, when we have unresolved tension, it's not unusual to hold the belly tight. The fact that the core muscles are often involved in tension is also evident in our language. We might describe receiving a surprising bit of news as a "punch to the stomach," or experience worry as a "stomach ache."


The 7-Chakra System
7. Crown Chakra
6. Third Eye Chakra
5. Throat Chakra
4. Heart Chakra
3. Solar Plexus
2. Navel Chakra
1. Root Chakra

For millennia, people who come from different disciplines and cultures have identified energetic centers, or areas of charge in the body. The chakra system is a map of energy centers from Vedic tradition. Psychotherapist Wilhelm Reich identified "Belts of Tension" that roughly correlate to the chakras.

The physical nature of our relationship with tension can assist us in letting it go. That's why somatic trauma-release programs focus on the physiological remnants of trauma—as opposed to emotional responses to it, or even the source of the trauma. Somatic release is so successful with PTSD sufferers because they do not have to recall, explain, or even consider "what happened," why it happened, how they feel about it, or what they could have done to avoid it. The reasons behind the trauma are often irrelevant. "What to do next time" can become relevant—but after we understand more about how trauma release works, and how to be more in tune with our bodies.

Let's consider Jorge. Now 32 years old, a husband and father, he has been moody, quick to anger, and less energetic than in years past. He is



7 Belts of Tension

a computer programmer, three years' out of the military. While in the Army, Jorge met his wife, and both are members of large families. They have plenty of help with childcare and share holidays with relatives. As with any family, "life" has expressed itself in theirs—two grandparents passed away in recent years, and both Jorge and his wife, as well as some of Jorge's siblings, have experienced work or financial difficulties. Then, about a year ago, Jorge's old car was rear-ended and totaled in an accident that caused him only a minor injury; he doesn't think this is his primary problem.

Jorge tells himself that this is the story of the "average person," and that nothing out of the ordinary has occurred that would explain his feelings. Especially since he was not posted to dangerous locations during his military service, Jorge cannot "blame" PTSD; he simply doesn't know why he is moody. "We have it pretty good," he says, "and things have improved in the last five years," he says. "Why don't I feel like myself anymore? Is it just stress?"

Because most people notice symptoms or behaviors, both Jorge and the people in his life probably report changes experiences with Jorge's moods. Everyone has seen the changes in him. If they thought about it, they probably could see changes in themselves, too. Trauma, whether identified or not, can manifest as distractions—ailments, distrust, fear, fatigue—that change our "factory settings." The subtle effects of trauma can shift our attention from "being" to "suffering." Trauma can become a filter through which we view life, relationships, work, play, and feelings.

When approaching the release of Jorge's trauma—thus easing his fatigue and calming his anger—all we need to know is that something happened that impacted him negatively. The event might have been a conversation, a physical altercation, his car accident, or even a thought. He might have worried about money, or experienced fear about a potential military transfer to a war zone.

Jorge also might have experienced similar events in the past that did not impact him in the same way. But this time, this "something" impacted Jorge's entire system, just by its existence. Even if Jorge's body was usually capable of processing or expressing it, this time, for some reason, it simply couldn't. We don't need to know why. We don't need to know what the "something" was, or if it will happen again. In the first phase of BBTRS, all Jorge needs to say is, "I'd like to release it."

Of course, sometimes a traumatic event is absolutely vivid, frequently recalled, and paralyzing, such as in a natural disaster, violent altercation, or terminal illness. In particularly arresting life events like these, the level of trauma can be very high, causing the nervous system to become aggressively stuck in the fight-or-flight mode. Along with overstimulating the nervous system, high levels of trauma also can cause people to repress aspects of themselves. For example, a soldier might subconsciously inhibit natural tactical abilities—becoming less good at skills he or she is actually great at. Imagine suddenly second-guessing or pausing in the face of split-second decisions that have been reinforced by years of training. This kind of change can crush a person's confidence, as well as invite more danger into already dangerous situations. In a less acute example, think of an outgoing, jubilant person who gradually represses his or her naturally occurring joy, or perhaps sexuality. Stress and its resulting trauma can cut off entire portions of personal expression and relationship, eventually changing our outlook on life.

When attempting to help people who have had experiences like this, it's important to avoid reactivating their fight-or-flight responses. As in other somatic practices, BBTRS focuses on the physical, so "talk therapy" is not required—or even desired. However, if a breather wishes to discuss a traumatic experience in session, we might talk sparingly and for a very particular purpose. Once a slight activation occurs, then we shift attention to how the body is responding. Our mission is to look for the tension related to the experience.

Release comes through a combination of specialized breathing and physical movement. Both these processes are intimately related to the sympathetic nervous system, the "automatic," primary actor in the fight-or-flight response. The combination of breath and movement "unlocks" the muscle fascia, allowing excess tension stuck in the nervous system to discharge and dissipate from tissues. In the process, an emotional response might occur. Crying or laughing can discharge some stuck energy, but foundational release occurs beneath those emotional responses.

After a successful release, the trauma loses its hold on its host. Let's see what this might look like for Jorge. After his car is rear-ended, he finds his hands in a death grip on the steering wheel whenever a car crowds him, even in the grocery store parking lot. Instead of focusing on reducing rush-hour driving—trying to outsmart his anxiety by avoiding traffic—he can focus on healing. With trauma release, the involuntary tension associated with driving will disappear. It might take several sessions, but the fight-or-flight stimulation will simply fade, along with its effects on his physical body.

Of course, Jorge might experience an emotional activation for some time, but this will also fade. That is, physical tension eggs on emotional reaction, reminding us from the inside that something is off balance. As the physical tension dissipates, Jorge simply becomes less triggered by traffic, and becomes better able to regulate his emotions. If any activation remains, it generally is so minimal that it is not bothersome. This is why the memory of a traumatic event is not as important as releasing its residual trauma. Without its underlying "hold," a memory simply loses traction. Eventually, it simply doesn't produce a response.

It's important for readers to know that BBTRS is a collection of modalities, thoughtfully chosen for their particular characteristics. Sessions are formulated with a beginning, middle, and end, so that participants can first orient themselves to the experience, and afterward, integrate their physiological, emotional, mental, and spiritual responses. The influence of BBTRS on any given person will be unique, and different each and every time. No one can predict how anyone might respond. Sometimes change is quiet and subtle, with only the "breather" sensing it. Sometimes mountains are moved—with change so obvious that witnesses are brought to tears.

Before BBTRS was "born," Chetana Lauren Barkan was a bodyworker and meditation leader. She describes her first breathwork session in Pune, India, as an experience where "the feelings and emotion running through my body were well beyond my ability to track yet." In that session, she says, "I understood that I needed to do more of this work for my continued self-growth and development, and that my professional life as a bodyworker had forever been changed. Breathwork was my new focus." And it has been. From the early days in the mid-2000s, as the kernels of the new modality were first coming together, Chetana was BBTRS's first witness, supporter, and collaborator. She became its first co-facilitator, and helped develop the infrastructure that supports BBTRS trainings around the world. In this key role, she also has held space for hundreds of practitioners each year.

In the process, Chetana has seen people blossom. When asked about the most moving, indelible transformations she has seen, she recalls two stories. One featured a woman who attended an early session in Goa, India. "We knew that she had experienced extreme developmental trauma, and due to those experiences, had lost hearing in one of her ears. My memory of her remains clear, because toward the end of her session, her ear just popped," Chetana says. "I remember the look on her face, and our wonder at the moment. Suddenly, after all that time, she could hear again."

Another meaningful moment occurred in a BBTRS session in Bali. "A young woman joined us, and she was a person of small stature. From what she described, her physical development seemed to have been interrupted because of the effects of developmental trauma," Chetana recalls. "She also barely communicated or connected with others, and she could not move her body in an open, free expression." Dance and conscious movement are important components in BBTRS trainings, as they create a supporting environment that encourages participants to feel comfortable in the exploration of their bodies, generates energy and celebration, and transitions between exercises. “This woman moved like a programed robot. She lived with shame and was fearful of her own joy and self-expression. It was quite tangible,” Chetana recalls.

While “telling one’s story” is not an essential part of BBTRS, this participant said that she associated her physical stature and inhibitions with childhood experiences. “She shared that her father used to make her watch pornography with him, placing her in a chair between himself and the television,” Chetana says. “She said he watched her while she watched the screen. He also told her she was unlovable, unworthy of care and respect and would always be alone—she grew up believing this.” Within days, this woman’s transformation was profound. “By the end of the training, she was dancing solo in the center of our circle. And she wasn’t just swaying with the music, she was in full celebration. She danced with a freedom she had never experienced in her whole life.”

Since the beginning of her time with BBTRS, Chetana has witnessed hundreds and hundreds of people choose to transform their lives from what she calls “trauma-affected to trauma-released.” “Every single group feels in my heart as the most committed and beautiful group of humans—until the next one,” she says. This rolling experience confirms that the world is full of people sharing the same intention: to live free, peaceful, happier, and more fulfilled lives. When asked what it feels like to be in the presence of so many people having so many pinnacle moments—across the world, year after year—she says, “I feel so full with gratitude, honor, and yes, joy. I am able to witness so many incredible beings come together—in the most unpredictable of territories, navigating for their freedom. I appreciate their deep, heart-full commitment and willingness to explore, see, allow, accept, and to move into present-moment reality with such vulnerability, tenderness, and grace.”

BBTRS workshop in Bali.

Trauma release works with people like these two women—whether or not they know why hearing is impaired, or engagement with life is shut down. It works with people like Jorge who can't put a finger on his problem, or those who experience an isolated, dramatic event. Regardless of cause, it can also assist people who experience specific manifestations, such as social anxiety, where leaving the house is very difficult. All of them can enjoy lives that are not defined or limited by their "problems." Their triggers can simply stop triggering.

Finally, periodically releasing trauma maintains a "clean slate"—as well as helping people to learn better how to deal with trauma when it occurs. If we can simply let it pass through, without taking up residence, then trauma's impact is low. An added bonus: releasing trauma is possible regardless of life experience, previous modes of bodywork, emotional release processes, various modes of therapy, or age. Please note, however, that trauma release in children is handled differently than in adults, with less stimulation. See more on this topic in Chapters 6 and 10.

Since trauma is real, and not an idea, there is no conquering it, or crushing it, or eliminating it from the universe. But we can move it. We can let it go. We can complete the interrupted fight-or-flight response, thus dispelling resulting trauma. And dispelling trauma invites a clearer experience of "being."

Reflection

This book contains a lot of information that spans a wide range of topics. Some of it might have particular meaning to you, or even activate a response in your body. Therefore, at the end of each chapter, there will be an opportunity for self-reflection. It will be the same each time, like a meditation or a mantra. Please take a moment to experience each section in this way.

Thank you for reading this chapter. Now, please take a moment to bring your attention to yourself. Please read the following paragraphs, and then close your eyes and scan your physical body. Notice how what you read reflects in your body. What is happening in your body—is it relaxed? Perhaps start with the breath, and consider whether it is flowing freely. Then consider your posture, whether you are sitting or lying down. Do you feel balanced, or does your physical body have tightness, pain, or discomfort? Look at different aspects of your body, including inner and outer tension. Take a deep, full breath, and sigh out your mouth. Next, consider your thoughts—are they generally centered outside of you? If something external is consuming your attention, can you let it go for now?

Now, consider if the material in this chapter resonates in you, and whether the resonance involves you or someone you know. If there is some triggering—positive, negative, emotional, physical, spiritual, or some other experience—then what does it feel like? Maybe it feels like you're reading something intimate or personal or true about yourself—or maybe you didn't like something you read. What was it? Was the response because the information was revealing, challenging, nurturing, or something else? It's possible that something that's gone unnoticed within you, or something you've neglected or forgotten about, might really be valid and making itself known. It's possible that this material might have awakened it in you. Or not. You be the judge.

After reading these things, and reflecting on them with your eyes closed, then open your eyes and bring your attention to your surroundings. Listen to the sounds. What do you see? Between chapters, perhaps take a brief walk, or dance to a couple of songs. Move around, reorient, and then settle in for the next chapter. Thank you.

Chapter 2

A matter of science

Please note: Some readers are especially interested in the scientific underpinnings of somatic release; others are more curious about the experiential aspects of the process. We have included a well-documented chapter that explains why our bodies naturally react the way they do, how fight-or-flight works, statistics related to trauma across the world, and how PTSD has assisted researchers and practitioners in helping all kinds of people to address their trauma. If this kind of background is not your thing, then skim through Chapter 2 and move on. However, be prepared—there's some interesting information in here!

People often ask how we know that inducing tremors in the body allows trauma to release. For those who have not experienced it firsthand, perhaps the easiest confirmation comes from watching the fight-or-flight process dissipate in someone else. And the easiest "someone else" is often found in the animal kingdom, especially in a mammal that is used to life-and-death situations.

In an amazing nature video,[10] an impala seems to have met its doom. A cheetah has caught it, and is in the process of suffocating the impala. Two strokes of good luck—for the impala, anyway—change the course of events. The first is a distraction, caused by a couple of hyenas; the second is the curious approach of two baboons, whose very presence encourages the cheetah to abandon its prey. Although the impala already looks dead, the camera remains focused on it; for a few moments, there is no visible breath or movement in the victim. Then, finally…one deep breath follows another—until the impala sits up and begins to tremble violently.

After several moments, the impala stops shaking, leaps up, and trots away. It appears fine—and armed with an exciting near-death experience to share.

How energy is held in the body

Think of that impala's body as a tightly wound rubber band. When released, the rubber band might fly across the room; likewise, the impala's unused "flight energy" became evident as tremors. If left to its natural processes, fight-or-flight energy dissipates automatically.

In physics, contraction requires energy. If we make a fist and hold it by contracting our arm muscles, then we're using energy. The moment energy is released, it mobilizes, unlocking the tissues it was stored in. The result is relaxation of the muscles. However, in a low-grade fight-or-flight pattern—otherwise known as anxiety—our bodies generate more energy than they consume, but it's not "available" to make a fist or run a race. We might seem hyper-aroused, always physiologically prepared for the next danger. Or, if our bodies cannot sustain that level of arousal, we might become dissociated and relatively lethargic—but the charge is still there, vibrating within us.

On the other hand, in the absence of anxiety, we relax—and our energy is available for other pursuits, such as creativity, inspiration, meditation, work, play. Anything. Like magic, our systems are wired to activate, and then to process and release the activation. It's just how our anatomy works; it's how an impala's anatomy works.

Since we cannot interview an impala, we can't truly understand its internal experience. However, the conversation about trauma in the animal kingdom is not new, and in fact has sparked some very interesting research—for both animals and people.

“The Ecology of Fear”

Scientists have long used “predator-induced stress” as a model to test for human stress. The question is: how does the chronic stress of a human compare with the impala’s always being ready to run for its life? In humans, studies tend to begin with natural reactions to ongoing, survival-related, physical challenges in the human experience, such as dealing with a shortage of food. Scientists call this “sustained psychological stress—linked to mere thoughts.” On the other hand, stress in animals was usually categorized as falling into non-psychological categories.[11]

By exposing prey animals to predators and “predator cues,” scientists have demonstrated what happens in the wild. For example, birth and survival rates changed according to exposure levels—the more predators, the fewer prey animals were born. Some of this was due to “intimidation” by the predators, along with the hesitancy of grazing animals to freely forage. In the wild, these researched prey animals exhibited a variety of physiological stress effects; in the laboratory, others demonstrated sustained psychological stress.[12]

With the discovery of psychological stress in animals, some scientists have proposed that prey animals live within an “ecology of fear.” They tested the concept by studying areas where a predator has been introduced to a new ecosystem—among prey species that had never been

attacked by them. To find out whether moose were "hard-wired to be terrified of wolves,"[13] conservation biologist Joel Berger studied two moose populations—one with wolf experience, and one without. Among other interventions, he broadcast several sounds: from wolf calls to the calls of animals from another continent. He also threw snowballs mixed with bear feces and wolf urine, to test the effect of different scents.

The result: moose that lived near wolves were afraid of wolf-related sounds and scents; the other moose were indifferent. "Berger thinks mother moose who survive encounters with wolves may teach their new fear to their calves, placing the next generation pretty high on the learning curve."[14] Researchers like Berger say this work supports the "ecology of fear" theory; that knowledge of a threat can cause an animal to be constantly on guard.

Scientists use the ecology of fear as a common model for PTSD in people.[15] It looks like this: Let's assume that the animal kingdom can help us to understand why we are afraid—and how we pass fear-related information to one another. With this knowledge in place, can we now learn from these furry "fear experts" how to deal with stress? Specifically, if an impala can process and "shake off" a near-death experience with tremoring, can we do that—or does our "large brain" mean that we're more complicated? Can we tap into and then surrender to our automatic processes? If so, will this allow us to help heal ourselves?

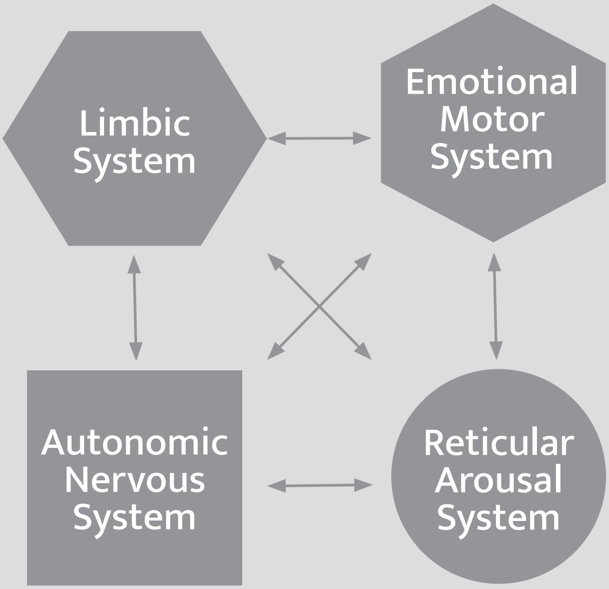
The short answer is yes, we can. The long answer is summarized below.

Us—on auto-pilot

Fight-or-flight responses originate in one "department" of the auto-pilot system that manages our physiological functions. The Core Response Network (CRN) does its job to keep us online—from salivation to heart functions to how memories work to our sleep balance—all without notifying our conscious minds of what's happening. The CRN is comprised of the autonomic nervous system—which houses the sympathetic and parasympathetic nervous systems—as well as the limbic, emotional motor, and reticular arousal systems. The sympathetic nervous system is the primary fight-or-flight center, although some of its associated "freeze" functions come from its parasympathetic neighbor.

These various processes work together to keep our bodies ticking—and one glance at the descriptions of their functions tells us how complex we are as machines. Just for fight-or-flight, consider these processes, among others, that instantly occur when danger presents itself:

- For extra energy, adrenaline and glucose production increase, allowing the heart and lungs to accelerate.
- Blood flow increases to large skeletal muscles and declines elsewhere.
- Blood clotting agents increase, in case we sustain injury.
- Our pupils dilate and often lose peripheral vision.
- We sweat to stay cool.
- Functions such as tear production and digestion slow or stop. In these moments, the bladder relaxes, and sexual responses are inhibited.


Limbic System
Emotional Motor System
Autonomic Nervous System
Reticular Arousal System

The nuts and bolts of biology: Core Response Network[16]

The Autonomic Nervous System, housed in the hypothalamus, regulates the body's unconscious actions, and is divided into two integrated parts.

(A) The Sympathetic Nervous System (SNS) stimulates the body's fight-or-flight response, as well as constantly maintaining homeostasis.

(B) The Parasympathetic Nervous System (PSNS) stimulates resting functions, especially those related to eating—such as salivation, digestion, urination, and defecation, as well as sexual arousal and tear production. The primary influence on the PSNS is the vagus nerve.

Connected to breathing, heart functions, and "gut reactions," the vagus also contributes to fainting and relaxation—and therefore sends calming hormones, enzymes, and proteins into the system during the fight-or-flight response.

The Limbic System (LS), housed in the amygdala, hippocampus, and septal region, is the home of the emotions and the formation of memories. It is comprised of a set of brain structures that support emotion, behavior, motivation, long-term memory, and the sense of smell. It's also the place that responds to pain and pleasure, regulates fear, and controls anger and aggressive or violent behavior.

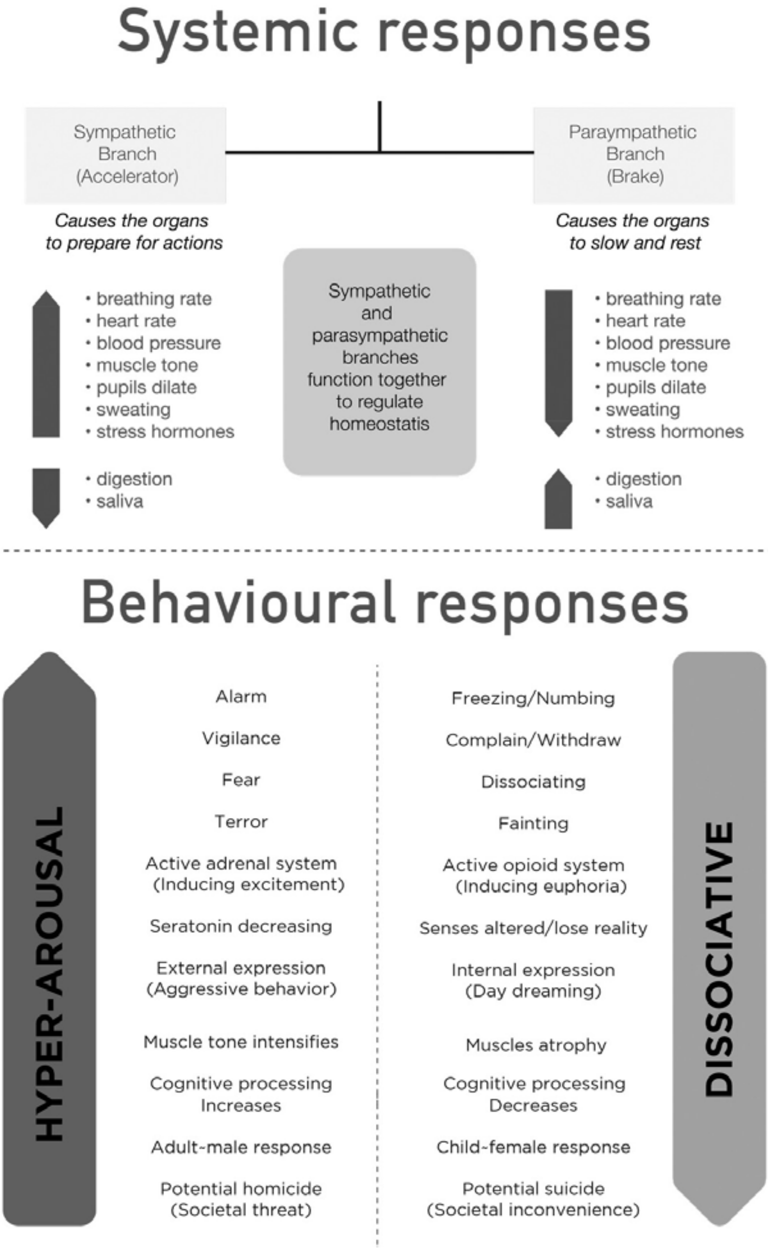
The Emotional Motor System (EMS) is a source of debate. Scholars generally agree that the EMS is related to the overall motor system that governs somatic, autonomic, and endocrine motor responses, including survival behaviors. However, while some consider the EMS from a psychological angle, others find it an "internal state, distinct from other aspects of cognition, and, therefore, perhaps not definable in terms of other cognitive functions."[17]

The Reticular Arousal Systems are a set of connected nuclei responsible for regulating sleep-wake transitions and wakefulness.

The sympathetic branch of the nervous system, the primary fight-or-flight center, is like a gas pedal—it gives us energy for any action we plan, and will help us prepare for any threat. Conversely, the parasympathetic branch acts like a brake pedal. It helps us to relax, unwind, and ultimately discharge the arousal of sympathetic activation. When a person is under chronic stress, the "gas pedal" can be pressed for days, weeks, months, or years—just like an impala's, or any animal who lives in an ecology of fear. Think of your life, or someone else's—a person under long-term stress, constantly tense, prickly, and on edge. We can all imagine the list of possible symptoms of traumatic stress that might result: anxiety, panic, hyperactivity, exaggerated startle response, inability to relax, restlessness, hyper-vigilance, digestive problems, emotional flooding, chronic pain, sleeplessness, hostility, and rage.

And then, consider what happens when the stress is just too much, and the body finally shuts down. The parasympathetic nervous system presses the brake pedal, seeking any kind of relief from the anxiety. And if the "pedal" sticks, we might experience depression, flat affect, lethargy, exhaustion, chronic fatigue, disorientation, disconnection, dissociation, pain, low blood pressure, and poor digestion.

Without actual relief and reliable trauma release, our normal regulatory systems are interrupted. Similar to driving with both gas and brake pedals fully engaged, the nervous system becomes over-activated. In these cases, every bit of the nervous system is under stress, and a person simply can't reach homeostasis.


Systemic responses
Sympathetic Branch (Accelerator)
Paraympathetic Branch (Brake)
Causes the organs to prepare for actions
Causes the organs to slow and rest
• breathing rate
• heart rate
• blood pressure
• muscle tone
• pupils dilate
• sweating
• stress hormones
• digestion
• saliva
Sympathetic and parasympathetic branches function together to regulate homeostatis
• breathing rate
• heart rate
• blood pressure
• muscle tone
• pupils dilate
• sweating
• stress hormones
• digestion
• saliva
Behavioural responses
HYPER-AROUSAL
Alarm
Vigilance
Fear
Terror
Active adrenal system (Inducing excitement)
Seratonin decreasing
External expression (Aggressive behavior)
Muscle tone intensifies
Cognitive processing Increases
Adult-male response
Potential homicide (Societal threat)
Freezing/Numbing
Complain/Withdraw
Dissociating
Fainting
Active opioid system (Inducing euphoria)
Senses altered/lose reality
Internal expression (Day dreaming)
Muscles atrophy
Cognitive processing Decreases
Child-female response
Potential suicide (Societal inconvenience)
DISSOCIATIVE

What it feels like when our bodies automatically respond to stress, whether we prepare to fight or flee—or we freeze.

This level of activation can cause important changes in the brain, or "dysregulation," which blur the body's ability to distinguish between past and present stress. That is, if trauma is not released from our physical and psychological systems, it not only remains trapped in our tissues, but also it echoes through our entire being—registering as a current experience. The body basically "doesn't realize" that we're not undergoing the trauma now, and it reacts as if we are.

This means that the gas and brake pedals can stay depressed because we walk through a constantly stressful life, or because our bodies just think we do. Regardless, the accompanying flood of hormones and chemicals that ready us for action affects memory, learning, the ability to regulate emotion, social and moral development, and metabolic functions. These chemicals also impact our neuropsychology, which is the balance between brain function and how we think, feel, and act. We have more difficulty sorting through information, and determining what is relevant. When under stress, we also lose our natural ability to trust our "gut instincts"—which are a result of nervous system functions. We might have trouble feeling anything at all. And, at the highest level of disconnect, traumatized people may actually feel unsafe in their own bodies.

Now consider those fortunate people who are not overcharged, whose "pedals" are working well and regulating stress responses just fine. They often tend to focus on what's "going on," such as the day's activities, needs of family members, and work obligations. All the autopilot operations tick along under the surface—along with their related physical and emotional indicators.

And then there's the average person, the person who is neither miserably out of sorts nor the perfect picture of balance. These people might be used to a bit of a roller coaster, coping with the day-to-day, hoping for things to improve, or working toward the next goal. "I'll feel better when I pay off the car," or "Things will smooth out when my child gets into high school." These people might not notice the physical or emotional signals that are trying to tell them that they're living with unnecessary stress. That means that on any given day—any normal day, under normal circumstances—even well-functioning people might not realize *when they are in trauma activation*, let alone *how it feels*.

It takes practice to "sense" where our tension might be, and to learn to release it. But more than that, it takes practice to truly sense whatever is happening in our bodies—even to recognize homeostasis, which is the other main function of the sympathetic nervous system. How are we "meant to feel"? How are we meant to be, or live? And how "should" we integrate threats, danger, and trauma? Finally, how does BBTRS harness the nervous system to allow us to heal?

Prema McKeever is a BBTRS practitioner and co-facilitator with a particularly scientific mind. Thanks in part to her Master of Science degree in Chinese Medicine and expertise in other somatic practices, she was able to connect the dots between what BBTRS "feels like" in the body with the actual internal processes that occurred during sessions. "I was especially curious about what was happening on a physiological level, and why the modality was so successful," she says. "By practicing BBTRS, I could feel changes. I was more connected to my body and my feelings. I felt more alive—but I wanted to understand why. I wanted to understand how this works, especially with trauma." As she delved into the science behind the modality, McKeever developed a few of her own personal theories.

The way she saw it, BBTRS helps people to enter states of sympathetic nervous system activation that are followed by parasympathetic deactivation, all in a safe environment. “People who have undischarged, trauma-related activation still held in their nervous systems often experience sensations from their nervous system’s ‘charging up.’ These include faster breathing, increased heart rate, tightening muscles, and increased energy moving through the body—all of which can be too scary for them to feel. So they avoid these feelings,” she says. “But when people are held in a place of safety, held by their facilitators, then they can feel these hard places and still feel okay. Their usual experience is to go into a traumatic activation and get stuck there. With BBTRS, they can feel the sensations associated with the trauma, and the trauma is allowed to naturally complete. Being able to cycle through sympathetic activation and then parasympathetic rest and safety is key—we expand our nervous system’s capacity to tolerate more activation, as well as deeper states of relaxation. BBTRS completes the cycle.”

For McKeever, BBTRS was different from other types of breathwork. “What I love about BBTRS is that there’s such a respect for our bodies. We’re not pushing ourselves into something,” she explains. “We’re inviting our bodies to open. There’s a deep respect for where each person is in a process. For those who have difficulty in connecting with their bodies, BBTRS is an amazing teacher. It teaches us to learn how to listen, to learn to breathe, to respectfully move, and to unlock. It’s almost as if you meet your body for the first time in your life.”[18]

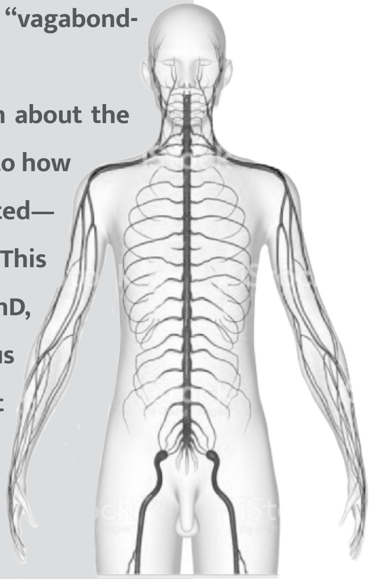
Talking to Ourselves

We will offer much more discussion of how to enhance our ability to sense and to feel, especially in Chapter 7. First, though, let's go back to biology—to the captain of the parasympathetic nervous system, the vagus nerve. It is the longest of our cranial nerves, and it extends fibers from the brain to visceral organs throughout the body. Because of this extraordinary reach, and its "vagabond-like" meanderings, it was named "vagus."[19]

Many research studies have been written about the vagus nerve, but its foundational connection to how we feel—in all the ways "feel" can be interpreted—can be explained through Polyvagal Theory. This theory, introduced by Stephen Porges, PhD, describes the various ways in which the vagus nerve helps us to adapt to the environment. It is the vagus nerve that engages:

- **Fight-or-flight;**
- **Immobilization, the "freeze" that translates to feigning death in animals and behavioral shutdown in humans; or**
- **Social communication, which involves our being able to sense the level of activation, or general status, of another person.**

"BBTRS helps us to move from the dorsal part of the vagus nerve—the freeze function, the state



The vagus nerve, the longest nerve in our bodies, provides both sensory and motor functions. Named for its "vagabond" journey throughout the body, it runs from the brain stem to part of the colon, and allows communication between the brain and the internal organs—including ears, throat, heart, and digestive tract.

of 'I can't make it'—into ventral part of the nerve, which is the social engagement system," practitioner Prema McKeever explains. "When we move through the fight-or-flight cycle, we release trauma, and we end up connecting with others. We can look into somebody's face, and our sense of safety is restored."[20]

All three of the vagal functions require swift communication between our bodies and our environments, and they all involve "neuroception." Neuroception is our ability to assess whether the environment is safe.[21] Porges says we get this information through "afferent feedback from the viscera"[22] —otherwise known as nerve impulses. *We feel ourselves.*

So think about it. If our bodies take stock of our environments, and then decide what to do, then we are in an auto-pilot, intimate relationship with the viscera. Our internal organs, glands, blood vessels, and tissues talk to us. If we listen, then we can sense discomfort, warnings, calm, and countless other pieces of information. These sensations also contribute to how we feel on other levels, such as in our emotions, attitudes, and relationships. Basically, each of us has the ability to sense our own balance—physical, mental, emotional, and spiritual—which in turn defines the optimal state of functioning and balance that Laurie Leicht relies upon in the Social Resilience Model. Homeostasis.

According to some world philosophies, homeostasis is, simply, happiness. But what happiness means differs from culture to culture. Consider Aristotle's humanistic ideas, in which happiness came from virtuous actions; he laid the foundation for an individual's search for meaning, self-realization, and becoming fully functional—otherwise known as psychological well-being.[23] Or hedonism, in which "well-being" meant pleasure—physical, mental, and spiritual.

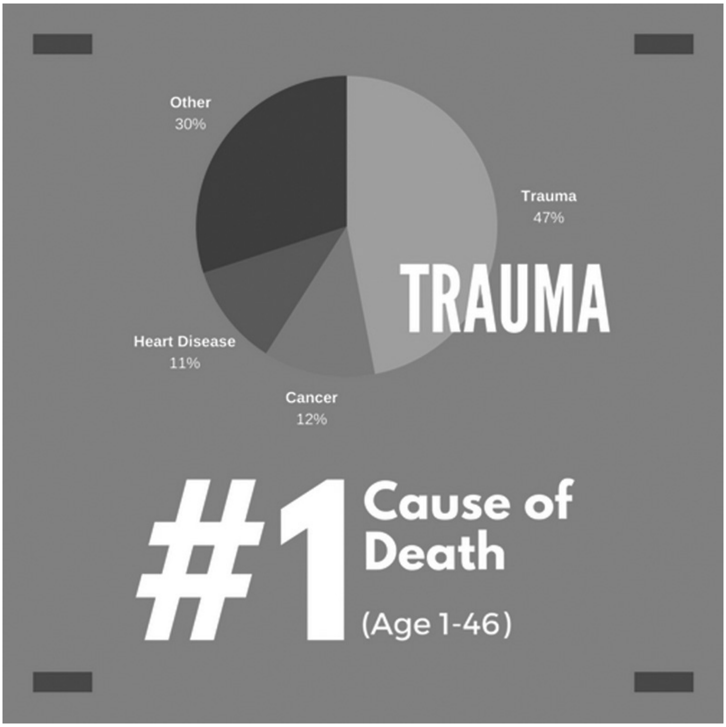
At the very beginning of Chinese civilization, there was no word like "happiness." *Fu*, its closest equivalent, has been interpreted to mean "blessings from the supernatural, and pleasures derived from human society." Later, that same word, *fu*, was more clearly defined in the *Shang Shu*, one of the Five Classics of ancient Chinese literature, to include "longevity, prosperity, health, peace, virtue, and a comfortable death."[24]

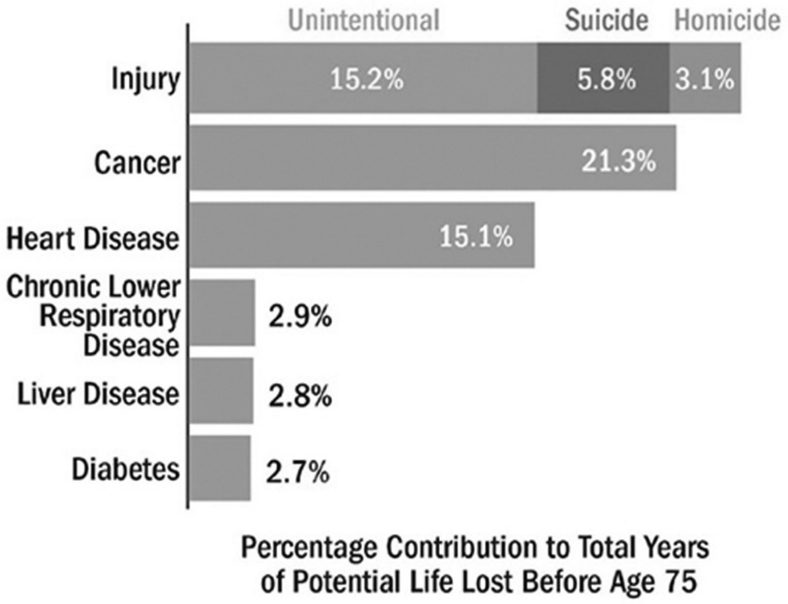
In today's psychology, happiness is considered as a state of mind, more a mood than a fleeting emotion, and now is the subject of the *Journal of Happiness Studies*, which launched in 2000. Happiness is generally defined as a "predominance of positive over negative affect, and as satisfaction with life, consisting of people's general evaluations of their life, affective and cognitive."[25]

This "harmonious homeostasis," inner peace, joy—this is what we're searching for. It is our natural way of being. It arises from real love, and from full self-acceptance of body, psyche, and life situation. This acceptance is our natural state; everything else requires energy, like making a fist. Being stressed, tense, and unhappy takes more work than does allowing homeostasis—uncluttered, satisfying, happy balance. For those who are all-too-familiar with traumatic cycles, stepping out of the paradigm and lifestyle of tension can be difficult. It requires attention and self-care first. Knowing what we need to achieve balance is crucial, both to our own quality of life and the energy we share—because a life of sacrifice and self-neglect does not cause a traumatic cycle to stop. It does not help us, and it does not help us to care for others.

Who experiences trauma?

It's easy to agree that a "bad day" is different from "trauma," and that "trauma" is a term that's reserved for people who have experienced something very unusual. However, research across the world shows that the experiencing of traumatic events is more common—and "routine"—than we would like to think. Trauma statistics are available from many sources worldwide, and they paint a startling picture. According to the National Center for Injury Prevention and Control (NCIPC), a division





of the U.S.'s Centers for Disease Control, trauma is the number-one cause of death for people aged 1 to 46, and is the most likely reason to shorten a person's life before the age of 75. "Trauma" in this reference can mean physical and burn injuries; intentional trauma, such as suicide or assault; and unintentional accidents, collisions, falls, drownings, and poisonings.[26]

But death is absolutely not the only result of trauma, and the primary person experiencing a specific event might not be the only sufferer. For example, witnesses to "someone else's" traumatic event might retain the residue of the experience in their own bodies. Personal trauma can result from a shock, such as a disaster or an accident, abuse, problems within a relationship, or negative interactions related to cultural, personal, or community identity. It can be obvious from the outside, or private and secret. Trauma can result from one event or repeated, prolonged, or chronic events. It can even result from situations that might not even seem "traumatic," such as shame. Therefore, in some form or other, trauma happens to everyone. Its impact depends both on the circumstances of an event and the resilience of the person's nervous system at the moment in question. Sometimes we're more able to process and release, and sometimes we're not.

Let's face it. More than seven billion people live on Earth, and they experience innumerable personal interactions, social and political circumstances, and natural and global events each year. Our ability to function in an ever-more-complex world requires fine-tuned bodies, minds, and spirits—and in the process of living, trauma is inevitable. Each individual exists on a continuum of personal development, all the while bumping into other people and unexpected moments.

Trauma around the world

Let's review some staggering figures, just for perspective. The Centre for Research on the Epidemiology of Disasters (CRED) and its Emergency Events Database (EM-DAT) provide worldwide statistics on what's happening on the planet in these topic areas: geophysical (earthquakes and volcanos, for example), hydrological (floods), meteorological (storms), climatological (anything from drought to wildfires), biological (human epidemics or insect infestations), and extra-terrestrial (impacts, or space weather). In 2015, CRED released a 20-year review of disaster impacts on the human population in an effort

to evaluate and promote global disaster risk reduction. Its review tells one story of what might traumatize us: each year, 218 million people were affected by natural disasters between 1994 and 2013. In that period, EM-DAT recorded 6,873 natural disasters worldwide, which claimed a total of 1.35 million lives—almost 68,000 on average per year.[27]

A slightly different focus on worldwide trauma comes from the World Mental Health Survey Consortium, which reports on the negative impacts to physical and mental health in people who experience traumatic events. In a study published in 2015, surveys in 24 countries across six continents polled nearly 69,000 adults regarding their exposure to 29 types of traumatic events. More than 70% of respondents reported an event; 30.5% were exposed to four or more events. Five types of events accounted for more than half of the exposures: witnessing death or serious injury; the unexpected death of a loved one; being mugged; being in a life-threatening automobile accident; and experiencing a life-threatening illness or injury.[28]

Other statistics report that most trauma—quantified as "percentage contribution to total years of potential life lost before age 75"—results from injury (roughly 24%), and most of that is unintentional. Cancer (21%) and heart disease (15%) are also high-ranking causes; chronic lower respiratory disease, liver disease, and diabetes are basically tied at just shy of 3% each.[29]

Now, let's add into these categories politics, whether global, international, national, or local—and including everything from war to governmental policies to the politics of race, sexual orientation, gender, workplace dynamics, or myriad other "political" situations. Then there's the matter of how we operate in relationship, and the subtle ways in which interpersonal interactions can affect us. No matter where we look, trauma is there. There is not a human life that avoids it; the challenge is how to manage it.

When we try to get a handle on what trauma actually is, we can consider it broadly or individually. However, when society has to manage trauma, help treat it, and pay for it, the medical model steps in with measures and definitions. The 5th Edition of the American Psychiatric Association's *Diagnostic and Statistical Manual of Mental Disorders* (*DSM*) defines a traumatic event as exposure to threatened death, serious injury, or sexual violence. A person who suffers from a disorder related to trauma must have either directly experienced or witnessed a traumatic event; learned that a traumatic event occurred to a close family member or close friend; or, like emergency responders, experienced "first-hand repeated or extreme exposure."[30]

The idea of suffering from a disorder comes from our medical system's attempts to quantify and qualify symptoms—deciding what level of impact results in what level of care. One result of this effort is Post-Traumatic Stress Disorder (PTSD)—a definition that allowed populations of sufferers to not only be recognized, but also to receive help. Over the lifetime of the *DSM*, the definition of PTSD has been one of the most controversial in the field of mental health[31]. Previously considered as an anxiety disorder, PTSD has proven difficult to get a handle on—because no two people experience an event in the same way. What is manageable for one person's nervous system might debilitate another. What works for one person might not work for another. Trauma is a moving target.

Trauma and the body

We know that trauma causes PTSD, but it does much more than that. Trauma is responsible for dozens of related conditions. Here is a sampling:

In extreme cases, people can lose sensory perception or experience other physical conditions, including auto-immune diseases, cardiovascular disease, and diabetes. Especially when children have experienced physical and mental abuse, domestic violence, and/or substance abuse, they show not only attachment deficits, but also an increased likelihood of auto-immune symptoms in adulthood—from migraines to seized muscles.[32]

One study of Vietnam veterans reports that PTSD symptoms remained more than fifteen years after military service had ended. Like past study subjects, these veterans had "increased resting heart rate, increased startle reaction, and increased heart rate and blood pressure as responses to traumatic slides, sounds, and scripts." All these were related to increases in the stress hormones epinephrine and norepinephrine—which can contribute to

cardiovascular disease,[33] or impact memory and attention, respectively.

PTSD can also speed the human aging process, accelerating cognitive decline and disability.[34] In young adults, "chronic stress from trauma" can prematurely introduce heart dynamics associated with older age. "Together, the PTSD related cardiovascular, metabolic, and inflammatory systemic changes affect the body in a manner consistent with accelerated aging."[35] Both Alzheimer's disease onset and dementia also have been correlated with PTSD, because of changes in the brain related to stress.[36]

In an impressive study that spanned twenty-two years, "women with the highest number of PTSD symptoms had a nearly 2-fold increased risk of Type-2 Diabetes compared to women without exposure to trauma."[37] The study reveals the "strongest evidence to date" of a causal relationship between the two.

How PTSD changed everything

Suffering can become a habit, a lifestyle, a part of our bodies that seems unshakeable. Of course, everyone experiences the ups and downs of normal life—triumphs and disappointments, birth and death, accidents and good fortune. Sometimes, we successfully process the "downs," and sometimes we don't. This is to be expected—and it's why soldiers and other sufferers of PTSD have provided such a great service for the rest of us. Some of them have been walking around with incredibly high levels of activation and really crushing symptoms. They have needed help, and that help has spilled over into the general population. That is, if something works for PTSD, chances are it will work outside of PTSD.

In general, of the adults in the United States who have experienced a traumatic event, up to twenty percent of them go on to develop PTSD.[38] Reports vary as to the number of Americans with PTSD at any given time, from approximately seven to eight percent.[39] Although statistics reported from other countries have been collected and managed in different ways—and therefore are not directly comparable to one another—estimates have been as low as three-hundredths of a percent in China to just over six percent in New Zealand.[40]

Studies commonly focus on a variety of underlying causes of PTSD. Along with military trauma, these usually include terror situations, sexual assault, and complex trauma—which results from prolonged exposure to traumatic relationships, often with parents or others with whom a person feels disempowered.

The somatic work that most informs and relates to BBTRS has been done by researchers like Bessel van der Kolk, MD, Peter Levine, PhD, David Berceli, PhD, and their colleagues. Their work helps breathers whether or not they are consciously aware of the source of the suffering. By focusing on the physical release of tension, they avoid potential reactivation or retraumatization that can sometimes result from other forms of therapy.

Van der Kolk is the founder of the Trauma Center at Justice Resource Institute, professor of psychiatry at Boston University Medical School, and former president for the International Society for Traumatic Stress Studies. He has published several books—the most recent of which is *The Body Keeps the Score*—and more than 150 scientific articles on subjects as diverse as neuroimaging, self-injury, memory, neurofeedback, developmental trauma, and Eye Movement Desensitization and Reprocessing (EMDR), a treatment designed to alleviate stress associated with trauma and traumatic memories.

Van der Kolk has worked in the field since the 1970s—and even was instrumental in defining PTSD. Because of researchers like him, work with Vietnam veterans' post-war trauma symptoms pushed PTSD into legitimacy, and caused the definition to be included in the 1980 edition of the *DSM*. However, Van der Kolk recognized early on that trauma affects people in all walks of life, especially children. And he noted that what happens to trauma victims happens in the brain—actually changing the ways in which people experience the world.

How people view the world is evident by where we put our attention. Memory forms the foundation of the story of our lives. In *The Body Keeps the Score,* Van der Kolk explains that trauma creates a different type of memory than do regular events, because a traumatic memory "fires up" the visual cortex with almost-cinematic detail. On the other hand, he says, "most day-to-day experience passes immediately into oblivion."[41] He recalls the Grant Study of Adult Development, which followed more than 200 Harvard men from 1939 onward. Many of these men would go on to fight in World War II, and how they coped with their experiences was evident in two interview series, one in 1945/46 and another in 1989/90. "The majority gave very different accounts from the narratives recorded in their immediate postwar interviews: With the passage of time, events had been bleached of their intense horror." However, the PTSD sufferers among the interviewees had bleached nothing. "Their memories were preserved essentially intact forty-five years after the war ended."[42]

In severe PTSD cases, trauma can "hijack the brain," causing both physical and psychological changes. Trauma can result in a smaller-than-normal hippocampus, the brain's memory center,[43] or it can short-circuit the rational prefrontal cortex—the part of the brain we use for reasoning and analysis. With the rational brain off-line, the "housekeeping" brain stem and the "emotional" limbic system can take over, and we simply can't override feelings of anxiety.[44]

"There is a mistaken notion that trauma is primarily about memory—the story of what has happened," Van der Kolk says. The reality is much more complicated, and includes a "variety of reactions in mind and brain" that can cause their own problems. "Traumatized people often become insensible to themselves. They find it difficult to sense pleasure and to feel engaged. These understandings force us to use methods to awaken the sensory modalities in the person."[45]

Research on PTSD has exposed the far-reaching impacts of trauma. It has confirmed, beyond a shadow of a doubt, that traumatic stress is more than a difficulty, or a passing condition. It can be debilitating; it can be life threatening.[46]

Reality check

It's interesting to see the difference between talking in theory about releasing trauma and actually doing it. Consider all those people you know who hesitate to attend psychotherapy; now imagine the hesitation about truly letting down one's physical guard. When considering somatic trauma release, it's important to be up-front about the comfort level required to receive its benefits.

Somatic trauma release works because it is based in science—in the realities of biology and physiology. This foundation can be important for many people. For example, westerners who lean toward the medical model can rely on the research that led to the development of somatic trauma release modalities. They can breathe a sigh of relief that this is not just some "alternative" therapy. Likewise, for individuals who have difficulty with feeling, let alone expressing themselves, it might help to know that they can rely on their bodies for the answers. This process is natural. On the other hand, those who instinctively or intuitively respond to BBTRS or other somatic practices use science to explain the underlying levels on which trauma release works. "I have no doubt that BBTRS works," says Claire Kearney, who has added BBTRS to her massage practice. "But I want to know the science so I can back up what I know to be true."[47]

If the idea of "acting like a traumatized impala" might seem off-putting, then think about all those PTSD-suffering veterans. If it works for them, then it can work for you. Forget "how weird you'll look" or "how vulnerable you'll be" when in the release process. Once you're busy experiencing what's happening in your body, any embarrassment or shyness about tremoring or other body movements will disappear. These feelings will quickly be replaced by warmth and appreciation—which will extend both to you, for being willing to enter the healing process, and to your facilitator(s), for holding a safe space to do so.

The text will build upon and expand on the scientific principles presented here, integrating them into your understanding of how you might feel or react to your life's situations on any given day. One of the broad applications of BBTRS is that anyone who practices it gains more resilience in daily life—so that any routine, potentially traumatizing event is simply processed and left behind.

Not everything worth doing can be proven unequivocally in a scientific experiment—or maybe "something we just know" hasn't been explained yet. Consider all religions, personal belief systems, coincidences, gut instincts, or any number of unexplained phenomena that you personally might have experienced. Or, consider a practice—such as holding and soothing infants—that we do instinctively. After unfortunate wartime or orphanage realities, we know that the absence of human touch can impede childhood development, to the point of impacting brain activity[48] and even DNA[49]. Similarly, somatic work pulls as much from science as it does from intention and energy, both of which are sticky subjects to define. We live in a mysterious existence, filled with the unknowable. The path toward releasing trauma—and toward living a more fulfilling, expressive, enjoyable life—requires open minds, open hearts, and a deep connection with our bodies. According to Bessel van der Kolk, "Our gut feelings signal what is safe, life sustaining, or threatening, even if we cannot quite explain why we feel a particular way. If you have a comfortable connection with your inner sensations—if you can trust them to give you accurate information—you will feel in charge of your body, your feelings, and yourself."[50] BBTRS supports us in expanding that "unexplainable" relationship with our bodies every day, all day.

Reflection

Thank you for reading this chapter. Now, please take a moment to bring your attention to yourself. Please read the following paragraphs, and then close your eyes and scan your physical body. Notice how what you read reflects in your body. What is happening in your body—is it relaxed? Perhaps start with the breath, and consider whether it is flowing freely. Then consider your posture, whether you are sitting or lying down. Do you feel balanced, or does your physical body have tightness, pain, or discomfort? Look at different aspects of your body, including inner and outer tension. Take a deep, full breath, and sigh out your mouth. Next, consider your thoughts—are they generally centered outside of you? If something external is consuming your attention, can you let it go for now?

Now, consider if the material in this chapter resonates in you, and whether the resonance involves you or someone you know. If there is some triggering—positive, negative, emotional, physical, spiritual, or some other experience—then what does it feel like? Maybe it feels like you're reading something intimate or personal or true about yourself—or maybe you didn't like something you read. What was it? Was the response because the information was revealing, challenging, nurturing, or something else? It's possible that something that's gone unnoticed within you, or something you've neglected or forgotten about, might really be valid

and making itself known. It's possible that this material might have awakened it in you. Or not. You be the judge.

After reading these things, and reflecting on them with your eyes closed, then open your eyes and bring your attention to your surroundings. Listen to the sounds. What do you see? Between chapters, perhaps take a brief walk, or dance to a couple of songs. Move around, reorient, and then settle in for the next chapter. Thank you.

Chapter 3

Taking on trauma: various paths

Let's assume that human society has always experienced stress and trauma—and that some individuals have always specialized in helping others to feel better. From tribal shamans to yogis to medical doctors to mediums to church elders to psychologists, humans have demonstrated their interests and abilities in sorting through the complexities of physical, psychological, emotional, spiritual, and energetic suffering. Today, the selection of modalities and practices can be overwhelming, even if someone has decided upon a somatic approach to trauma release.

BBTRS has been carefully designed to include various key components, each of which accesses and soothes the body in its own way, adding balance to the modality. Each component—like fight-or-flight or PTSD—is a study in itself. Chapter 4 will dig deeper into how each core element adds its strength to BBTRS, but first let's put the practice in context. How does BBTRS fit into the both Western and Eastern approaches? How does it compare to other somatic release or breathwork modalities? Has somatic trauma release been proven to work? Does meditation really add to trauma release, or is it just "good for you"? How does BBTRS utilize "energy"?

West and East

Western medicine has proven its achievement in the human condition. The medical model has provided countless scientific studies, reaching across countries and continents, and even eradicating disease. Doctors can prolong people's lives and minimize the death rate during birth, disease management, and in thousands of other circumstances. The Western system also has allowed for a network of scientific research that has contributed to our collective understanding about countless physiological and psychological conditions and advances—and explains the underpinnings of why somatic trauma release works. In the end, however, whether Western medicine fully embraces somatic work depends on its effectiveness. So far, so good. For example, it's exciting that body-based studies on PTSD have been so successful that the U.S. Department of Veterans Affairs has embraced some somatic modalities.

In Old English, the word "health" translates to "wholeness," or "being whole." However, with no disrespect implied, the Western medical model was conceived from the perspective that the body is a machine, a collection of symptoms or specialty areas. It's an interesting idea—dividing the body into systems, and then training doctors as specialized "mechanics." This process also allowed for the medical insurance system to be developed, where certain treatment protocols could be linked to certain problems—and then paid for in a systematic way.

On the other hand, Eastern approaches consider the body as a multi-layered, integrated system. "Holistic" approaches—from the Greek *holos*, which means "whole"—target the underlying causes of our imbalances. Importantly, in Eastern medicine, our "problems" might or might not be evident based on our symptoms. As Western medical practitioners have seen the success of holistic work, they have begun to integrate some basic

Eastern practices. For example, crossover in recent years has included prescriptions for mindfulness exercises, meditation, and yoga.

While the average Western doctor might not prescribe herbal tinctures, homeopathics, or other such medicinals, many are becoming more aware of them. And some doctors are actively integrating breathwork into their practices. Thomas Ischler, MD, started his practice in Germany as a surgeon in 1986. His career has spanned patient care, corporate pharmacology, consulting in business process re-engineering for hospitals, and teaching. Today, Ischler works with breathwork clients in Germany and ayurvedic patients in India. A graduate of BBTRS, Ischler has successfully integrated his study and experience with both Western and Eastern traditions.

"There are engineers or physicists, and they know how one thing should work. We cannot do that with a human body. Medicine is not an accurate science," Ischler says. "I am now working as a holistic doctor, because Western medicine is not yet prepared for many of the results that come from psychotherapy, Native practices, or even tests with MDMA. The success I have with BBTRS shows me that other kinds of work can help heal people from within."[51]

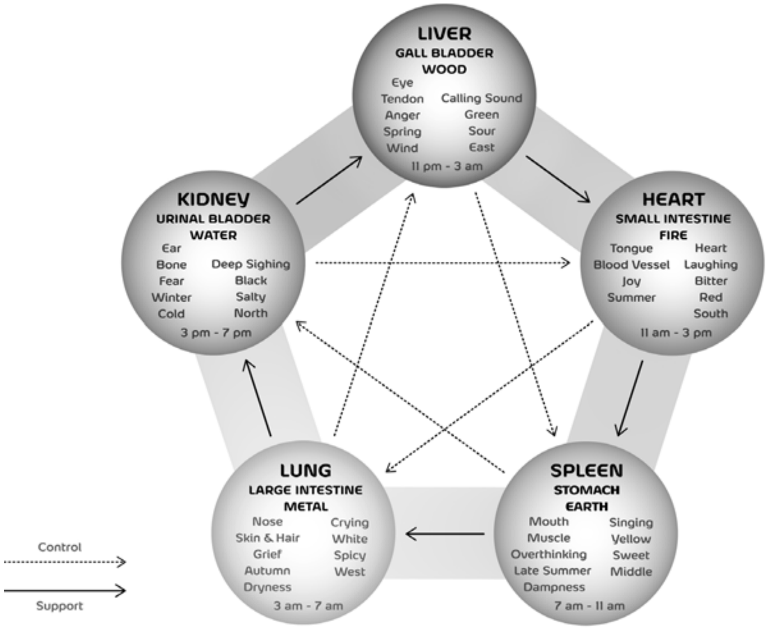
One client of his from Geneva, Switzerland, provided this review[52] of BBTRS in Ischler's medical practice:

Catherine:

> "I'm still amazed how with my own breathing I was able to release all the tensions and traumas. I feel so free of all the shells built along the experiences of my life.

> My posture has changed, and I once again feel confident in myself and my abilities. I sleep beautifully; nothing seems to be too big a challenge. I want to learn again, and all my curiosity is back.

In BBTRS, Eastern philosophies to approaching illness or imbalance are added to our scientific understandings—about fight-or-flight, the ways in which signals travel through our bodies, and the various methods that have proven effective in physical and mental health care. We build from the Chinese approach, in which the body is a fine, energetic garden, where all parts interrelate. Balance and harmony throughout the system


LIVER
GALL BLADDER
WOOD
Eye
Tendon
Calling Sound
Anger
Green
Spring
Sour
Wind
East
11 pm - 3 am
HEART
SMALL INTESTINE
FIRE
Tongue
Heart
Blood Vessel
Laughing
Joy
Bitter
Summer
Red
South
11 am - 3 pm
SPLEEN
STOMACH
EARTH
Mouth
Singing
Muscle
Yellow
Overthinking
Sweet
Late Summer
Middle
Dampness
7 am - 11 am
LUNG
LARGE INTESTINE
METAL
Nose
Crying
Skin & Hair
White
Grief
Spicy
Autumn
West
Dryness
3 am - 7 am
KIDNEY
URINAL BLADDER
WATER
Ear
Bone
Deep Sighing
Fear
Black
Winter
Salty
Cold
North
3 pm - 7 pm
Control
Support

Traditional Chinese Medicine's "Five Element Theory," which shows how the organs are related to each element, and how the various elements control and rely on one another for support.

(TCM), the ancient Five Elements are correlated to different parts of the body—and they're also correlated to everything else. All natural phenomena are organized into groups—Wood, Fire, Earth, Metal, and Water—and they relate to a seemingly limitless set of understandings.

In the Eastern approach, the doctor is viewed as a gardener who's maintaining and restoring the fine balance and homeostasis of the garden of the human body. The mission is to keep all elements in balance with one another according to time of year, time of day, and the particulars of the patient. When considering an individual's emotional, physical, psychological, or energetic balance, we might have a conversation linking season, climate, patient's age, stages of development, the body, the soul, and the senses—because everything is interrelated and connected.[53]

Various aspects of Eastern philosophies will be incorporated throughout this text, because various tenets of holism apply to BBTRS and its components. In addition, the idea that Eastern thought requires harmony among our different elements applies to the very foundation of BBTRS—which also has "spokes on a wheel." Each spoke supports and enhances the others, creating an integrated whole.

How culture integrates BBTRS

Likewise, the spokes of the wheel impact practitioners and facilitators who want to work with people from other places. Like anyone crossing boundaries, we need to educate ourselves as much as possible. In addition to doing research, please feel free to talk to your clients or classmates from other cultures. After all, it is their experience—not a country's—that might impact yours. And you theirs.

Kaveesha Prem is a BBTRS graduate, translator, and group coordinator for a variety of programs. She lives in Taipei, Taiwan, and has translated and facilitated several BBTRS groups on the mainland of China. Although she didn't want to "generalize about all people from a certain place," she agreed to discuss an example of how different cultures integrate BBTRS experiences—especially the deep, activating breathwork associated with trauma release. "I have experienced what seems to be a cultural difference between Western and Chinese participants," she says. "The Chinese tend to vomit and burp more." In fact, early in a BBTRS session—regardless of what country the group might be in—she and other facilitators report that Chinese participants have a tendency to vomit, whereas participants from other cultures do not. "I have always assumed this comes from a deep-rooted concept in Chinese medicine, where vomiting, spitting, and burping are ways to cleanse the body," Kaveesha adds.

Traditional Chinese Medicine is about balance, and vomiting is a signal of imbalance—specifically "impaired homeostasis and reversed upward Qi flow within the stomach."[54] Later in this chapter, we'll learn more about Qi, the life force that flows through all of us. But for now, according to TCM, there are at least three types of vomiting, and they can be stimulated by "six excesses, wind, cold, summer heat, dampness, dryness, and fire/heat"—all influencing how the Qi flows in the body. Basically, the stomach's function, which relies on supporting organs, requires a downward energetic flow. "Various unfavorable conditions associated with environmental factors, food, mental health, or physiological stress can compromise the stomach, thereby leading to the loss of gastric homeostasis, which often triggers vomiting."[55] It follows that deep, connected breathing, which charges the Qi, surely could change the balance of the stomach.

But there is more to trauma release than energy coursing through the physical system. There are also emotions—and they can rise with increased activation. In fact, practitioners have also noticed heightened vocal expressions in Chinese participants. Again, according to tenets of Chinese medicine, "one very common cause of emotional imbalance in TCM is repressed emotions. If emotions are not being expressed, they are being stuck, which can lead to a blockage of the flow or a stagnation, which in turn can lead to disease. Releasing emotions can heal disease."[56]

These kinds of insights allow BBTRS practitioners and facilitators—as well as breathers who are new to the modality—to consider their own belief systems, healthcare approaches, and general assumptions about change. Cross-cultural awareness, curiosity, compassion, respect, and clear communication all assist when working with the very intimate experience of trauma release, especially in the presence of language and life-experience differences.

Working Across Cultures

"In a recent training session in Bali, a Chinese woman volunteered for a demonstration session directed at the abdominal region. Although I did not know this person, I have interacted with hundreds of Chinese participants in BBTRS sessions over the years. Some of these participants have discussed some cultural conditioning with me, particularly a tendency to avoid emotional expression in their day-to-day lives. In this session, I tried not to make any assumptions, but to be ready in case she shared this tendency.

Like many Chinese participants in the room, when she began her breathwork, her body charged quickly with activation. Early on, she found her voice with a lot of emotional expression, much of it disjointed and cathartic in nature. With all of this energy flowing, I continually paced her, asking her to slow down, calm her nervous system, and then reactivate. At first, I simply held my hand over her belly, instead of demonstrating several interventions. Then, when I finally put my hand on her belly, an intense process began.

She was comfortable in the demonstration because she saw me as someone who didn't want anything from her. My mission was simply to support her—with my presence and with my touch. She later said this allowed for an intense, safe expression that could transcend her initial vocalizations. After she rested and self-regulated, I massaged with a bit more weight on her belly, in clockwise circles that moved in synchronicity with her breath. At first, her muscles were tight, protecting her core—but after just one or two circles, her belly started to soften. With this softening, her emotional expressions became less cathartic and more connected with the movement of her body. The intentional pacing of the session kept her connected to her felt sense. Eventually, we naturally worked with her entire abdominal area. I was very careful to go slowly and keep her present. If she started to go too much into a charge, I brought her back. She was able to rest, then recharge and release, then rest again.

The session progressed beautifully, with her vocalizations eventually fading, and her relationship with her body growing. I could both feel and see the tension flowing out. After the session, she explained what had happened for her. Without knowing what sensations or memories she might encounter, she organically had gotten in touch with very deep feelings. They involved the neglect and misuse of her sexuality, which had made themselves plain to her when we worked on her lower belly. Like people the world over, and many women from every walk of life, she had felt the need to use sex to gain something from others. Until this session, there was a lingering feeling that she "thought she had to," and that she was not "enough" on her own.

You might expect a turning point like this to produce tears or hurt feelings, but no. Like most BBTRS breathers, after the session, she sat up with huge smile. Everyone in the room was astonished; her face had changed entirely. Her eyes were soft, and she was radiating light. By nurturing her at the speed she needed to feel safe, she now looked and felt like a different person. She had released so much tension in this single session, she felt as if she

BBTRS practitioner Natalie Keany works with a breather's abdominal area in a workshop setting.

could truly breathe freely. She was finally in touch with her body, and with her life.

Mental health & psychotherapy

Once society began to understand that trauma was more than wartime "shell shock" or simply an unspoken family reality, then psychotherapy began to take its place in solution-building. Over the period of growth in the field, several theories of "how people tick" have led to distinct therapeutic approaches. And with the increasing focus on trauma research, an inevitable dovetailing has occurred. Just as Western medicine has adopted some Eastern tenets, so psychotherapy grows to include the body—creating mind-body therapies.

Beyond adding the now relatively common tools of mindfulness, breathwork, and meditation, some therapists are adding trauma-specific specialties in their work. For example, anxiety is a common complaint, and it might involve fearful thinking and beliefs, along with an activation of the fight-or-flight system. Or chronic fatigue might be the result of unhealthy lifestyle patterns that also exhaust the adrenal system—basically leaving the fight-or-flight "switch" turned on. "Pairing psychotherapy and BBTRS works brilliantly in these instances," says Kate Macnamara, who offers both psychotherapy and BBTRS in her Australian clinical practice, Jesmry Counseling Services. "In addition to supporting clients to heal their fears and patterns with psychological support, BBTRS has the capacity to directly deactivate the nervous system."[57]

Understanding the differences between nervous system activation and psychological responses is important, but as in any healing process, there can be no “preferred method” when actually treating a client. The trauma-focused interventions work in tandem with psychological trainings, which can vary greatly. According to the American Psychological Association, psychotherapy generally falls into five broad categories, based on different theories of how people develop.[58]

1. Psychanalysis and psychodynamic therapies focus on the unconscious meanings and motivations behind our behaviors, feelings, and thoughts. While this branch of therapy is still closely identified with Sigmund Freud, “analysis” has expanded since its inception.
2. Behavior therapy focuses on how we learn, and how positive and not-so-positive behaviors are developed as a result. Key in this branch of therapy were heavy hitters Ivan Pavlov and his classically conditioned dogs, as well as E.L. Thorndike, who implemented rewards and punishments in operant conditioning.
3. Cognitive therapy is about thinking, and how dysfunctional thoughts lead to dysfunctional emotions or behaviors. Albert Ellis and Aaron Beck were prominent figures in showing that if we change what we think, then we can change how we feel and what we do. Cognitive-behavioral therapies are a more recent variation, and reflect a blending of the named schools of thought.
4. Humanistic therapy approaches the human condition through each person’s individuality and uniqueness, which determine how we reach our maximum potential. Philosophers Jean-Paul Sartre, Martin Buber, and Søren Kierkegaard influenced this branch. Sub-categories that grew from Humanism include:
    a. Client-centered therapy, which prioritizes the individual’s expertise in his or her own life over the therapist’s opinions.

    b. Gestalt therapy, which focuses on personal accountability, and living in the "here and now."
    c. Existential therapy, which focuses on the search for meaning in life, as well as our ability to determine our own paths.
5. Integrative or holistic therapy, also known as eclectic therapy, blends elements from a variety of modalities, allowing the therapist to tailor individualized treatments for each client.

Within this general framework, therapists can specialize in working with children, families, adults, couples, people suffering with addiction, or those from specific backgrounds or cultures, such as military, clerical, corporate, or ethnic or minority groups. Therapists lead support groups and teach classes; they might specialize in grief or anger management, among many other focus areas.

In approaching methods to care for people who are suffering, it's easy to see how some researchers and therapists would focus particularly on trauma. As an enormous subject with worldwide impacts, trauma requires specialists from all regions and walks of life. Working with trauma victims can involve deploying to areas in crisis—floods, fires, warzones, humanitarian interventions, local accidents or emergencies, or broader acts of terror. It also requires day-to-day, local helpers, trained therapists who are capable of working with trauma in all forms, across the population.

Obviously, working with trauma in the field of psychology includes how the individual is in relationship. "Couples often trigger each other's nervous systems," Macnamara says. "For example, if one individual holds unhealed trauma—where loud yelling was a component—and the other partner raises his or her voice, the first individual's nervous system may activate into a familiar response. This response was used in the past to attempt to avoid harm. In the present, any attempts to communicate effectively falter."[59]

Macnamara explains that this kind of activation disengages us from the outer cortex of the brain, which is required for effective communication. This disengagement happens when the limbic system boots into gear, choosing between "fight, flight, freeze, or fold." That's why a counselor's support—both through psychology and trauma release—can reset someone's internal scene, from the brain down. "This approach plays a role in healing core patterns and themes that clients may have struggled with for years, and which can show up in all aspects of life, including relationships," Macnamara says.

In her own life, Macnamara is well aware of the effects of trauma. "Even with all my training and skills, and my own work in therapy, my nervous system was still triggered at times. In certain circumstances or situations, I found myself vulnerable to disconnecting from myself and others. I struggled to voice what was true for myself, and at times I felt very alone and isolated," she says. "BBTRS allowed me deactivate the 'freeze' response. It began long ago in childhood—to cope with and survive with sexual and psychological abuse—and was remembered in my nervous system. This was life-changing for me, and since then I have been able to live my life in a much more empowered and truly authentic way. Even though I have used the breath personally and with clients for a long time, I was amazed how effective it can be when it is used in the context of BBTRS."[60]

Breathwork

In general, modulating the breath is known to impact our emotional and physical states, but that's not all. Changing how we breathe changes how our nervous system functions. Remember the "gas and brake pedals" associated with the sympathetic and parasympathetic nervous systems? Physically, as we inhale and exhale, blood is drawn between the heart and the vasculature of the lungs, causing subtle changes in our heart rate and related responses of our nervous system. The abridged story: the more slowly and deeply we breathe, the more our parasympathetic "brake" is engaged; conversely, faster deep breathing engages the sympathetic "gas"—a direct link to the entire fight-or-flight auto-pilot response system.[61] This is why breath is the first of the primary aspects of BBTRS. It opens the door for somatic work.

Breathwork has a long history, and many diverse applications. Deep understanding of the breath has been shown to both change the quality of our lives and impact the way our subconscious and conscious minds work together. Breathwork is something that anyone can do, anywhere in the world—and it has been shown to have immediate, positive impact for those suffering from grief, anxiety, and other challenges.

For example, Max Strom teaches personal transformation, mindfulness, and yoga through the Inner Axis method. His focus is on stress reduction, improved sleep, and higher day-to-day functionality, all accessed through changing patterns of breathing. Quoting statistics that show increased levels of life dissatisfaction and depression in people across the world, Strom advocates a free, simple, no-side-effects practice of breathwork that can be done even during work breaks.

Like the breathwork in BBTRS, the intentional breathwork in Strom's practice allows us to calm ourselves naturally, and to enter a space of non-reactiveness. Strom says this is one component to a more sustainable life—but he also notices a very specific correlation when working with groups. “The people that have the most anxiety, (when they) learn breathing exercises, almost immediately start to weep,” he says. According to his work, anxiety is often associated with unexpressed grief.[62] We have seen similar correlations in BBTRS sessions.

“People feel alone,” Strom says, which might seem counterintuitive, since we seem to be “connected” through current modes of technology. He believes that by focusing on screens, however, we have developed a lack of face-to-face, interpersonal intimacy—and therefore lack effective methods to deal with our own and others’ suffering. We might yell at a passing driver, without apology to our car’s passengers, for example, but become embarrassed if we cry in public. We also don’t know what to say to a friend who is grieving, thus isolating that person when he needs support. For the record, Strom suggests this approach: “Don’t try to cheer them up. Just say, ‘It’s going to hurt really bad for a while, and I’m not going anywhere. I’m here. This year, it’s your turn; next year, it might be my turn. We’ll all get through this together.’”[63] And breathe.

Another well-known practitioner who works primarily with the breath is Stanislav Grof. Considered a “consciousness researcher,” Grof describes his Holotropic Breathwork as a method to access our deepest creativity—reaching a place where we develop ideas. Although the foundations of human consciousness will always remain a mystery at some level, Grof and other consciousness researchers describe it as a combination of sensory signals, memory, and the subconscious mind. From this perspective, the subconscious even has been considered as the realm from which our ancestors developed human intelligence.[64]

He has spent four decades studying ancient shamanic practice, in which consciousness was altered through breathing exercises that provide a mild form of oxygen deprivation in the brain. What happens in this "transpersonal realm" is hidden from everyday awareness. "The western industrial civilization is really the only group of people throughout human history that does not hold the non-ordinary states in high esteem," Grof says. "Every other group has tremendous appreciation of these states, and they spend a lot of time and energy trying to develop very safe and effective ways of inducing these non-ordinary states."[65]

In today's complex world, Grof advocates doing our "homework" by becoming deeply aware of the problems and questions in our minds. Then, when entering the non-ordinary state through breath, creative answers are more available. "The ideas, the inspiration that can come from non-ordinary states, can help later when the person returns to the ordinary state to substantially improve our way of being in the world," Grof says.[66]

In the area of non-ordinary states and consciousness, the use of psychedelic substances has been helpful to many researchers. Grof worked extensively with LSD before developing Holotropic Breathwork. In addition, hallucinogenic plants, such as peyote and ayahuasca, have been used for generations, especially by indigenous populations in sacred ceremony. In successful experiences with these substances, a "knowing" beyond the mind can occur for the practitioner.

It's important to distinguish breathwork used for healing or shamanic practice from recreational forms of breathwork. In the latter type, achieving an altered state of consciousness is the sole purpose. For seekers simply looking for an interesting experience, the more beneficial aspects of breathwork are usually secondary—and might not even be noticed. Recreational breathwork practice has little, if anything, to do with somatic work. One other important note about BBTRS and shamanic exploration: Although the BBTRS structure was partly inspired by an ayahuasca experience, the use of mind-altering substances is not allowed during sessions. The stimulation of the nervous system through breathwork can be an intense experience. Both safety and ethical considerations contraindicate mixing therapeutic, trauma-release breathwork with other activities.

In BBTRS, the breath links our conscious and subconscious aspects; it provides grounding in our bodies; it assists us in maintaining connection with our facilitators during sessions; and it opens the door to our fight-or-flight auto-pilot nervous system. While other somatic practitioners conduct successful sessions without conscious breathwork, BBTRS practitioners find the breath to be an intimate partner for the somatic releasing process. It also provides a direct connection for the breather to identify where tension is held in the body, and to monitor the stages of tension release. Conscious breathing takes somatic trauma release to another level.

Somatic therapy

If breathwork is our direct access to the sympathetic nervous system, then somatic work is our primary exit path for trauma to release from tissues. Recalling that the word “somatic” means “of the body,” it’s clear that any therapeutic trauma release focusing on tissues requires a strong protocol for safe, effective movement. There are many body-based practices, including somatic yoga, somatic psychology, somatic dance therapy, and conscious somatic movement—in which movement is focused on the internal experience and not the external appearance or result of the movement. Our focus is on somatic trauma release, which is a form of somatic therapy.

It's easy to distinguish somatic therapy from two of the most common and popular trauma treatments in mental health—talk therapy and medication. While useful, according to somatic pioneer Bessel van der Kolk, both of these approaches are limited because they don't bring the person out of trauma and back into reality. In the case of drug therapy, brain chemistry might rebalance—and a person can even receive comfort—but the root of the problem is not touched.[67]

In broad terms, body-centered therapies can focus on stress, anxiety, depression, and grief, as well as related issues, including addiction and sexual dysfunction. Mind-body exercises are often employed to help release tension that negatively impacts both physical and emotional wellbeing—thus creating a bridge between somatic therapy and elements of psychotherapy. In addition, some effective trauma therapies primarily target the brain, such as neurofeedback and Eye Movement Desensitization and Reprocessing (EMDR). Others primarily target the body, such as singing, dance, and massage. Recent research conducted by Van der Kolk, along with that of other researchers, has shown that even more integrative practices, especially yoga, are helpful for people suffering with PTSD. Yoga incorporates conscious breathing, mindfulness, and movement, resulting in emotional regulation.[68] In fact, yoga has documented affects in restoring pre-trauma physiology.[69]

Peter Levine, who holds doctorate degrees in both medical biophysics and psychology, has studied stress and trauma for more than thirty years. He has been a stress consultant for NASA, a member of the Institute of World Affairs Task Force with Psychologists for Social Responsibility, and served on the American Psychological Association's initiative for response to large-scale disaster and ethno-political warfare. He is the founder of the Somatic Experiencing® (SE®) Trauma Institute. SE has been used successfully in studies that reduced symptoms of PTSD—most recently, in an Israeli study conducted during active political unrest and terrorist attacks.[70]

SE's foundation relies on the client's becoming aware of inner physical sensations, such as tightness, tingling, numbness, and warmth, which act as primary guides to traumatic memory. By becoming aware of these sensations—and then developing an even more intimate interaction with sensory messages throughout the body, called the "felt sense"—a practitioner gains the first handhold toward releasing the trauma itself. Bodily sensations are like beacons, pointing to the traumatic activation that was left behind in the tissues after a traumatic experience. We know that the amygdala can't tell if the experience is happening now or ten years ago, so the feelings we experience might be a trauma-produced "echo." Just the process of accessing a memory or its related symptoms causes fight-or-flight activation to begin. However, in a therapeutic setting, the experience is not allowed to escalate as it might have in the original experience.

SE's basic method of releasing the activation is to very slowly and gradually come into awareness of various aspects of the traumatic experience. If the traumatic experience itself is not recalled, then attention is directed to the sensations that are happening "today." At each gradual phase of response, the client receives support to (1) slow down the fight-or-flight "checklist," and (2) allow each phase of activation to fully complete, to fully express itself so as to exhaust the charge.

SE has provided an excellent foundation in the field, and has proven to be very successful. In addition to working well with individual sufferers, Levine's scientific approach also has defined terms for other practitioners. He named or applied several concepts or functions that are used in SE, including the following, many of which will be used in this text.

- The "felt sense," which was coined by psychotherapist Eugene Gendlin, and described above. It will be discussed in more detail later in the text.
- "Discharge," the process of dissipating the tension held in tissues.
- "Titration," referring to releasing the tension in small "doses," so as not to overstimulate a person.
- "Resources," which are supports—feelings, sensations, areas of the body, memories, people, physical objects, or thoughts—that calm, enhance feelings of safety, and reduce arousal.
- "Pendulation," in which one utilizes the body's natural rhythm of moving between the contraction of tension and the expansion of relaxation. In somatic trauma release, breathers can consciously shift attention from areas of "contracting" tension/trauma to their "relaxing" resources.

Finally, one of the leaders in "pure" somatic trauma release is David Berceli, PhD. A board-certified psychoneurologist, he created Trauma Releasing Exercises® (TRE®), a protocol intentionally structured to be easily accessible to anyone who wants to easily stimulate stress-relieving tremors. The protocol is comprised of six exercises that initiate mild tremors in the legs; once the tremors begin, the person reclines, allowing the tremors to spread naturally through the body. "The (tremors) take any pattern that you may still have in your body—that could be from stress or anxiety or trauma—and they begin to relax that tension pattern," Berceli

says. In this modality, the person simply observes his or her body as the tremor moves through different muscle groups.[71]

Berceli's work has shown improvements in sleep, stress and anxiety, muscle tension and back pain, worry during chronic illness, anger and over-reactivity, relationships, resilience, and optimism.[72] However, Berceli has taken these healing impacts a step further. With Trauma Recovery Services, he founded a company that provides workshops and recovery programs for international organizations around the world. For the last two decades, he has lived and worked in Israel/Palestine, Sudan, Uganda, Kenya, Yemen, Egypt, and Lebanon, where he integrates his awareness of religion and ethnic customs, enabling people to move beyond personal trauma. His purpose in taking his program to areas around the world where conflict and trauma are prevalent is to facilitate reconciliation. In his model, then, trauma release can be individual, cultural, and societal.

BBTRS builds upon the work of these somatic researchers and practitioners, and integrates the body's natural movements to release tension from the tissues. Although the physical aspects of encouraging the body to shake and undulate might be unusual in a person's day-to-day life, the process truly has no equal.

Meditation

As with breathwork and yoga, hundreds of scientific articles and studies correlate the benefits of meditation with various physical, psychological, and emotional benefits. For example, just one website, Transcendental Meditation (tmhome.

com), lists sixty research papers from a variety of sources showing the positive effects of meditation on blood pressure, heart disease, addiction, cancer and other diseases, mental health in various populations, anxiety, employee stress and burnout, PTSD, hyperactivity and ADHD, violence, autism spectrum disorders, brain functioning, cognition, creativity, work and school performance, and relationships, among other topics.[73]

One of the most renowned programs documenting the positive effects of meditation is Mindfulness-Based Stress Reduction (MBSR), from the Center for Mindfulness in Medicine, Health Care, and Society, a department of the University of Massachusetts Medical School. Founded in 1979 by Dr. Jon Kabat-Zinn, MBSR melds East and West—combining science, medicine, and psychology with Buddhist meditative traditions, described as the "refinement of attention and awareness, compassion and wisdom."[74] This work has shown "consistent, reliable, and reproducible demonstrations of major and clinically relevant reductions in medical and psychological symptoms across a wide range of medical diagnoses,

including many different chronic pain conditions, other medical diagnoses, and in medical patients with a secondary diagnosis of anxiety and/or panic."[75]

The primary attributes taught and supported by MBSR are the same ones BBTRS recognizes during and after trauma release—a "heightened sense of self and self-in-relationship, and a greater ability to find coherence and act effectively under high degrees of stress."[76] Meditation is a key component to BBTRS because of these attributes. Furthermore, a daily meditation practice is the vehicle by which BBTRS's positive results are carried on and integrated into our lives.

Energy

The human experience of energy has long presented a rich, diverse, complex, and sometimes divisive dialog. From a purely scientific view, the term "energy" is used in physics to describe the force that heats, moves, or otherwise "works" upon an object. Types of energy include kinetic, elastic, chemical, radiant, magnetic, electric, or the potential energy held in a force field, such as in gravitational energy. School children are taught the law of conservation of energy, which states that energy can be converted in form, but never created or destroyed. The term "energetics" is another scientific term; it refers to energy under transformation, from the quantum level to the cosmos. This term encompasses many diverse disciplines, including thermodynamics, biochemistry, and ecological energetics.

Long before any of those "energy terms" were coined, however, the Chinese had identified a different type of energy. At least 4,000 years ago, Qi—also known as chi, or vital energy—formed the basis of a modality in traditional Chinese medicine, Qi gong. This modality was, and continues to be, based upon the belief that the human body circulates this vital energy through a network of energy pathways. Qi gong utilizes the breath to balance the body and maintain good health. According to the *Natural Medicine Journal,* "Qi gong is an umbrella term that subsumes a variety of energy-based healing practices based on Taoist philosophy and principles of Chinese medical theory. There are reported to be several thousand schools of Qi gong practice in its long history in China."[77] In Qi gong, an individual can work with his or her own energy, or a practitioner can infuse Qi into a patient.

Qi is far from the only description of "energy" that has been named across the globe in the last 4,000 years. Life force—in individuals, the earth, and the universe—has been embraced and utilized by healers and thinkers in cultures across the world; it has been identified as an aura that extends beyond our physical bodies; although refuted by scientists, many believe it has been photographed in Kirlian photography. Differences of opinion about energy arise generally when Western ideas of quantifiable science clash with Eastern ideas of intuitive knowing. Still, humans on every continent agree that there are literally countless aspects of life that cannot be proven scientifically; we share experiences, sensations, and interactions that transcend science. The energy of life force is indisputable in its presence, but difficult to "catch." Still, people have tried.

The National Center for Complementary and Integrative Health (NCCIH) is one of twenty-seven institutes and centers within the National Institutes of Health (NIH) in the United States. Its mission is to utilize scientific protocols and investigation to define the safety of alternative medicine interventions, which it describes as "health care approaches developed outside of mainstream Western, or conventional, medicine." Of course, NIH's posture assumes that the Western medical model is the standard, and that older, Eastern approaches are alternative—but that's okay, our mission here is to gather and share information.

Beyond yoga, meditation, acupuncture, and other therapies, the NCCIH also tries to consider approaches that do not fit neatly into their concept of integrative or complementary approaches. These include Ayurvedic medicine, traditional Chinese medicine, homeopathy, and naturopathy—and what is known as energy medicine. Qi gong and Reiki are examples of practices that cannot be scientifically duplicated and verified. Although Western scientists have tried to gather information on some of these outlying practices, few studies have been undertaken. In these cases, scientific conclusions on the NCCIH website are inconclusive, such as the one listed for Reiki, which says it is ″based on an Eastern belief in an energy that supports the body's innate or natural healing abilities.″ However, it adds, ″there's no scientific evidence supporting the existence of the energy field thought to play a role in Reiki.″[78]

For those in the energy medicine field, two distinct energetic areas have been identified, "veritable," or bona fide, and "putative," or reputed but unproven. Veritable energy includes methods that can be measured, such as mechanical vibrations—sounds, for example, which form one of the six pillars upon which BBTRS is founded—as well as light, and monochromatic radiation, including lasers. On the other hand, putative energies are based on that same idea of Qi or prana, a life force infusing living systems.[79] The therapeutic touch involved in Reiki and BBTRS utilizes this kind of energy, and "evidence" of its presence is obvious to practitioners.

While scientific proof of the energetic component of Qi, or prana, is unnecessary in BBTRS, we also find it intriguing to note that some scientists, and lay-scientists, have been thinking outside the box. In a recent experiment, the Dubai branch of IKEA, the Swedish homegoods store, collaborated with an ad agency to test the impact of bullying—on plants. School children conducted an experiment in which two identical plants were watered, fed, lit, and otherwise treated similarly, except that one was spoken to harshly and the other was praised. After the bullied plant wilted, some worldwide observers doubted the test. “While there seems to be a consensus that plants respond better when talked to, there’s not much evidence that plants really care what you say,” *Adweek* wrote on May 8, 2018, shortly after the study was published. However, *Adweek’s* and others’ follow-ups were answered by project spokespeople similarly: those involved “reaffirmed that it was a legitimate project.”[80] It certainly makes sense to BBTRS practitioners.

Perhaps the most notable among out-of-the-box thinkers come from the Global Consciousness Project (GCP), a multidisciplinary collaboration of scientists and engineers. In an unprecedented expression of collaboration, these scientists have used verifiable laboratory procedures to record what can only be described as what might be the first Western evidence of Qi. In simple terms, “random event generators” (REGs), which produce a random output of 1’s and 0’s, are located in approximately seventy sites around the world. For more than fifteen years, random, synchronized data from these “high-speed electronic coin-flippers,” has been collected.

On an average day, completely unpredictable sequences of 1’s and 0’s result—but on remarkable days, both good and bad, the sequences are not random. “When a great event synchronizes the feelings of millions of people, (the) network of REGs becomes subtly structured. We calculate one in a trillion odds that the effect is due to chance. The evidence suggests an emerging noosphere or the unifying field of consciousness described by sages in all cultures.”[81]

In an effort to quantify the results, the GCP has searched for words. "When it comes to 'what we are measuring,' the story becomes more complicated, because there is no real understanding of the mechanism whereby an REG's behavior can be altered by thoughts and emotions or intentions…. We do not know how a mental state such as an intention or emotion is able to inform the physical system to affect its behavior."[82] Further, after recording spikes in the data on days like September 11, 2001, or World Peace Days, or the deaths of world leaders, the scientists behind this work add: "The devices are designed to be independent, and they are separated by hundreds or thousands of kilometers, and yet we see the correlations—that is anomalous, and it is linked with consciousness. The implication is that we are not isolated from each other as seems to be the case, but linked in a subtle, unconscious and inaccessible way. Learning more about that, and tapping into the potential of our interconnection is the next phase of human development. We are at the beginning, and ready to move forward."[83]

In BBTRS, the idea of energy is foundational to the practice. It is energy that is caught in the tissues of our bodies—energetic activation that has not dissipated. Energy arises in us as hope, when we anticipate feeling better. Energy passes between breather and facilitator, too, in the process of witnessing and supporting. Energy assists with release if the facilitator applies touch to the breather's body. Even more, energy resonates between breather and facilitator, sometimes allowing the breather to "pick up" the intention of the facilitator and move accordingly. When this happens, we tune in, like transmitters and receivers dialing into the same frequency. "Touch" happens without physical contact. These kinds of experiences, which we see every day in BBTRS, can seem mystical. But they're just life.

Life runs on energy. It runs in currents. It circulates through our bodies. When we pay attention, we feel the quality, the balance, the power of the energies within our bodies, our lives, and the world around us. In BBTRS, we embrace both the science that explains our physical realities and the majesty of the mystery. By welcoming the greatness of the unknown into our experience, nothing is dismissed. In the Lakota language, there is one word that says it all: okipichasni—all things are possible.

Reflection

Thank you for reading this chapter. Now, please take a moment to bring your attention to yourself. Please read the following paragraphs, and then close your eyes and scan your physical body. Notice how what you read reflects in your body. What is happening in your body—is it relaxed? Perhaps start with the breath, and consider whether it is flowing freely. Then consider your posture, whether you are sitting or lying down. Do you feel balanced, or does your physical body have tightness, pain, or discomfort? Look at different aspects of your body, including inner and outer tension. Take a deep, full breath, and sigh out your mouth. Next, consider your thoughts—are they generally centered outside of you? If something external is consuming your attention, can you let it go for now?

Now, consider if the material in this chapter resonates in you, and whether the resonance involves you or someone you know. If there is some triggering—positive, negative, emotional, physical, spiritual, or some other experience—then what does it feel like? Maybe it feels like you're reading something intimate or personal or true about yourself—or maybe you didn't like something you read. What was it? Was the response because the information was revealing, challenging, nurturing, or something else? It's possible that something that's gone unnoticed within you, or something you've neglected or forgotten about, might really be valid and making itself known. It's possible that this material might have awakened it in you. Or not. You be the judge.

After reading these things, and reflecting on them with your eyes closed, then open your eyes and bring your attention to your surroundings. Listen to the sounds. What do you see? Between chapters, perhaps take a brief walk, or dance to a couple of songs. Move around, reorient, and then settle in for the next chapter. Thank you.

Chapter 4

The foundations of BBTRS

Everyone you know is likely using energy to hold onto tension. People might notice fatigue, a nagging slowness, and even apathy. They might be dissociated, and feel exhausted. Or, on the other hand, their whole systems might be flooded with nervous energy that vibrates deep within. They might be hyperaroused, always ready to fight or flee. In any case, they'll likely lack "juice" in life—and they might have gotten very used to it.

BBTRS is a somatic modality that relies on a suite of elements, which, when applied together, maximize the potential of trauma release, along with the return of balance and "juice." It draws from the best-researched modalities in the field—including various aspects of somatic approaches; scientifically proven mindfulness and meditation applications; anatomy, massage, and physical therapy; breathwork; sound vibrations; emotional release; and energy.

This modality can be practiced in groups or one-on-one with a practitioner/facilitator—and eventually, after several sessions, alone, with modifications. As with any somatic approach, this one focuses on where the body holds tension. However, BBTRS is different in how it assists a person to sense this tension, to participate in the release, and to achieve a long-lasting, productive relationship with managing stress.

Even extremely traumatized people have found this nurturing, multimodal approach to be faster and more comprehensive than is any single component on its own.

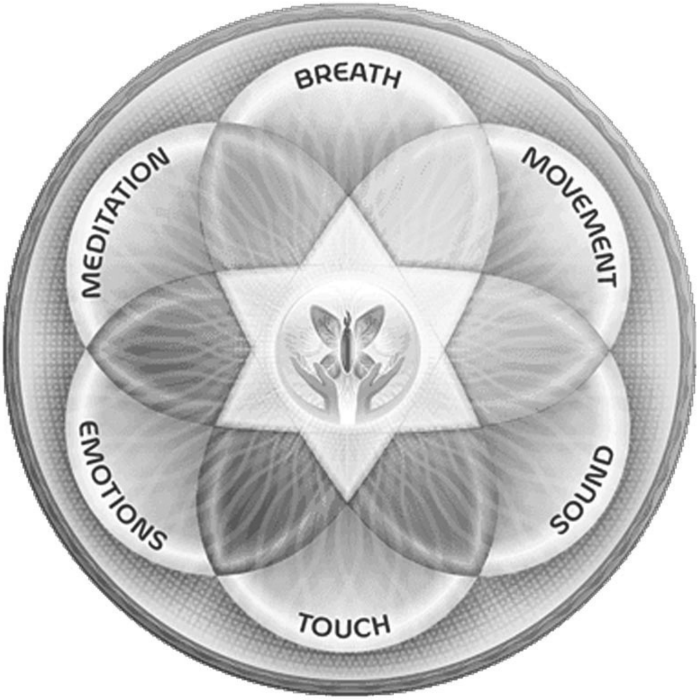
The six elements of the BioDynamic Breathwork & Trauma Release System function naturally in our bodies, utilizing the autonomic processes of the sympathetic nervous system, physics, and energetic connections between people. In order of importance, they are:

1. Breath
2. Movement
3. Touch
4. Emotional Expression
5. Sound
6. Meditation

Please note: When working with people who are extremely traumatized through PTSD, it's important to first assess how resilient their nervous systems are. The foundational, efficient method of breathing used in BBTRS might initially prove too stimulating. Therefore, in these cases, we often begin with meditation or movement through tremoring as a gentle beginning; breathwork can be introduced after familiarity with the method is established.

Also, in later chapters, detailed accounts, approaches, and tips regarding each of the BBTRS components will be described. What follows is an overview.

Components



This mandala, which has become the logo for BBTRS, represents the six elements at the foundation of the modality.

Breath

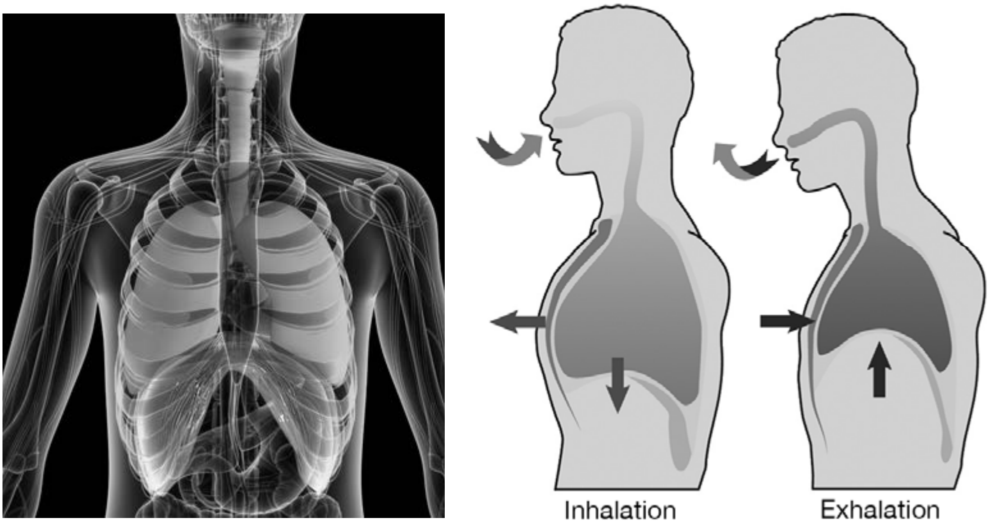
Breathing indicates how we're feeling. Just think about it: what happens when we're worried or tense? So often, people find themselves saying, "I keep holding my breath!" We've been trained since childhood to manage our emotional expression, to "control ourselves," and this often means we tighten our breath. Breathing affects posture, complexion, energy levels, and an individual's capacity to live life to its full potential.

Long-term tension causes many people to constrict their breath, causing a lack of oxygen in the system—which is called hypoventilation.

Hypoventilation is also common in heavy smokers or in people who are simply not used to breathing properly. It often goes unnoticed, because its symptoms include a gradual increase in sluggishness, depletion of energy, and even irritability. Learning to breathe freely and joyously leads to a variety of benefits, not the least of which is increased lung capacity.

On the other hand, hyperventilation occurs when we breathe too quickly and forcefully. Sometimes, hyperventilation is intentional, as in the case of yoga pranayama practices that aim to stimulate and awaken the full breathing capacity. In these practices, drawing in breath is the equivalent of drawing in prana, or life-force. However, sometimes people unintentionally draw in quick breaths without using the full capacity of their lungs, especially when they are anxious or stressed. This results in very shallow breathing, and can lead to a drastic lowering of carbon dioxide levels in the blood, which in turn can cause capillaries to become constricted. Closed capillaries mean reduced blood flow to the cells, and hence no fresh oxygen—and lightheadedness, nausea, a sense of "spinning," and other symptoms.

People who are anxious or stressed on a daily basis experience altered breathing habits—whether hypoventilation or hyperventilation—which reset autonomic, involuntary breathing patterns. This behavior becomes particularly damaging; over time it sparks a series of physiological adaptations that engage our survival mechanisms. One physiological benefit of BBTRS is that it retrains our breathing patterns, thus resetting any "faulty" habits. Natural, clear, relaxed breathing also increases our ability to adapt to tense situations that may compromise our capacity to breathe calmly. The most healthful breath flows freely, naturally increasing or decreasing in pace and depth as appropriate to our activity and stress levels.



Illustrations showing the position and movement of the diaphragm and lungs during deep breathing.

In BBTRS sessions, "right kind of breathing" is deep, and "connected," meaning there is no pause between inhalations and exhalations. This kind of breathing stimulates the sympathetic nervous system's fight-or-flight center, and is accomplished through targeted breathing exercises. Breathing in this way "charges" the system and provides access to the physical aspects of the process—where movement, or tremoring, allows the tension to dissipate. Breathing also is the method we use to monitor progress; it's implemented in stages. We charge the system, release tension, slow down, rest, and then enter the next round. Each round allows for a deeper level of release. One session generally invites at least two rounds, and sometimes three.

Deep, connected breathing is done through an open mouth, which requires the use of the diaphragm. Sometimes called "belly breathing," each breath expands the belly, then moves up into the chest. The ribs expand as the chest rises. In exhalations, the chest contracts first, and then the belly.

As this breathing process continues—with long, connected breaths, with a focus on the self—relaxation settles in, and the body becomes oxygenated. With this oxygen comes energy. It might seem counterintuitive that with more energy, the mind actually quiets, but it does. Early in the session, try using this quiet moment for a short meditation. Simply inhale to the count of eight, then exhale to the count of eight.

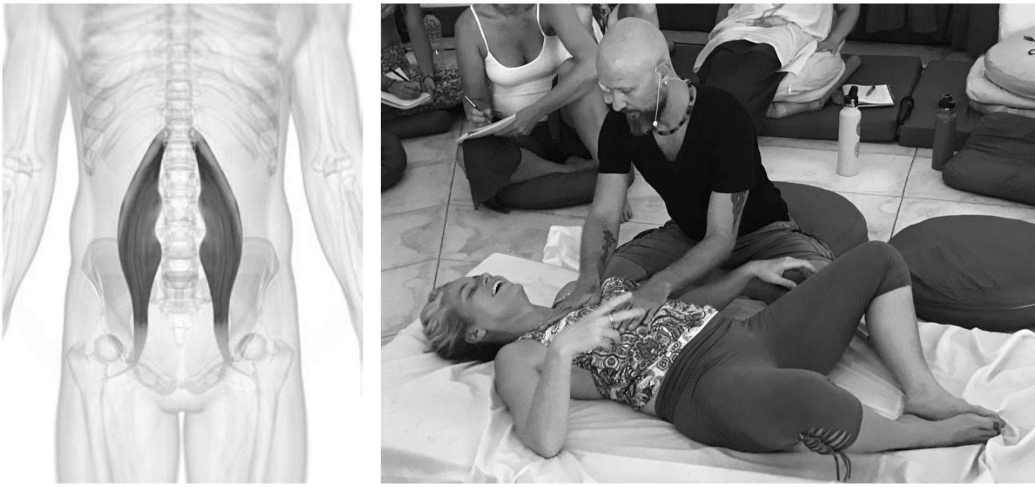
If this technique is practiced in day-to-day life, outside a session—inhale to the count of eight, exhale to the count of eight—breathing alone can have a tremendous impact on the body, the emotions, and the ability to manage stress. In the moment. At any time.

Please note: A BBTRS session does not have to have a "sexual component," but conscious breath extends the pelvic floor and psoas major muscle, and thereby moves the bladder and internal sexual organs. By embracing the fact that the breath awakens the root chakra, we invite whatever sensations come our way. For those who have issues with sexuality, or if sexuality is a source of trauma, then the breath might consciously or unconsciously be inhibited. More about sexuality is discussed in several areas of this text, especially in a section specifically devoted to it, and in sections related to the second chakra and its related "belt of tension."

Movement

Involuntary physical movements are the most direct and fastest route for tension to escape the body. Various somatic approaches were outlined in Chapter 3, some of which focus specifically on stimulating the tremor response. However, tremoring is not the only involuntary movement common to somatic work. Another is best described as "unwinding," in which movement originates in the core of the body and spreads toward the limbs. In some people, the experience and the appearance of this movement is snakelike, or undulating, and it accesses tightness in deep muscular and myofascial tissues. Where tremoring is often fast, unwinding can be slow and measured, sometimes more like a rhythmic dance. Tremoring and unwinding movements can each trigger the other, and both release arrested energy.

When breathwork is not a component of a somatic modality, tremors and unwinding are generally stimulated by activating various muscles in the body, which set off a chain-reaction. To trigger the physical element of trauma release, the breather can begin by engaging the psoas muscle—the primary muscle of the hip flexor, the primary muscle activated in



The psoas muscle—the core muscle involved in the tremoring process—is most easily stimulated to tremor in this reclining posture. Tremoring is not so easy to capture in still photographs, but videos of workshop sessions can be found at the BioDynamic Breathwork & Trauma Release Institute YouTube channel.

walking, and the ultimate "core muscle." Perhaps the single-most important postural and structural muscle in the body, it connects the upper and lower halves. Because of its key location and attachments, the psoas controls large movements of the body; it also lies in the pelvic bowl, on either side of the sacrum, and therefore involves both our center of gravity and the center of energetic circulation.

Although there are various physical positions that can initiate involuntary tremoring, one is often the easiest for people to access. People lie on their backs, knees bent, with soles of the feet touching. The knees are open at a 30-degree angle or so, thus engaging the psoas. By keeping the pelvis "lifted" and the knees open, shaking in the legs should begin in about five to ten minutes.

This position also engages the pelvic floor muscles, which activates the first chakra, our energetic foundation. Movement usually begins in the pelvis; once our foundation starts vibrating, everything above it becomes looser. Sometimes movement is expressed by breathers' legs' moving as if they are running; their arms might make hitting motions. People might demonstrate disorganized movements in the joints; their muscles engage, contract, and mobilize.

Natural unwinding movements can include leg actions as if people are running or kicking. Facilitators can engage a breather's leg musculature during a session, by pushing in sync with these natural motions. In the process, the facilitator supports the breather to complete the fight-or-flight response stuck in the lower part of the body.

In most cases, movement starts slowly and then builds, eventually becoming bigger and bigger, until the entire body is engaged in a very congruent and graceful expression. Waves of tremoring and unwinding of the spine travel through the entire system. With each wave of tremoring, muscles relax and unwind.

If movement becomes "stuck" anywhere in the body, the facilitator likely will be the first one to notice, since that area likely will remain still, or at least less active than other areas. In these cases, various supportive interventions can be employed. As the tissues loosen, a person's movement reflects it. Also, waves of movement generally wax and wane. When they wane, the person can jump-start the tremoring by repositioning the legs in the original stance.

Please note: when "body-only" somatic work is augmented with conscious breath, the movements described here are supported. For example, by beginning a session with connected breath, the body's physical movement usually begins more quickly. Also, the bent-knee position not only engages the psoas, but also the diaphragm. The psoas and diaphragm are physically interrelated in various ways, all of which work together—along with breathing—to access the sympathetic nervous system. As the tremoring and unwinding progress, our breath deepens—opening additional pathways of energy that move through the body. Finally, when the natural "rounds" of movement wax and wane, the most direct and efficient way to stimulate or calm them is through conscious breathing.

Touch

One of the primary reasons to approach trauma release in a group setting, or at least one-on-one with another person, is to have a shared, safe experience. Trauma is troubling enough on its own—and often makes people feel alone with their pain—and so many of the aspects of BBTRS involve witnessing, supporting, and facilitating. Touch is one of the ways in which the healing process is enhanced and advanced through the active participation of someone else.

Conscious touch, bodywork, and soft-tissue manipulation add a dimension to somatic work that allows a practitioner to assist a person in the physical aspects of trauma release. When we apply touch to contracted muscles in the body, we support a flow of energy to areas cut off from their own physical and energetic circulation. The supporting facilitator acts as an external, safe, and knowledgeable resource for a person, ensuring that he or she is not alone. Through loving kindness and a caring touch, the facilitator helps the person to shift attention from one area of the body to another, so energy can dissipate.

Touch might be directed toward pressure points, connective tissue, muscles, joints, or nerves. Contact with these areas can be achieved energetically or physically—and only after asking the person for permission to reach out. There are five stages of touch in BBTRS practice, and they all require that facilitators are in relationship with their intuitions. The stages are:

Verbalization: Always make sure that the person is okay with your touch; simply ask. Even the process of speaking permission can be significant for the recipient in the healing process—especially when we consider that we live in a touch-deprived society. So many people lack the concept of a non-sexual touch that the idea of feeling safe with a massage therapist or BBTRS practitioner could prove very significant.

Intention: Sometimes the combined power of a facilitator's heart and mind is enough to assist in the movement of energy in another person’s body. This is particularly true if a good connection has been established between two people. By deeply holding space, and feeling the other person’s process, a facilitator can assist without any physical movement.

Approach: If more attention is needed, the facilitator can simply move toward the person, extending one hand, but stopping at a distance of 10-12 inches (25-30 centimeters). At this distance, you can feel the subtle energy fields of another person, and a “stuck” area in the other person can gently release.

Pre-touch: As with Reiki, the next stage of energy movement is conducted at a distance where the facilitator can feel the heat of the person’s body, at about 1 inch / 3 centimeters.

Touch: When one makes physical contact, it’s with a gentle, tender touch. The contact directly draws the person’s attention to areas that are stuck, or constricted from tension.

At whatever level, touch works by supporting the body’s movement to become more congruent and fluid. Through a combination of watching the flow of movement in the other person and using intuition, the facilitator knows where and when to touch. It’s also important to know what muscles and connective tissues lay underneath a constricted area, so knowledge of anatomy, physiology, and energy is crucial.

Massage and physical therapists, as well as nurses and others trained in Western medicine have excellent foundations to bring to the physical aspects of BBTRS. It's also helpful if a facilitator knows the foundations of Chinese medicine and Five Element Theory, and what properties, emotions, and elements relate to specific organs. Earth, Water, Fire, Metal, and Wood all support, control, and nourish one another, which makes targeted touch especially therapeutic. One correct contact can affect the entire nervous system.

In general, when using touch on another person, we approach the higher chakras first, and then move from the top down—which makes a pathway for the energy to rise. By the time touch comes into play, the body's movement already is well underway, having started at the root. Unwinding and tremoring originate at the core—usually in the pelvis, belly, or heart—and then flow upward. Once movement reaches the crown of the head, it then reverses direction, transforming as it goes and becoming more and more refined.

Think of initiating the tremor in the knees, thus igniting the root chakra. Once the breather has kindled the fire of the pelvic energy, he or she must allow "oxygen" and space for the fire to burn. The freedom of movement, augmented by touch, if necessary, creates that space.

With all of this in mind, it's easier to understand how touch can be the "grounding wire" between physical tension held in the body and the energetic charge that releases the tension. For example, consider watching a person during a session and noticing that his chest and shoulders are not involved in movement. The rest of his body is moving freely, but the upper torso remains quite still. The facilitator might consider:

- Releasing a joint in the shoulder can trigger movement of the arm.

- Relaxed pectoralis muscles open the heart and allow deeper breath into the chest.
- The heart meridian moves through the arm and hand, into the pinky finger;
- A pressure point for the pericardium, the sack holding the heart, is in the center of the palm.
- The heart is related to the Fire element, which is also associated with spiritual transformation and the crown chakra.

Here, Giten supports a participant to unlock and open the shoulder joint. Muscular tension in shoulder joints is related to an interrupted impulse to reach out for others—for affection and contact.

The facilitator brings forward his own heart energy, and then accesses intuition and experience to determine which of these approaches to begin with. After supporting others in sessions and observing the patterns that come up over time—with the same breathers and across breathers—a facilitator's intuition and resonance grows when it comes to reaching out, whether with intention or physical touch.

When we touch someone with the presence of heart energy, the other person will feel it. We might also consider whether deeper bodywork or myofascial work might be beneficial. Sometimes deep, slow, bodywork supports the breath coming to the places that are restricted. The process is never the same twice, and every decision depends on what a person needs in that moment.

The Power of Touch

"I wasn't supposed to have the first breath session I ever experienced. I was translating for a Russian participant who was taking a specialized massage training in India. The facilitators decided to include a breath session, and I was allowed to participate. I had no idea what I was getting into, or that it would change the trajectory of my life as I knew it.

It was in a sitting position, engaging deep, connected breathing. My facilitator had one hand on my back. I could feel my body charge with energy, but nothing was moving inside. Then, as part of the session, the facilitator pushed two fingers into my solar plexus. This gentle movement opened a floodgate of tears. Not since I was a little child had I cried like this. Something opened I was not aware of, something not seen by me, not acknowledged. It came to the surface all at once. The feeling was healing, but also overwhelming. At the same time, it felt good, relaxing, and as if it were something that needed to happen.

It was also funny. The moment I came out of this session, I had to begin translating again. I couldn't put two words together. It was also difficult, translating and crying at the same time. I would cry on and off for the next two days, feeling so present in my body, with so much compassion for everyone. My translation client was very compassionate; he was in a similar situation. It was a beautiful experience for both of us. We managed, somehow, to finish the course. However, for the next section, I found an alternate translator—and went in as a participant. The rest is history.

Emotional expression

Emotional expression is a primal connection to past trauma, and it is one of the pillars of BBTRS. Emotion is held along with body tension, and both come to the surface during somatic work. However, emotional expression during a BBTRS session must be carefully managed. Somatic trauma release approaches are structured to avoid reactivating people, and therefore they steer away from overstimulation. Just as extensive verbal descriptions of traumatic experiences can retraumatize a breather, so can prolonged, extremely loud, or highly charged emotional expression. If breathers move past organic emotional expression related to physical sensations, or stay engaged in emotional too long, they are prone to "falling into" catharsis.

In BBTRS, we find that a short emotional expression can be very useful, especially for people who have experienced a long period of trauma. Sometimes people need to find their voices and express feelings that have been repressed. Therefore, when emotional expression appears—crying, laughing, yelling in anger—we support people in staying with the feeling for a moment, and then moving to the next step relatively quickly.

This process usually works very efficiently. By honoring what is coming out, and at the same time remaining connected to the breath, people become masters of their expression. This occurs through considering how the experience is reflected in the physical body. For example, if a breather feels great joy during a BBTRS session, and begins laughing, the facilitator might ask, "Where is the joy in your body? What does it feel like?" By linking the emotional expression to the physicality it produces, along with the presence of breath, people become more sensitive. With each breath, sensitivity to the body increases, which allows more tension to release. Please note: release is still occurring with the experience of pleasant sensations. The presence of joy in someone's body encourages the relaxation of tissues, which allows tension to escape—even if it's not the "subject" of the moment.

Conscious emotional expression is demonstrated here, where a breather emotes sound for a brief period during her session. Emotional expression can be expressed in laughing, crying, or organic vocalizations, the latter of which are part of the sound element of BBTRS. Emotional expressions work best when integrated with movement, so that the energy travels through and out of the body. This process helps to unlock dense and contracted tissue.

For overall healing to occur, it is important to allow the energetic flow to happen. The flow might include both pleasant and unpleasant sensations, but all of them simply move through and out. Since we're not staying in the emotional expression for long, then nothing becomes overwhelming. Once we realize that we can express how we feel without the experience's being too painful to endure, then we are much more connected to our entire system. This process aids in the development of our emotional intelligence, because we can process available, free-flowing feelings through the physical body. We can cry, laugh, or become angry without being overtaken.

The emotional expression is simply the energy we generate in motion—we hear it on its way out of our bodies. This way we're "enlarging our container," developing our ability to be with whatever is, without being overwhelmed by it. Please note, however, that if an emotion or its expression "takes over" our awareness, we generally forget to maintain the consistent breathing that is an integral part of BBTRS. This glitch diverts us into the mind, which disengages us from the somatic experience and distracts us. People might hold their breath, or begin breathing shallowly. In these cases, the facilitator can simply call attention to the breath—allowing the expression to reintegrate and the process to continue. On the other hand, if the emotional expression requires more attention, then the breather might "take a break," and resume after a few moments.

Sound

The entire universe vibrates. Each of us is part of a cycle of life that includes everything from the birth or death of a star to the rhythm of a cicada's wings. NOVA, Public Broadcasting Service's science programming resource, explains this complex interrelationship quite simply: "According to string theory, absolutely everything in the universe—all of the particles that make up matter and forces—is comprised of tiny vibrating fundamental strings….The only difference between one string and another…is its resonant pattern, or how it vibrates."[84] String theory says that all objects have resonant patterns, from a desk to a flagpole to the entire Earth.

Various parts of the human body have been measured to operate at specific frequencies—the brain, the heart, the eyes—and recent scientific research shows that the proteins in our bodies are constantly vibrating. "The tiny motions enable proteins to change shape quickly so they can readily bind to other proteins, a process that is necessary for the body to perform critical biological functions like absorbing oxygen, repairing cells, and replicating DNA."[85]

In the midst of all this micro-level vibration, consider the impact of bulky scar tissue that mends torn or surgically cut tissues. Visualize how that scar tissue can cause an adhesion to neighboring tissue, inhibiting that tissue's natural movement, on both the micro and macro levels. Now, consider this scar tissue as a model for how trauma impacts our cells. The National Institute of General Medical Sciences says that when proteins "stress out," they can begin to unravel and stop functioning. "If they unravel too much, they tangle up with each other and form a clump that can kill the cell." In this particular example, parts of the cell stick together, because the environmental temperature is too high.[86]

An exact number of ways in which cells might "stress out" in response to trauma is impossible to guess. Not all trauma causes cells to heat enough for their proteins to unwind, but that single example illustrates how easily—and seriously—the body does respond. Trauma causes an orchestra of tiny cellular reactions to occur within an overall, physical contraction throughout the tissues. Fortunately, sound can help "loosen things up."

Both negative and positive impacts of vibration—coming from the outside, into or onto the body—have been studied. In one case, a particular frequency was found to be the cause of "many forms of industrial illness," because it caused "maximum displacement between the organs and skeletal structure."[87] On the positive side, a study published in the *Proceedings of the National Academy of Sciences* in 2014 shows that sound waves "sort cells without having to expose them to chemicals or damaging forces." This function allows doctors to both separate cancer from non-cancer cells, and also to check if a tumor is likely to spread. "Acoustic pressure is very mild and…a most gentle way to separate cells," says Dr. Ming Dao, a principal research scientist in MIT's Department of Materials Science and Engineering.[88, 89]

Sound is a vibration, a release of energy. Sound affects us deeply—from loud, jarring construction noise to soothing, tranquil birdsong. When sound moves through air, it causes the "molecules to bump into their neighboring molecules, who then bump into their neighbors, and so on." This "progression of collisions" creates a sound wave, but the air does not travel with the wave. "Each air molecule moves away from a rest point and then, eventually, returns to it."[90] We hear a sound when a wave caused by the vibration of an object hits our eardrums. However, the sound obviously doesn't stop at our ears. And when we intentionally apply sound to our environments, we can assist how sound moves through our bodies.

It might be hard to imagine how sound can help on a cellular level, until you "see it in action." For example, put some water in a Tibetan singing bowl or crystal bowl and play it. You will instantly see how the water molecules react to the vibration; they change shape and dance into different geometric patterns. Since our bodies consist of up to 75% water, this is the same response we have to sound. By selecting the nature and frequencies of the sound waves that move through us, we can impact the nature and quality of our unwinding.

The precise use of sounds in bodywork or somatic release is not something that "everyone" agrees on—and very different interpretations of how exactly sound affects the body are found within the metaphysics and science communities. However, BBTRS practitioners have witnessed interesting impacts of external sound on the nervous systems of breathers during sessions. Therefore, those who are especially attuned to sound are encouraged to experiment with different tones and styles of sound—such as recorded music, live instruments, and gongs—to see how sound can benefit or hinder BBTRS sessions.

For example, because they have such a wide harmony and range of sound, gongs usually are seen to impact the entire body, while other musical instruments target different body regions. Bass instruments, such as African drums, the shamanic frame drum, and the didgeridoo open energetic flow, especially in the lower parts of the body around the pelvis and belly. Meanwhile, flutes of various kinds affect the heart area, which supports people to merge into a soft, spacious connection with themselves. Shakers and rattles, which are used in many shamanic traditions to facilitate healing sessions, have a finer vibrational effect, and deeply impact the nervous system. It's advantageous to gently use them at the end of a session over the entire body.

We know that deeply held adhesions are common in most people, and these adhesions might be due to dehydration, circulation problems, tightness, unwinding proteins in a heat-sensitive cell, or trauma of many kinds. We know that somatic shaking and unwinding loosen contracted tissues of the muscles or fascia. With the targeted addition of sound, practitioners report feeling deep, physiological release. If sound physically releases tissues by changing our body's internal vibration, then perhaps it is working at the cellular level. Perhaps the actual spaces between cells shift and widen—allowing fluids to circulate more freely, blood and nutrition to nourish the cells, and waste products to exit—all of which contributes to the homeostasis of our entire organism.

Music and other sounds can be used to evoke emotional states, providing great benefit when setting a particular mood or inviting someone into a desired state. However, since sound has different effects on different people, BBTRS practitioners must get to know individual clients, and apply musical choices with care, whether during the beginning, middle, or completion of sessions.

Finally, an incredibly powerful instrument is our voice. When we open our emotional expression, we put it into sound. However, trauma often constricts the throat, and therefore the voice, thus inhibiting a person's ability to express. Sometimes our expressive limitations originate in childhood; although we begin life as free and unrestricted beings, we might slowly "shut up and sit down" over time. In session, we might shake loose our voices—in song, toning, words, or even gibberish—unsticking something that has been stuck. Keep in mind, though, that in BBTRS, we express with voice for only a short moment, and then come back to breath. This is not a cathartic process; if we get distracted in sound, then the energy of breath diminishes.

Because people are so responsive to sound, you might like to try several BBTRS "soundtracks" in your own practice or as part of sessions you offer to others. These soundtracks were created for a variety of session types, and can be downloaded from Spotify—check playlists under "Giten Tonkov."

Meditation

Meditation allows us to develop our inner witness, our inner watcher. It is used to look objectively at ourselves—our physical sensations, emotional responses, the body as a whole—without being involved. Meditation is just observing; it's not necessarily "being quiet." In fact, some meditations include physical movement, music, or other activities. Anything can be a meditation, such as walking or even doing the dishes.

If your mind is busy, then just observe it, and eventually some of the energy will come to the observer. It's not difficult. If necessary, start by observing the closest thing: your breath. Meditation is not a mystical experience, but it can lead to one. It can lead to something deeper, beyond ourselves. It's a practice, like going to a gym.

When it comes to a BBTRS session, meditation has several purposes. The first occurs during the session itself. Meditation in the midst of breath and movement refers to the process of sensing what is happening as the process unfolds. As we breathe and sense our bodies, we must observe. The same energy that first went into our thoughts, our minds, and our tension now goes into witnessing and observing. As meditation progresses, the watcher gets more energy than the thought process, which is very healing. Meditation is the most effective path to integrate experiences during sessions.

After a session, meditation integrates everything that occurred; it is the element of BBTRS that brings everything together. In this application, meditation is quiet. The deepest insights come to us when the mind is quiet and we connect to our true essence, which is still and silent. Meditation also gives the nervous system a chance to calm down and rest. This is especially necessary after an intense breath session.

Finally, meditation is a tool between BBTRS sessions, and is most effective when it becomes part of a person's life routine. Daily meditation is a big part of healing, because it allows us to look at our healing from a non-attached point of view. By continuing to practice breathwork, sense the physical body, and observe, it becomes easier to shift something that doesn't serve our greatest good.

For those who might have difficulty with stillness, or with observing the self, the mystic Osho Rajneesh introduced the idea of active meditation. He said that the today's world is much more complex than the world was when meditation was developed. People were not as busy then, and the influx of information we receive now is overwhelming. Osho said that to successfully quiet ourselves, we might first need to put the physical body into movement; we express, so that we can sit silently and be present to our state. He prepared the body for silence by first executing several active steps, which are outlined in the Osho Dynamic Meditation: chaotic breath; active vocalization or movement; activating the root chakra by jumping and shouting the sound "hoo" (also hü); freezing in place; and free-flowing dance. After all this movement, profound stillness flows.

All six of these elements—Breath, Movement, Touch, Emotional Expression, Sound and Meditation—combine to create a unique

Active meditation uses movement, breath, and sound to support the mind's natural silence to arise. Active meditation can take many forms, and is used in concert with silence to prepare a person for the day, or for a BBTRS session.

experience that has proven consistently beneficial to people holding tension in their tissues. From veterans suffering from PTSD to "the average Joe," people from around the world have experienced relief—as well as enhanced connection to their physical and emotional bodies. They have gained both relief and the ability to process experiences in the moment with more peace and less residue. They report improved day-to-day experiences, more resilience, and more joy.

A TASTE OF BIODYNAMIC BREATHWORK

Active breath meditation, 15 to 20 minutes.

Start by sitting or lying comfortably and bringing attention to your breath. You can choose the level of breath, either (1) through the nose, (2) in through the nose and out though the mouth, or (3) if you're comfortable with it, use connected breath—in and out though the mouth, without pausing after each inhalation and exhalation—for a deeper result. Consider how the breath enters, and what parts of the body expand with it. Chest? Belly? Pelvis? Just notice how the body moves and expands with each breath.

Now, when inhaling, imagine that a rubber ball has been placed inside your belly. Notice how the ball expands with every breath. Notice where the breath naturally flows. On an exhalation, notice how the body comes back to its original shape. The ball expands with inhalation and comes back to original size with the exhalation.

Next, shift this ball down, as if it were placed on the bottom of the pelvic floor. Actually feel the pelvic floor as it expands toward your feet. Notice all physical sensations that are present. Notice the physical presence of the movement of the breath. Notice how the pelvic floor moves back up to its original shape when you exhale.

Take a few breaths this way, and notice if anything starts to change. You don't have to count, but take deep, full breaths in and out.

After two to three minutes of breathing this way, then shift the attention—and the ball—up a bit, into the entire abdomen and lower back. To feel that, bring your hands to the lower back area, and allow yourself to experience how this area expands with the breath. Stay aware of all the physical sensations present in your body, all the physicality. This is how we begin to understand the somatic experience, the "felt sense."

After two to three more minutes, shift the ball to the lower diaphragm. Bring hands to the lower ribs, and feel how they flare out with the breath. Again, this ball is present, in the center of the body, and you fully expand your body with the breath. Pay attention to the natural physical movement that begins to arise in your body—the natural movement of the spine, pelvis, and neck. Slowly expand on this movement and allow it to spread throughout your body.

After two to three minutes, shift a little higher, to the chest area, the heart area. Notice how the chest rises and falls with the breath, and how the breath expands from the inside out. Imagine that you're filling a glass with water, from the bottom up. Fill the lungs and body with a breath. When exhaling, empty the breath, and the glass—but without forcing or pushing anything. With relaxation, the diaphragm allows the breath to come out on its own.

As you continue to breathe, ensure that you allow no pauses between inhalation and exhalation.

Now, while the chest is expanding, notice that the area around and behind the heart begins to soften. Its armor, its protection begins to soften. The awareness of the chest comes from the inside out.

Next, move the awareness to the throat area. Ensure that your throat and neck expand with each breath, and that each breath flows freely and clearly. It is common for blocks in expression and creativity to restrict breath, so strive to experience no blocks on inhalation or exhalation.

Notice how this area softens with breath, and how any blocks clear.

After two to three minutes, bring attention up to your head. Imagine the ball expanding in your entire skull. If your eyes are closed, then keep your gaze directed into the third eye. Notice how the area between your eyebrows expands with the breath; in this way, you stimulate the main breathing organ, the brain. Breathing in this way stimulates the entire brain. In this meditation, we are bringing the attention to how the breath goes through the entire body.

Finally, go down the same way you came up: throat, chest, diaphragm / rib cage, belly, pelvis—about thirty seconds in each area. After coming back down, notice all sensations. Very likely, your breath and heart rate have come down. Notice your thoughts. Notice your state, without changing anything. Notice your general feeling in your body—and if there's any movement that wants to happen naturally for you, then just follow it. Let your body move however it wants to. If you feel any tremoring or unwinding movements happening, then simply allow them to manifest…until all movement slows and eventually stops on its own.

Keep observing all the physical sensations that are present right now. If emotional responses come up, try not to push them away; simply continue breathing and shift your attention on how these emotional responses are manifested in your physicality. For example: do you feel vibrations or tightness, sensations of energy moving, temperature changes, or body movements?

When your experience naturally comes to a close, thank yourself for allowing this space to express yourself.

Reflection

Thank you for reading this chapter. Now, please take a moment to bring your attention to yourself. Please read the following paragraphs, and then close your eyes and scan your physical body. Notice how what you read reflects in your body. What is happening in your body—is it relaxed? Perhaps start with the breath, and consider whether it is flowing freely. Then consider your posture, whether you are sitting or lying down. Do you feel balanced, or does your physical body have tightness, pain, or discomfort? Look at different aspects of your body, including inner and outer tension. Take a deep, full breath, and sigh out your mouth. Next, consider your thoughts—are they generally centered outside of you? If something external is consuming your attention, can you let it go for now?

Now, consider if the material in this chapter resonates in you, and whether the resonance involves you or someone you know. If there is some triggering—positive, negative, emotional, physical, spiritual, or some other experience—then what does it feel like? Maybe it feels like you're reading something intimate or personal or true about yourself—or maybe you didn't like something you read. What was it? Was the response because the information was revealing, challenging, nurturing, or something else? It's possible that something that's gone unnoticed within you, or something you've neglected or forgotten about, might really be valid

and making itself known. It’s possible that this material might have awakened it in you. Or not. You be the judge.

After reading these things, and reflecting on them with your eyes closed, then open your eyes and bring your attention to your surroundings. Listen to the sounds. What do you see? Between chapters, perhaps take a brief walk, or dance to a couple of songs. Move around, reorient, and then settle in for the next chapter. Thank you.

PART 2

OUR INTERNAL WORLD

We are not human beings having a spiritual experience. We are spiritual beings having a human experience. *~Pierre Teilhard de Chardin*

Chapter 5
Xin

For young people the world over, it might sound simplistic, or slightly passé, to reminisce about the "old days"—or wax poetic about how chaotic society has become. How areas of conflict have escalated. How the dialog about race still translates into violence and jail time and slurs. How mass shootings and terror networks have filtered into daily life, and not only in far-removed places with exotic names. How lifestyle choices, or gender, or age still impact a person's ability to earn enough income to support a family. How city dwellers who speak every language we know are more watchful, and sometimes less trusting. How abuse is a story behind closed doors on every continent, and addictions hobble us. How politics has split otherwise reasonable people into factions that don't even recognize one another.

In past studies of intense trauma, researchers and laypeople alike have had no difficulty in recognizing resulting wounds in survivors, whether the wounds are physical, emotional, or psychic. Our history of war, including the list of past ethnic cleansing "campaigns,"[91] reveals hundreds of times when humans have turned on one another, and the horror that has resulted. And let's not forget the natural disasters, or routine accidents and random traumatic events that fall into categories of forces we cannot control.

With all of this, humans have no shortage of study subjects, no lack of trauma symptoms. We also have limitless compassion, curiosity, love, and healing power—or we would not have made it this far. Thanks to the sacrifices of countless individuals who have suffered, and the hard work of those who want to help them, science has demonstrated not only that tension can be stored in our bodies, but also how it gets there. In recent years, mainstream thinkers have come to identify successful, verifiable methods to access trauma-related tension and to physically release it from tissues.

In creating BBTRS, then, we gather the interventions that work best into a multi-modal, holistic approach. But the work doesn't stop there. Successfully applying the program's six elements requires an understanding of a variety of related topics, techniques, and subtleties. It asks of its practitioners and explorers a softening of agenda, an opening of the heart and mind, and a willingness to dive deep.

Power of the mind

Imagine sitting on a bench in a park—just taking in a pleasant day, feeling the breeze on your skin. Maybe you're eating a sandwich. Listening to some kids playing nearby. Then a person—just an average person in work clothes, like you—sits next to you. Maybe you share a glance as you move your jacket to make more room. All is well, until…you start to feel it. Anxiety, maybe a sense of nervousness starts creeping across the bench, like a little parade of ants. It marches up your arm, crawls to the back of your neck. Your food starts to taste like dust and you can't concentrate. Your stomach goes into a knot. You realize that the person next to you is clenching his teeth; his leg is vibrating like a jackhammer. He is fiddling with the seam of his cuff. You realize that your stomach is in a knot because of the energetic vibrations from this other person. We can sense these vibrations, knowingly or unknowingly, all the time, from individuals, groups, and even entire communities.

That ability to sense what's happening in the environment is a function of our nervous system, but outside of a fight-or-flight response, what we do with the information happens in the mind. What do you do, if you find yourself feeling unwanted tension? If you're aware of what's happening, then you can send those ants packing. A few healing thoughts, a decision to walk away, or perhaps a simple question, "Hey, are you okay?"—any of those can change not only your body's response, but also how you feel. It might also help your neighbor.

Mental shifting—call it thought intervention, or intention—works when we're alone, sitting next to someone on a bench, or standing in a group of people. It works when managing anger flare-ups or other emotional swings. For those who live with mental health challenges, learning to intentionally shift our thoughts can become an invaluable tool—even if it seems that we "can't help" how we feel. Both practical experience and research show that what we think and believe can absolutely affect how we feel—not to mention what we do and how we live—but there are many perspectives on the process.

Of course, in the study of trauma, we've talked most about the fight-or-flight activation center, the "autopilot" part that stimulates our brain's emergency responses. Anecdotally, people report that these autopilot responses happen so quickly that they sometimes feel more organic, even more real than any conscious thought could. We almost "trust" our autopilot brain more than the interpretation that follows.

However, our autonomic "reflexes" might sometimes get us into trouble, and they might fire off a warning when we're not in danger. Now we know that with BBTRS we can step in to manage the fight-or-flight activation process and its aftermath. But there's another layer to how the mind can impact us every day, all day—beyond our fight-or-flight. We might find it by looking in a mirror.

Unless we're practiced at keeping a "poker face"—as are actual poker players, or people in law enforcement, psychology, politics, and, unfortunately, crime—what's happening in our minds is often obvious to others, even if not to ourselves. We might "wear our thoughts" in plain view. For example, if we think of ourselves as victims, then our bodies often assume a posture of protection, as if we're looking for the next danger. Fear looks like collapse, and it takes our whole energetic field with it. Conversely, if we feel vital, powerful, and creative, then we tend to stand tall, shoulders back—revealing our vital organs. Strength looks like invincibility and aliveness.

When we notice that our shoulders are slumped, or our jaws are clenched, we might stand up straight and make space between our teeth. And then, perhaps in the next breath, we notice we're right back to those habits again. It's as if we're just built that way, that we're "snapping back" into position. Is thinking about managing a traumatic experience easier than managing our posture? How does it compare with reconsidering and reformatting how we think about ourselves—as people?

We have a built-in system in our brains that can help with the magical dance between the nervous system and conscious thought. It developed as humans started to live within larger social groups—to accommodate the changes that "society" brought. "This thinking system helps us handle more complex mental activities, such as managing individual and group relationships, logical reasoning, probabilistic thinking, and learning new information and patterns of thinking and behavior."[92]

This model is different from our auto-pilot processes because it requires effort. Still, the more we use it, the easier it gets. "Fortunately, with enough motivation and appropriate training, the intentional system can turn on in situations where the autopilot system is prone to make errors, especially costly ones."[93]

AUTOPILOT SYSTEM	INTENTIONAL SYSTEM
• Fast, intuitive, emotional self • Requires no effort • Automatic thinking, feeling, and behavior habits • Mostly makes good decisions, 80% of the time • Prone to some predictable and systematic errors	• Conscious, reasoning, mindful self • Takes intentional effort to turn on • Drains mental energy • Used mainly when we learn new information, and when we use reason and logic • Can be trained to turn on when it detects Autopilot System might be making an error

It's literally impossible for us to think intentionally about every single thing we do each day. Many of our actions are influenced by the nervous system, intuition, and habit—and some chunk of all of those is influenced by emotion. However, "over time, you can use the intentional system to change your automatic thinking, feeling, and behavioral patterns. In this way, you will become a better agent in terms of taking charge of your life and reaching your goals."[94]

This idea of "changing your automatic thinking" is something BBTRS helps to develop. Specifically, releasing previous trauma from the tissues is only one benefit of the modality; others are our increased resilience, ability to process as circumstances in our environments change, and ability to "let things go" in the moment. The better we process what's happening in the moment, the less of it we carry with us. This requires the brain. It works hand-in-hand with intuition and perception, because the more sensitive we are, the more likely we are to feel what's happening in our environment, in people around us, and in ourselves. The more sensitive we are, the more likely we are to be able to work with modalities like BBTRS, and truly "get inside ourselves."

The fact that the mind is a powerful tool is no secret, and many people, from business experts to philosophers, have discussed how to integrate strategic, positive, inspirational thinking into our day-to-day lives and our long-term visioning. Conversely, negative thinking can drive us into complete disaster, full of anxiety, worry, self-induced stress, physical illness, lack, and just plain misery. Our minds can support us to reach the highest heights, or the darkest depths—and they can be a primary tool in deciding to change our habits, common thinking patterns, and negative behaviors.

Even more, our minds can shift the entire paradigm of how we operate—from any given day, onward.

The mind is a key that can open us to another dimension. How to access that key, or "train the mind" might seem counterintuitive. The first step is to realize that there is something larger than the science explaining which part of the brain manages which reactions, thoughts, or feelings. There is something beyond being a mammal that responds to danger with hormone and blood changes. As human beings, we have awareness.

Humans have the ability to "stand back" and observe ourselves, witness ourselves, and enter something beyond ourselves. The vehicle into this space is meditation. When we enter deep meditation, there's a part of us that's not involved. The more we practice, the more the mind becomes a useful servant instead of a master. Our minds might continue to process, whether we sleep or talk or dance or meditate—but this does not mean that we cannot learn to witness the process. Pure conscious awareness eventually can be achieved; with practice, we can enter a state of no-mind, a connection with higher consciousness. By using our minds, we can transcend our minds, becoming aware of breath, mind, body, self—and eventually, more.

Power of emotions

Although somatic work focuses on the physical, and we know our brain is a crucial tool in our ability to manage our lives, we cannot diminish the importance of how we feel. Our emotions play an enormous role in our health, our relationships, our spirituality—and our ability to release trauma. Just reflect on almost any crisis you might have experienced or witnessed. Did you or someone else involved cry spontaneously? Did the tears help to dissipate energy? Probably so—and it's not the only emotion that does. Laughter changes our physiology by releasing feel-good chemicals throughout our bodies. It stretches muscles throughout our face and body, raises our pulse and blood pressure, and causes our respiration to increase, sending more oxygen to our tissues.[95] Either of these emotional responses minimizes the impact of tension. Even so, the emotional "story" goes much deeper, and it does not function on its own.

Emotions and the Brain

Research on emotion is filled with dozens of approaches to how our brains process our experiences. Some studies have focused on whether different mental states, such as fear, love, or even concentration, might "light up" different parts of the brain. Other research has contrasted how emotion and cognition work in the brain. Three alternative, classic views have suggested either that (1) emotion occurs independent of thought, (2) emotion precedes thought, or (3) emotion happens automatically, and helps us to "evaluate" what to do in an unexpected circumstance.[96]

Emotions seem to have a life of their own because they feel lightning-fast—and because they have been tracked along a direct path, or "low route" to the amygdala, the same part of the brain that houses fight-or-flight responses. But recent research shows that our emotional responses to our environment are complex. Yes, they surely have a neural handshake with the amygdala, but they also travel down "large-scale, distributed brain networks" across various parts of the brain.[97] Our emotions actually work together with our thoughts, not before or independent of them.[98, 99] Some studies also refute the idea that emotion occurs automatically, showing instead that emotions "may actually have required attention after all."[100] At the most general level, scientists now say that emotion "modulates and mediates basic cognitive processes"—and that how we

feel translates to what we see in the world. For example, when we're sad, a hill looks steeper than it looks when we're happy; also, when we're sad, the hill looks steeper than its actual incline measurement indicates.[101]

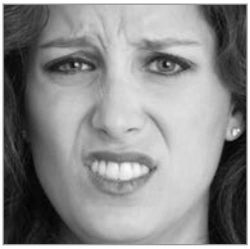
Research sifting through these details assists us in understanding how our minds and feelings work together, especially when we are under stress or exhibiting the effects of trauma. According to research coming from MIT, "some mental illnesses…including depression, addiction, anxiety, and post-traumatic stress disorder…may stem, in part, from the brain's inability to correctly assign emotional associations to events. For example, people who are depressed often do not feel happy even when experiencing something that they naturally enjoy."[102] Therefore, understanding the actual "state of mind" of trauma clients becomes a key part of trauma release. We process our world differently when we're happy; we reach for more, make more connections, and feel more effective. When stress has us by the throat, not only might things "look bleak," but they also might not "stack up" as usual.

Emotional intelligence

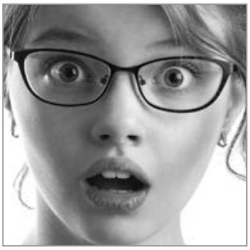
The study of our ability to process and understand emotion is not new. Even the evolutionist Charles Darwin published a book in 1872, *The Expression of the Emotions in Man and Animals*. In it, he explained his premise that both people and some animals experience a few "universal" emotions, and that emotion has "an evolutionary history that could be traced across cultures and species."[103] Today, whatever their cross-species ideas, many psychologists agree that humans—regardless of culture and language—recognize select universal emotions on the faces of others. Since the 1800s, researchers have considered as many as sixty different emotional expressions, but leaders in the field now focus on six: anger, fear, surprise, disgust, happiness, and sadness.[104]

Just as understanding how our thoughts and emotions might get derailed during or after traumatic experiences, the ability to recognize emotion is another tool in our work. At a foundational level, researchers learn from those who might not properly recognize emotion, such as people living with autism or schizophrenia. Other studies measure skin temperature and heart activity in those who "make faces" for scientific purposes. They begin neutrally, without feeling emotions corresponding with the expression. As they hold the expression, these people experience physiological changes that correlate to the emotions. "For example, performing a facial expression related to anger would lead to an increase in the temperature of the extremities (such as the fingers). Ultimately, the researchers were able to distinguish the physiological responses related to expressions…showing that we not only react to the emotions displayed by others, but also the emotions displayed by ourselves."[105]

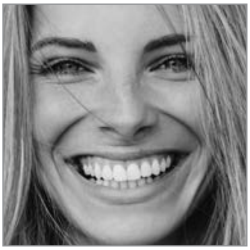
Researchers also learned that these universal facial responses occur involuntarily for a split second, before we consciously "rearrange" them. Those trained in reading "microexpressions" can glimpse evidence of how someone is feeling, despite the person's wish to hide it. Leaders in this field include Paul Ekman, who essentially defined facial expression analysis, and his colleagues Robert Levenson, Wallace Friesen, and Carl-Herman Hjortsjö. Their Facial Action Coding System (FACS) documents which muscle contractions and head or eye movements are autonomically triggered during specific emotions.



Microexpression research shows that when we're disgusted, the nose wrinkles and the upper lip raises—different from anger's low brows, glaring eyes, and narrow lips.



Of the six universal emotions, surprise causes raised eyebrows, wide eyes, and an open mouth. In fear, there is more tension around the eyes, and open lips stretch horizontally.



When truly happy, our crow's feet lines wrinkle and the muscle around the eye orbit moves. The cheeks push upward. Sadness is opposite, with drooping eyes and mouth.

For helpers in many fields, microexpressions tell us *that* someone is feeling a particular emotion, but not necessarily *why*, so it's important not to make conclusions. For example, some experts in law enforcement might be able to detect that a person feels contempt for someone, or is being untruthful, but not necessarily that the person is guilty of a particular crime. That is, the subject might feel conflicted emotions about the death of a person with whom there was a bad relationship, but this response does not indicate that the subject was involved in the death.

Breaking the recognition of emotion into physiological components has allowed us to understand how our feelings and our bodies interact, but it's just the beginning of a comprehensive understanding of the importance of emotion. Assisting people with trauma release also requires a core understanding of the depth and breadth of how people process experience, how we describe feelings, and how we relate to one another. Recognizing the influence of emotions means being aware that feelings impact and are impacted by our experiences, thoughts, behaviors, and interactions with others. It means realizing that what we do impacts other people, as well as ourselves. Our ability to understand and manage this balance—especially when we're under pressure—is called "emotional intelligence (EI)."

EI was coined in 1990 by professors Peter Salovey and John D. Mayer. "There's an inconsistency in how people use the term," Salovey says. "It's often linked to anything that someone thinks is important, for example in the workplace or in schools, that isn't measured by traditional IQ or ability measures. We much prefer using EI to describe a set of skills having to do with perceiving, understanding, managing, and using emotions."[106] While temperament has a strong genetic component in how we interact with the world, the skills involved in EI are both teachable and learnable. They include understanding your own emotions and those of others, being able to describe and reflect emotions using language, and regulating emotions. Salovey says that when people learn these skills, they "can perform more adaptively and more functionally" in school, in the workplace, and in family life.[107]

Salovey explains that his work grew from asking why humans have an emotional system, and how this system helps us. In the safety of the laboratory, his team asked subjects to arouse specific emotions so that researchers could observe how people in different emotional states behave. "What do they pay attention to, what can they remember, how do they reason, how do they make decisions, how do they interact with other people, are they more or less willing to be helpful, how do they make friends, form relationships—everything having to do with the impact of emotions on other psychological processes," Salovey says. "Our goal was really to show that although emotions occasionally make other processes worse, often they can improve decision making, reasoning, creativity, help us with memory, and help us pay attention to what's important."[108]

Emotional expectations

“Emotions can be tricky to work with. We “think we know how we feel,” or we “want to feel” a certain way. Maybe we think we “feel too much” or not enough. And when we enter into a process like BBTRS, we also might arrive with assumptions about what being “open” looks like. Sometimes people are afraid to be too open, and therefore they don’t allow emotion, which adds to the tightness they’re working to release. Sometimes people over-emote, tumbling into the pit of catharsis—and right out of their bodies. As facilitators, we are careful to support “helpful” emotional expression, and to show the way out of less helpful processes. For example, deep emotional catharsis can become disconnected venting, expressing the ideas of the mind. Not only does this distract our physical processes, but also it can retraumatize. Trained facilitators are vigilant, and waste no time in bringing breathers back into more supportive emotional expression.

Also, we try to educate breathers about what to expect. Or, more accurately, what not to expect. BBTRS, like meditation, works best when we remain in the moment and stay present in the body. By leaving our preconceived ideas at the door, we gain spontaneity and creativity—and these are some of the strongest tools we have when it comes to release. This is especially true when dealing with developmental traumas.

I once worked with a German bodyworker named Hans. He was a young guy, very good-natured and open. I would describe him as ready to connect with people, eager to interact. He also shared that he was missing a deeper connection with people. His social skills were great, but they were superficial, and he craved deeper connections. He asked how to drop what we called a "veil of separation," which he thought was probably a protective mechanism. We tried an easy exercise that Hans had done before. It had worked well for him, and had caused him to feel deeply, so he was eager to try again.

The exercise is intended to open the heart. Hans began by raising his hands out to me, and then opening the arms wide. The intention is to open the arms slowly, allowing the heart to naturally feel safe in opening. However, Hans opened his arms quickly, expecting an emotional response—as he had before. Nothing happened. Or, more specifically, he "felt open," but that was it. He was disappointed.

I stopped him, and we talked about how he had structured his session according to his expectation—both from previous experience and his desired outcome. I asked him to close his eyes and repeat the exercise, this time very slowly. I guided him through this experience, supporting him to use his senses—to feel what was happening within his body on that day, in that moment. Again he put his arms out, breathing, and this time he connected to his emotional

charge. Then he opened arms very slowly, and a dam of tears and vulnerability broke through. This was different from the last time, and he didn't expect it. He was surprised, but instead of thinking about that, he simply continued to connect to his body in that moment. This turned out to be a most beautiful, breakthrough session for Hans. For the first time in his life, he felt vulnerable in front of someone else without protecting his heart. I was sitting in front of him, also deeply moved. I began to cry. It was such a touching space, and I allowed it to touch my heart—without losing myself in his experience. I simply witnessed him, and allowed his experience to move me.

Hans had accomplished so much that day. First, he had expressed the ingrained desire of his heart to connect. Second, he didn't become disheartened when his first expression "didn't work." Third, he then allowed himself to drop his expectations and try again, with a clean slate. Because of this, he allowed himself to experience something brand new. Fourth, Hans had volunteered for a demonstration session in a group, and this experience occurred among 40 to 50 people, who all were watching in pin-drop silence. He allowed everyone else to hold space. He received their support and presence.

After the session, he allowed himself to look around and make eye contact with all those people. Hans not only had opened himself like never before, but also he

had allowed himself to be seen by a roomful of people. He knew that if he chose to, he could remain with his "arms open," no longer carrying an armor plate around his heart. Everyone's eyes were full of tears. Each person recognized his level of need. Everyone knew what it was like to wish for deeper connection—and also to lean toward protecting ourselves from pain. It took only a few moments in Hans's life for it to change forever.

Using emotion in BBTRS

We know that emotions are deeply connected to the physical form and are reflected in the body. We know that people have different levels of EI, and fall somewhere on the spectrum of skills in identifying and working with their emotions. When it comes to BBTRS, we use this information to assist us in facilitating trauma-release sessions. Consider the different ways in which you might approach people who are in different emotional states. For example, the emotionally alive person is more relaxed, more receptive. Think of a child, who often expresses more freely than adults, and moves quickly from one feeling or situation to another. Likewise, we can identify repressed emotion through one glance—in physical tension, in resistance to the natural flow of events. Such tension might be distributed through the body in dozens of readily observable ways.

It's important to note that if a person "freely expresses emotion," this does not necessarily correlate to an absence of trauma. It just means that both the breather and facilitator can approach the session by taking into account the breather's access to emotion. Similarly, "repression of emotion" does not necessarily mean that a person is hiding something very volatile, or is "on the edge." Repression is a natural outcome of social conditioning, because average people have been trained not to express themselves in certain ways in "polite society." They might be repressing feelings related to love or fear, but anger and rage are more common examples. These "negative" emotions often arouse fear or distrust in others, and so are more likely to be repressed. And repression causes us to tighten up, literally—which causes us to breathe more shallowly.

In working with thousands of breathers, we see that healthy breathing habits are essential for a healthy body and healthy emotional expression. We see people who want to be emotionally alive beings, who don't want to feel disconnected from their internal lives and from the environment. When we live with tension and restrict our breath, we cannot feel touch, we stop tasting food, we don't experience orgasm when making love—we don't enjoy the things we once enjoyed. We stop enjoying life.

Therefore, consciously using the breath can influence a person's emotional state. Although scientists have documented the correlation in many applications, we also intuitively "know" that a person's emotional response and corresponding level of tension are directly tied to how that person breathes. The more relaxed we are, the more freely we breathe. The more freely we breathe, the more relaxed we become. When we encounter a difficult situation, or are just simply tense, we breathe shallowly. Sometimes even hold our breath. In emergencies, we often breathe quickly. We might even "overbreathe," or hyperventilate, causing the body to lose carbon dioxide. Hyperventilation triggers gasping, choking, and the feeling of being smothered—which makes people even more emotionally upset. In BBTRS, we have found that the connection between emotion and the breath is a crucial element in successful trauma

A breather early in a session, being supported by a facilitator. Deep, connected breathing literally infuses the physical body with energy, which can be seen by the facilitator as movement develops.

release. It is a two-way street, with emotion influencing both the breath and tension, and successful tension release influencing both emotion and the breath.

Breathwork paves the way to improved quality of life. By helping to remove what's holding us hostage, it naturally lifts our spirits. By connecting so intimately to our emotions, breath provides the direct path to making us feel better.

The breath does more than connect us to our emotions. It also connects us to the divine. In Latin, both "inspire" and "expire" originate from the Latin *spirare*, or "to breathe." *Spriare* is related to *spiritus*, the root of "spirit," which means "breath." The word "inspire" was originally associated with a supernatural being's "breathing into" or imparting a truth into a human being, while "expire" means the end of something, including a life. Of course, inspiration and expiration can be considered as synonyms to inhalation and exhalation, whose root is *halare*, also "to breathe." It's possible that the semantics of the time differentiated between *spirare* and *halare*, with the latter referring only to the physical act of breathing. Still, "breathing in"—with air or ideas—invites spirit into our bodies. It is our primary connection to prana, or Qi or life force—and therefore our primary connection to our emotional center. In BBTRS, we think of the breath as our moment-to-moment connection to everything that matters.

Trauma & spirituality

After working with thousands of breathers, across the entire spectrum of traumatic activation, we see many similarities between traumatic and spiritual experiences. Both can create deep, authentic experiences that might feel separate from "normal life." During traumatic activation, people might feel distanced from everything outside their bubbles. They might seem to be standing still while the outside world speeds up or slows down. Depending on fight-or-flight wiring, they might have run toward danger or become frozen, and might or might not be aware of tension remaining in the body. In any case, like meditation or a spiritual experience, trauma can bring us into a space of "no-mind."

For many people, trauma provides their first contact with spirit. Although packaged with unpleasant and unwelcome aspects, a traumatic experience might shock people out of their routine thought processes. Whether in the immediate moment or long after an event, the energy of trauma can jump-start a mystical journey of personal exploration. Therefore, trauma can become a stepping-stone for those who want to heal, release thought and behavior patterns that don't work anymore, step out of the role of the victim, and expand their spiritual experience.

It might seem odd to say, but trauma release requires a courageous willingness to step out of our comfort zones. "Comfort zones" don't always have to be positive. Anything can become familiar, including sensations, reactions, behaviors, and decisions that hurt us. When the nervous system stays in reaction, it changes our digestion, ways of thinking, and ways of relating. Eventually, our "new normal" produces a systemic reaction that becomes familiar. The first step toward healing is to realize that "familiar" doesn't necessarily mean "healthy." And making healthy choices, whether physiological, emotional, cognitive, or spiritual, requires consciousness about our status quo.

In BBTRS, we look at ourselves objectively in order to create space for deeper relaxation. With this relaxation, the tension departs, and we are left in a profound state. This is why our sessions might invite mystical experiences. These experiences reach much deeper than emotional responses; they lead to complete silence and relaxation, a meditative state, and a communion with God or the universe or whatever resonates.

Sometimes, people have become so used to living with trauma that in its absence they have to reassess who they are, on both human and spiritual levels. At this stage, it's important to distinguish between authentic spiritual experience and what psychologist John Welwood coined in 1984 as "spiritual bypassing." Although many believe that every human has the capacity, and perhaps the innate drive, to connect with something greater than him- or herself, not everyone does it. And sometimes people divert their attention from their own authentic experiences and focus on the divine. A spiritual journey can be a great distraction that psychotherapist and author Robert Augustus Masters calls "avoidance in holy drag" and a "persistent shadow of spirituality."

Masters describes spiritual bypassing as a relatively common "use of spiritual practices and beliefs to avoid dealing with our painful feelings, unresolved wounds, and developmental needs…. Aspects of spiritual bypassing include exaggerated detachment, emotional numbing and repression, overemphasis on the positive, anger-phobia, blind or overly tolerant compassion, weak or too porous boundaries, lopsided development (cognitive intelligence often being far ahead of emotional and moral intelligence), debilitating judgment about one's negativity or shadow elements, devaluation of the personal relative to the spiritual, and delusions of having arrived at a higher level of being."[109]

Spiritual bypassing can come in many forms and wear many faces. For example, moving toward religion or a comforting belief system can be an excellent, enriching addition to life. However, if used as a diversion from the pain of past trauma, this practice becomes a pale stand-in for its true function and can prolong the healing process. Similarly, otherwise helpful social circles and support groups for people with similar experiences—such as Anonymous groups, rape and incest survivor groups, and veterans groups, among many others—can be unconsciously misused. Even yoga has been appropriated into a popular cycle of "spirituality." A yoga class twice per week can do wonders to tone the physical body, but the *Asanas*, or postures, are only one of the eight "Limbs of Yoga." True yoga practice is a true spiritual practice, but it includes: *Yamas*, which are moral disciplines or standards; *Niyama*, spiritual observances and self-discipline; *Pranayama*, breathing techniques and control; *Pratyahara*, sensory withdrawal and transcendence; *Dharana*, focused concentration; *Dhyana*, meditation; and *Samadhi*, or ecstasy/enlightenment.

Work on the self is an individual process. While yoga class, a support group, or a religious affiliation can provide temporary comfort, it will not take care of deep trauma release. It might even provide an unintentional, comfortable space to live with the experience we're trying to heal. People like to share experiences. They like to feel seen and supported, but letting go of trauma can lead to true transformation. The true religious experience we're looking for arises as a byproduct of working on the self.

BBTRS is meant to root organically in each individual's own experience. Like a true spiritual path, it requires dropping the ego. As Masters says:

> "True spirituality is not a high, not a rush, not an altered state…. Our times call for something far more real, grounded, and responsible; something radically alive and naturally integral; something that shakes us to our very core until we stop treating spiritual deepening as something to dabble in here and there. Authentic spirituality is not some little flicker or buzz of knowingness, not a psychedelic blast-through or a mellow hanging-out on some exalted plane of consciousness, not a bubble of immunity, but a vast fire of liberation, an exquisitely fitting crucible and sanctuary, providing both heat and light for what must be done."[110]

Finding spirituality in the body

Not everyone wants to follow a spiritual path—and trauma release can occur in the absence of this goal. Relief is available for anyone who simply wants to leave pain behind. However, for those who are interested in a spiritual path, trauma release can help clear the way. When we work on our bodies, by getting rid of trauma-related tension and behavior, then we open the door to becoming better, more conscious human beings.

How is this possible?

For many people, the experience of spirituality begins in the mind. We believe something, or think something. We have been told something, and what we see in our environment supports that reality. Sometimes we hear of a spiritual awakening that occurs in connection with a near-death experience—where a person's entire reality is turned on its head. Everything looks different because of a door that opened, a vision that was seen, or some other experience that connected the person to another dimension of reality.

What is always available and accessible to all of us is the experience of being physical. When we are able to sense something in our bodies, then we know it. When we personally experience a transformation from traumatized being to freer, lighter, more energized being, then we know it. When we realize that we have the ability to allow this transformation, then we cease to be objects and become subjects—authors of our experience. We realize that we feel better. We feel more integrated with life, the planet, and other people. We impact the world differently. And importantly, by releasing our most traumatic experiences, we give support for others to do the same.

To be able to feel, give, and share, a person must be able to receive. While trauma release is possible with a tentative "yes," real healing requires full self-love and acceptance. It requires "YES!" And to receive, we open the body first. We relax, deeply, all the way to the core. With this relaxation, the first thing to flow in is breath. With breath comes spirit. And with spirit comes the full potential of being.

Xin & Kokoro

Scientist, writer, and mediation teacher Jon Kabat-Zinn has been credited with "being responsible for bringing secular mindfulness into the world over thirty years ago."[111] In an effort to explain how we attend and relate to our environments, he refers to a "non-conceptual knowing, which we call awareness. It's before thinking comes in." He says that Buddhists speak of this awareness as the sixth sense, along with sight, hearing, taste, touch, and smell. Mindfulness comes into play by creating a bit of "extra space" between awareness—what we're experiencing—and all of the judgments or thoughts we might have about the experience. Importantly, mindfulness is not just a mental exercise. In Kabat-Zinn's exposure to

Asian cultures and languages, he learned that the words for “mind” and “heart” are the same. “When you hear the word ‘mindfulness,’” he says, “you have to hear the word ‘heartfulness,’ or you’ll misunderstand it as simply one more cognitive exercise, and it’s not.”[112]

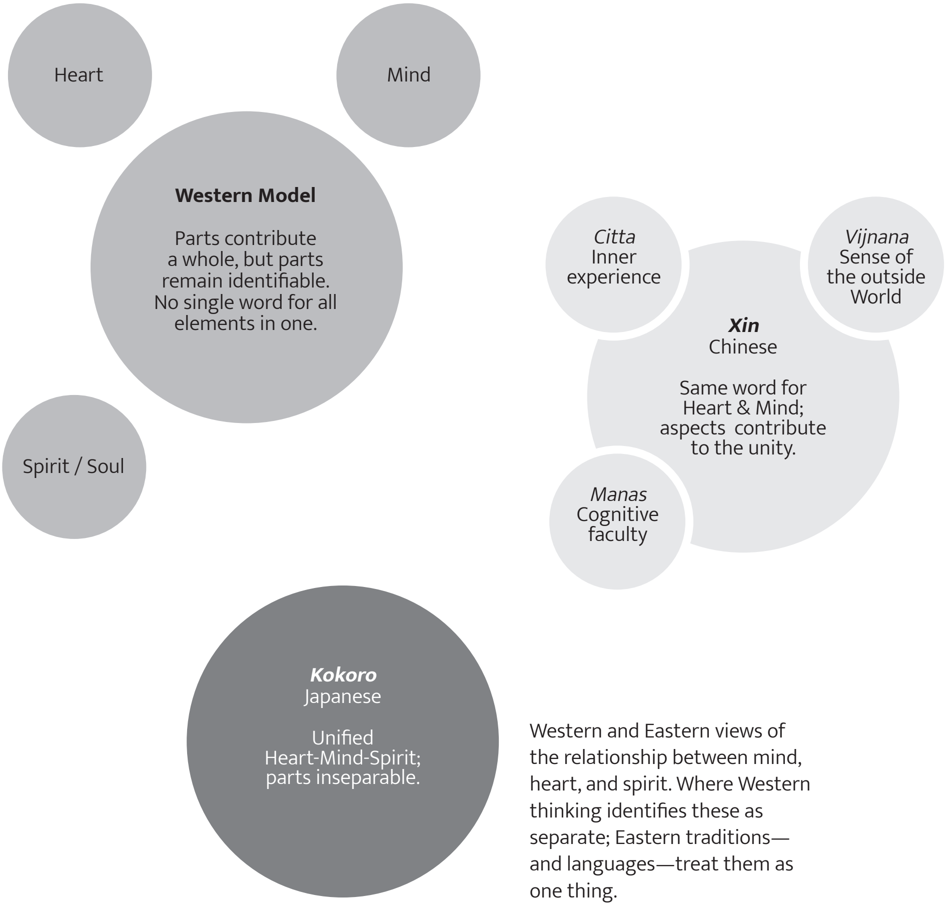
In Chinese, the word is *xin*. In a modern Chinese-English dictionary, *xin* is defined as “heart; mind; feeling; intention; centre.”[113] Soto Zen priest Shohaku Okumura expands on the subtleties of *xin* by considering Sanskrit’s three words for “mind”—*citta, vijnana,* and *manas.*[114] When taken together, the three terms can be seen to encompass what Westerners might consider as our essence—those parts of us that experience both the internal and external. Specifically, *citta* can be associated with our subjective, inner experiences; *vijnana* embraces our sensory experience of the outside world; and *manas* relates more to cognitive faculty, and probably is the closest to the English interpretation of “mind.”[115]

Likewise, the Japanese word *kokoro* also assumes this oneness in how we process and perceive our environment. Okumura quotes a Japanese dictionary of classic words, which reads: "Originally, *kokoro* referred to the beat of the heart, which was considered to be the essential organ of life and the source of all activities. By extension, *kokoro* refers to all human activities affecting the outside world through intention, emotion, and intellect."[116]

He goes on to write: “*Kokoro*, then, has three basic meanings: the heart and its functions; mind and its functions; and center, or essence.… It is very interesting to see that Sanskrit and English have completely different words to point to heart or mind, and have no word that combines both meanings.”[117] In Japanese, the word *kokoro* does not link heart, mind, and spirit—because that idea assumes these are three separate things. Instead, *kokoro* is all of these at once. Heart, mind, and spirit are one thing.

In BBTRS, each person who crosses our path, or sits on the mat with us, will have an independent view of how our consciousness works. Each one will think differently, feel uniquely, reveal microexpressions and emotional intelligence, and relate to spirit in his or her own way. As breathers or facilitators, we can consider, evaluate, and reflect our concern and good intentions with each interaction. But what is the point? What are we after?

Perhaps we are after connection.


Heart
Mind
Western Model
Parts contribute a whole, but parts remain identifiable. No single word for all elements in one.
Spirit / Soul
Citta
Inner experience
Vijnana
Sense of the outside World
Xin
Chinese
Same word for Heart & Mind; aspects contribute to the unity.
Manas
Cognitive faculty
Kokoro
Japanese
Unified Heart-Mind-Spirit; parts inseparable.

Western and Eastern views of the relationship between mind, heart, and spirit. Where Western thinking identifies these as separate; Eastern traditions—and languages—treat them as one thing.

Kokoro and I

“Honestly I never thought that BBTRS would be my life’s work. I did it because it felt good to me to do. Workshops and seminars were places of learning, vulnerability, meditation, and connection. As I kept coming back to this kind of environment, and with my background in massage, ideas of the workings of a new modality started to emerge. These ideas were natural expressions of my personal self-awareness, my connection to body, mind, and something beyond body and mind. I found that my ability to look at things objectively started to expand. I was able to better look objectively at myself, at my thought process, and to be in the space of no mind. I became more comfortable in the space of just being. When I’m in the space of no mind, I am opened up to a particular kind of knowing, which is beyond the mind. This knowledge feels rooted in the laws of existence, of the universe. I don’t know how else to say it.

These experiences changed the way I perceived things—from reality, to how I related socially, to the way I relate to myself. I hold space for others by first feeling safe and relaxed within my own body. I want and expect nothing of the other person, which usually allows him or her to relax. I approach each individual as if he or she is complete, with nothing needing to be changed. Everything that needs to be accessed is already there, and trauma can be healed and shifted.

> This relationship with myself and with others stems from this kokoro—a mind-beyond-the-mind experience. Whatever is happening with me is manifesting out of the bigger, universal mind that we're all a part of. This has caused me to feel more connected to others, and to trust more deeply. In my experience, once you trust, you drop your defense mechanisms. I see this between participants and me, and also among participants.
>
> It really feels like I am in the right place, living my life's purpose. I am here not because I chose this job, but because it chose me. I trusted my gut feeling. I feel grateful, aligned with what is destined for me. I'm doing what I should be doing, and nothing other. To be of service gives me pleasure. Serving is a humbling experience. I feel humble in the trust that people put in me—and I trust in their highest potential. I trust they can find peace and happiness beyond the trauma and their particular life experiences. I put my trust into that. It feels right.

Shunryu Suzuki Roshi, in *Zen Mind, Beginner's Mind*, wrote about the "universal mind, always the same, not different from another's mind. It is Zen mind. It is big, big mind. This mind is whatever you see—this mind is at the same time everything."[118] Okumura replaces this use of the word "mind" with *xin*, or even "life." He also relates the Zen mind to something Dogen Zenji quoted in *Shobogoenzo Sokushinzebutsu* (*The Mind Is Itself Buddha*): "The mind that has been correctly transmitted is the one mind…and all dharmas are one mind. The mind is mountains, rivers, and the great earth; the sun, the moon, and stars."[119]

As seekers of a trauma-free life, we are seekers of greater health. We are seekers of joy. We do not assume that life will unfold without difficulty and trauma, but we do assume that joy is possible. We assume that health is possible. We choose to release negative emotions, thoughts, and experiences. We choose to shed the skin of agony and to be reborn.

The very process of this action contributes to big, big mind—big, big *xin*. It contributes to life on earth. It contributes to life in this universe. One person at a time.

Reflection

Thank you for reading this chapter. Now, please take a moment to bring your attention to yourself. Please read the following paragraphs, and then close your eyes and scan your physical body. Notice how what you read reflects in your body. What is happening in your body—is it relaxed? Perhaps start with the breath, and consider whether it is flowing freely. Then consider your posture, whether you are sitting or lying down. Do you feel balanced, or does your physical body have tightness, pain, or discomfort? Look at different aspects of your body, including inner and outer tension. Take a deep, full breath, and sigh out your mouth. Next, consider your thoughts—are they generally centered outside of you? If something external is consuming your attention, can you let it go for now?

Now, consider if the material in this chapter resonates in you, and whether the resonance involves you or someone you know. If there is some triggering—positive, negative, emotional, physical, spiritual, or some other experience—then what does it feel like? Maybe it feels like you're reading something intimate or personal or true about yourself—or maybe you didn't like something you read. What was it? Was the response because the information was revealing, challenging, nurturing, or something else? It's possible that something that's gone unnoticed within you, or something you've neglected or forgotten about, might really be valid and making itself known. It's possible that this material might have awakened it in you. Or not. You be the judge.

After reading these things, and reflecting on them with your eyes closed, then open your eyes and bring your attention to your surroundings. Listen to the sounds. What do you see? Between chapters, perhaps take a brief walk, or dance to a couple of songs. Move around, reorient, and then settle in for the next chapter. Thank you.

Chapter 6

Passing trauma to loved ones

Alim was born in 1947, during the Israeli war of independence. His parents had survived concentration camps during the Holocaust. Like many others who lived through that particular horror, they didn't discuss their pain. They couldn't discuss it. Still, their past was evident in the family's day-to-day life, because both of Alim's parents were angry, afraid of their anger, and abusive. They didn't know how to manage these feelings, but they were reminded of them every day. Every day, they walked through their world with extra difficulty, because their Holocaust experiences had resulted in the loss of their eyesight.

Understandably, the environment in the family home was toxic. As the only child, Alim was the primary recipient of his parent's outbursts. He also bore the responsibility of managing the accommodations for his parents' lack of sight. He did all of this by swallowing any emotion he ever had. By the time he had his first BBTRS session, he had lived with their pain buried deep inside for more than fifty years. His body was like a rock. Underlying decades of routine life events was a layer of hate and frustration. Since Alim had never experienced true joy, his laughter was hollow. His parents had successfully injected him with agony that filled every molecule in his body.

During BBTRS sessions, we had to move very slowly and gently. Even mild stimulation caused Alim to go into reaction. Every emotion or moment of vulnerability was read by his body as danger, and his fight-or-flight kicked into gear. With no experience in how to understand or manage his feelings, Alim's emotional intelligence was very low. When triggered, he simply didn't know what to do with himself.

Facilitators saw so much vulnerability and softness in Alim. He was just a little boy underneath; he wanted to cry, but could not. With practice, however, he learned about body awareness, and believed that people cared for him. Until he successfully worked with BBTRS, he didn't know that love could come from the outside, and he surely had never felt it from the inside.

BBTRS worked for Alim. What's interesting is how his new understandings changed his views about himself, his parents, his culture, how people treat one another, the future, and the past. His family was just one small constellation of Jews who had lived with a constant lack of safety. They were just three people among millions living under pressure of armed conflict. Even as Alim began to set himself free, he knew that he was part of the solution. If he could not just survive, but thrive, then he could help change the tide that had been roiling for thousands of years. He saw his parents. He saw the concentration camps. He saw Russian and Ukrainian pogroms before Hitler's time. He saw Romans destroying the temple in Jerusalem before that. All of this history was brought into Alim's newfound state—and still, he found hope. Within one week, Alim's body unwound. He softened. He stood taller. He smiled with calm, sure energy. He found relief. And with his relief came the possibility of healing, for himself and others.

In several sections of this book, scientific research is used to underscore or illuminate topics. While it's not always necessary, or even possible, to "prove" a concept or a train of thought, science is very useful in strengthening understanding. However, science is not the only place we find answers. When it comes to whether and how we pass trauma to loved ones, both science and common sense have a vote.

It doesn't take a rocket scientist, or a doctorate in psychology, to know that our environment impacts how we grow. "Everyone knows" the nature/nurture concept for children, but it's also clear that circumstances in our adult lives influence our mental, psychological, emotional, and spiritual worlds. We're experiencing and learning all the time. We're living and working with other people. This means that we're illustrating on a daily basis how we think, feel, and believe. Those closest to us know what we fear, love, take pleasure in, and avoid. They also know what triggers us.

So, do we "pass on trauma"? Of course we do. We talk about it. We react to it. If we can articulate it, we tell our loved ones what has hurt us—and if we can't say it in words, then we act it out. We definitely say what we believe about people, places, situations, prior actions, and belief systems. We share our fear of elevators or spiders. We rant about politics and religion. We model our most intimate and personal strengths and weaknesses every day. Of course our children know how we feel—and often they feel that way, too, even before they come to any personal conclusions on their own.

But how much of our experience actually changes us on a cellular level, in our biology? Some answers to this challenging question might never be known, but some are being hinted at in our genetic code. Epigenetics is the study of changes in our DNA, particularly how circumstances in our environment might cause a gene to turn on or off, become damaged, undergo a chemical reaction, or interact differently with neurotransmitters, hormones, or other functional elements.

Let's look at a few examples where environmental circumstances changed DNA in people who experienced particular situations. The first involves telomeres, the caps at the end of each strand of DNA. Like the plastic tips at the end of shoelaces, they protect our chromosomes. Our DNA can become damaged if the caps lose integrity. One significant epigenetics study showed that in families where children were exposed to "family disruption and violence," telomere length was significantly shorter.[120] Another study shows that certain childhood environments can cause improper regulation of a person's inflammatory response in adulthood.[121] Inflammation, which is controlled by inflammatory response genes[122], is studied because it is key in the development of potentially fatal illnesses, including diabetes, cardiovascular disease, and dementia. Likewise, stress hormones, especially cortisol, allow scientists to see if a person's fight-or-flight system is over- or under-firing. A cortisol-based study grew from a population of women who were pregnant on 9/11, and were either at or near the World Trade Center during the attack. Of this group, the women who later developed PTSD had significantly lower levels of cortisol in their saliva than did those without PTSD.[123] Cortisol affects metabolism, blood sugar, inflammation, learning and memory, blood pressure, immune function, weight regulation, cholesterol, heart disease, and bone density, among many other functions. Too much or too little cortisol can cause a number of negative symptoms and serious illnesses.

These studies show that what we experience physiologically impacts our bodies. The next epigenetic question is whether and how changes to our DNA can be passed down—and if any documented correlations between parents and children are actually genetic. For example, in the World Trade Center example, all of the babies of the pregnant women were tested about a year after the women were tested. The children born to the PTSD/low-cortisol mothers also had lower-than-normal cortisol levels and exhibited higher distress reactions when presented with new stimuli.[124]

The same researcher, Rachel Yehuda, also studied Holocaust survivors and their children. In one study, she found that Holocaust-surviving parents with PTSD produced offspring with low cortisol levels, even if the children did not have PTSD.[125] In another study, she researched the presence of a common chemical reaction called "methylation." When a methyl group is added to a molecule, scientists can see extra carbon and hydrogen atoms, which alerts them to the fact that "something happened." However, "what happened" might not be so easy to determine. In Yehuda's Holocaust research, she found methylation variations in the same site on the same gene in both parents and adult children. Since these results "could not be attributed to the offspring's own trauma exposure, their own psychopathology, or other examined characteristics that might independently affect methylation of this gene," Yehuda described the research as "the first demonstration of an association of preconception parental trauma with epigenetic alterations that is evident in both exposed parent and offspring, providing potential insight into how severe psychophysiological trauma can have intergenerational effects."[126]

Basically, Yehuda says this is the first proof of genetic transfer of trauma, although she says we cannot infer the mechanism of the transfer. Other experts say, rather reluctantly, that her conclusion is not necessarily correct. The Center for Epigenetics at the Albert Einstein College of Medicine in New York City published a critique of her study, writing that there are many other reasons methylation might occur, and that the study does not rule out or even explore them. From this perspective, "the story is typical of many in the field of epigenetics, with conclusions drawn based on uninterpretable studies."[127]

A broad, unrelated review of epigenetics research states that the evidence for epigenetic inheritance in humans is weak. "In plants, evidence for heritable epigenetic variation is more than half a century old…. In animals, by contrast, there are relatively few examples of heritable epigenetic variation at individual genes…but many examples of quantitative epigenetic traits appear to respond to environmental, and especially nutritional cues experienced by former generations."[128]

Does it matter? Is it important whether our genes are switching on and off or adding molecules, and then impacting how our children's genes are behaving? Or is it really most important to recognize that regardless of the physiological details, trauma is generational?

Seven generations

The Russian word for family is *semya*, which is the combination of "seven," or *syem*, and "I," pronounced "ya." *Semya* means that the next seven generations are all packaged into our concept of the individual. Many indigenous cultures share this understanding, not only recognizing that the next seven generations are part of each one of us, but also that each decision we make as a people impacts the future. Everything we do changes the future for seven generations. When our descendants are considered in our day-to-day decisions, we behave differently, whether personally or governmentally. We behave collectively.

Paying it forward

“The idea of seven generations might have another interpretation, too. For example, sometimes a thought or feeling comes up in my session, and I have no idea what it’s about. I know that it doesn’t belong to me; some sensations and feelings simply are not mine. Am I carrying it from a past life? Maybe, but my view is that bits and pieces are carried from generation to generation. Often, these bits are unresolved pain, pain that was never addressed. In today’s world, many of us have pretty good lives. If we think back just two generations in the western world, even our parents and grandparents experienced war, hunger, and abuse. And just as these generations made decisions to make it better for their children and grandchildren, it’s our obligation to heal trauma for future generations. It’s how we change the world. It’s how we create better, more evolved people.

Just as it’s a challenge to scientifically prove how epigenetic changes are truly written in our DNA, it’s almost an act of faith to know that our actions today will impact the planet in a grand way. We simply must believe that relations among people will change for the better when any person has released the tension held in his or her tissues.

Consider the thousands of locations that have been exposed to war over human history. The wartime itself is bad enough—filled with constant fear, oppression, injury, and worse. After the war is “over,” however, the environment doesn’t suddenly shift to unicorns and popsicles. Usually, violence and crime continue, and the stress that everyone hoped would end remains. For example, after the Soviet Union fell, hell broke loose. Years' worth of pent-up anger, frustration, and repression came out in the expressions of average people. This is normal, because people have no way to express this level of pain in day-to-day life.

This is also true where war is not a constant, looming reality—in just the average city or town, under the normal stressors of the world. In these places, people work to survive, to manage their lives, and hopefully to protect and raise children. Even in these places, we usually hold in our pain. We push back our anxieties, fears, and worries, trying to focus on work and dinner and getting kids off to school. Even when we're not actively engaging in our trauma, we still inadvertently, unintentionally pass it on to our children.

With trauma release, we can truly accomplish what we are trying to accomplish through repression. When we "ignore," push away, or numb our feelings, we almost will the feelings to disappear. Of course they don't, but for a moment, we feel rid of them. On the other hand, when trauma is actually cleaned out of our bodies, it really does leave us. We really do feel different. In the process, we also lift the cloud from the shoulders of our children. And, miraculously, when we release our own tension, somehow the worries of our parents are reduced. Just try it and see. When a son or grandson, daughter or granddaughter, successfully shakes free of past agony, something shifts in the elders. Our positive efforts actually impact the past. This means that we are able to cease carrying anger, resentment, injury, illness, and fear for ourselves and our ancestors, and we remove from our children the burden of carrying our troubles into the future.

Is this really possible? Yes. Just in case we need a little science to help bolster us, the groundwork has been laid in traditional psychotherapy.

Family Systems Therapy is a form of psychotherapy in which individual work is done in the context of the family unit. The approach grew from the same idea that we see in trauma: that we are part of a system; that what we do impacts others, and what they do impacts us. Psychiatrist Murray Bowen developed this system in the 1950s while working at the National Institute of Mental Health. He believed that "personalities, emotions, and behaviors of grown individuals are a result of their birth order, their role within their family of origin, and the coping mechanisms they have developed for dealing with emotional family issues…. One of the most important premises of family systems therapy is that what happens to one member of a family happens to everyone in the family."[129] Every family creates a unique family dynamic, and it's easy to see how each parent is part of two family systems—the one in which he or she was raised, and the one in which he or she parents children.

We've talked about the energy of trauma release, and the energetic interaction between the breather and facilitator. Likewise, energy is naturally part of the process when people share experience and history. One modality that blends energetics with traditional concepts in psychotherapy is called Family Constellations, developed by Bert Hellinger. This work is inspired by the groundbreaking ideas of several pioneers in related fields, including: family therapist Virginia Satir; the founder of psychodrama, Jacob Moreno; Iván Böszörményi-Nagy, who developed transgenerational systemic thinking; and Eric Berne, who conceived of "life scripts."

When psychotherapist Alfred Adler coined the term Family Constellations, he referred to the foundational idea that we are bonded in relationship to members of our family. Hellinger took this premise of a constellation into a different, and sometimes controversial arena. As a former Catholic priest, Hellinger had experienced a prolonged immersion as a Zulu missionary. After leaving the priesthood, and studying gestalt therapy, he integrated several Zulu concepts of family into his work—including a deep and continual connection to deceased ancestors in present-day life. In his Family Constellation sessions, however, the participants are not a family group. Instead, one participant comes forward with a personal issue, and he or she selects other participants to stand in as "representatives" of important family members whose participation can assist with the issue. The crucial family members might be dead or alive, and the representatives tune into their corresponding roles within an energetic field of understanding. The representatives become a model of the subject's original family system, even in the absence of detailed discussion about the family's dynamic.

At its best, Hellinger's work helps individuals and even businesses in working through the energetics of relationship, helping people to let go of unconscious burdens of other constellation members. However, the process has been criticized by those who do not believe that energetic work can impact us on a biological level—and/or who do not believe that people are universally connected. In addition, criticisms have been published about how facilitators are only minimally trained in the method, and how the practice reflects some of Hellinger's old-school social views—especially on violence and traditional roles of women.[130,131]

This brings us to the reality of working in a modality that crosses the line into the unknown. In every field where pioneers have exercised their hearts and minds, we find their vulnerabilities, preconceived notions, prejudices, and errors. We find remnants of their traumas. Likewise, our ideas and modalities are organic. As a new idea develops from concept into successful, strong practice, it might have shared wall space with dozens of less developed, and even unworthy ideas. As the modality is then practiced by human beings, both the modality and the humans remain organic and imperfect. Even as the modality helps people, it can always improve.

Agony connects us. We know this even if we can't prove it absolutely through scientific or other external, linear mechanisms. To those of us who practice trauma release, it is obvious. Letting go of the agony and its resulting trauma can also connect us. The chain that is our family goes on and on, progressing through time until we are the top of our family tree. It's up to us to take care of the seven generations that come after us. It's our responsibility.

Constellations

"As with everything, our interpretation and understanding changes with personal experience. Although I might have believed that all people who heal themselves will impact people around them—and not just those in their gene pools—one profound experience made me know for sure. In one short Family Constellations session, my entire family experienced a mysterious, energetic shift.

I was born in 1971, less than thirty years after WWII, into a Jewish family in Odessa, Ukraine. It was a stressful, repressive time in the Soviet Union. My grandfather on my father's side was an aeronautical engineer, and always worried about being arrested by the KGB as intelligentsia. But the trauma our family carried came from my mother's side, and it was related to the Holocaust. When her brother was ten, he had been taken away and was never seen again. This was a loss that was deeply felt by my mother and my grandparents. It was a subject we all knew and lived with long after the war ended. I still remember my mother's father listening to a program on Soviet radio. People who had been separated during the war were still looking for each other, and their names were broadcast. So-and-so parent was looking for a son or daughter, named so-and-so, from this or that town. My grandfather was glued to this show, on the dot, every time it was on. The receiver was in the kitchen, and had only one channel. He would sit on a chair at the kitchen table, staring at the little black box. He did this religiously until his death, when I was five.

I didn't think about my long-lost uncle every day as I grew older, and of course I never knew him. Still, in my physical and energetic system, his loss remained. I connected this loss to the unnerving fact that I never felt comfortable around German people. Intellectually, I knew that Germans had never done anything bad to me. Strangely, I also knew that the sensations I felt did not belong to me. Somehow, the sensations belonged to my grandfather.

What I felt was a man's pain. It was a man's pain that results when he loses his son.

I became attracted to participating in a Family Constellation session because of this inherited situation. When I volunteered to be the subject, I was asked by the facilitator to give a brief family history. The way I said it was, "My grandfather lost his son." I then chose representatives in the group to stand in for the various members of my story, using intuition to choose each person. The entire group engaged in the dynamic with great energy and integrity. Leon, the man who represented my grandfather, really embodied him.

During the session, I sensed a lifting of the burden. This thing, which had never belonged to me, but which I dutifully shouldered, was gone. Afterward, in a powerful closure, Leon told me something about his own life. At the time of my uncle's capture, Leon's father was a high-ranking SS officer—and he was stationed where my grandparents, my uncle, and my mother had been living. Leon told me that he grew up feeling completely out of his element in his family, and had continuously rebelled against his racist, Nazi father. He and I sat together and cried. As we talked, I realized that I had known Leon was German, but this fact hadn't registered consciously when I chose him to represent my grandfather in the session. I had intuitively chosen this rebellious son to bravely stand in for my grandfather, at the same time representing the crimes his own father might have committed. As Leon shared his own emotion, he realized that his father was responsible for the pain in his family. It's possible that this particular father was also responsible for the pain in ***my*** family, but we will never know for sure.

After this, my whole attitude toward Germans changed. Completely. Thanks to Leon's bearing witness, I now sense no difference between Germans and any other people. I have been to Germany, and have seen the scar the whole country carries for the Holocaust. I believe that the trauma from that experience will remain in a web of humanity across the world for many generations to come, unless we learn collective ways to release it. Until then, it's up to each one of us—anyone related to victims and perpetrators, anyone who is connected.

I never saw Leon again, but I carry our shared healing as my current state of being. Together, we shifted a small part of the universal energy. Later, more and more profound experiences added to my understanding of how this energy works. In my gut I became positive that if we take care of our own traumas, then we won't pass them on as harmful conditioning. It's like cleaning out the basement, the "root chakra" of your house. The more we toss what doesn't serve us, the more freely energy flows through our lives, and through the lives of us all.

Keeping family trauma free

How do we do it? Aside from releasing our own trauma, and living with good intention and action in the present day, how do we keep our family free of trauma? The basic rule of thumb is to remember that love is the answer. Think about it: when our needs are not met, pain results. Pain has to be healed. However, if our needs are met—and we are supported during uncomfortable, scary, injurious moments—then trauma simply doesn't have to result. If it doesn't settle in, then it doesn't require healing. Children in healthy families reach to their parents for support when difficulty presents itself. Adults in healthy relationships reach to their family and friends for support. Governments in need reach to one another for support. When the human impulse to reach out is responded to with love, then trauma is avoided. This is true for physical, psychological, social, and developmental trauma.

On a day-to-day basis, try these approaches:

- Bring laughter into the family.
- Recognize your children's natural talents and support their development.
- Educate yourself in sensing your own bodily responses and know what to do when stressful moments occur. Help your children to do the same. For example, if a child has been scared and expresses emotion, or begins to shake, let him. Don't say, "Man up, don't cry." A situation remains traumatic only if a person's responses are incomplete. Trauma doesn't have to get stuck in the body.
- Use exercise and playful movement meditation to support children to know themselves.
- Talk to children, and touch them lovingly.

- Share the power of community; children need to know other loving adults besides their parents.
- Model social support by participating in your community. Take your children along to assist those in need. Serve soup at homeless shelters. Do *something*. Show your children that each gesture can change a life.

Reflection

Thank you for reading this chapter. Now, please take a moment to bring your attention to yourself. Please read the following paragraphs, and then close your eyes and scan your physical body. Notice how what you read reflects in your body. What is happening in your body—is it relaxed? Perhaps start with the breath, and consider whether it is flowing freely. Then consider your posture, whether you are sitting or lying down. Do you feel balanced, or does your physical body have tightness, pain, or discomfort? Look at different aspects of your body, including inner and outer tension. Take a deep, full breath, and sigh out your mouth. Next, consider your thoughts—are they generally centered outside of you? If something external is consuming your attention, can you let it go for now?

Now, consider if the material in this chapter resonates in you, and whether the resonance involves you or someone you know. If there is some triggering—positive, negative, emotional, physical, spiritual, or some other experience—

then what does it feel like? Maybe it feels like you're reading something intimate or personal or true about yourself—or maybe you didn't like something you read. What was it? Was the response because the information was revealing, challenging, nurturing, or something else? It's possible that something that's gone unnoticed within you, or something you've neglected or forgotten about, might really be valid and making itself known. It's possible that this material might have awakened it in you. Or not. You be the judge.

After reading these things, and reflecting on them with your eyes closed, then open your eyes and bring your attention to your surroundings. Listen to the sounds. What do you see? Between chapters, perhaps take a brief walk, or dance to a couple of songs. Move around, reorient, and then settle in for the next chapter. Thank you.

Chapter 7
Engaging the body

So far, we have discussed how trauma works in general—by overwhelming the normal coping mechanisms of the body—and that it involves our minds, emotions, spirituality, and the people around us. We know that a variety of circumstances can interrupt our instincts, thus allowing trauma to have lasting, adverse effects on our functioning and well-being. We also know that releasing the trauma is good. It's good for us, and it's good for our families. A lot of us think it's good for the universe.

That sounds like a lot.

It might even sound like it's overwhelming to figure out—and that while being stressed out is "obvious," the stress itself is elusive, nebulous, and hard to "catch." It feels bad, but we're not so sure we can truly do anything about it.

In this chapter, we'll get specific about how to sense what's happening in the body. Once the body's signals are more evident, we can do something about releasing stress and tension. We'll start the process of understanding the body's signals by focusing on two elements: tuning in to feeling what's happening, and understanding how some very key areas in the body work.

Felt sense 1.0

Exteroception is the ability to sense stimuli from the outside. We use our five senses to take in information from the world around us—and that information can be distracting. Even overwhelming. It's easy to imagine that most of what we sense comes from exteroception. It seems logical to say that most of what we're doing involves what's outside us.

However, Peter Levine says that the "most basic experience of being alive" comes from within, through the physical sense of the self. Interoception is that sense of self—from the inside. Our inner experience coexists with what's happening outside our bodies. The more we understand it, the more balanced our life becomes. It's important to say from the outset that our focus here is on the physical. We've already discussed our minds and emotions, and these also require interoception. However, to grow in our ability to move tension from our systems, we must be able to sense our physical bodies.

If we think about the wide variation of stressors and the ways in which they affect us, it might seem impossible to "feel the difference" in sensations in different areas, or locate stress in our bodies with accuracy. With practice, however, we can. This is true whether the stress or trauma originates from your own experience or comes from your reaction to other people's stories; it's true whether the trauma is physical or emotional, and whether it happened in an instant or over a lifetime. Our most primary tool is interoception, which in BBTRS and other somatic modalities is called "felt sense."

We all know what the felt sense is. Some describe it as an inner knowing, or a body sense. The sense might have an emotional aspect, but it's more than emotion. It might be a fuzzy, vague feeling of pleasure or a sense of unease. It might be a heavy feeling that you describe as "not feeling yourself." It might be expansive warmth that accompanies a beautiful morning or a rainstorm.

How Levine explains the senses

Exteroception (Orientation)

By using our senses, we receive direct information from the external environment. Exteroception uses:

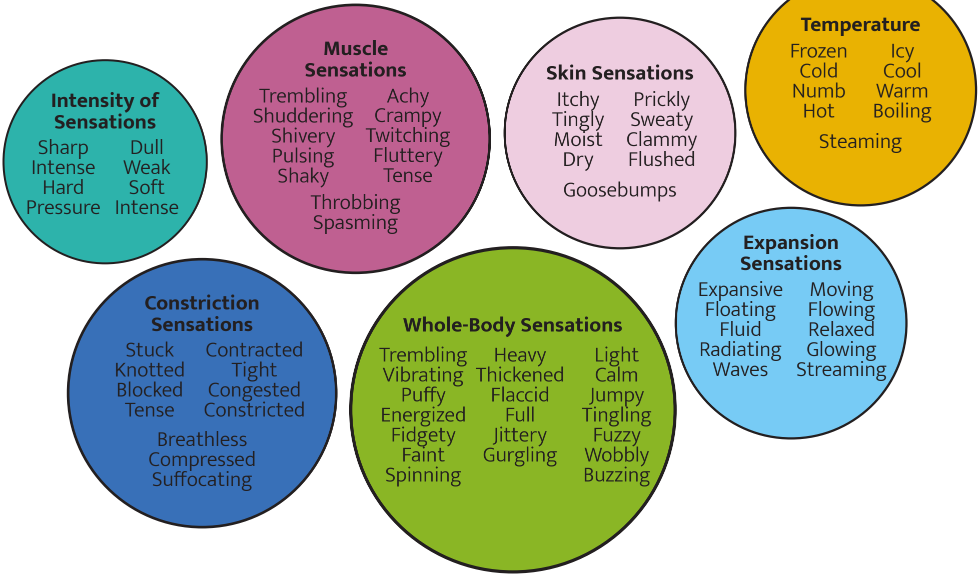
- Sight
- Hearing
- Smell
- Taste
- Touch

Interoception (Felt Sense)

The felt sense is an inner awareness of the internal states of the body. We use interoception to detect and inform us of internal regulation responses, such as:

- Respiration
- Heart rate
- Body temperature
- Balance
- Hunger
- Thirst
- Need for digestive elimination
- Emotions
- Pleasure/pain

Felt sense: the language of sensation



Intensity of Sensations

Sharp
Intense
Hard
Pressure
Dull
Weak
Soft
Intense

Muscle Sensations

Trembling
Shuddering
Shivery
Pulsing
Shaky
Achy
Crampy
Twitching
Fluttery
Tense
Throbbing
Spasming

Skin Sensations

Itchy
Tingly
Moist
Dry
Prickly
Sweaty
Clammy
Flushed
Goosebumps

Temperature

Frozen
Cold
Numb
Hot
Icy
Cool
Warm
Boiling
Steaming

Constriction Sensations

Stuck
Knotted
Blocked
Tense
Contracted
Tight
Congested
Constricted
Breathless
Compressed
Suffocating

Whole-Body Sensations

Trembling
Vibrating
Puffy
Energized
Fidgety
Faint
Spinning
Heavy
Thickened
Flaccid
Full
Jittery
Gurgling
Light
Calm
Jumpy
Tingling
Fuzzy
Wobbly
Buzzing

Expansion Sensations

Expansive
Floating
Fluid
Radiating
Waves
Moving
Flowing
Relaxed
Glowing
Streaming

Start with something easy—your heartbeat or digestive processes—and then zero in on particular spots and work your way out. For example, as you read this book, choose one part of your body and attend to sensations there. Then move to another part. Maybe begin with sensations in a finger, then the whole hand, then feel the whole arm. As you stand up from your chair, choose a leg muscle and sense how it helps you stand. Feel it propelling you across the floor. With practice, slowly you will be able to scan your whole body relatively quickly.

The next element of the felt sense—after "feeling your body"—is to understand how we gain information from these sensations. Specifically, we learn to read our bodies. Practice by looking at someone else. Look into the face of someone you love dearly. Connect what you know about that person with what is happening today. Recall how that person's face expressed when you shared wonderful news, or when the person was suffering. Trace the lines of joy or despair on the person's face, but also in his or her stance, voice, and energy. The better you know the person, the better you can immediately sense how the person is feeling. The felt sense works the same way, except you turn your attention inward. Loads of information pass through our own bodies. How much we "read" from our own systems is determined simply by how much attention we pay.

The term "felt sense" was coined by psychotherapist Eugene Gendlin. He identified it alongside the concept of "focusing," which involves using the felt sense with intention. Gendlin said that the felt sense allows us to become more clear about situations, reach goals, receive insights, or uncover and resolve problems. In his book *Focusing,* Gendlin wrote, "A felt sense is not a mental experience but a physical one. *Physical*. A bodily awareness of a situation or person or event. An internal aura that encompasses everything you feel and know about the given subject at a given time—encompasses it and communicates it to you all at once rather than detail by detail."[132] (Emphasis in the original.)

A breather tunes into his body during a session, with support from a facilitator. The felt sense is a primary tool in keeping in touch with the physical body, whether in a BBTRS session or daily life.

Awakening

“Anna felt numb. She didn't think too much about it, but when she attended a BBTRS session, she was asked to describe her numbness. She was able to point. She pointed to her abdomen, past her pelvis, and to her legs. She said that she could feel the ground under her feet, and she could feel the touch of someone's hand on her knee—but from the inside, she felt "numb, from her diaphragm to her knees." When she was asked if she knew the reason she felt like this, Anna said it was due to some sexual trauma she had experienced as a child. She had disconnected from the experience, and, some thirty years later, she still had never reconnected. Her self-protection and rigidity had obviously disconnected Anna from her sexuality. Anna had never experienced an orgasm, and called herself a "dead fish."

Her BBTRS session started gently, with slow movement, mild breathing, and only light, feathery touch by her facilitator. At first, the intention was oriented at inviting sensation on the surface of her body. Anna began to feel superficial sensations in her legs and feet, then, gradually, in her belly and pelvis. Her facilitator supported Anna's organic movement, which developed in her upper body, to drift downward, into her midsection, pelvis, and legs. She started to feel the aliveness in that lower part of the body. She felt heat, which she said was generated from her own movement. Her facilitator saw pink patches develop on the surface of Anna's skin, which indicated an opening of the capillaries.

As the session progressed, and Anna became more comfortable with feeling, the facilitator slowly started to bring Anna's attention to the physical sensations throughout her body. After decades of being disconnected from the body, she had to relearn the experience of sensation. She gradually shifted from associating pelvic sensation with pain to neutrality—and then to gentle pleasure. She simply began to feel again, and identified tingling, heat, love, and caring. She experienced so much acceptance from her facilitator that she cried tears of joy.

"This part of my body has simply been ignored," she said. "I thought I would just encounter pain there and nothing else." In the BBTRS session, she did experience some pain—but it was just part of her process, along with pleasure. She did not know that both could exist at the same time. She left the story behind—of what happened, why it happened, associated guilt and shame—and she rejoined her body, experiencing the subtle physical sensations that emerged because of the breath, the movement, the touch, and the feeling. "I could not do it on my own," she said. "At first, I was too afraid I would feel pain. My facilitator helped me to come out of my fear, and to be free of my numbness."

Shifting sensations

The felt sense is a tool. It is like holding a stethoscope up to your inner self. You can "listen to" a particular spot, or to the entire body, all in one go. Importantly, Gendlin noted that the felt sense is also a moving target. As soon as you notice a sensation, it shifts. And in that process, your experience shifts with it. Through the very act of paying attention, your inner experience changes. "When your felt sense of a situation changes, *you change*—and, therefore, so does your life."[133] (Emphasis in original.)

Peter Levine, who also uses the felt sense in Somatic Experiencing®, agrees with this observation. He writes:

> "The felt sense is sometimes vague, often complex, and ever-changing. It moves, shifts, and transforms constantly. It can vary in intensity and clarity, enabling us to shift our perceptions. It does this by giving us the process as well as what is needed for change. Through the felt sense we are able to move, to acquire new information, to interrelate with one another and, ultimately, to know who we are…. You can imagine going through a day without emotion, but to live in the absence of the felt sense is not just unthinkable; it is impossible. To live without the felt sense violates the most basic experience of being alive."[134]

What do these experts mean? Of course we know that the sensations within our bodies change, but how does this make our life change? How does the felt sense allow us to "acquire new information"?

Consider our daily "rhythms." We can check in with periods of more or less aliveness, times when we need to regroup, periods of intense concentration, periods of rest. We also can immediately remember what a certain pain feels like, or what happens in our bodies when we feel worried or elated. Intuitively and cognitively, we can agree that these moments shift throughout a day. But Gendlin and Levine are saying that the moment we are aware of a sensation, *we* shift. More than our awareness of the sensation, or pain, or fatigue—we ourselves are different because of the experience of the awareness.

Linguists claim that the moment a thought, or a thing, or an idea is attached to language—the moment a tree becomes "tree"—the thing changes from the potential concept of what it might represent into an object, a word. Tree. In the process of "naming a tree"—or in our case, of identifying a sensation—we gain information. We learn about ourselves. We discover connections between the world around us and our reactions to it. We find out what we like and what hurts us, what feels good and what our intuition tells us to avoid. By becoming aware of a physical sensation, the sensation shifts from a state of being to something we interact with, observe, and can change. We are able to witness the sensation, and that makes it different. Although it would have flowed on its own, just our attention causes the feeling to become more fluid.

An added bonus to this process is immediacy. Once we realize that no "state of being" is permanent—that the sense we have changes from moment to moment—then we are living more in the now. We realize that whatever is happening in our lives will flow. It's here and gone, better and worse, interesting and dull, rich and thin. Likewise, we're not stuck in how we feel. In order to live, to really *live,* we begin to "go with the flow."

Somatic work adds process to this flow. The moment we step away from the emotional and cognitive—the moment we move into physical expression—we initiate the shift away from being stuck. Think about the last time you felt really angry about something. Can you recall how you felt physically? Did your body feel correspondingly tense? What did "tense" feel like? Now, imagine if the *physical sensations* had changed first—if the tightness, heat, hair-standing-up sensations you had took your attention, more than the situation that angered you. If you had been able to shift those physical sensations, then your body would have "felt better." Can you imagine that your emotional feelings also would have shifted?

We learn about ourselves by paying attention. We "see ourselves" if we notice how our bodies feel—and by noticing, we feel different. Just as changing what we think impacts our emotions, changing what's going on in our body shifts our emotions. Importantly, by remaining in the physical, even as we manage a negative moment, we don't feel an emotional overcharge or re-traumatization. And frankly, it's easier to focus on a physical sensation than an emotional one.

Managing the moment

"Dana was in Poland on a holiday—relaxed, enjoying a walk along a crowded street—when he was happily drawn into a "shell game" conducted by a street artist. He believed that the surrounding participants were, like him, observers. They were "festively playing and exchanging money for wins and losses." Dana joined in, calling out which shell he thought held the pea. The group encouraged him to bet. "I generally do not bet, but thought it would be fun, so I reached into my zipped pocket to pull out $2. Before I could even look up, the game leader snatched the whole fold of money out of my fingers, proceeding to 'place my bet for me.' Instead of $2, my 'bet' was for $50," he says. "I objected repeatedly and loudly, but the crowd ignored me. The game leader let someone else call the cup, which lost, and everything moved forward as if I were not there. I realized that the group were all part of the scam—which was later confirmed. I realized that no one was going to help."

Dana had not been physically abused. He had not been endangered. He had, however, been violated. But trauma comes in all forms. What was a whimsical, voluntary experience became a charged, involuntary one. It was theft with a false front. Dana lost more than money in this exchange. As the realization swept over him, he stood there feeling distraught, helpless, frustrated, and angry. He felt his throat tighten, his heart race, his stomach churn. An experienced BBTRS practitioner, he noticed that his breathing had become shallow, and that his fists were clenching.

"I realized that I had no recourse, that this was basically sanctioned theft, and that I could not even go to the police. As I watched the thief nonchalantly going about his day, I even had thoughts of physical violence. Instead, however, I decided it would be better to just walk away, take my loss and lesson, and dispel the trauma that I was feeling."

Dana already had developed a daily breathwork meditation that brings him into his body. He went to a less-traveled area, went inside himself, and conducted a personal breath session. "I let myself breathe, walk, shake, cry, and even growl, until the energy was dissipated and my body was feeling normal again. After about ten minutes, I was able to continue with my day and the remainder of the trip without crippling effects." Dana was still bothered by what had happened, but he was able to count it as an unpleasant life experience. He doesn't carry fear or anger when traveling. He doesn't carry negative feelings about Poland. He is more diligent about pickpockets and other potential pitfalls during travel, but he experiences no residual trauma from the theft.

Dana has, however, become very good at the shell game.

If we learn to use the felt sense in our daily lives, then we can learn to not feel overcharged as a general way of being. Like Dana, we can separate from emotional charges when they happen, thus diverting emotional overwhelm. The shift of attention from emotional feelings to physical attributes allows the separation from the emotional charge. Then, by tracking the internal physical aspects of the experience, we can release the associated emotional charge—immediately or later.

Putting the felt sense into words

In a BBTRS session, the facilitator asks the breather what is happening in the body, where the body feels good, stressed, relaxed, tense. Importantly, breathers are asked to use language to describe the sensations, even though this can be difficult.

Gendlin explained that the felt sense does not arrive in the form of thoughts, but as an often "puzzling and very complex" bodily feeling. "Since a felt sense doesn't communicate itself in words, it isn't easy to describe in words."[135] However, even though the experience is broader than language, language improves our facility with the felt sense. We know that language will never fully capture a feeling or a body sensation. It can't. But as we strive to capture our experience, and notice that it is shifting, our relationship with our felt sense evolves.

In BBTRS, we do try to use our words. We try to express the felt sense enough to identify where we're feeling stress and where we feel safe in our body. We begin with narrowing our general senses into specifics. If a breather says she feels hurt by betrayal, and that she wants to shout, "How could my sister do that to me?", then we ask: "What does that feel like in your body? Where does betrayal live in you?" With experience, people can answer these questions in their own bodies, and help others to find the answers in theirs. By practicing this process of awareness, we step out of activation and watch it dissipate. We step into a state of relaxation. This is why it's important that breathers stay present to physical sensation during entire BBTRS sessions. This attention helps to avoid getting emotionally overcharged in catharsis.

Breaking the cycle of emotional charge is what the experts mean by using the information in our bodies to change our lives. We find a physical sensation related to emotional charge, track it, and describe it. This very process causes the emotional charge to shift. It loses its power. The sensation, the emotional charge, and the language we use to describe them all affect one another. As the sensation shifts, it becomes a diffusing agent for the circumstances that caused it.

Some people—especially those who have been traumatized—live in a disconnected relationship with their physical bodies. For these people, it might be difficult to imagine change. It might be difficult to imagine sensing the body, or describing how they feel in a therapeutic session. It might feel completely impossible to integrate this process of using the felt sense into daily living. If this sounds familiar, then it's best to start the process of sensing the body little by little. It isn't impossible. Learning to feel and change the body's responses really does work. And remember: as soon as you begin, your body changes. The effort to learn is worth it. When all a person encounters is tightness and pain, life cannot be very fulfilling or expressive. Even if the felt sense is challenging to learn and articulate, living without pain is easier than living with it.

With BBTRS, we recognize that we will never feel the same way again. Think about that. Instead of being a slave to emotions and vague sensations, we participate in our physical experience. We feel, we notice, we describe, and then we feel different. Our entire process of feeling changes. And it changes everything. We get better at sensing, better at talking about it, better at working with it, and better at releasing trauma. And with practice—even when words fail us—our relationship with the physical body becomes much more intimate, expressive, and healing.

Anatomy & functionality

This entire chapter is about "feeling things." The felt sense will play an important role in the success of moving tension out of the body during a BBTRS session—and even just walking through the day, once it becomes a staple in your life. Using the felt sense and releasing tension are complemented by at least some awareness of basic anatomy. Most helpful is understanding the physiology most related to fight-or-flight activation. Let's discuss the important role of a few crucial parts of the body. The more you know how they work, the better they'll work for you.

A general rule of thumb: the more we experience healthy movement and flow in the body, the better. Movement therapist Bonnie Bainbridge Cohen has influenced the fields of bodywork, movement, dance, yoga, and many other movement-based disciplines. She describes how different aspects of the body work together to facilitate not only functionality, but also consciousness. She says that the "skeletal-muscular system guides our external movement through space, (while) organs occupy our inner space and therefore guide our internal movement. Our bones align and move us through the environment; our organs provide the internal integrity for that alignment. Muscles provide the visible forces for the mobility of our bones; organs provide the internal patterning that contributes to the organization and patterning of muscular coordination."[136] All the while, "the cells of our tissues live within the interstitial fluid, our internal sea," and circulatory channels carry life-supporting nutrients and other substances.[137]

Basic anatomy and physiology tell us the plain facts. For example, in the joints, optimal mobility and flexibility describe the range of motion possible when bones, muscles, ligaments, and tendons function as a unit. Range of motion is defined and limited by strength, injury, and muscle length—not to mention what might be happening with neighboring organs. Now, think beyond simple anatomy. Every tissue impacts the tissues around it, and every tissue responds to much more than locomotion and metabolism. For example, Cohen says that "organs provide us with our sense of volume and organic authenticity. They are the primary habitats for our emotions, aspirations, and memories of our inner reactions to our personal histories."[138] From her perspective, then, tension in the system ties knots in much more than biology. Likewise, movement and release throughout the body involve much more than mobility.

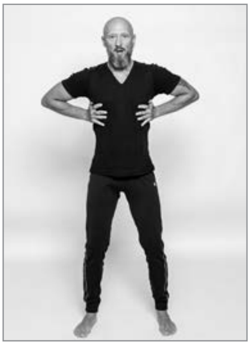
When we really open the body to release tension, we enhance—or even craft—our well-being. True opening requires a willingness to heal. It requires receptivity, which interrupts a domino effect of constriction and tension. If one muscle is tight, then it pulls on other muscles, organs, and connective tissue, activating, contracting, overstretching, compensating, inhibiting circulation, slowing nutrient delivery, and clogging the process of waste removal. Nerve impulses keep firing, fighting the problems with the purpose of regulation, of finding homeostasis. Tension brings further tension, discomfort, and imbalance. It sounds stressful, and it is.

Let's begin to heal, and to open, using the processes that enhance relaxation and flow. Let's begin with prana.

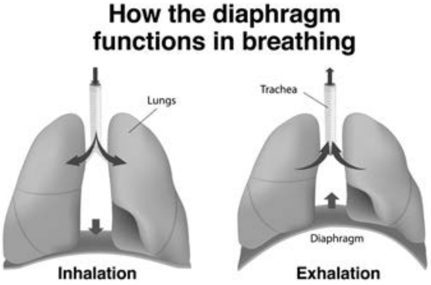
The breath

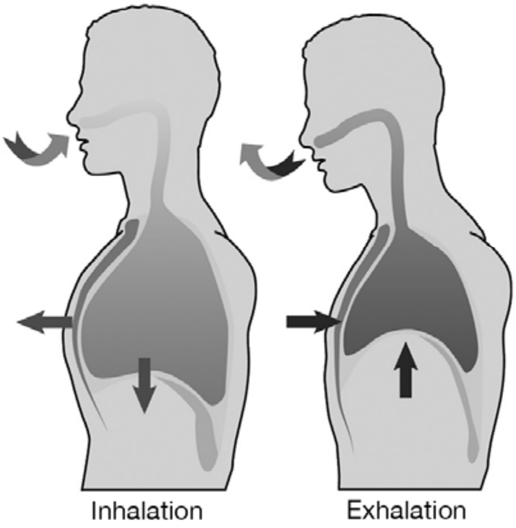
The act of breathing relies on the dome-shaped diaphragm, which works in concert with the ribcage to expand and contract the lungs. Put simply, the diaphragm is a sheet of muscle connected to the lower ribs, front of the spine, and the sac that holds the lungs. When it contracts to its full capacity, the diaphragm pushes down on the abdomen, expands the ribs, and increases the volume in the chest cavity. This change in pressure allows air to rush into the lungs. When the diaphragm relaxes, the air flows back out again.

Exercise: For proper breathing, both a toned, healthy diaphragm and a flexible rib cage are necessary. Try putting your hands on your sides—in the same position you would use to put your hands on your hips, except at chest level. Squeeze your hands against your sides as you take a deep breath, and notice that your hands expand with your ribs. Exhale, your hands return to the original position.



The diaphragm also interacts with the sac that holds the heart, the pericardium. The diaphragm's connective tissue attaches to the spine, the front and back of the abdominal wall, core muscles, and both hip flexors, called the psoas major muscles. It plays a vital role in the function of the circulatory, lymphatic, and thoracic nervous systems—and when the abdomen is activated, the diaphragm impacts digestive health. The simple act of breathing—fully, expansively breathing—is one of the most basic ways to keep us healthy.





The diaphragm acts as a “bellows,” assisting the respiratory system to push air in and out of the lungs.

As with any muscle, stretching and using thoracic muscles and their connective tissues maintain their strength and vibrancy. When we limit the diaphragm's movement, it atrophies and loses flexibility. Although no one intentionally limits the movement of thoracic muscles, flexibility naturally diminishes when we don’t breathe deeply. And the most common reason we breathe shallowly—or clamp down our breathing—is to consciously or unconsciously control emotional responses.

Trauma is one of the most common reasons we control our emotional responses. Prolonged fight-or-flight activation changes our breathing. Think back to when you’ve cried or laughed until you were gasping for breath. These are basically moments of hyperventilation, in which quick, shallow breaths through the mouth do not allow for full inhalation. In these situations, we simply do not receive sufficient oxygen in the blood, and our respiratory system becomes tight and constricted. Imagine doing that for days, weeks, or years, to a lesser degree.

John Douillard, DC, explains the biological purpose of this type of breathing by imagining suddenly coming face-to-face with a bear. “Your initial response would most likely be a gasping, fear-based mouth breath. This gasping breath fills the upper lobes of the lungs first, which activates the stress receptors and engages the fight or flight response. Ideally, this would happen quickly enough for you to get a burst of energy and speed to save your life."[139] Douillard adds, however, that we don't need a bear to breathe like this. "The act of shallow breathing alone will activate stress receptors in the upper chest…(which) triggers an excessive and unnecessary production of degenerative stress hormones."[140] Sound familiar? It should. Douillard is saying that simply by breathing shallowly through the mouth is enough to activate the sympathetic nervous system.

The more stress we live with, the worse it gets—and the more the lower lobes of the lungs are held tight with each exhalation. They become "tight and rigid" in a positive feedback loop. "More stress leads to more shallow breathing, and continuing to breathe shallowly leads to feelings of increased stress; the lower lobes become even more rigid and inaccessible. Soon the rib cage literally becomes a cage squeezing the heart and lungs and forcing all the breath into the upper chest."[141]

"Breathing through the nose, on the other hand, delivers the air deeper into the lower lobes of the lungs because of the structure of the nasal passages and sinus cavities," Douillard says. "Breathing into the lower lobes of the lungs is preferred because we find 60% to 80% of the lungs' blood supply waiting for oxygen delivery and gas exchange. The receptors for the parasympathetic nervous system are concentrated in the lower lobes of the lungs."[142] Therefore, relaxed, full breathing is calming.

When we inhale normally, preferably through the nose, the amount of flexibility and movement in our lungs, ribs, and diaphragm, as well as our emotional state, determine how much oxygen enters the lungs. In BBTRS, we use this normal, calming breathing as "base camp." We begin there and return there throughout the session, ensuring that the breather is not overstimulated. We also use faster breathing, through the mouth, which is intended to stimulate the sympathetic nervous system—for the very targeted purpose of eliciting unconscious movement. This is how we release the tension held in the tissues. The difference between the deep, conscious breathing in BBTRS and shallow mouth breathing is the completeness of the breath. Although we stimulate the fight-or-flight system, the breathwork in BBTRS is not hyperventilation. It is not shallow. Throughout the session, plenty of oxygen is flowing, and the breath is monitored, adjusted, and carefully attended to.

In general, we are both activated and relaxed in a BBTRS session, with complete access to our diaphragm, ribs, and lungs. Through our breath, we charge the body with energy, expanding the tissue that contracted because of trauma. The breath supports the opening of the entire body, and is closely connected to our emotional responses. Without the use of the breath in BBTRS, the other elements would not fall into place.

Exercise: First, try breathing shallowly, with the breath remaining in the top of your chest. Keep your belly contracted; don't let the air expand your torso below the clavicle. After a few breaths, you should feel constricted, and as if you need to take a deeper breath. Next, relax your torso, and inhale deeply, allowing the belly to expand first. Notice that the as the breath is completed, the movement is like a wave, beginning in the belly, and then traveling up into the chest. If you have trouble with this at first, imagine filling a glass with water, from the bottom up. Relax the belly, let it fill with air; then let the lungs fill with air. On the exhalation, you will feel the wave retreat; first the chest falls, and then the belly.

The moment we think about observing the breath, it automatically becomes deeper. Like the felt sense, at that moment of attention, our breath changes. On the other hand, if we're stressed and also not paying attention to being stressed, the effort of breathing is constricted. It's so much harder. And the mountain we're trying to climb becomes steeper. When we do not breathe deeply, we decrease the energy we have access to. We're already suffering over something, and then we compound the suffering by restricting our air and our energy. In turn, we feel less.

When we don't breathe—really, truly breathe—we become numb.

The fascia

Most people don't think much about fascia, but it literally holds us together—and is key to the "unwinding" process of BBTRS. Fascia is an uninterrupted collagen/fiber network known as the "extra cellular matrix," or ECM, that connects everything to everything else in the body. It's kind of like a one-piece suit that runs under the skin, at the same time encasing, surrounding, separating, and/or binding together muscles, organs, and other tissues. Fascia is one of four types of connective tissue, including "proper" connective tissue, blood, bone, and cartilage.

There are different kinds of fascia, including: the superficial layer under the skin; deep fascia covering and permeating muscles, bones, nerves, and blood vessels (the target of deep-tissue or myofascial release); and visceral fascia, the deepest layer, which holds organs in their cavities.

According to Functional Fascia, a company that teaches Integrated Allied Health Professionals about anatomy, the fascial network is as important—if not more important—than the cells that are contained within it and make all movement and function possible. "ECM is like the inner ocean of our bodies. All the cells that we have require space around them, and this space is filled with an inert fluid that protects, cushions, and holds the cells and tissues in place."[143]

Moist, flexible fascia not only creates a framework for muscle to move around bone, but also allows for the transmission of force. That is, fascia allows physics to take place; it also provides stability for any tissue that requires a strictly controlled environment. For example, muscles slide in a particular and limited plane, while nerves are not supposed to move around at all. Fascia allows each organ, muscle, tendon, ligament, vessel, and nerve to do its job.

Without movement—from a sedentary lifestyle, restricting illness, stress, or trauma—fascia becomes dry and brittle. When we freeze up, the fascia becomes contracted, which diminishes our range of motion and overall quality of life. There are many modalities that support the fascia, but the most crucial one is daily, regular, wide-ranging movement. Only through movement can the circulation of essential fluid occur. It makes sense, then, that the deep, relaxing, snakelike movements associated with BBTRS's unwinding are effective and also feel so good. The same movement that supports the fascia allows the tension to leave the tissue.

One great exercise not only supports general fascia health, but also mimics the unwinding portion of a BBTRS session. It can be initiated in a seated or standing position, and it requires the ability to allow organic movement from a deep place within. This means we stop thinking, except to ensure that we keep stretching. And stretching. And stretching.

Exercise: From a sitting or standing position, reach both arms out to the side, as if they're pointing at "3" and "9" on an imaginary, life-sized clock. If you're standing, place the feet shoulder-width or wider apart, with knees slightly bent.

Begin by stretching the arms out simultaneously, reaching away from each other. In much of the exercise, the arms will maintain this opposing posture, regardless of which number of the clock they're targeting.

As you feel the polarity, which initiates a stretch in the fascia and myofascia, your body will naturally "want" to move in its own way. You might end up with one hand touching the floor and the other straight in the air, or in a twisting position.

Throughout, breathe in a connected, relaxed way, with no pausing between inhalations and exhalations. Use deep, fluid breath to support and inflate your movements. Even more, visualize bringing the breath in to the areas where you feel the biggest stretch. Create a feeling of inflating this area with the breath from the inside.

Continue allowing natural movement, ensuring to maintain a bodywide stretch. Keep the movement present and slow, with the stretch traveling smoothly through your body, from one area to the other.

As the exercise progresses, you might find yourself on the floor, undulating with snakelike movements. If this occurs, don't worry if your arms lose their opposing positions. However, as you move around and regain an upright posture—or if you're flat on your back or stomach—the 3-and-9-o'clock position is a good starting point to return to.

Continue for several minutes—fifteen to twenty at least (but start with only five, if that's all that feels comfortable), and then continuing for as many as thirty, if you like. Remember: the key is natural, organic, connected movement tied to the breath.

Once you get familiar with the exercise, and have extended it to at least twenty minutes, consider stopping every ten or fifteen minutes to employ the felt sense. Tune in to the slight body sensations that are present. You might notice a physical sensation reminiscent of a slight "electrical charge" running right under the surface of the skin. Opening the myofascia supports nerve impulses throughout the body—and it also feels great.

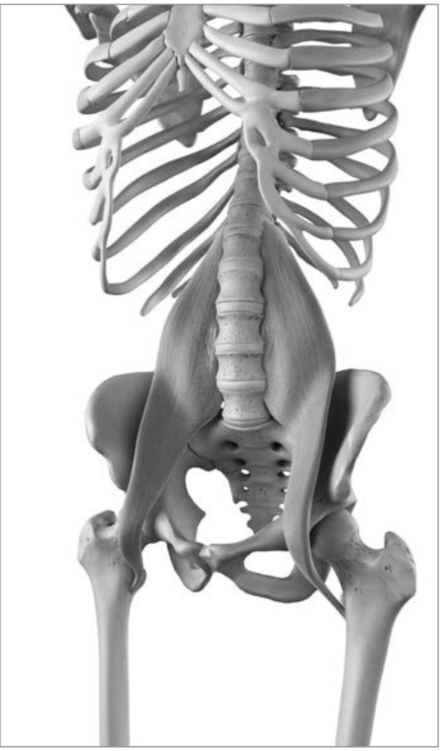
This is a foundational self-care exercise that you can do anywhere with enough space. If you're cramped in an airplane, you can still polarize your arms in the aisle, or bend forward in your seat, twisting gently and organically from side to side. For a wonderful demonstration, see John Barnes's YouTube video at https://youtu.be/1QM-8_DwArU.

Think of fascia as either the prison that helps us hold tension inside, tying us in "knots" and "locking us down," or the supple, open, flexible, smooth ocean that carries tension away. It seems pretty obvious which kind of body we want to live in.

The psoas

We often think of our "core muscles" as our abdominals, our "six-pack" area, but the core also includes everything from the pelvic floor to the diaphragm to muscles supporting the spine. In fact, "minor core" muscles include back muscles as high as the trapezius and latissimus dorsi, and as low as the gluteus maximus. The hip flexors—the psoas major and iliacus, which assist us in raising the knee toward the chest and bending at the waist—are definitely part of the core. As the main muscle involved in walking and kicking, the psoas could be considered our primary fight-or-flight "action center."

The psoas connects the spine to the legs. It travels from its attachment points on the lower back (Thoracic 12 and Lumbar 1-4), across the rim of the pelvis, to its other attachment on the back half of the inner thigh at the top of the femur. Jonathan Fitzgordon, founder of the CoreWalking



This medical model shows the position of the psoas major and how it attaches to both the femur and the spinal column.

Program, explains: “The psoas muscle, if properly aligned, is capable of supporting the spine and moving us through space. Its downward pull on the front of the lower spine (lumbar vertebrae) allows many muscles at the back of the body to lengthen and tone upwards, providing support for the head at the top of the spine.”[144] As the left and right psoas muscles move in opposition, they pull the back leg forward, stimulating a healthy rotation of the pelvis and proper spinal alignment. When the psoas muscles are toned and stress free, they do the heavy lifting of walking, thus relieving the quadricep muscles. Therefore, the psoas is our main go-to when danger approaches.

The psoas is also a key element in BBTRS because of its intimate relationship to the breath. Walking and breathing are connected through the psoas muscle and the diaphragm, thanks especially to a diaphragmatic ligament called the medial arcuate that wraps around the top of the psoas. A particular grouping in the fascial network also connects the diaphragm, psoas, and other hip muscles. With all of this interrelationship, the psoas and our respiration work in tandem. “With each inhalation, the diaphragm and pelvic floor should lower, and with each exhale they should rise back up moving the contents of the trunk with them. Add to this up-and-down movement the rotational possibilities of the psoas moving in rhythmic opposition, and every breath and every step we take can tone and massage the entire contents of the trunk.”[145]

Finally, while it might seem odd that a hip flexor impacts the heart, the psoas muscle is indirectly connected. Specifically, the diaphragm connects to both psoas and pericardium—so when the psoas becomes tight, it pulls down and restricts the movement of the diaphragm. This constriction in turn pulls down the pericardium, which restricts the physical space in this sac. Therefore, in a chain reaction, a tight hip flexor can reduce the amount of room available for the heart to beat. Unresolved trauma can literally be the cause of a "closed heart."

The core of the matter

"Lara, a former athlete, found herself in middle age with some chronic back and leg problems. Nothing tremendously serious, she thought, but they inhibited her attempt to get back into hiking, biking, and walking her dog. Occasional chiropractor visits addressed her temperamental SI joint, and she felt hyper-sensitivity in her Achilles tendons. In what she thought was unrelated news, Lara also had visited a massage therapist who had worked a little on loosening her hysterectomy scar. She was especially protective of that part of her belly, where the nerves had never regenerated. The whole lower belly felt like it should not be touched; her skin "crawled" there. She didn't mention this to any other practitioners.

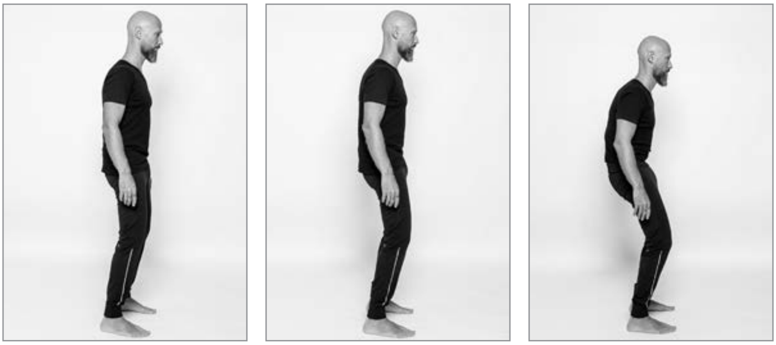
Eventually, Lara's discomfort caused her to seek the advice of several intuitive bodyworkers. One sensed a connection between the fight-or-flight mechanism in her brain and her Achilles—but he didn't know why. Another told Lara that her psoas muscles were so tight that they were pulling her hips out of alignment, so that Lara was leaning forward slightly. Then, both her massage therapist and her Chinese Medicine practitioner filled in the last piece of the physical puzzle: Lara's hysterectomy scar had adhered to her psoas muscles, pulling them tight. Her hips' flexion was inhibited, and even her breathing was restricted, because her diaphragm was being activated. The involvement of the psoas explained the lower back pain, and why her SI joint was occasionally compromised.

By connecting all of these pieces, Lara became much more proactive. She toughed out the painful process of disengaging her scar, and feeling flooded back to her lower abdomen. Her psoas muscles relaxed, and she stood taller, walked with more flexibility in her joints, and experienced less pain down her legs. She attended more yoga classes, some taught by a medical doctor who helped practitioners with previous injuries. She gradually began to breathe more freely. But she didn't understand the foundational "why."

A BBTRS session focused on the lower abdomen provided the unexpected answer. Her hysterectomy had been traumatic, and related to various negative sexual experiences in Lara's youth. She had instinctively leaned into protecting herself, "folding inward" at the hip, not giving her heart freely. She had worked hard in psychotherapy to release her past traumas—and like many newcomers to BBTRS, Lara had a sudden realization that some of those traumas had stubbornly remained. In the end, all of her practitioners had been right: she had been in chronic fight-or-flight activation for decades, and this had been expressed in a chain-reaction from her diaphragm to her ankles—one that physically tied her hysterectomy to her core muscles and emotionally held her heart in a tight grip. The session brought all of this to the surface, in an intimate, glorious expression of her femininity. Lara told her facilitator, "I have never felt so safe and alive!"

After BBTRS sessions and seminars, posture changes are often evident. Like Lara, people stand taller and radiate confidence. This change is so because the psoas, as a fight-or-flight "first responder," also ends up holding the tension when the fight-or-flight activation is stopped prematurely. Unresolved trauma literally stops the process in its tracks, causing the psoas to tighten, constrict, and lose flexibility as a result.

When we stand with a psoas contraction, we feel collapsed. The hips and back are forced out of alignment and folded forward, the



These photos demonstrate how tight psoas major muscles can affect structural integrity and posture. Here, three illustrations show a continuum of contraction that collapses the lower back.

abdomen and diaphragm are constricted, and associated pain can appear throughout the back, hips, legs, and feet. With BBTRS relaxation, this primary stabilizing structure in our bodies relaxes and realigns, resulting in an evident, physical experience of opening and self-correcting posture. With this improved posture, we create more room to breathe. “The relationship between these two titans of walking and breathing cannot be understated,” FitzGordon says. “Health in one can and will encourage health in the other.”

Sexuality in trauma release

People usually experience involuntary body movements only in unguarded or “unguardable” times, such as while giving birth, during injury or accident, during a fight-or-flight response, while dying, and while engaged in sexual activity. Somatic trauma release, whether in BBTRS or other modalities, encourages the body to release tension and stress through unconscious, autonomic movement. There are at least three reasons, and probably many more, why sexual sensations might arise during somatic work. Because this is common, it’s important to discuss it alongside our other “what-things-feel-like” topics. In addition, because sexuality is such an important part of our relationship with the body, sexual sensations can be used and managed as part of the BBTRS experience, if they arise.

The three basic reasons the genital and belly regions might “wake up” during a session are:

- The tremoring and unwinding feelings originate in the core of the body, near the sexual center of the body. During the organic movement, which can be rhythmic and involve the hips and spine, it makes sense that sexual arousal could occur.

- Because of cultural assumptions and repression around sexuality, or sexual trauma in the breather, tension might be locked in the pelvic region. Challenges around sexuality might be theoretical, emotional, or physical, and we'll address how to approach sexual abuse histories later in this section.
- Even for those who do not have negative or highly charged issues around sexuality, the sex organs are natural areas of "release." Sometimes a sexual charge naturally accompanies other systemic activation.

Sex and repression

It might be picking low-hanging fruit to blame the Victorian era for Western ideas of proper sexual behavior, but we're all aware of how these norms trickled through some 130 years. Then, the sexual revolution of the 1960s created a platform for new cultural norms, not to mention a permanent reaction against Victorian ideals. The result was a progressively more permissive environment, until today we are used to sexually explicit material and activity in the media, on advertising, on selfies, and in public. Children are sexually active at younger ages, and for many people, "intimacy" has drained from the dating world. In about two centuries, we've gone from dresses covering the ankle to dresses barely covering the behind. In the 21st century, only fetishists wear corsets and chastity belts—but still…the dominant culture measures sexuality against norms of "proper behavior." Somehow, despite all the change, all the backlash, all the experimentation, a palpable tension remains between sexual expression and sexual repression.

The Victorian era tied "propriety" to religious teachings. In fact, the period saw the greatest burst of church building since the Middle Ages.[146]

When the sexuality discussion is expanded beyond the West, even more traditional religious cultures enter the conversation. What for Westerners is an overlay of criticism and personal tension becomes the law for many Eastern people, especially women. One example comes from an online Iranian blog called IranWire.com. A recent story discusses a summary of four surveys conducted in 2006, 2007, 2010, and 2014. Respondents were asked their opinion as to whether women should be forced to dress conservatively, especially regarding traditional head coverings called the *chador* and the *hijab*. In 2006, 34.7 percent agreed that the government should not interfere in how women dress; by 2014, the figure had risen to 49.2 percent—which is a tide rising in opposition to Islamic law.[147]

The idea that women must dress conservatively has to do with modesty. It has to do with sexual propriety—and cultural mores place the majority of the responsibility, and the blame, on women. One cleric was quoted as saying that when women dress immodestly, they "lead young men astray and spread adultery in society, which increases earthquakes." While this level of repression is not present in the West, sexual repression is pervasive throughout the world.

Sexual repression affects how we walk on the planet, how we work, how we play, and how we engage with others. It also impacts how we release tension in BBTRS. Repressing the power of sexuality is different from containing sexuality. Containing, managing, and being in relationship with sexuality is productive, healthy, and necessary. It's part of having appropriate boundaries and of managing our personal needs. When we contain sexuality, we allow it to move within us, to live within us. We don't "stuff" it. On the other hand, when we repress, we cut off from feeling sexuality, and that's when physical tension starts to build. Often, the blockage involves shame and guilt. Most of those programmed feelings are connected to one of our most intimate belief systems, our religious upbringing. That's why the conflict is so personal, and so enduring.

Repression can arise from very serious issues, such as feeling endangered because of clothing choices. It can arise from much milder worries, such as when a married person feels a passing attraction to someone other than his or her spouse. All of these are ideas, disconnected from our biology. However, since we are biological, sexual creatures, the conversation becomes complicated. To discuss healthy sexuality is not the same thing as preaching free love and promiscuity. That's not the goal. It's not even the subject. A productive conversation about sexuality should include our aliveness. It should be an energetic, biological, and creative conversation, not only a political or religious one. Once people let go of shame around being alive, which is one of the biggest catalysts of repression, then the sexuality issue fades into normalcy.

Healthy sexuality is a matter of building a container where we can be with our sexual feelings without spilling them out inappropriately. It's about learning how to use touch in general, sexually and otherwise. It's about teaching people how to touch with affection and appropriateness—from childhood onward. Adults and young people are so stressed about inappropriate touching that they're not touching at all. They're afraid to connect in a physical sense, and in many ways are inexperienced with nonsexual touch. Once we relax about "SEX," then touch takes on a new meaning. It's not intimidating to touch someone, because touch in general is not charged with sexuality. When we are at peace with our sexuality, our nonsexual interactions are clear and clean.

Longitudinal studies have shown that newborn, premature babies who were cuddled during in-hospital stays did better—from their premature births through age 10. They had better sleep patterns, steadier heart and breathing rates, and better stress reactions.[148] Touch literally has the power to heal. Our heart's desire is to reach out to care for someone, or to feel connected. It can also be to feel sexual pleasure. These are all different expressions of the same energy. The creative, sexual impulse is also part of our creativity. When we feel safe and free in our ability to connect and express ourselves, when our energy is free to circulate, it is transformed into our relationships and also what we make. On the other hand, when repressed, it binds us, triggers us, and makes us afraid. It usually locks up the pelvis. This is why so much clearing occurs in the pelvis during trauma release. This is why people stand straighter afterward—because we enter the session with so much energy blocked in our core. BBTRS unties the energetic and physiological knots tied by this sexual, repressive tension.

Sexual trauma

The World Health Organization (WHO) reports global statistics in many topic areas, including an important estimate: that about one in three, or thirty-five percent of women worldwide, has experienced either physical and/or sexual violence. Most of this abuse is perpetrated by intimate partners.[149] The United Nation's Children's Fund (UNICEF) reports that around fifteen million adolescent girls worldwide, aged fifteen to nineteen, have experienced forced sex in their lifetimes.[150] In the United States, statistics are even more detailed. According to the Rape, Abuse, & Incest National Network (RAINN), one out of six U. S. women (more than seventeen percent) is a victim of an attempted or completed rape; eighty-two percent of all juvenile victims are female; ninety percent of adult victims are female. However, one out of ten rape victims is male, and three percent of U. S. men, or one in thirty-three, have had this experience. In addition, twenty-one percent of transgender, genderqueer, or nonconforming (TGQN) college students have been assaulted, as compared with eighteen percent of non-TGQN females and four percent of non-TGQN males. Native Americans are much more likely to experience sexual violence than members of other races—and unlike in other statistics, most of these attacks are perpetrated by strangers.[151]

After-effects of sexual violence include PTSD, sexually transmitted disease, contemplated or attempted suicide, drug use, unwanted pregnancy, and a large variety of social, work, relational, and emotional distress and/or associated problems.[152] That's a single sentence with a lot of trauma packed into it. One sexual attack can change a person's life forever. It can change how a person interacts with others, and how he or she forms intimate relationships. It can forever alter one's perception of self, one's idea of whether the world is a safe and sustaining place, and whether striving for one's goals is even worth it. In any therapy, experiential, or trauma release group, it's certain that sexual trauma will play a part. It's certain that someone in the group—and probably several "someones"—carries the residue of this most intimate of assaults, deep in the tissues.

However, as injurious, insulting, painful, and transforming as sexual assault can be, it is not impervious to repair. People around the world are able to find relief, and sometimes they can leave their sexual trauma behind. Stories of success in this arena are plentiful—sadly, because the problem is so familiar to us all. In a recent BBTRS training seminar, the wide-ranging expression of sexual trauma was visible and palpable to the entire group. And then, surprisingly, the power of healing gobsmacked each participant.

While personal "stories" are not the foundation of BBTRS sessions or trainings, the complexity of the human experience becomes apparent as strangers participate in a shared experience over a week or ten days. In this particular case, over the first half of the workshop, a few of the younger women—let's call them the trio—were visibly shaken during exercises that involved personal reflection on intimacy. For example, in one exercise, participants were asked to consider ways in which emotion was visible on their faces, and how they could intentionally change their emotions. They were asked to walk through the group, meeting one another's eyes, with expressions of anger, joy, disgust, or sexual interest, among other feelings. Another exercise involved dancing, especially isolating and loosening various parts of the body, including the hips. These simple exercises were not so simple for people who associated negativity, pain, injury, fear, or self-doubt with the pelvis, sensual expression, feelings of physical freedom, or intimacy. Members of the trio occasionally collapsed, sometimes to the floor, unable to hold back tears.

The first expression of love and acceptance that occurred in the room was the group's instant allowance for whatever was needed. The participants neither ignored nor hovered around the women during their emotional expression. Everyone simply allowed, with warmth. Facilitators, of course, provided more specific support, and the trio were encouraged to participate or not, as was comfortable to them.

As the days went on, the women seemed to become more comfortable in their discomfort. They cried more openly, and displayed no embarrassment; they stayed within the group, and participated again as soon as they felt safe. Then, something amazing happened.

In a group sharing, the men were talking about their experiences during the workshop—about how they were showing their vulnerabilities, crying together, being willing to take emotional risks, letting down their guard. Then Jared, a man in his early thirties, gently said that he had noticed the undercurrent revealed by the women in the group. Something about the unguarded expression of these women had touched him, and he apologized. He apologized to all women on earth "for any harm they had experienced at the hands of men." He apologized on behalf of all men. Jared's heartfelt proclamation not only touched everyone in the room, but also it caused many women to weep—those few, as well as several others. It was clear that they, too, had been victims of sexual trauma at some time in their lives, and they received Jared's words with open arms.

The after-effects of the apology changed the dynamic of the entire group for the remainder of the event. Months later, participants were still discussing it. Change was evident in the demeanor of the trio; they had been heard. Their faces were more open, their smiles were wider, they were able to connect with everyone in the group more freely, and their participation was less constricted. They were, unsurprisingly, especially fond of Jared. Several other women thanked him, too—and so did the men. The cohesion of the entire group increased.

Thanks to Jared, the group learned many things. One was how essential intimacy is to humans, and how much they want to connect. Another was how essential acceptance is—not "tolerance," which assumes distance, but a true reaching out and drawing together of people with different experiences. Another was the importance of resilience. Everyone in the room was willing to change. Everyone did change. It didn't matter in what way exactly, or in what direction. But change occurred, and it helped the community. If it works among thirty people, it can work among more.

Working with sexuality

Sexual repression and misuse can lead to trauma in many forms, which can be perpetrated by a stranger, a friend or colleague, a partner, or even oneself. Inappropriate contact can span a wide range of acts, from overly enthusiastic hugging, to sexual harassment and abuse, to private obsessions with pornography. And, let's not forget: sexual repression, domination, and abuse is perpetrated on women, men, and those who do not identify with gender. It is perpetrated on children of all ages.

When working with trauma release in BBTRS, we need to be aware that we might be working with a person who lacks appropriate boundaries, someone who has suffered sexual trauma, or both. In any of these cases, we must begin the session slowly, careful to avoid retraumatization. First, since the deep, connected breathing in BBTRS can cause overactivation, the breather is encouraged to breathe relatively shallowly. Second, facilitators must use extreme care and consciousness when coming in physical contact with breathers dealing with sexual issues. Specifically, if the breather "leaks" sexuality—by allowing sexuality to interject itself into this nonsexual interaction—then boundaries must be set for both parties. There can be no hidden meaning in the interaction, because the purpose is to assist someone in deep, vulnerable release.

Third, when working with the energy of sexuality, it's also important to maintain a clear dialog regarding the felt sense, supporting the breather to be aware at each moment of what is happening within the body. If sexual energy arises, then the breather might be asked, "How does sexual energy feel in your body?" Usually, such energy generates warmth in pelvis. We ask, "If you allow yourself to feel it, where does this energy want to flow? Does it rise up, or stay where it is?" As discussed earlier, just accessing the felt sense allows the energy to shift and transform.

Movement in most sessions, whether focused on sexual energy or not, begins in the core. The hips are often the first area to move. When we awaken energy in this area, most of the time the vibration wants to move up the body. The energy builds and then turns into tremoring or unwinding that releases deep core tension. The movement can involve what psychotherapist Wilhelm Reich called the "orgasm reflex"—an involuntary movement of the pelvis that we often correlate with sex—but the movement is contained and redirected. It does not become sexual, and that's when it reaches the heart. Sexuality that comes from the mind is cut off from the heart, and lives in the genital area. When we allow it to move up the body in a wave of energy, the sexuality expands into something greater.

Sometimes, however, a purely sexual response does occur in the midst of a session. How this is dealt with depends on various factors. However, BBTRS work in general is not a genital-focused sexual experience. Most people can redirect sexual energy back into a full-body experience, thus intensifying the trauma release aspect. For example, in an afternoon seminar in Boulder, Colorado, a female participant was midway through her session when she realized that she had become sexually aroused. As a natural response, her breathing quickened, became more shallow, and finally stopped for short periods. As she held her breath, which is common in sexual activity, her focus shifted away from her whole-body experience. The facilitator realized what was happening and invited the participant to return to her deep, connected breath. The session shifted again. Afterward, the participant reported that she had been able to "direct" that sexual charge back into the rest of her body, moving beyond a traditional orgasmic response. She said her overall experience was much more profound than a sexual release would have been.

We find that even with the typical pelvic movement associated with BBTRS, men generally do not experience erections. If they do, the session is handled in the same way as described above. However, we find that the release experience is usually energetic, and can be completely blissful without being expressed genitally.

Kundalini rising

One exception to the arousal norm is an intense Kundalini response. Kundalini has been likened to having an electric current running up the spine. It can be intense, spiritual, and sometimes "ecstatic," and is often the result of intentional yogic, meditative, and spiritual practice. A term that is often used in tandem with kundalini, "tantra," in Sanskrit means "weave," or "system," suggesting its essence as a model. Some yogic and tantric practices and texts are geared to awaken and unlock the kundalini energy in the body, with the ultimate purpose of spiritual awakening and enlightenment.

However, because of the component of the kundalini experience that includes ecstatic energetic flows, some Westerners oversimplify both kundalini and tantra as primarily sexual experiences. In practice, however, the opening of the free flow of energy through the body can result in an experience where the "electric current" in the spine is extremely intense. The energy travels up the central nervous channel, and can create a sudden energetic release accompanied by a period of "no mind." A few of the many expressions of a kundalini awakening—which can occur intermittently over months or years—are: shaking, a felt sense of heat or light, out-of-body experiences, emotional or psychological fluctuations, a sense that something important is happening, and an increased interest in spirituality. Sexual responses also might spontaneously occur.

An ecstatic showdown

“Ron has been meditating and following a spiritual path for several years. He practices daily meditation, breathwork, and yoga. However, he was not prepared for what he would later understand as an ongoing kundalini response, which he documented in present-tense journaling. “Over several months, I have a series of out-of-body experiences. These occur almost daily—at least several times each week,” he wrote. “A wind comes through the room, and sweeps me up off the ground. I float gently into the walls and ceiling, cascading and bouncing along in the air. The feeling is delightful—expanded, carefree, sensual, unbounded, blissful...such a release and feeling of ecstasy.”

As he meditated, Ron sometimes spontaneously shifted into intense kriyas, or “locked” yogic postures. He became aware of an energetic blockage in his abdomen, which felt “tight as a drum.”

“The kundalini energy is pounding up from the base of my spine into this block, and is being redirected throughout my entire body. The hose in my sushumna (central channel up the spine through the chakras) is crimped. Not enough energy can pass through this block.” At this point, Ron wasn’t sleeping. His eyes were stinging. He couldn’t focus. This went on for months. Ron visited specialists in various fields, wondering if his problem was spiritual, psychological, physical, or what.
One internal medicine doctor conducted tests on Ron, and

diagnosed “stress.” He prescribed meditation. How ironic. Every time Ron meditated—which he believed was the key to his enlightenment—he would shake for hours afterward. He was basically releasing. And releasing. And releasing.

One of the most important resources Ron finally found was a facilitator. With support, he didn't feel as isolated, and the facilitator was able to suggest practices that offered some relief, thus calming his nervous system. “I am intensely relieved to find a guide who understands my experience,” he wrote.

Years later, Ron’s process continues. He has found balance in his life and is working on healing. “I bring my focus on a deeper softness and gratitude for being human—for being in my body.”

Because of the wide range of kundalini responses, BBTRS facilitators are advised to have some awareness of the scope of kundalini practices and responses. BBTRS breathers activate their autonomic nervous systems, and thus invite vulnerability and free physical expression. Their most intimate feelings and experiences are on display, and facilitators are responsible for holding a safe space. Therefore, if a kundalini response happens in a session, whether sexual or not, then facilitators monitor the intensity of it, and support the breather through the process.

The majority of breathers do not have Ron’s experience, with too much free-flowing energy. Everybody has history, however, and that history might include pain, dysfunction, fear, and repression. BBTRS facilitators usually see stress, tightness, blockages, illness, fatigue, poor posture, poor sleep patterns, repressed emotion, and disconnected sexual energy. BBTRS can assist in regulating anatomy, sexuality, and *xin*—mind and heart. It can help to release tension held in the tissues. It can unlock tight pelvic structures, allowing release and the return of dormant physical sensations. It can help us to be more energetically, physically, and sexually aware. With time and practice, even significantly constricted people can relax. They can feel more free, connected, and able to sense their bodies. Their breath and movement can become more natural, whether in session or out. With these tools, they can deepen and enrich their relationships with self and others.

At the end of the day, we are physical beings in a complex, multidimensional experience. Our job is to find full, compassionate self-acceptance. From this place, real love can arise—supporting body, psyche, and life.

Reflection

Thank you for reading this chapter. Now, please take a moment to bring your attention to yourself. Please read the following paragraphs, and then close your eyes and scan your physical body. Notice how what you read reflects in your body. What is happening in your body—is it relaxed? Perhaps start with the breath, and consider whether it is flowing freely. Then consider your posture, whether you are sitting or lying down. Do you feel balanced, or does your physical body have tightness, pain, or discomfort? Look at different aspects of your body, including inner and outer tension. Take a deep, full breath, and sigh out your mouth. Next, consider your thoughts—are they generally centered outside of you? If something external is consuming your attention, can you let it go for now?

Now, consider if the material in this chapter resonates in you, and whether the resonance involves you or someone you know. If there is some triggering—positive, negative, emotional, physical, spiritual, or some other experience—then what does it feel like? Maybe it feels like you're reading something intimate or personal or true about yourself—or maybe you didn't like something you read. What was it? Was the response because the information was revealing, challenging, nurturing, or something else? It's possible that something that's gone unnoticed within you, or something you've neglected or forgotten about, might really be valid and making itself known. It's possible that this material might have awakened it in you. Or not. You be the judge.

After reading these things, and reflecting on them with your eyes closed, then open your eyes and bring your attention to your surroundings. Listen to the sounds. What do you see? Between chapters, perhaps take a brief walk, or dance to a couple of songs. Move around, reorient, and then settle in for the next chapter. Thank you.

Chapter 8

Where trauma lives

Humans are born fragile. While many animals come into the world able to fend for themselves—some without any parental supervision—others require care. For example, sea turtles hatch on lonely beaches and make their way to the ocean. Their mom is long gone. In other species, such as the giraffe, horse, or deer, the mammal mothers are large enough to carry babies within their bodies for a long gestation. Like turtles, these young are born in a well-developed state. However, many animals give birth to hatchlings or babies that require a lot of care. Their offspring might be born with their eyes closed, and they might need their parent's body heat to survive.

Beyond these foundational physical needs, one other variable seems to play an important role in how much care offspring require. If most survival information is "hard-wired," then little ones hit the road running. But if offspring need training and context and subtlety, well, that's another matter. The more we need to learn from our parents, the longer we hang around the house.

When human babies are born, they need constant care for several years. According to Centers for Disease Control statistics, babies can't lift their heads without help for about the first two months. At about four months, they usually roll over for the first time, and sit up two months later. They don't even start standing until about nine months of age, and take their first tentative steps at around one year old. But this extremely delicate state is just the beginning. Survival skills, such as walking and feeding themselves, require another year or more. A general dependency on adults to navigate the world lasts more than a decade, and some development in the brain continues into early adulthood. This is the price we pay for complexity.[153] "The extra time that humans need to acquire these abilities is part of the evolutionary trade-off for having highly developed brains capable of managing complex reasoning, communication, and social interactions, alongside the physical requirements and capabilities of our adult bodies."[154]

Just think about all the opportunities for trauma in that drawn-out process. For a deer, trauma can be waiting just behind a tree, or around a bend. Very early in life, perhaps right after birth, those babies might need to run. But for human babies—just as in the rest of life—trauma can be much more complex.

The origins of trauma

We vulnerable humans are likely to be traumatized no matter how good our parental care is. Something is bound to happen that upsets or hurts us, environmentally or interpersonally. It could happen in utero, during birth, immediately after birth, or in the first few years of life. Once we have more complex social skills, trauma could result from family dynamics, social dynamics in the school or community, worries about a parent—perhaps in a stressful job, or away in the military—or the fallout from a single event. Perhaps a school shooting, whether near or far away, or the loss of a family member to violence. It could derive from medication taken by the mother, an accident, a variety of types of abuse, or the death of a caregiver. And consider the potential for trauma in children with physical, mental, developmental, or learning deficits. "Normal life" throws plenty of curveballs; extraordinary challenges are especially taxing on young minds and bodies.

Because of how our brains are wired to manage stress, both children and adults might not remember some traumatic events. Even so, this "developmental trauma" is common in the human experience, and it takes up residence in the body.

How we cope is a conversation that has fueled dozens of fields of study, many of which are touched on in this text. However, to streamline, let's begin with some assumptions:

- We all experience and store trauma as life goes by.
- No two people experience a given event in the same way. Beyond simple "point of view" differences, evaluating how people interpret the world or a specific event is impossible. The number and types of personal, environmental, biological, and psychological variables are too high to count and too nebulous to analyze.
- One measure of coping involves the resilience of each person's nervous system.[155] Some of us simply take longer to come back to neutral. Not to mention, "neutral" is different for each person, too.
- How trauma takes hold in our first years of life shapes us.

There are dozens of systems, theories, and approaches in the study of human nature. All of them agree that our first years are crucial to the process of who we become, how we think and feel as we grow, and the decisions we make in life. Psychologists and counselors study these theories and systems, and ultimately choose a methodology to approach their own work. They might focus primarily on conditioned behavior, cognition, personality, social psychology, or human development, among other fields. Often they choose a particular model developed by a theorist, and then build their own practice from a set of "tools" defined by that modality. Several theories have become mainstays in our dialogue about the human condition.

Lifespan and personality development

It's really quite amazing to review the various ways in which psychologists and other researchers have looked at how people develop. Freud introduced his famous psychosexual stages, and Eric Erickson modified them to develop a concept of psychosocial development. Jean Piaget focused on cognitive development, observing how children's reasoning, logic, and morals changed as they grew. Lawrence Kohlberg was most interested in moral development, which he associated with a person's understanding of justice. Although researchers and psychologists critiqued all of these and other approaches, everyone agrees that considering lifelong development changed how people think about people—and even added entirely new fields within psychology.

Carl Jung, who founded analytical psychology, also impacted many areas of thought—from psychiatry to literature to philosophy and architecture. His foundation involved how people individuate, differentiating themselves from others through elements of their conscious and unconscious. Jung identified several pivotal concepts, including extraversion/introversion, the collective unconscious, and archetypes. The archetypes are especially interesting, as they are thought to be universally understood forms that all humans share. They include: the relevance of major life events, such as birth and marriage; motifs, such as creation and the apocalypse; and figures, such as god, mother, father, wise elders, the hero, and the trickster. Archetypal figures appear in music, literature, and social and psychological dynamics—such as when people refer to their shadow side, or to opposite aspects of a single person (i.e.: the maiden and the crone). Terms like these have become cornerstones in our language.

More recently, Jeffrey Young developed schema therapy, which is based upon the assumption that when childhood needs are not met, people cope by adopting specific styles of behavior. Schema therapy, which draws on many concepts related to psychotherapy and attachment, focuses on easily triggered patterns that cause difficulty for people throughout their lives. For example, someone who struggles with an abandonment schema might be triggered regarding his or her value to others, which then could cause stress in relationships. Young identified eighteen specific schemas, including mistrust/abuse, abandonment/instability, emotional deprivation, defectiveness/shame, dependence/incompetence, failure, entitlement, and self-sacrifice, among others.

Young says that schemas are stored in the amygdala, the fight-or-flight center, and that the way people respond to triggers is similar to our other auto-pilot responses. "All organisms have three basic responses to threat: fight, flight, and freeze. These correspond to the three schema coping styles of *overcompensation*, *avoidance*, and *surrender*," he says (emphasis in original). "In very broad terms, fight is overcompensation, flight is avoidance, and freeze is surrender."[156] For example, "schema surrender" might involve passively giving in to our abandonment or shame issues—or "freezing." In avoidance, a person "runs away" from the trigger; perhaps someone focused on failure turns down a new job. "Fighters" might use overcompensation by doing the opposite of what the trigger suggests. Here, a person with a defectiveness schema might present as "perfect," which likely would lead to criticism.

Schema therapy has proven successful in the most challenging settings, such as with criminal offenders, those suffering with eating disorders, and people with the most chronic, entrenched mental health diagnoses, such as borderline personality disorder and narcissism.[157]

In all of these psychological and sociological approaches, thinkers have viewed human development through particular lenses that made sense to them. They identified ways to think about individuals, families, and larger social groups. In BBTRS, while many of these topics make sense and can be part of the "toolbox," one man's perspective has proven especially helpful. Because he combined how we think and feel with how our bodies work, Wilhelm Reich was the first person we know of to experiment with somatic release.

Reich and the body

Psychotherapist and author Wilhelm Reich began as a student and employee of Freud, but soon became the first practitioner to notice how the physical body held the tension that the client was talking about in session. One of his coined terms was "muscular armoring," which he said expressed an individual's personality in how his or her body moved. Reich noticed that this armoring seemed to be distributed in seven "belts" in the body, which inspired him to integrate therapeutic touch in his sessions. With this foundational concept, the idea of somatic release was born. Reich's work would go on to influence Gestalt therapy, bioenergetic analysis, and primal therapy, along with creating the field of body psychotherapy—the first "body-mind" approach.

As brilliant as Reich was, his life was evidence that humans are born fragile. He was both a wounded person and an imperfect scientist. His own developmental life was complex. Born to a non-practicing Jewish family of means in what is now Ukraine, Reich grew up on a farm. Because of the isolation of the location, Reich and his younger brother were home-schooled—until he discovered that his mother was having an affair with his tutor. He wrote in his journals that he struggled with whether to tell his aggressive and jealous father about the affair, but he eventually did. Afterward, home life changed forever, for which Reich reportedly felt terribly guilty. The children were sent to an all-boys school, and their father began a protracted period of abusing his wife. She finally committee suicide in 1910. The father died in 1914. The Russians invaded in 1915, and Reich fled, never to see his home or possessions again. Along with this tale, which was the subject of his first published paper, Reich's journals also contain much detail about his youthful sexual life, which was varied, precocious, and would inform some of his future research.[158] He was a conflicted soul with big ideas and nothing to lose.

In subsequent professional relationships, critiques, and published literature, Reich's no-holds-barred way of living and expressing himself caused controversy. For example, in his bodywork practice, he noticed pulsations and "electric currents" that patients described as "coursing through their body."[159] Sound familiar? It should. BBTRS practitioners work with these "currents" every day, and scientists have confirmed their source. What Reich's patients were describing was the tremoring and unwinding that comes with autonomic tension release, but this was definitely out-of-the-box thinking at the time.

Further, as discussed in Chapter 7 of this text, sexual energy has been identified as a powerful method to discharge tension. In the 1930s, Reich and others subscribed to a "bioelectric theory of life," postulating that "libidinal energy was electrical in nature and that the fundamental life process was its spontaneous pulsation, its rhythmic expansion and contraction."[160] He also found that his patients were generally repressed sexually. Controversially, Reich labeled the energy associated with sexuality as "orgone," and he associated it with universal energy, and even God. He talked about sexual relationships as opportunities to expand emotional health. One of Reich's most controversial efforts was the development of an orgone energy accumulator (ORAC), a box he hoped could gather this energy of life. The response of the scientific community to this concept was critical, and eventually would lead to Reich's partial fall from grace.

However, Reich still has avid supporters. Richard Blasband, a psychiatrist who specializes in the interface between consciousness and life energy, integrates Reich's work in his practice. In a book review, he explains the essence of Reich's ideas about sexuality. "Emotional armoring against pleasure, established in childhood and perpetuated by a sex-negative society…(affects) the quality of satisfaction and gratification in the sexual embrace."[161] Foundationally, Reich believed that only sex with love provides the richness of human experience. "Primary drives are impulses moving out towards the world unimpeded by armoring, from the deepest part of the self—love fused with eroticism. Secondary drives are primary impulses that become distorted as they are expressed through the psychic and muscular armoring—loveless sex, sadism, pornography. In Western society, sadly, the primary drives are hardly known; the secondary drives are considered to be the 'norm,' one's 'nature.'"[162]

Reich found that these energies were also a mirror into a person's childhood and family development. He proposed a model of the human condition in which energy is a primary component of all matter and space. He suggested that energy was important for all living organisms, and is a universal life force. He wasn't the first to suggest such a thing. For thousands of years, Vedic tradition has called it Prana, the breath of life. The Chinese call it Chi. With this general outlook, Reich worked with individuals on their personal and relational issues. He saw how energy flowed in their bodies, how tension blocked their movement, and how they held themselves differently after tension was released. While sexuality was a major focus area, it was not the only one. He also believed that childhood development was critical to how a person viewed every part of his or her life. Reich was into holism.

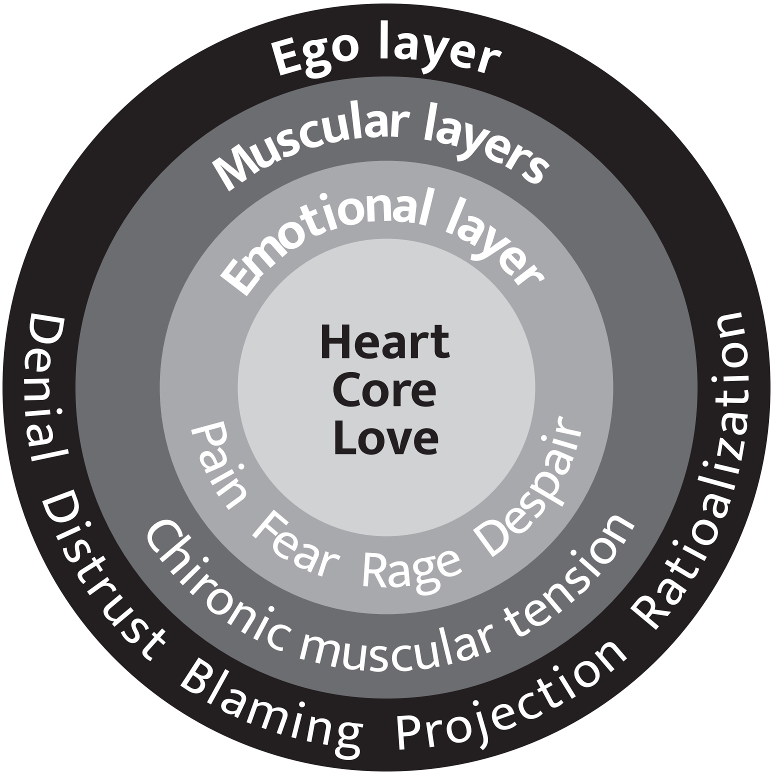
All of these elements are primary tools in the BBTRS toolbox. They bring together family, biology, environment, DNA, world events, society, growth, and temperament. They require emotion, cognition, and spirituality—*xin* and *kokoro*. Throughout this book, we have shown that BBTRS integrates the thinking and approaches of many different scholars and modalities, some thousands of years old. Childhood development is the last foundational keystone that we consider when approaching our breathers' bodies, because it can be such a rich source of information. It can also be a rich source of healing.

Let's now look more closely at how Reich and his colleagues believed that tension is written into our bodies.

Reich's personality structure

One of Reich's students from the 1940s and early 1950s was Alexander Lowen. With another colleague, John Pierrakos, Lowen developed a new form of mind-body psychotherapy, called bioenergetic analysis. Like Reich's theories, bioenergetic analysis assumes that energetic blocks are present in the body as chronic muscle tension. Beyond Reich's theories, Lowen added additional focus areas, including grounding with the earth, and several Freudian concepts, such as transference, countertransference, slips of the tongue, and Oedipal issues.

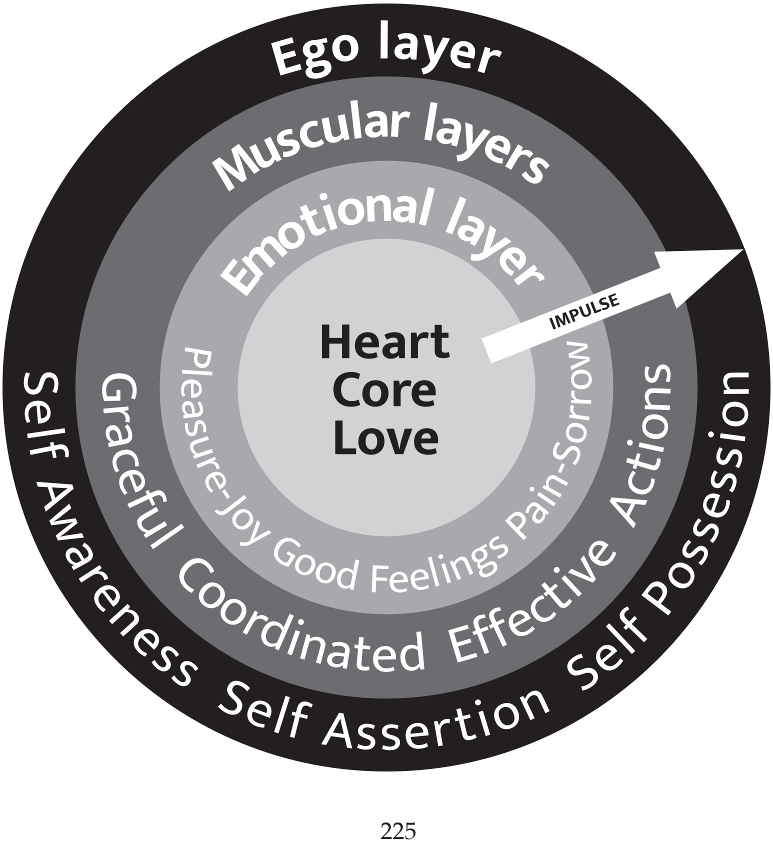
Lowen also developed a theory that expands on the importance of childhood development in the structure of personality. He postulated


Ego layer
Muscular layers
Emotional layer
Heart
Core
Love
Pain Fear Rage Despair
Chironic muscular tension
Denial Distrust Blaming Projection Ratioalization

that we all have three "layers" built around a core, like a bullseye, or an onion. Called the Formation of Personality Structure, the model was built upon the basic premise that children reach to their parents for pleasure and comfort. If they are met with deprivation, frustration, or punishment, anxiety naturally results. Depending on how chronic and strong the deprivation is, defenses move deeper and deeper toward the core.

The innermost layer is the heart, from which we reach to the parent for love. Covering that layer are: emotions; our physical bodies, or the

These "bullseyes" depict Lowen's Foundation of Personality Structure. The graphic on the previous page illustrates human development in the face of deprivation. Below, the positive manifestations of love and support "light up" the core, soften the emotional and muscular layers, and allow the heart/core energy to flow outward.


Ego layer
Muscular layers
Emotional layer
Heart
Core
Love
IMPULSE
Pleasure-Joy Good Feelings Pain-Sorrow
Graceful Coordinated Effective Actions
Self Awareness Self Assertion Self Possession

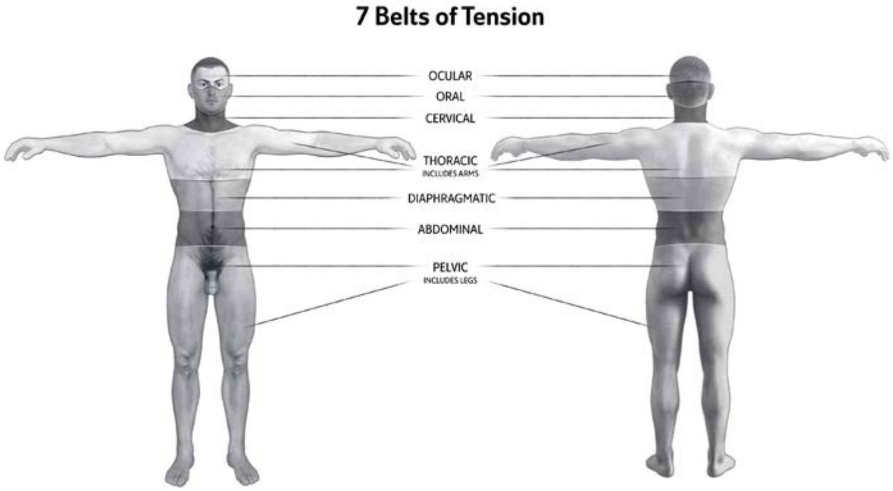
muscular layer; and the outermost ego layer, the one we present to the outside world. Lowen assumed that tension, anxiety, and trauma exist in everyone's life, and that each of us manages how these anxieties manifest. The model shows how deeply tension lodges—and therefore, how deeply we must go to release it. In BBTRS, the focus is on the physical, or Layer 2. However, as we work on muscular tension, the two surrounding layers are also relaxed and supported. Therefore, as the physical body gains freedom of movement, energy is able to move throughout the system. This allows for positive and supportive aspects to replace the defenses.

Once we get to the core layer, we unlock the mission of the heart energy, which is to be met by another person. The heart can make this connection once chronic muscular tension dissipates—which allows the ego and emotional manifestations to open, surface, and release. With release, the heart's energy has a chance to reach to the periphery and effect the outermost layer of the personality. The heart's energy is powerful, and affects every layer as it moves outward. This entire process happens when we access the muscular layer. Our "portals" to the muscles are specific segments, or "belts" in the body.

Felt sense 2.0 — Belts of tension

Reich believed that when the muscles contracted, energy would not flow through the body. He used the metaphor of armor to describe the contraction, since it is a protective response. However, as we know, the protective effort is actually harmful. By attempting to avoid trauma, we inadvertently lock it in our tissues. As we avoid feelings, sensations, emotions, and sexual responses, we bring on muscle spasms, decreased motility, and postural misalignments. Reich believed we even armor our "characters," which leads to emotional rigidity, a lack of aliveness, and poor relational skills. He said that armoring is our attempt to prevent the spontaneous release of emotions, and evidence of armoring is obvious not only in how we carry ourselves and interact with others, but even in each person's body type, which is determined in childhood.

In BBTRS, we see armoring in how bodies move, and where they seem stuck. Our observations correspond to Reich's Segmental Armoring Theory, in which he identified seven segments or belts of tension, which roughly correlate to the chakra system. He was the first Western thinker to relate this energetic system to psychology, although he did not use the term "chakra" in his work. Still, the concept was the same. Energy flows up and down the spine, from the pelvic floor to the crown of the head. Like dams across a river, the belts of physical tension work at right angles to the flow of energy. The tension literally stops the flow of our life-force energy, and the result is obvious in the body. The tissues armor these segments in an effort to protect from trauma, and to achieve psychic equilibrium. Of course, with time, it's very clear that this



The Belts of Tension, as identified by Wilhelm Reich.

"protective effort" actually impairs our equilibrium, retains the trauma, and diminishes our quality of life.

The type of trauma a given person experiences in life determines the body's expression of the trauma. In addition, childhood, family, and social conditioning around certain parts of the body cause us to be more or less sensitive, or armored, in those areas. For example, a household of repressed sexuality likely will produce people with contractions in the lower part of the body. A devastating heartbreak might result in armoring around their heart. Not being allowed to express verbally can constrict around the throat and jaw.

Walking on the sun

Andreas was thirty when he came for a week-long intensive with BBTRS practitioner Prema McKeever. Andreas described being paralyzingly fearful. He had an overly sensitive startle reflex, difficulty relaxing, and an inability to express himself in relationships. "In fact, when he was 28, he had a long-distance relationship, which was great—at first," McKeever recalls. "When they met face-to-face, the couple had an amazing connection—but when she told him she loved him, Andreas simply froze. He wanted to push her away. He said the bones in his face started to hurt—he said that when she came close, it was like walking on the sun." Those were pretty startling, intense descriptions.

A triplet, Andreas had been only three pounds when he was born. Photos of him as an infant showed him in an incubator, covered with wires. He said his family never discussed how long he had remained in the incubator, but it was clear that he had not been held for an extended time as a baby. McKeever says that when she met him, Andreas could not make eye contact, and his speech was fragmented. "He was in a functional-freeze response," she says. "He was just so afraid."

Their BBTRS sessions began with the top belts of tension. "There was a lot of release, and Andreas said he felt very open," McKeever says. "By the time we got to the fourth belt, where he did some heart work, Andreas was able to look me in the eyes. He started talking and sharing. At one point, he stopped himself and said, 'Wow, I have lots to express. My heart is open.' He finally didn't have that feeling of shame. He felt warmth in his heart—and he felt love."

Understanding what is associated with each belt helps a BBTRS facilitator and the breather to target the felt sense during a session. The process works by noticing tension, constriction, or other sensations in a particular area, and then "breathing into" that area, or focusing on that area while inhaling. Also, depending on how that part of the body is moving, the facilitator might apply touch to assist, or unlock movement.

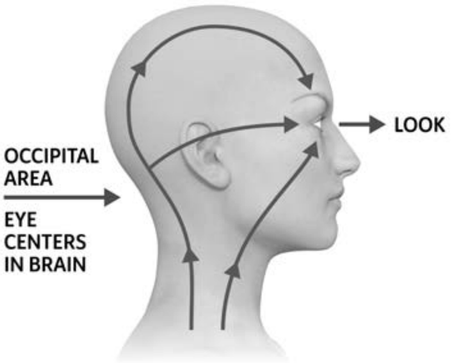
In BBTRS, we move from the top down, which allows trapped sexual energy to move up and out as it is released. Like water finding the path of least resistance, energy is always looking for empty space. Therefore, we clear a path by removing muscular tension and emotional blockages, starting with the head.

With this approach in mind, the belts are outlined and described below. Here, we include what the belts are, the emotional expression that is related to each one, and how tension can be held there. In Part III, we will discuss how to work with specific belts in sessions. Supporting exercises and meditations for each belt are included in the Appendices.

Ocular Belt: Eyes, Forehead, Top of the Head

Emotional Expression: Suspicion, amusement, contempt, detachment, guarding, fight, anger, and grief

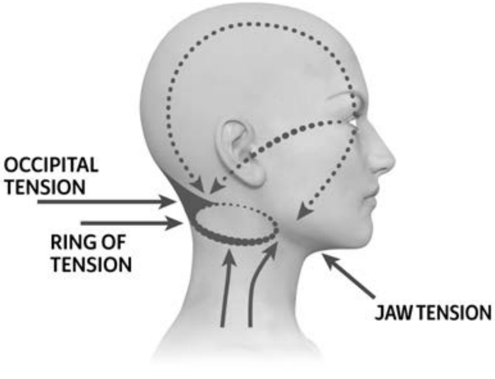
In Russian, we say, "Fear has big eyes." Fear saturates everything, and it is very obvious in the face. What do we do when we suddenly look up, startled to see a bear standing in front of us? Our eyes open wide. The same is true for a person who is constantly looking, listening, and preparing for danger. Therefore, tension in the Ocular Belt is like asking, "Where is the next danger coming from?"



The eye gathers visual information, assesses the safety or threats in the environment, and sends nerve impulses to the occipital area for processing. Our bodies then respond accordingly, by either moving or storing any tension that results.

Continual fear and stress can create headaches; tightness in the forehead, cheeks, temples, and occipital lobe; tension around the ears and in the muscles around the eyeballs and pupils; and even hair loss in the scalp. In extreme cases of fight-or-flight activation—such as in war environments or unsafe home environments—fear can cause tunnel vision. In these cases, people automatically block out what they are seeing. The ocular belt is also affected by frustration. We can't concentrate well when we're stressed and frustrated, despite the level of energy pouring into an overactive mind.

In BBTRS, we have worked with some individuals who, after releasing fight-or-flight tension, experienced significant change in this belt of tension. On occasion, diminished hearing has improved; more often, vision sharpens. In one case, a breather could again read a medicine label without glasses. In addition, we have seen improvements to some ocular-belt deficits related to psychosomatics—conditions caused or aggravated by internal conflicts or stress. For example, although Ray



A more detailed graphic demonstrates how tension collects based on the flow of energy between the ocular belt and the back of the neck. Depending on the threat we experience, tension can gather in the ring near the occiput, as well as in the jaw and upper neck.

Charles's blindness has generally been attributed to glaucoma, people in our line of work wonder if his loss of sight was actually caused by his witnessing the traumatic drowning of his brother. At the time, Charles was only four years old; he was blind by the time he was seven. Wouldn't it have been interesting to have worked with him on trauma release?

Oral Belt: Mouth, Chin, Nose, Jaw, Ears

Emotional Expression: Contempt, disgust, longing, desire, fear, pain, anger, connection, self-awareness, and presence

When measuring strength in the body, there are a few different muscles that win in different categories. The heart muscle is considered the hardest working. The soleus, in the calf, can pull with the greatest force. The gluteus maximus is the largest muscle in the body, and is capable of explosive and dynamic strength in hip extension. The uterus, of course, can stretch and contract with enough force to control childbirth. Among these top-ranked muscles is the masseter. The bite strength of an average man is between 117 to 265 pounds.[163] This means that traumatic stress can express itself in extreme tension and significant pain symptoms in the temporomandibular joint (TMJ). Our mouths are key to our expression, and they are intimately involved in our consumption of what sustains us. When communicating, we often watch another person's mouth move; close contact with the face is also very intimate. The whole area around this part of our bodies is powerful.

Now, consider the Oral Belt of the high-intensity courtroom attorney or stock broker, for example. We often see "Type A" people with a strong lower jaw that projects forward. The muscles in their jaws can be in constant constriction, or you might see the muscles pulsing. These people often grind their teeth, when asleep or awake. They get headaches. If they are junior in the hierarchy, they might struggle to balance confidence with a fear of being reprimanded. They might feel strong anger, but can't open their mouths to speak about it. If they are senior in the hierarchy, they might feel the weight of the world hanging on their every word.

Natalie breaks through

Natalie first attended a BBTRS training workshop because she suffered from depression, anxiety, and addiction. Early into the multi-day workshop experience, she approached these issues through working on the Oral Belt. Natalie's expression—her voice, which is associated with the throat, jaw, and neck—was "frightening territory." At first, she encountered great fear when she heard her own voice. This is her account of this pivotal day in Goa, India.

"During the pre-session exercise, we partnered up for an exercise called Lion Heart Meditation. In this exercise we faced our partner, made eye contact and used the body, facial expression, and voice to mimic a lion who is protecting its territory. Both partners participated at the same time. In essence, it is an exercise in asserting and holding boundaries—a concept that was very new for me.

"When confronted with my partner's 'ROAARRR,' I immediately cowered and collapsed. And when I attempted to use my own voice to stand up to my fellow lion, my body became weak, wobbly, and full of nausea. I vomited, cried, and collapsed in a heap on the floor. Then, with the gentle encouragement of facilitators standing on either side of me, I was helped to my feet and continued with the exercise. The nausea and collapse continued, until I really found my voice.

"This was a moment of liberation. Not only had I found my voice to express boundaries, but also I could clearly see myself as a child—in fear of my father's rage all those years ago. It was as if part of my being had been frozen in this trauma-induced state. Here I was, finally, moving through it as an adult. And I could see my own behavioral patterns of life in general—collapsing in overwhelm, staying small to survive.

"And the fact that my own vocal expression caused nausea and vomiting really made sense. Throughout my childhood I was continuously reminded that 'children should be seen and not heard,' and that personal expression simply was not an option in my family environment. In this BBTRS exercise, I met with a lifetime of swallowed, stuffed-down, and repressed expression. It came to the surface, and was fully released thanks to the safety, guidance, and resourcing that I felt and received during this workshop.

"The full breathwork session that followed this exercise was an organic continuation of the previous exercise. During the breath session, specific bodywork was applied to work with the fascia in the throat and neck, the sternocleidomastoid muscle, and the tension in the jaw, all whilst maintaining a conscious, connected breath pattern.

"At some point during the session I 'checked out,' encountering a vivid memory that was somehow associated with the distinct odor of wet sheepskin. In the memory, I saw a scene from my parents' bedroom when I was around seven years old. My father was being violent with my mother, and I witnessed his raping her. In this memory, I could hear her screams. I could see in detail a picture that hung on their bedroom wall, a picture of horses running, something I had completely forgotten about. I could see and feel the sheepskin rug on the floor under my feet. I collapsed onto the rug, buried my face in the fur and cried...the smell of wet sheepskin.

"What felt like an eternity was actually just a few moments before my partner brought me back to my physical body, inviting me out of the collapsed and coiled-up position I was in. I continued to breathe deeply and felt a surge of shaking in my core, a desire to move and stretch my body, to kick and push with my arms and legs, and then an emotional release of tears.

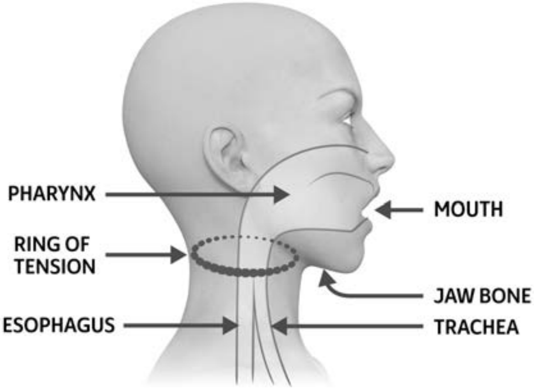
"After returning to resource, a huge sense of resolution and forgiveness occurred. I felt so much love and gratitude for my parents and all that we had experienced together, for everything that led me to be there in that workshop at that very moment in time. I also felt a very strong sense of the lifting of something very heavy—like a thick, dark blanket. Something that had always been present throughout my life was being lifted from my being. Later, I realized that this was depression leaving my body. I was left with this new spacious, light feeling. I had never felt so alive, never felt so connected with myself or others. I could maintain eye contact with others without feeling the urge to look away in shame. I felt grounded and joyful.

"This session was more than a breakthrough. This session changed my life."[164]

Cervical Belt: Deep Neck Muscles, Tongue

Emotional Expression: Self-pity, helplessness, longing, hurt, fear, pain, rage, freedom and creativity, self-expression

The Oral and Cervical Belts work intimately together. Although the mouth is contained in the Oral Belt, think of the Cervical Belt as the "voice." Even when some people are fully charged with breath and want to make a sound, sometimes they just can't. Their throats simply will not allow sound to escape. They might have been reprimanded as children, and prohibited from using their voices to express themselves. As adults, we hold back our words for many reasons, personal and professional; we might wake up with a sore throat the morning after repressing what we wished to say. We might experience chronic tension in the throat and neck, stiffness in the trachea and vocal cords, and a restriction of breath. Neck tension causes people to sink their head and neck down into their shoulders, pulling themselves into their "shells," like a turtle. These constrictions do more than keep our physical mouths shut. They also impact creativity, the expression of our inner selves.



A person's "voice"—his or her ability to speak, literally or figuratively—can be impacted by a variety of influences. Often, the oral and cervical belts contribute to the tension in the cervical belt, which is distributed in a ring just below the jaw.

Consider a backyard full of children, racing about with natural, unbound energy. Suddenly an adult enters the fray and tells everyone to settle down. The children swallow both their sound and their creativity. People who have been “shushed” a lot might have natural talent, but even as adults find themselves unable to sing in a choir, for example.

Thoracic Belt: Shoulders and Muscles around Scapulas, Chest, Intercostals (muscles between ribs), Heart, Lungs, Arms and Hands

Emotional Expression: Deep heart feelings, grief, mourning, longing, love, fear, rage, anger, joy

The Thoracic Belt is the area deeply impacted by the childhood impulse to reach out for pleasure. When our developmental needs are not met, and we are met instead with deprivation or punishment, chronic tension results. For adults, traumatic heartbreak also can cause knots in this area.

In either case, sometimes years might pass before a person recovers; some people never recover. The armoring we put in the Thoracic Belt protects our hearts, but it doesn't stop there. The heart meridian travels from the armpit, down the inside of arm, to the pinky. This is why tension around shoulder joint can also interrupt the flow of the heart to the periphery. Along with shoulder stiffness, thoracic belt of tension expresses as a collapsed chest, thereby affecting the lungs. The person looks like he's hanging on a clothes hanger. Suspended. In other cases, tension in this part of the body is vividly expressed through interaction. The best example is when greeting someone with a hug.

If we hug someone who is not expecting a hug, he or she might stiffen up. The hug becomes a pat on the back, and the pelvis juts backward so as to avoid contact. The non-hugger goes through the motion, but does not surrender. This means that when we come close to a new person, we can understand his or her emotional state. Expressive, or stiff and withholding? Our nervous system instantly reacts; we instinctively know if we face threat or safety.

In BBTRS, we can see thoracic tension or relaxation in different cultural or religious groups with different interpersonal conditioning. For example, we've noticed that South Americans and Mediterraneans are more likely to touch one another and to generally express themselves than are those from Scandinavian countries. Perhaps it's the weather. In other cases, it might be related to high population levels. In India, people generally greet without touching, choosing instead a chest-protective *Namaste* gesture. In still other cases, whether we allow our thoracic region to contact others is firmly rooted in culture. In general, the more orthodox the religion, or in colonialized areas, the less we find interpersonal touch. For example, traditional Jewish and Muslim men cannot even shake the hand of any woman who is not directly related; in colonialized Asian islands, those who follow Christian teachings are much less likely to express themselves physically than those who are more traditional.

These cultural norms are passed from generation to generation. In populations where touching is supported, we see that full-body hugging supports oxytocin production and cardiovascular health. How is that? In the same way as when babies are held—we feel safe and comforted. Our whole system relaxes with full contact. When we connect through the chest, and touch the person's back, it feels like our hearts meet. The moment we start to receive comforting, healing touch, the energetic field of our hearts extends outside of our bodies, and we get sort of high on oxytocin. This happy chemical is a feel-good hormone that releases naturally and impacts us physiologically and emotionally. The hug, then, is full engagement of the Thoracic Belt.

Speaking from the heart

“My father died of a massive heart attack when he was 72. A victim of circumstance, he had never expressed himself in his life. He was a tool-maker with dreams, a man who felt stuck in his profession because of post-war needs in the society where we lived. Even years after the war, he did not change his work, because he was afraid of not being able to support the family. By nature, my father was an introverted, artistic man. Classical music was his escape, and the only time he showed emotion was when he listened to it. Even though he was sensitive, this was the only time I saw him cry. In my family's conditioning, this kind of expression indicated weakness.

In other circumstances, my father would have been a musician or critic or writer, and a much happier man. That was his calling in life. My dad shared this story with me when he was in his late 60s. He said, "If I could do it all over, I would be a musician. I would never marry. I would travel the world like you do." Although he was very schooled in music, he never learned to play an instrument. He lived vicariously through me. My heart went out to my father, and I saw his decisions written in his body. When his heart finally failed, I realized that he had carved it out of his chest for his entire adult life. He had, in essence, given it to me.

Diaphragmatic Belt: Diaphragm, Stomach, Solar Plexus

Emotional Expression: Pain and pleasure, blocking of all sensing and feeling of the lower segments, such as sexual feelings, excitation, rage, hate, terror, strength, and empowerment

As the bridge between the upper and lower segments of the body, the diaphragmatic area involves the rib cage and all supporting structures. As the diaphragm is so intimately involved in our ability to breathe deeply, stuck emotion obviously affects the volume of the breath. Likewise, our breathing affects how we activate and use our diaphragm. Therefore, the inability to emotionally express affects our ability to flex the rib cage and expand the lungs. If we stop breathing, then we stop feeling.

In Chapter 7, we talked about the relationship between the diaphragm, the heart, and the psoas, or hip flexor. The volume of breath we inhale directly impacts the expansion of the lungs, our ability to feel emotionally, and digestion. When the diaphragm moves freely, it activates the internal organs in the abdomen, enhancing digestion, lung and heart movements, overall energy level, and long-term well-being. Because of the relationship between breath and general health, we know that tension in this belt causes us to "cave in," resulting in significant pain and diminished oxygen levels, which in turn are related to many kinds of disease.

An extreme example is the "dowager's hump," which is an abnormal curvature of the thoracic vertebrae of the upper back. In the Western medical model, this condition is usually associated with osteoporosis that causes compression fractures of the associated vertebrae. The inward folding of the spine also causes a restriction of movement in the diaphragm and heart. Emotionally, therefore, the dowager's hump is related to unexpressed feelings—which in turn are connected to collapsed breathing. These sufferers are basically "not breathing." If they truly breathed, they would fully feel their pain.

Abdominal Belt: Abdominal Muscles, Lower Back Muscles (Latissimus Dorsi and Sacrospinalis)

Emotional Expression: Pain, fear, pleasure, trust, all feelings related to nourishment and being nourished, relaxation of the center of the body

Everyone knows what a "gut reaction" is. Everyone has experienced a stress-related stomach ache. Closely connected to the Diaphragmatic Belt, constriction in the abdominal region closes off our internal spaces and our breath. By holding our abdominal muscles tight, we literally can't move or breathe. Extreme stress causes us to tighten the belly, which constricts the diaphragm. This kind of constriction is common with emotional trauma, sexual abuse, and anger, and can result in Irritable Bowel Syndrome, constipation, indigestion, gastritis, and problems with the liver, spleen, and gallbladder.

Pelvic Belt: Almost All Pelvic Muscles, Including Genitals, Urinary Tract, Buttocks, and Leg Muscles (Thighs to Lower Legs)

Emotional Expression: Sexual feelings and arousal, sense of power, anger, rage, anxiety, terror, panic, and pleasure

The pelvis houses the primary core muscles, including the psoas. This area envelops and supports the bladder and sex organs. A healthy core is important to the health of peripheral muscles. Therefore, with its crucial participation in fight-or-flight, the Pelvic Belt is also a primary location for tension. It causes the core to become weak and overextended. Along with general fight-or-flight related tension, sexual abuse, rape, and misuse of sexuality all take hold in this region.

In BBTRS, we've found that people manifest diseases related to how much trauma they're holding in specific belts. When people are contracted or conflicted about sexuality, the resulting tension has been seen to relate to sexually related disease. Because of the intimate connection between sexuality and quality of life, pelvic discomfort, contraction, and disease also blocks the channeling of sexuality to the heart. On the other hand, the moment we relax into sexual energy and heal traumas throughout the body, we gain access to our creativity.

Our legs are also part of this belt. They keep us connected to the earth. With sexual and/or creative repression, we feel disconnected. By forgetting to go barefoot and have direct contact with the planet, our "roots" are cut. Our legs and feet bring us full-circle, back to the holistic view of life-force energy, Prana, Chi. Our Pelvic Belt is the source of foundational, nourishing vibration. When we sacrifice a natural lifestyle for modern conveniences—even rubber-soled shoes—we slowly disconnect from our roots. But it's never too late. Start by having a stroll on the beach or a patch of lawn, or stand on a rock somewhere.

Pairings across belts

When working with belts of tension, we have learned that some belts work in tandem with others. Although we usually focus primarily on one segment per BBTRS session, we include adjoining segments as well. This process causes facilitators to bear in mind how tension in neighboring belts might present itself, and how interventions can be paired naturally. For example, when working with the cervical belt, the oral belt will have to come along for the ride. If the jaw is tight, then we can't release what the throat is holding. Likewise, the diaphragmatic and abdominal areas are intimately connected.

In addition, it is important to consider the rules about the direction of the work. For example, opening in an upper segment, like the throat, may lead to a spontaneous opening and sudden flow of energy through a lower, connected segment, such as the diaphragm. As the energy flows, the higher segments might not be prepared for the emotional material flooding up, which can lead to even stronger constriction of the upper segment. Therefore, it is important to support the integration of neighboring segments through conscious grounding and maintaining eye contact with the breather. More on these techniques will be discussed in Part III.

Eyes / Arms, Hands, Feet, Legs

The eyes, along with our physical “contact points” to the earth, are the primary agents of our contact with reality. We see reality. We stand on the ground and grasp the world with our hands. In order to strengthen a person’s grounding—the actual interaction with the world—we connect the energy flow into the extremities. Energy flows particularly into the eyes, through movement, and the hands and feet, through pushing. By using the felt sense, the breather begins to develop a lasting awareness of these body parts and boundaries.

Mouth, Jaw / Genitals, Pelvis

The movements of the jaw are closely related to those of the pelvis. If the jaw is locked, the pelvis will also be locked, and vice versa. Similarly, a loosening of the jaw will influence a spontaneous loosening of the pelvis.

Throat / Diaphragm, Ribcage

Since both are involved in the formation of word and speech, we find a close connection between these two regions.

Mizuki finds her voice

“In a BBTRS demonstration session that began with a Pelvic Belt focus, Mizuki’s life unexpectedly changed. Her body simply took over.

Although Mizuki was nearing the end of her BBTRS training, and was very familiar with the protocols, she felt nervous as she settled into position. She described a “tension in the heart,” along with “stiffness in the shoulders and tension in the pelvis.” She was aware of the other trainees who were watching, but she felt supported by their presence. She had briefly reflected that a major problem of hers had been “solved” in her last training, and so she was not expecting anything in particular on this day.

Mizuki’s definition of a “major problem” was debilitating abuse at the hands of her mother. Mizuki later said that her mother had been “beating me, shouting and yelling at me” throughout her youth. “She even tried to kill me with a kitchen knife, although she doesn’t remember it. She was drunk.” The mother’s unpredictability and violence had wreaked havoc on the entire family. Her father had had an affair; her brother had put his head down, taking care of himself. Mizuki’s response had been to feel like a victim, and to blame her mother for everything negative in her life. They had fought all the time.

But those days were over, she thought. In BBTRS sessions, she had released the trauma associated with her mother’s treatment of her. She had seen what happened between them in a kind of internal, distancing movie. "Once I knew how to watch myself, I could stay out of the drama," she said. Therefore, she no longer felt victimized. She no longer identified as a victim—and therefore, had no idea what might happen next. As she sat across from the facilitator, she certainly wasn't thinking about the potential connections between different belts in her body. She didn't anticipate that her voice—expressed through the throat, mouth, and jaw—would soon be in relationship with her pelvis.

The session began with four exercises associated with the body's root (see the Appendix for exercises that complement each belt.) The exercises began on the floor, with Mizuki on her back. She arched her back as in a yoga bridge, bounced her pelvis on the floor, and invited tremoring by holding her knees apart. Even during these initial exercises, she began emitting vocal tones. Her trainer explained to the group that this emotional expression was a bit unusual, and especially for Mizuki. Mizuki was always very quiet, and even one year ago, in the last training, she would never have let herself do this. Because of her nature, everyone in the room supported her expression.

As the session progressed, Mizuki's toning continued. For at least thirty minutes, she was almost never silent, and at some points, she even yelled. Possibly because of her experience with BBTRS, and with self-regulating—and with support from her facilitator—these vocalizations never fell into catharsis. Mizuki maintained connection to her physical body, and released tension through unwinding and tremoring. But her voice simply would not stop.

"I was finally able to speak," she said afterward. "I was perfectly into the session. Even though I was in front of everybody, I felt no panic, no drama. 'This is what's going on, this is what I'm supposed to do,' I thought. I didn't know what was going to happen. I felt every tension as it came up—each one came as an image of what happened when I had stopped talking in my life. And then each feeling of tension went right into the throat."

Mizuki realized that the trauma she had previously released was about her mother's actions. Today's session was about expressing herself. "I had never screamed and shouted," she said. Every single moment in the session, that's what my body wanted to do. I surprised myself. Now I feel like a different person."

Mizuki had experienced many challenges throughout her training, especially a tendency to "check out." At first, she simply had avoided looking at the pain. For several years, she also had turned to meditation. There, she relied on silence, a quiet mind, a place of distance from her history, and an integration of her Japanese culture. She had been raised with spoken and unspoken rules, etiquette, politeness. Her mother had used alcohol to lose control, but there had been no room for anyone else to lose control. There was no talking back. There was no talking at all.

For Mizuki, this pelvic belt session was about owning her right to express. After the session, which she said was the best in her life, she looked like a different person. "I enjoyed it so much. I used to be ashamed and shy. Now I just wanted to hear my voice," she said. "Joy is inside me. Wow! That's really inside me."[165]

Mizuki after her breakthrough session.

As practitioners begin to experiment with the felt sense, they find their own truths about how tension is held in their bodies. They might see correlations with what Reich identified, and they might find their own new associations between tension and physiology. The key to understanding how we develop as children—and how we carry this development through our adult lives—is that we have the ability and the freedom to change. So often, we feel stuck in our circumstances. We might feel like our family history hangs like an albatross around our neck, and that we forever will carry our inflammation, our tight jaws, our diabetes, and our knee pain. We often simply assume that "everyone has pain," and this laundry list of ailments is simply ours.

There is another possibility. Without suggesting that all people can experience pain-free lives, we can say for sure that relief is available. By attending to these belts of tension, exploring what is held there, and releasing what we can, perhaps the laundry list will become shorter. Perhaps the spring will return to our step. Perhaps we will laugh more.

And perhaps we will allow ourselves to live the lives we would like, instead of the ones we feel chained to. Perhaps we will use our own hearts and throats to express ourselves, and nurture our children by example. Perhaps we will inspire them to imagine large and full futures—and they will inspire the seven generations after them.

Reflection

Thank you for reading this chapter. Now, please take a moment to bring your attention to yourself. Please read the following paragraphs, and then close your eyes and scan your physical body. Notice how what you read reflects in your body. What is happening in your body—is it relaxed? Perhaps start with the breath, and consider whether it is flowing freely. Then consider your posture, whether you are sitting or lying down. Do you feel balanced, or does your physical body have tightness, pain, or discomfort? Look at different aspects of your body, including inner and outer tension. Take a deep, full breath, and sigh out your mouth. Next, consider your thoughts—are they generally centered outside of you? If something external is consuming your attention, can you let it go for now?

Now, consider if the material in this chapter resonates in you, and whether the resonance involves you or someone you know. If there is some triggering—positive, negative, emotional, physical, spiritual, or some other experience—then what does it feel like? Maybe it feels like you're reading something intimate or personal or true about yourself—or maybe you didn't like something you read. What was it? Was the response because the information was revealing, challenging, nurturing, or something else? It's possible that something that's gone unnoticed within you, or something you've neglected or forgotten about, might really be valid

and making itself known. It's possible that this material might have awakened it in you. Or not. You be the judge.

After reading these things, and reflecting on them with your eyes closed, then open your eyes and bring your attention to your surroundings. Listen to the sounds. What do you see? Between chapters, perhaps take a brief walk, or dance to a couple of songs. Move around, reorient, and then settle in for the next chapter. Thank you.

Chapter 9
Resourcing

In the fight-or-flight process, *even if we triumph over a threat*, we come face-to-face with danger. The danger can be real or imagined. It can be very sudden or long-term. Regardless, in the moment of activation, we survive thanks to our innate and learned strengths, our support systems, and our comforts. If we're suffering with residual trauma, it might not feel like our strengths are strong enough—but they do exist. They are tools in the toolbox. They might be literal, figurative, emotional, mental, physical, physiological, or spiritual, and they are with us every day. Employed intentionally, these tools help us to structure both a trauma-release session and, with practice, a trauma-free life.

In BBTRS, we call these tools "resources," and we use the word "resource" as both a noun and a verb. Resourcing is calling into play anything or anyone that supports our well-being. Active or passive, internal or external, resources can be people, nature, pets, dance, music, art, a feeling, exercise, physical touch, a scent, a memory, and a million other things, ideas, or actions. Once we're adept at identifying what resources might be useful on a given day or in a given situation, they can come to our aid at a moment's notice in other situations. In *Waking the Tiger*, somatic expert Peter Levine writes, "the resources that enable a person to succeed in the face of a threat can be used for healing. This is true not just at the time of the experience, but even years after the event."[166] This means that resources can become lifelines. They can help us "just get through" a situation, and they can literally save our lives. Therefore, it's difficult to overstate the importance of understanding how to identify and utilize resources.

When we put our attention to the subject, resourcing can seem kind of obvious. And part of it is. Most of us resource all the time—we take a deep breath when we're frustrated; we take walks to clear our minds; we hug a friend or partner when we're sad. Our natural tendency to call upon resources is built in. However, the process of learning how to call upon these resources deftly, quickly, and easily is learned.

In this day and age, we're used to technology, conveniences, constant streams of information and products. We're simply not used to delving into our innate resources, our animal instincts. "What we need to do to be freed from our symptoms and fears is to arouse our deep physiological resources and consciously utilize them," Levine writes.[167] To expand our natural resourcing capabilities, we think critically about what supports us in our lives—and then we do more of that. We buy a bigger toolbox and fill it with more tools. By looking around us, Levine says we can take stock of our starting points:

- Life context—family, friends, general health and fatigue, ongoing stress, nutrition
- Physical characteristics—genetic resilience, physical strength, fitness, age, physiological development and resilience
- Learned capabilities—life experiences that define our familiarity and coping abilities with various situations
- Self-confidence—a personal sense of our capacity to cope or defend ourselves, which might or might not relate to existing resources.[168]

When we experience a story featuring someone who has conquered hardship, real or fictional, we applaud when they succeed. Whether underdog or superhero, we tend to place a premium on that person's combination of strength, intelligence, and experience, just as Peter Levine outlines above. Consider Navy Seals who face an obstacle. Healthy and physically fit, they have developed a support system of highly trained comrades and a background of elite training. Because of this context of physical and learned strengths, they exhibit deserved self-confidence. *They* know they can handle the problem, so *we* know they can. When we look at ourselves and our own lives, we can remember that we have our own strengths—and goals—and we can walk in the world with the same level of confidence. We don't have to jump out of airplanes to feel competent, resilient, and able to take on the world.

By taking an honest look at our lives, we can begin to assess what we have "on our side." If we instantly say, "if I only had ____, I could feel stronger," then we know the first place to bulk up. If our physical bodies are not healthy, or if we have physiological deficits that invite stress, then we can consider ways to either strengthen our bodies or work around our weaknesses. If we find our social network to be a little sparse, then we can consider ways to grow our human connections. If we feel "unable," or "incompetent," then we practice whatever skills need reinforcement. Just like anything else we've learned to do—a sport, a job, playing the piano—with attention, we get better.

So that's the context, the setting from which we walk into the world. The next step is to grow the number of resources we plug into this context.

Felt sense 3.0 — Internal resources

By now you've practiced the felt sense. You've practiced breathing. You've felt the belly and ribs expand and contract with each inhalation and exhalation. You've focused on a leg or an arm, perhaps identified a twinge in a muscle or an organ. Perhaps you've explored the cascade of sensations throughout your body as you jogged this morning or enjoyed your yoga class. Now comes the next stage—utilizing the felt sense in the process of releasing tension.

In BBTRS and other somatic modalities, the felt sense allows us to access and relax tissues, and thus release the tension held there. This means that no one can release tension for someone else. Each person must turn attention inward, and then control that attention. We must feel what's happening, modulate energy, and allow it to shift. We learn when to dive in, when to relax, when to stop for the day. In a trauma-release session, an organic dance unfolds. The breather dances with himself.

Our felt sense is the ticket to this energetic dance. We discover not only our strengths, but also our internal places of safety, where we feel physically good and comforted. We don't have to feel good there all the time—just "right now." We also sense the places where we're holding an energetic activation or charge. Right now. By feeling into both the charged areas and the comforting areas—scanning from head to toe—we begin to tell the difference between where we feel good and not so good. For example, we might feel tension in the chest and throat, but calm and expansive in the belly. As we try to describe the sensations associated with both areas, we become more familiar with our own bodies. The resources become touchstones. They become even more safe and comforting. They make us feel stronger and more resilient, right now. Some words people frequently use to describe these areas are “spacious,” “warm,” “tingling,” or “calm.” Importantly, the same spot might be tense and a resource, all at the same time. Maybe a track star feels tension in the psoas, but also finds great strength and comfort in the hips and legs.

Levine reminds us that internal resources are born from our instinctual responses, or “innate action plans” that are embedded within us. “All animals, including humans, use these instinctive solutions to improve their chances of survival,” like ducking a fraction of a second before hitting your head on a low-hanging branch.[169] Therefore, internal resources can seem relatively abstract or involve the whole body. Some of these resources might be strength, agility, spiritual awareness, intelligence, guidance, inner relationship with spirit, instinctual wisdom, talents, a resilient nervous system, and a sense of humor. We talked about the physiology of laughter in Chapter 5, but the benefits of humor and laughter go beyond tension release and endorphins. Those are amazing benefits, but the access to humor provides an excellent illustration of how many layers a resource can have. For example, just being able to see the humor in a frustrating, or even dramatic or uncomfortable situation can be healing. It can immediately lift the tension, reduce the charge in the air, and allow participants to consider constructive alternatives. In the absence of the charge, anyone experiencing the situation can “be with the moment” without becoming overwhelmed. Imagine being the person who has easy access to that lightness. You might become an external resource for other people, but internally you get to live with your own ability to let go of unnecessary seriousness. You get to be “unstuck.” With humor, you no longer identify as a victim or a traumatized individual.

With conscious attention to the possibilities, any person can suddenly realize how many internal resources he or she really has. Internal resources are personal. They create an inspiring, permanent, reliable, internal chain reaction. They are the spark to our lightness of being.

External resources

In today's existence, we tend to relate to the outside world more readily than we feel our internal world. Therefore, it's quite easy to list external resources. These are the things that give us pleasure or comfort, so they are often other people, animals, environments. They can be activities, sports, arts, music. They can be angels or god. The temperature of a room can be a resource. Taking a bath to relax can be a resource. "What the environment provides in the way of potential safety (e.g., a tall, sturdy tree, rocks, a narrow crevice, a good hiding place, a weapon, a helpful friend) contributes to our inner sense of resourcefulness, if our developmental level is such that we can take advantage of it," Levine writes. He suggests the examples of a child who finds safety in a place where abuse does not occur, or with a non-abusive adult.[170]

Just as laughter has layers in internal resourcing, another human being's presence can become a multi-layered resource in external resourcing. Consider the comforting elegance of a "safe" person's simply holding space, or witnessing. Then there is the layer of a "safety line," where the breather might need to know that she can draw on someone else's strength in a given moment, or just look in someone's eyes. A trained facilitator also provides experienced support if someone feels very vulnerable. If activation gets too intense, the facilitator can actively assist in many ways. For example, that person's external touch can both soothe and release stuck tissues, joints, or extremities. External resources help us to release tension and feel safe. If we're exploring sensations that have been neglected for years, knowing that the world can be comforting can expand our possibilities. Support from the outside can change our entire outlook.

Making myself a resource

“Sometimes a facilitator can be an excellent resource for a breather. This works if the facilitator is able to maintain a stance of both witness and supporter. In this case, holding space becomes multi-dimensional.

I once facilitated a session with a young woman who had a very difficult time finding her internal resources. I offered to be her external resource. Later, I would learn that she was used to putting up boundaries, to protecting herself, because she accustomed to being approached by men with sexuality in mind. She was physically attractive, and after the session we would reflect on how this protective stance was a common experience. In the session, therefore, allowing me to represent her external safety line was a significant expression of trust.

Acting as an external resource during a session.

Throughout the session, I asked her a number of times to open her eyes in order to make contact with me, to touch my hands and arms if she needed to hold on, to connect with the safety that I represented. For her, this was a new experience. She lived in a world where someone always wants something from her. To be next to a man who represented safety, someone emitting absolutely zero sex vibe, was entirely foreign. It was important that she could receive support from any male person.

It is often a good idea for a facilitator to perform this role of resource, but in this case it was especially useful. This woman allowed herself to feel safe next to a man. She could experiment with growing her self-confidence, and her experiment worked. She experienced no contracting, no additional tension—and she was able to release her deeply held trauma. She was so pleased. It was as if a light had turned on. Now, even if a man approaches her, she feels... different. Her container has changed. How she responds has changed.

Resources:

Anything or anyone that supports the sense of emotional, physical, mental, and/or spiritual well-being. Keep an ongoing list of internal and external resources; use them in your practice.

Internal	External

Pendulation and titration

Whether we're in the midst of a traumatic experience or in a trauma-release session, we tune into the body. The process requires: (1) identifying the areas of tension and safety; (2) releasing and relaxing the areas of tension; (3) retreating to areas of safety when needed; then (4) going back to (2) and (3) until the process completes. Throughout most sessions, the "dance" moves repeatedly between charge and resourcing. It speeds up and slows down; it reduces and increases activation. The process of expanding and contracting the energy you're using—alternating between the charged area and the resource, whether internal or external—is "pendulation.".

In some cases, the breather is 100 percent aware and in charge of the balance, speed, and rhythm of pendulation. In other cases, the facilitator might assist in avoiding over-activation, or keeping the breather present. In his somatic work, Peter Levine has identified three levels of activation, low, medium, and high. In BBTRS, trained facilitators often use a similar model, determining what level is appropriate for the breather's resilience. For example, it's always safe to begin slowly, with a low level of activation, and then build up throughout the session. With each round, the activation can increase so as to encourage deeper trauma release. Each session, however, is unique. Even a person who has experienced several medium- to high-activation sessions might arrive one day with lowered resilience. That day, a gentle session might be all that is necessary.

"Titration" is a complementary tool for balance in the releasing process. Specifically, it allows us to regulate the process of release, so as to avoid going too far into activation. Titration is the release valve that modulates us, which allows a session to keep going until the tension has comfortably released and departed the body. Without attending to "how much" we let out at once, we can move out of the body and into emotional catharsis. When this happens, we lose our attention to our grounding in the physical experience. Once we go into the mind and emotions, we lose track of the breath, which is our link to the autonomic nervous system—which in turn is like letting go of the oar in the canoe. We can't steer anymore, and eventually we just run aground.

This process is true whether in a BBTRS session or in daily life. Remember back to a moment when your whole system felt like it was on the edge. Maybe you were stuck in "doing," focused on responsibility, forgetting completely how the situation was making you feel. Reach back into your felt sense. What did your body feel like when it was overcharged and overwhelmed? Did you forget all about what you love? Did you forget in that moment what brings you strength, comfort, and pleasure? Were you focusing on the problem, ready to flip out or burn out? If little stressors are "pushing your buttons," and causing more reaction than normal, that's a sign of overwhelm. With resourcing, the charge can leave your body, allowing the system to regulate.

In a BBTRS session, we gently, consciously modulate the breath, which allows the unwinding movements and tremoring to ebb and flow as needed. A person can do this same thing at home, or near a street fair in Poland, or on a mountaintop. We can move from activation to relaxation, from areas of tension to resources. We can breathe more deeply and then more quietly, using titration. One conscious session of release might involve two, three, or more rounds of intense activation and release coupled with the same number of periods of rest.

Through the deft use of resourcing, BBTRS allows a person to learn how to steer the boat. With resourcing at a maximum, we eventually are able to gently activate and release tension in any moment, when a problematic event is happening. We begin to just feel better, and also give permission for those around us to feel better, too. Our bodies naturally want to seek out pleasure. The more we learn the skills of maximizing the release of trauma in our bodies, the more we are able to gravitate toward joy, and then inspire it in others.

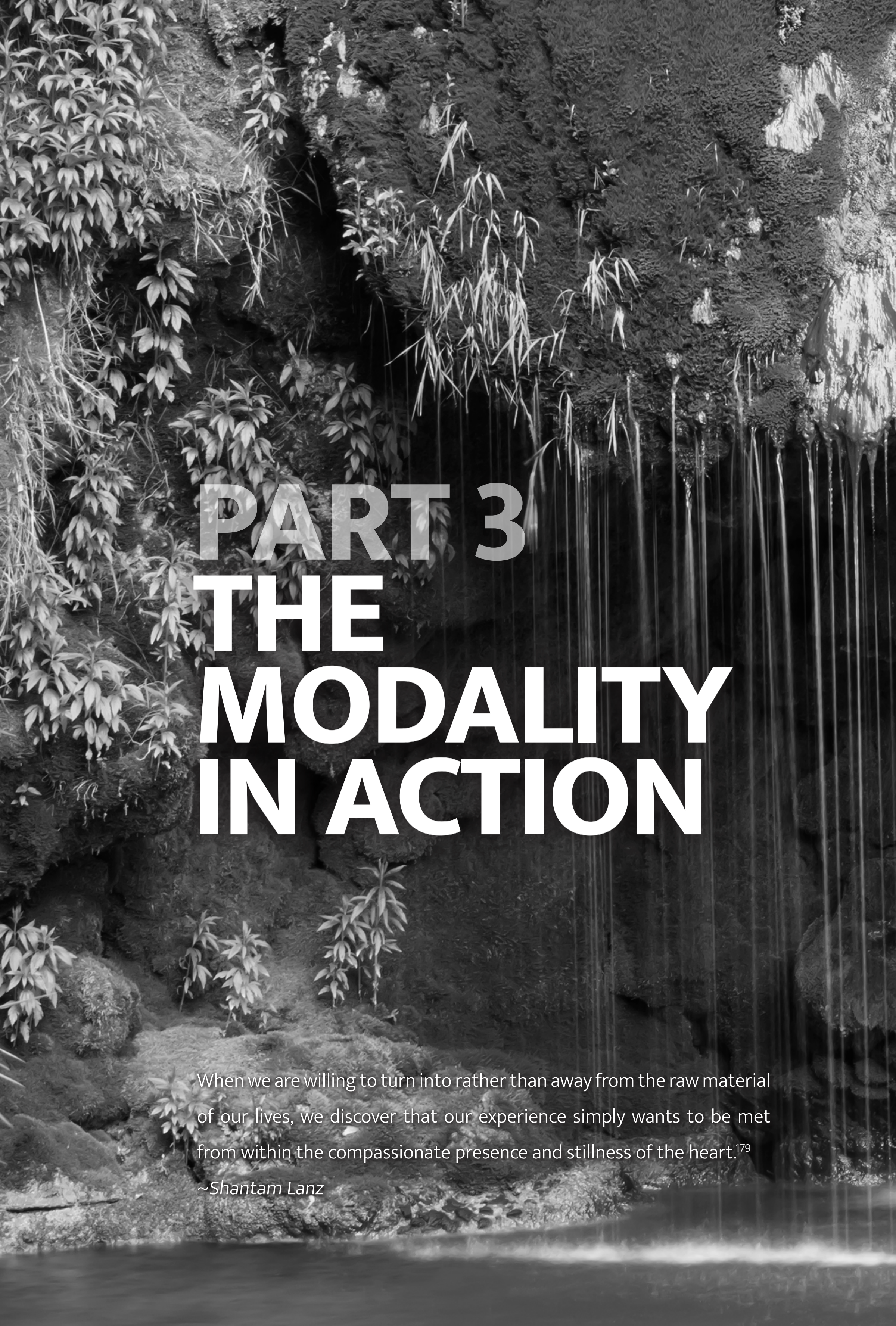
Joy is contagious. Take some for yourself, and then pass it on.

Reflection

Thank you for reading this chapter. Now, please take a moment to bring your attention to yourself. Please read the following paragraphs, and then close your eyes and scan your physical body. Notice how what you read reflects in your body. What is happening in your body—is it relaxed? Perhaps start with the breath, and consider whether it is flowing freely. Then consider your posture, whether you are sitting or lying down. Do you feel balanced, or does your physical body have tightness, pain, or discomfort? Look at different aspects of your body, including inner and outer tension. Take a deep, full breath, and sigh out your mouth. Next, consider your thoughts—are they generally centered outside of you? If something external is consuming your attention, can you let it go for now?

Now, consider if the material in this chapter resonates in you, and whether the resonance involves you or someone you know. If there is some triggering—positive, negative, emotional, physical, spiritual, or some other experience—then what does it feel like? Maybe it feels like you're reading something intimate or personal or true about yourself—or maybe you didn't like something you read. What was it? Was the response because the information was revealing, challenging, nurturing, or something else? It's possible that something that's gone unnoticed within you, or something you've neglected or forgotten about, might really be valid and making itself known. It's possible that this material might have awakened it in you. Or not. You be the judge.

After reading these things, and reflecting on them with your eyes closed, then open your eyes and bring your attention to your surroundings. Listen to the sounds. What do you see? Between chapters, perhaps take a brief walk, or dance to a couple of songs. Move around, reorient, and then settle in for the next chapter. Thank you.



PART 3

THE MODALITY IN ACTION

> When we are willing to turn into rather than away from the raw material of our lives, we discover that our experience simply wants to be met from within the compassionate presence and stillness of the heart.[179]
>
> ~*Shantam Lanz*

Chapter 10

Preparations

The complexity, diversity, and richness of life result in the inevitable experience of trauma. As human beings, we cannot avoid it. As social beings, we share and co-create the circumstances that give rise to it. Even so, once we understand how trauma works in animals and people—and especially how particular parts of the human body process or hold onto trauma—we are able to release it. In this section, we glimpse how BBTRS sessions are structured to allow individuals to experience profound relaxation, from the autonomic-inside out.

But don't light a candle and plop down on the yoga mat just yet. If you're working alone, your first mission is to act as your own facilitator and prepare both the space and yourself. If you're facilitating for someone else, then you're responsible for preparing the space, and also for maintaining proper boundaries and providing guidance to the breather. Since no two sessions will ever be alike, both breather and facilitator must grow skills in managing this organic, intimate process.

As with most things, when integrating BBTRS into your life, practice is paramount. With exposure to your own responses—and, for facilitators, the responses of others—we become attuned to what can help most at any given moment. In the meantime, even experienced participants must expect the unexpected. Therefore, let's begin at the very beginning.

Personal accountability

“Self-help” materials are always to be used with consciousness, caution, and with the participant’s awareness and understanding of his or her physical, emotional, and spiritual health and limitations. BBTRS’s component modalities—some over millennia—have been shown to support millions of people around the world. Still, practitioners must assume responsibility for researching any modality that is new to them. For example, when considering BBTRS, there are six different aspects to consider—especially the systemic, autonomic activation that accompanies deep, connected breathwork, and the organic physical movement of somatic release. Anyone with physical ailments, chronic conditions, extremely delicate nervous systems, or other concerns should consult a doctor to confirm BBTRS’s suitability in a personal protocol.

Age, fitness & accommodations

As we travel across the lifespan and accumulate experiences, we also tend to accumulate tension. Along the way, there are many reasons why we might lose mobility, flexibility, muscle tone, and lung capacity. For example, the natural aging process diminishes our physiological structures. Socially, “healthy” has looked different over the generations, and not everybody has created an exercise regimen that carries into their later years. In other cases, natural functions can be affected at any point in life if people suffer injuries or chronic ailments, or work in industrial, wartime, or other challenging or dangerous settings. Sometimes, people lose connection to their nervous systems, resulting in partial or total paralysis that adds to the congestion of trauma and tension in their bodies—even if they can’t feel it.

Fortunately, while these states of affairs might be difficult in many aspects, they surely do not prevent our ability to practice BBTRS. To practice responsibly, however, physiological limitations must be accommodated.

Reduced Lung Capacity—Lung capacity defines the ability to charge the body with energy and discharge tension. For those with limited lung capacity, attempting to fully charge the body through deep, connected breathwork could cause dizziness—which is not helpful, especially in a consciousness-based practice like BBTRS. Therefore, the depth and length of the breathwork component can be modified to suit an individual's situation. For example, if inhaling through the mouth is too stimulating, then inhaling through the nose and exhaling through the mouth could help. Breathing in softer, shorter increments also can be helpful, with breath rounds starting at just two or three minutes, and building up to eight or ten minutes each. With practice, even a person with limited lung capacity could do as many as three short, productive rounds in a BBTRS session. Details on specific accommodations are found in Chapter 12.

Felt Sense Relating to Age and Illness—Tuning into the felt sense is especially helpful, because it notifies us of where our bodies are working well and where they need attention. Breathers can achieve excellent, albeit gentle results, even if the body's movement is less than it used to be, or has always been restricted. For example, an elder who must modify the depth of each breath still can sense what is moving and changing in the body. The path of energy and related movements in the body might be fine and small, but absolutely can result in significant change. In fact, energetic charge and discharge can happen even in the absence of visible tremors or unwinding. The breather is the one who feels the releasing of the nervous system.

For those who suffer from chronic pain, such as arthritis or other illnesses, one area might scream for attention, while other areas are rather left out. In these cases, the felt sense can assist in "looking past" areas of pain to check in with other areas of the body. On the other hand, those who have experienced an injury often tend to avoid the area entirely. Here again, body awareness—and understanding—are key. Keep in mind that BBTRS is a conscious practice; the breather is in charge, even when autonomic movements present themselves. The body will not unpredictably jerk an injured area "without your permission." This means that the breather knows how much, when, and for how long to activate. The breather also knows when enough is enough. It's important to remember that extremely small movements can do wonders; with time and careful practice, the movements might get bigger.

Finally, for those confined to wheelchairs or bed, please know that as long as tissues are alive—whether or not they can be felt—energy is traveling through them. Circulation, lymph, and other functions continue, even in the absence of nerve impulses or muscle movement. Energy also continues to circulate, as is shown by scientific research that can be particularly beneficial to those with nerve damage. In a study that compared physical exercises of fingers and elbows to "mental training-induced strength gains," both groups increased physical strength. That is, *even those who did not physically exercise experienced increased strength*, whereas a control group—who conducted no exercises—did not change. The study instructed the test subjects to perform "mental contractions," which were found to "enhance the cortical output signal, which drives the muscles to a higher activation level and increases strength."[171] This kind of research underscores our experience with BBTRS clients—where activation and trauma release have occurred, even if people could not feel it.

Whatever the limitations and reasons for them, people with these limitations should not push past their abilities. They can work within available mobility, use internal and external resources, and consider a gentle breath session as the best method to get reacquainted with the body.

Children—The above considerations are directed to elders and adults with chronic ailments, because we assume that adult bodies have completed their growth cycles, and adult minds and spirits have enough life experience to understand the sensations in their bodies. However, the state of development of a child's nervous system is something else entirely. Intense breathing techniques should not be undertaken by or used with children under sixteen years old. However, facilitators can surely teach young people about breath awareness, felt sense exercises, and meditation, among other modalities.

Contraindications

In addition to simply encouraging self-agency in a person's selection of appropriate modalities, we have identified several circumstances or conditions contraindicating one or more of the BBTRS components. Both breathers and facilitators are encouraged to carefully screen everyone—themselves and others—who expresses interest in participating, so as to avoid potential problems with any of the following circumstances.

- While well-managed prescription medication regimens should be maintained during BBTRS practice, people under the influence of **recreational drugs or alcohol** should abstain from BBTRS.

- **Pregnancy**, especially the first trimester, is a period when women should avoid the release associated with deep breathwork.
- While mild **asthma** might be improved with BBTRS, common sense says to avoid intense breathwork for most people who suffer from asthma.
- **Severe heart disease, epilepsy, or a history of seizures** are conditions that do not respond well with systemic activation. Meditation or quiet, conscious-movement exercises might be better in these cases.
- **Mental illness**, especially schizophrenia, bipolar disorder, borderline personality disorder, and any condition that can include psychosis can be triggered by the intensity of BBTRS. People in relationship with these experiences are encouraged to engage in quieter modalities.
- **Diabetes** is an illness that involves blood chemistry, and blood chemistry can be impacted with the activity in BBTRS. Many people with very mild diabetes, and who are proactive with diet and medication, are often able to tolerate BBTRS. However, those with severe experiences with diabetes generally are advised against practicing.
- **Acute physical injuries** can be exacerbated by a variety of movements, and the movements in BBTRS are autonomic. This means that they are not directed by conscious thought, and therefore could inadvertently aggravate an injury. After healing, however, it is possible that BBTRS can be introduced. In these cases, several elements are required, including but not limited to: a solid understanding of the injury and how it happened; a clear felt sense of the body, so as to assess healing; clearance from

medical practitioners associated with treatment; commitment to rehabilitation and other physiological therapies so as to avoid further injury; and commitment to releasing tension associated with the injury. In the period before BBTRS is indicated, those with acute physical injuries might consult with a physical therapist to choose more appropriate modalities, until they have recovered enough to participate in BBTRS.

Possible reactions & side effects

Both how tension resides in our bodies and how we let it go are moving targets. That's why no two people will respond to a BBTRS session in the same way, and the same person will have distinct experiences with every new session. Each session is new. It might traverse difficult ground, bliss, or a very mild, quiet release. Still, experienced breathwork facilitators have noticed some common responses when people charge their bodies with breath. Therefore, before initiating your first BBTRS session, it's important to consider how to manage them if they manifest during your practice. Most side effects fall into a small grouping, which is detailed below.

Don't forget: The fact that a given person might experience a particular response in one session does not mean that the same effect will reappear in a following session—because our internal terrain changes with each release. Therefore, although both breathers and facilitators are better prepared when they know how bodies, minds, and emotions might respond, the mission is to flow with what is happening in the moment.

Hitting or Kicking — One common reaction can relate both to organic release and to the emotional-expression component of BBTRS. Sometimes a brief, organic emotional expression occurs in a session, such as slapping the floor or kicking the feet and legs. In these cases, facilitators help the breathers to maintain slow, monitored, conscious movements instead of allowing physical expressions to escalate. The deepest trauma release happens organically through unwinding and tremoring, not through extreme physicality often associated with thoughts or emotions. Therefore, if systemic activation stimulates thoughts or emotional responses about what is happening, we safely transition back into the body. Any bits and pieces of the breather's story are relevant only in how the feelings or content might be stuck in physical tissues.

Hyperventilation — Because of the intense and deep breathwork component, some participants might experience hyperventilation, or overbreathing. Both breathers and facilitators should be prepared to deal with this situation by helping the breather to lessen oxygen intake. A few tips include[172] :

- Breathing through the nose, or through pursed lips, as if whistling;
- Slowing the breathing rate by counting, perhaps with one breath to every five seconds;
- Belly-breathing with the diaphragm, which fills the lungs fully and thereby slows the breathing rate;
- Breathing into a paper bag for six to twelve natural breaths; the bag is held over the nose and mouth. Please note: people with heart or lung problems or those at altitudes above 6,000 feet should not use the paper bag method.

Temporary Discomfort — We know that BBTRS and other somatic release practices are meant to locate and release tension—sometimes long-held tension. We anticipate encountering "blocks," areas where the body does not move or flow on its own. It stands to reason, then, that when we approach tension trapped in our tissues, we might initially feel uncomfortable, both psychologically and physically. In *Body, Self, and Soul,* Integrative Body Psychotherapy founder Jack Lee Rosenberg wrote that some of this discomfort is based in a person's natural "hesitation" to interact with tension, and how that hesitation can manifest.

> A charge can be built up by altering a person's normal breathing pattern.… Many people are uncomfortable with the charge and they attempt to control their excitement so they can stay "calm, cool, and collected." The reason that they do this is that breathing deeply and fully amplifies their awareness of feelings. Many of the feelings that emerge with the deep breathing are uncomfortable ones, so most people avoid awareness by restricting their breathing. Unfortunately, while restricting the breathing to repress uncomfortable feelings, they restrict feelings of pleasure as well.[173]

As we avoid feelings associated with our energetic amplification, of course we bring our physical bodies along for the ride.

> Muscular interruptions to the charge…may take the form of thinking, dizziness and/or nausea (usually a sign of anxiety), splitting off, "spacing-out," fidgeting, scratching, squirming, yawning, falling asleep, muscle spasm, cramping, ear ringing, urge to urinate, talking, laughing, and a number of other responses that dissipate the intensity of the charge.[174]

Rosenberg saw these interruptions as "expressions of resistance," and considered them as areas to target for attention. He recommended that breathers "stay with the block, to experience the discomfort of the holding rather than changing or releasing it immediately."[175] In our experience, this kind of discomfort really can be considered as a "symptom" of healing, or the outward expression of tension as it unlocks. Think of the release of bound-up energy as it finally "elbows" its way through tight tissues and bound-up joints. The more we engage our sympathetic nervous systems, and trigger undulating movement and tremoring, the more open we are to whatever is happening in the tissues.

For many people, increased discomfort or even minor physical pain might occur in any area in the body in which tension is stored. Adrian Ealy, a massage therapist with a natural affinity for somatic work, has both personally experienced this temporary discomfort and seen it in his clients. "I don't want to take away the pain, in case it takes something else you need away," he says. "I'm supporting people through whatever it is they're doing, trusting that divine intelligence in the system will guide them into the place they should be."

Specific, Physical Side Effects — Professional facilitators have also noted some particular physical reactions, occasional side effects that can impact a minority of breathers. These reactions are not harmful for most people—but they can be off-putting, especially if a person is not aware of their potential onset. Two that are worth discussing more thoroughly are short-term apnea and tetany. Both of these usually appear as symptoms of organic illnesses, but they also can manifest briefly from the stimulation of the autonomous nervous system and related changes to blood chemistry. In breathwork and somatic practices, these side effects usually diminish after a short period of time. Still, anyone who experiences these two manifestations should consult a physician to ensure that they are not suffering from an underlying imbalance.

- **Short-term apnea** — Occasionally, light-headedness or fainting can result from a significant shift in breathing patterns, but a particular breath-related side effect can be particularly disconcerting. If apnea occurs—in which the breath might stop briefly, even for several seconds—it usually happens at the end of a breathwork session. To consider the cause, let's recall the potential range of experiences when the fight-or-flight system is activated—and think back to the impala, whose breath stopped and then restarted. Sometimes the system goes into a freeze response. Although the relaxed end-state of a BBTRS session does not involve a super-stimulated or -activated freeze response, some breathers experience a brief pause in breathing reminiscent of that "playing dead" breath cessation. After a few moments, the breath returns to normal—usually with a relatively sudden, deep inhalation—requiring no intervention.

- **Tetany** — Rosenberg identified a few other possible breathwork-related side effects, including "nerve symptoms such as numbness and tingling in the fingers and around the mouth," and muscle spasms—or tetany.[176] When spasms and stiffness progress, they can result in tremors or contractions, especially of the hands, feet, wrists, and ankles. In medical terms, tetany is usually the result of over-acidity or over-alkalinity in the bloodstream, which in turn comes from a drop in calcium, magnesium, or potassium.[177] Whereas medical treatment of illness-caused tetany requires intravenous administration of calcium or other minerals, tetany in BBTRS generally reverses on its own. If it occurs, it most likely manifests as hand cramps that cause fingers to curl toward the palm, and it can be painful. BBTRS facilitators have noticed that

> for those who experience tetany in sessions, the incidence usually reduces in severity and eventually stops as treatment progresses.

Overstimulation or General Unease — While not extremely common, occasionally a person under significant stress or with a lot of underlying trauma can respond—quickly, strongly, and sometimes uncomfortably—to the various elements of BBTRS. Agitation, overstimulation, feeling on edge, or "freezing" can occur as the result of an extremely sensitive nervous system. The person might or might not be aware of this preexisting sensitivity, or its cause, before the session begins. If a strong reaction like this happens during a workshop or individual session with a facilitator, then the facilitator will assist the breather with a bit of extra care—helping the breather to connect to the present moment and the safety of the session. Common interventions include: opening the eyes; connecting to the physical space; connecting to the others in the space as external resources; and connecting to internal resources. Shifting attention to safe, comforting parts of the physical body as internal resources might be all a breather needs to diffuse the level of activation.

If a breather is practicing solo, however, then a couple of preparations can help. First, before the session, assess your level of energy and inner resilience—and make yourself very familiar with Chapter 9, Resourcing. Second, be aware that changing the energy of a session, even without outside assistance, is definitely possible. Third, in the event of overstimulation or freezing, open your eyes, connect to the physical aspects of the environment, use your mind to remember your personal strengths, and sense a place in your body that feels quite stable. Any of these—or dozens of other resources—can allow the nervous system to regain equilibrium.

After calming the system, either alone or with a facilitator, a gentle session might continue without a problem. Or, if the breather feels quite tired, then the session can stop. Either way, the breather very likely will have accessed some significant, deep trauma—possibly something that has not been accessed in years. After such an experience, the breather might feel liberated; and there generally is no residual negative result from one uncomfortable session. And remember, even a rocky session releases trauma. Very likely, the nervous system will have strengthened through the experience, and the next session will be quite different. Still, it's best to wait a week or two before a follow-up session, whether solo or with a facilitator.

Personal support

As a rule, those who embark on healing work are encouraged to tell someone else about session details. That is, when we dive deep into our personal growth, it's important that a friend or intimate is aware of what kind of session is occurring, along with when and where it is. This ensures that support is available, if needed.

For many people, aspects of BBTRS can be practiced safely without supervision or in-person support. Ideas on how to practice individually are included in Chapter 12. However, especially early in the process of learning the modality—and any modality that includes breathwork or trauma release—it's best to work with a facilitator. If no one with training is available, then at least a "sitter" is recommended. An objective, caring space-holder does much more than provide emotional support, although that is a crucial gift. Along with witnessing, a facilitator can assess how the breather is moving, reacting, emoting, and expressing. The facilitator can determine if a person is too activated and needs to return to resource, or even move toward closing a session. The facilitator can assist in protecting the breather's body as it organically moves; for example, common interventions include placing blankets or pillows under the breather's arms, hands, and feet. The facilitator can provide a hand to squeeze, resistance to push against, a drink of water, a tissue, and eyes to look into. Most importantly, the facilitator is there to react if the breather experiences any physiological or emotional discord. On occasion, for example, a breather might hyperventilate or develop a muscle cramp. Trained BBTRS facilitators, like many bodyworkers, chiropractors, psychotherapists, and others commonly attracted to BBTRS, are familiar with how to support a variety of breather experiences.

The physical space

Participants need to feel safe and comfortable in any successful practice, and this includes considering the physical space. For those who practice meditation or bodywork, setting up a practice space might seem second-nature. However, for people unfamiliar with such settings—whether working alone or with a facilitator or a group—below are several hints for arranging the physical space for BBTRS.

At a minimum, the area to conduct a session should be clean, private, and large enough to accommodate a yoga mat with a few feet of space all around it. In trainings, we generally use at least two stacked mats and two blankets, to provide adequate cushioning for the breather to lie down, kneel, and roll from side to side. Additional blankets and/or flat cushions are useful to protect elbows, wrists, hands, and feet as the breather moves. The space also should provide insulation from outside sounds, as well as allow the breather to make sounds without feeling inhibited. Once a session begins, the breather should not be interrupted.

As in the search for real estate, "location, location, location" can provide an external resource for breathers. For BBTRS trainings, we often choose beautiful locations around the world, because nature and the environment support the process. It's easy to be open and sensitive in Sedona, Mexico, Colorado, Bali, Costa Rica, Greece, and the countrysides of Poland and China. However, even if your corner of the world is less expansive, consider how nature can assist with the work of trauma release. In a city, for example, consider choosing a private space for yoga or meditation that supports self-exploration, and also might be nearby a garden or park for a walk afterward.

Common sense

When embarking on any new practice—or expanding a familiar one—it's important to use common sense, intuition, and caution. Some reminders:

- Be careful not to overcharge the nervous system in the pursuit of relaxing the nervous system.
- Before every session, be aware of what resources are available that day, whether outside the self, in the physical space, and within.
- If something unexpected—an intense memory, a feeling, a reaction, excessive activation, a side effect—suddenly arises in a session, remember to de-escalate and ground through resourcing. Slow the breath, return to the body's felt sense, and switch gears away from an overwhelmed state into an experience that supports release.
- Remember that our mission is to release tension, and that the great majority of people experience BBTRS with relief, release, peace, and ever-increasing resilience.

Reflection

Thank you for reading this chapter. Now, please take a moment to bring your attention to yourself. Please read the following paragraphs, and then close your eyes and scan your physical body. Notice how what you read reflects in your body. What is happening in your body—is it relaxed? Perhaps start with the breath, and consider whether it is flowing freely. Then consider your posture, whether you are sitting or lying down. Do you feel balanced, or does your physical body have tightness, pain, or discomfort? Look at different aspects of your body, including inner and outer tension. Take a deep, full breath, and sigh out your mouth. Next, consider your thoughts—are they generally centered outside of you? If something external is consuming your attention, can you let it go for now?

Now, consider if the material in this chapter resonates in you, and whether the resonance involves you or someone you know. If there is some triggering—positive, negative, emotional, physical, spiritual, or some other experience—then what does it feel like? Maybe it feels like you're reading something intimate or personal or true about yourself—or maybe you didn't like something you read. What was it? Was the response because the information was revealing, challenging, nurturing, or something else? It's possible that something that's gone unnoticed within you, or something you've neglected or forgotten about, might really be valid

and making itself known. It's possible that this material might have awakened it in you. Or not. You be the judge.

After reading these things, and reflecting on them with your eyes closed, then open your eyes and bring your attention to your surroundings. Listen to the sounds. What do you see? Between chapters, perhaps take a brief walk, or dance to a couple of songs. Move around, reorient, and then settle in for the next chapter. Thank you.

Chapter 11
The session

BBTRS sessions can be structured around an individual, and provide intimate experiences in which one breather and one facilitator flow together through an organic release process. Sessions can also involve small groups—from perhaps a few to thirty breathers—facilitated by one to several facilitators. In some special circumstances, sessions can serve hundreds of breathers in one large area, with one primary facilitator on a microphone and dozens of others filtering through the group. Because of the varying needs of different people, the availability of practitioners, and cultural styles, it's quite obvious that no two sessions will ever be the same. For that matter, no two sessions of one single person in individual sessions will be the same. Still, detailing the general approach to preparing a breather to experience trauma release is possible.

As this approach is described, it's important to remember that the primary element for anyone involved is to remain present in the moment. BBTRS was developed to help the body to function and feel better by releasing traumatic residue from the physical tissues, mind, emotions, and spirit. The moment-to-moment experience of engaging and releasing this tension can appear unexpectedly calm and placid—or, for brief periods, the body can become boldly expressive and active. One session can roll through several different tempos or remain relatively static. Therefore, while the following material addresses a wide variety of specific interventions—in general, and for the different belts of tension—preconceived notions of "what something looks like" or "what will happen" are limiting. With more experience, practitioners simply become more able to allow and assist, both in our own bodies and when helping others.

Moving from the inside out

From beginning to end, even during "resting" periods, a breather's body might exhibit some level of movement. When a session is successful, this movement is organic, occurring naturally from the autonomic nervous system. The movements might involve one finger's tapping on the floor or the entire body's writhing like a snake. No matter where we are in the world, the openness needed to allow the body to move in this fashion is contrary to the average cultural expectation in today's society. Let's recall our example of a person who might be seen tremoring in a public space. The average onlooker might wonder if the person were in need of help, and if the help should begin with medical, mental health, or even legal intervention. That is, tremoring is not something we see every day, and it might be associated with a seizure, a psychotic episode, or a street-drug-induced reaction.

As a breather in a BBTRS session, therefore, a level of trust is the first order of business. Trust in oneself to acknowledge the tension and trauma in the tissues leads to trust in the possibility of release. Trust in the facilitator, if there is one, allows the breather to feel comfortable enough to enter a place of vulnerability. The second order of business is to release preconceived ideas of what is socially acceptable or expected. BBTRS sessions are set up to embrace new experiences, to celebrate new ways of being, and to witness ourselves and others as we literally step out of our past and into a new future.

To make this possible, we breathe into our bodies and move "from the inside out." This means that we rely on our felt sense, thus connecting with the physical body from the core. In BBTRS, we refer to the core of the body as the space from the pelvic floor through the areas involved with the diaphragm. Movement often originates in the hips, because the psoas muscle is a primary fight-or-flight activation point—and it is intimately involved in movement all the way up through the belly, lungs, and heart. This is why "core awareness" during release work is so crucial. It supports spontaneous movement that arises during the session, and allows this movement to feel more fluid and connected. This quality of movement is evident to experienced facilitators, who can assist breathers in even deeper release.

Everything in the BBTRS session supports release, and while not all release involves a lot of activity, much of it does stimulate slow, graceful, connected movement. This grace in movement arises when the core becomes free—when previously locked energy is released. As release occurs, every gesture feels and looks different.

We begin here in the description of a BBTRS session not because movement is the first thing that happens—but because the shift in attitude to allow movement allows any session to do its job. We walk around the world taking advantage of our core muscles, using them as containers for our tension, agony, and past stories—causing restricted movement, muscle weakness, and intensified wear and tear of joints and ligaments. All of this invites both physical injury and more tension. Let's shift our ideas of humans-as-mechanisms into humans-as-energetic-beings. Let's shift from churning on auto-pilot through daily routines into consciously choosing our engagement with life. This is not an abstract notion. Sensitivity to our lives becomes concrete when we are able to unlock the tight and restricted tissues in our core. It is from this single starting point that the connection with our internal and external worlds becomes injected with life's juice. This single starting point connects our feet on the ground with our spirit in the ethers. We are living beings, and by connecting with our physical bodies, we are both tethered to the earth and free.

So. To truly experience any BBTRS session, we begin first with this invitation: find your core, and then move from the inside out. The movement will change as "stuck" areas release. Keep in mind that these areas are not metaphors, but physical areas in the tissue where the tension of fight-or-flight activation remains. They act like dams stopping the flow of water. The dams disintegrate—not just "ka-bam, they're gone," but through our presence. By using the felt sense, we know what each place feels like, and what movement is both organic and needed to carry away any tension remaining there. Surprisingly, the physical movement and expression in somatic trauma release usually does not result in physical exhaustion. The muscles are not "spent," or sore. Instead, usually the breather achieves deep relaxation and peace. Some report a sense of feeling "clean," "clear," or "light" within the body.

All the additional elements of the BBTRS modality support this transformation.

- The breathwork component stimulates the system, awakening energetic pathways that Chinese acupuncturists have relied on for millennia. It also "turns on" the sympathetic nervous system.
- Touch and bodywork support muscular opening and softening, increased range of movement, and further release "stuck" areas—

all of which also allow energy to move through energetic pathways. Therefore, through both the internal physical process and the contact with the facilitator, touch can be an amazing resource.
- Emotional expression acknowledges and punctuates muscular unwinding, connecting the physical and emotional bodies.
- Sound resonates with the body's vibrational and emotional aspects, encouraging both movement and release.
- Meditation allows for deep integration.

With this entire package of resources, a breather has total control over each BBTRS experience, and therefore can self-regulate throughout. The knowledge that the breather is in charge, coupled with the "autopilot" aspect of the autonomic nervous system, creates an unmatched, unexpected, personal dance.

Overall structure

While there is no typical BBTRS session, a general format provides structure and also allows for an understanding about "today"—where the breather is in that moment, how the body feels, and what, if anything in particular, is desired. Please note: in this description, the method of working with one breather and one facilitator in an individual session is provided as a template. If practicing alone, a breather can do most of this work without another person, with relatively obvious accommodations. However, more on solo practice will be addressed in the next chapter. If practicing in groups, training is a must. That is, for facilitators who wish to work with groups, training addresses many issues related to breather safety and the logistics of managing the experiences of multiple breathers simultaneously.

Please note: Before launching into a solo practice, it is recommended that a practitioner first experience a series of sessions with a trained facilitator. This process will allow a new breather to ask questions, troubleshoot any confusing experiences, and become accustomed to the sensations associated with BBTRS.

Step 1: Set-up

As noted, the space is prepared with a soft place on the floor, facial tissues, a bottle of water, and blankets or pillows. In BBTRS training sessions, the practice mat is generally a large foam mat or two yoga mats and two yoga blankets, with additional blankets available to pad elbows, hands, and heels.

Since a good deal of the experience usually involves the breather's lying on the floor, looking up, ceiling lights are dim or off, with light sources coming from elsewhere. Water and tissues should be easily accessible for the facilitator to hand to the breather when needed. In addition, sound-producing elements can be added, based on the breather's wishes and the facilitator's knowledge base. For example, music tracks similar to those played in massage sessions might be useful to begin. With practice, however, created soundtracks can assist a facilitator in shifting the tempos and transitions in a typical session. In addition, flutes, rattles, drums, and a variety of percussive instruments can be at the ready, for use at opportune moments throughout the session.

To begin, the breather and facilitator sit across from each other, with the facilitator toward one end of the mat/blanket and the breather toward the middle. They take a moment to join in a shared experience. They can hold hands, touch knees, or simply comfortably meet each other's eyes. For the breather-facilitator partnership to work, direct availability is important. In addition, this interaction allows the facilitator to assess whatever is present in the breather's eyes, such as fear, fatigue, disconnect, anger, or seductiveness. If needed, the facilitator then takes a moment to address whatever is detected so that the session can begin without distracting feelings or energies.

Please note: When working with cultures in which looking directly into the eyes is too invasive, then achieve a posture of availability by sitting across from each other. If the facilitator is unfamiliar with the breather's cultural customs, then simply ask the breather what works to allow for clear communication. The idea here is to create comfort for the breather, so information is key. It's possible that the facilitator can look at the breather while the breather is talking, and vice-versa, but the speaker's eyes are averted.

Step 2: Connection

Before any real dialog pushes the breather too far into the mind, the facilitator asks the breather to close his or her eyes and connect with the body. The connection might happen immediately, or it might take a few moments to "arrive," to sense the safe space, to feel the interior of the physical body. The facilitator might support the breather to connect with the breath, and even guide a short meditation to travel from head to toe, just sensing both body and breath. Commonly, the breather might attend to where breath is flowing, without pushing it in any way; how the body is resting on the chair or cushion; how the weight of body feels; how gravity is working in the limbs. After a few moments of connecting with the self, the breather opens his or her eyes.

Step 3: Check-in

The opening dialog should remain short, because somatic trauma release is not meant to focus on the mind, or on discussion. It's not a counseling session. However, even by bringing attention inward to consider answers, the sympathetic nervous system is stimulated. Plus, of course it is important to share relevant information in words; the spoken words—even if not fleshed out in great detail—can be used effectively during a session.

Any practitioner can develop his or her own list of check-in questions, but some common ones are:

- Why today? What's happening with life that brings the breather into this space?
- How is the body? Are there ailments, injuries, or anything acute to be aware of?
- How are the emotions? Is there something bothering you that you can simply name, so that we can be aware of it? *Examples might be "annoyed with a colleague," or "trouble with my child."*
- What would you like to have happen today? *Please note: a breather might want only to dip a toe into the process today, or he or she might be ready for full expression. As a facilitator, this information will help in pacing, resourcing, and other support. For the breather, this goal-setting can provide safety, but also it might be just a starting place. Breathers have reported that they were more comfortable and "ready" than they first imagined, so flexibility on goal-setting is important.*

This process usually goes relatively quickly—perhaps two to five minutes—but there might be circumstances that require additional time. For example, someone who has suffered some now-healed injury might need to explain areas the facilitator has to pay special attention to. Also, if a breather is new to BBTRS, a bit of dialog might occur regarding what to expect in the session regarding goals.

One caution: Some people walk in the door fully charged with a troubling situation. They might already be on the edge of their emotions, and are looking to release associated tension. If a person's charge is obvious, or if it becomes obvious during the check-in process, the facilitator does more than active listening. The facilitator carefully watches for how much activation is present or arising in the breather's body as the story is told. Since the session should not be spent calming down heightened emotions, the facilitator brings the breather back to sensing and feeling the body as well as supporting him or her to find inner resource. Breathers in this state of activation who arrive at a BBTRS session looking for support can be assisted, but careful preparation is key. However, if they remain highly activated early on, it is not recommended to invite breathwork, which would stimulate the nervous system even further. More on this in Chapter 10.

Step 4: The modality

Once the goals and status of the breather are established, then the facilitator takes a moment to explain some elements of BBTRS. If the breather is new to the modality, then the facilitator provides a brief overview of how the modality works, including the six elements, how tremoring feels and functions, and the possible side effects of somatic work. Then, based on the check-in from the breather, and whatever the facilitator has observed, the facilitator summarizes how they will approach the session, and the session's intended goals.

Step 5: Locating tension

With eyes open or closed, the breather then pays attention to the body, sensing areas of tension. The breather describes how the tension feels, including any emotional sensations that arise when considering the tension. It's important to provide details about what the tension feels like, without inviting too much activation. Examples of words that breathers often use to describe tense areas include: tight, stiff, angry, deadened, flat, charged, or full of energy.

Step 6: Locating resources

While finding where we are stuck is usually quick and easy, locating areas of resource can take time, especially for those new to BBTRS. Many people are so disconnected from the physical body that it's difficult even to understand this concept. They might say, "My whole body is tense."

In these cases, the facilitator can assist by asking the breather to concentrate on something quite straightforward: the differences between parts of the body that feel better and those that don't. The "better" areas might simply feel "normal," or devoid of pain. The differences might be quite tangible and pronounced, or slight and faint. No matter how pronounced a difference, once it is noticed, then the breather practices feeling the physicality of it. For example, at first, a breather might sense only a slight constriction in the chest, and not much everywhere else. With further attention, however, he or she might notice subtle aches and pains, such as a twinge in the shoulder or thumb. With further attention, "good" areas might actually feel quite flexible and relaxed.

Areas of resource are not only different from person to person, but also within a person from day to day. Or moment to moment. A lot of people point to the belly or hands or feet as places that feel safe or reliable, but there is no limit. A person can name five places or one, and resources might even shift within a session. Some common descriptors for resource areas include: soft, tingly, expansive, warm, or safe. If describing feet or legs, a person might say they are strong or fast, which might relate to the inherent capabilities of those areas. Regardless of the resources identified by the breather, the facilitator takes note of the particular words used to describe their qualities. During the session, these terms can be used to remind the breather of his or her “comfort zones.”

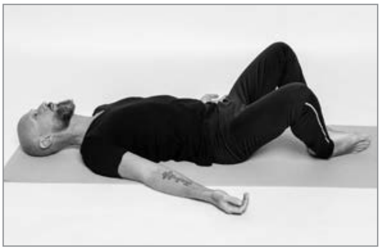
If a breather simply can’t locate an area in the body that feels “better” or “safe,” then the facilitator can offer to be a resource. Breathers should always be able to rely on a facilitator as a solid support who will never abandon or bring the breather into harm’s way. Usually this idea works well, helps the breather to relax. It also eliminates any anxiety the breather might have developed in the unsuccessful attempt to identify an internal resource. In the early stages, it’s important to feel free to proceed, even without a strong felt sense.

It’s also important to remember that trauma release can involve some pretty intense history. Especially if a breather is new to the modality, the awareness and assurance of being supported can be very comforting. The onset of trauma, even if experienced with others, happens individually. Each person experiences trauma uniquely, and we tend to feel alone with our experience. This is one reason a facilitator can be so important. Sometimes a facilitator can see on the breather’s face the moment of realization, the “clicking in” of possibility, the idea of exploring and releasing a traumatic experience—but not alone. This shift can change everything, both in a session and in a person’s life moving forward.

Step 7: Diving in

With resources in hand, the facilitator brings the breather to place of activation. At first, this simply means to shift attention back to the places of tension, the places that require release. The breather senses if anything has already changed, just since the areas were first discovered and discussed in the session. This part of the work introduces pendulation, where the breather easily moves between the place of activation to the resource, and back again. This shifting allows the nervous system to receive the signal that it's safe to release. At this point, the breather begins with deep, connected breath.

Usually, we begin the breathing component while the breather is still seated, but some people prefer to begin lying down. In the latter case, the knees should be bent and drifted out from center into the "tremoring position." The angle of openness is somewhere around 30 degrees, or wherever the breather finds that the muscles must be engaged to maintain the position. The simple action of holding the muscles in this posture will assist in engaging the autonomic nervous system.



Whether seated or lying down, the start of connected breathing shifts the person's experience inward. If the pace of breath is a challenge, or the breather is unfamiliar with how to do it, the facilitator can model, and even take a few breaths with the breather.

The path through "diving in" then flows organically. It's impossible to predict how any given person will respond, but the average experience involves the spontaneous appearance of movement, whether initially mild or bold. If the breather is seated, then the movement might originate in the head or shoulders, with just a "getting-used-to-it" gesture of engaging the body. It's also common to see movement originate at the hips, with the torso moving front to back, or even writhing a bit, with the first expression of the "snakelike" movement familiar to BBTRS.

When the movement becomes significant, or when the breather prefers it, the breather's posture moves to a prone position, usually on the back. This allows for a maximum range of expression. Everything from head to toe might shake, flex, roll, and undulate. The facilitator ensures that the natural movement does not cause the breather to hit a nearby wall, and uses extra blankets to protect hands and feet from impacting the floor. As the movement continues, the facilitator also attends to the speed of the breather's movement. The key is for the breather to stay present as much as possible to the physical sensation of the connected movement, which is originating from the inside. If the movement is fast, then the breather will find it more difficult to stay present and aware of it. Maintaining a slower, more integrated movement will give the breather an opportunity to track the felt sense, as well as all sensations present during the movement.

With experience, a facilitator also can integrate touch into the practice. This involves close observation of the breather, combined with an understanding of what the breather's body is doing. "Stuck" places simply don't move in synchronicity with the rest of the body. Therefore, the facilitator can add a light touch, trigger a pressure point, or assist a movement that's inhibited—such as "helping" an arm as it tries to lift, or adding momentum to a rib cage that wants to circle. The facilitator can also provide resistance when feet and legs "bicycle" in the air or lightly kick. Successfully adding these interventions requires experience and intuition—and is greatly enhanced with a background in bodywork. Much more information on appropriate support is included in the hands-on BBTRS training program.

Notes for elders or others with limited lung and/or physical capacity: For those who are chair-bound or unable to lie on the floor, the entire session can occur in a seated position, on a cushion, or else on a chair. If lung capacity is restricted, then avoid the open-mouthed, deep, connected breathing that BBTRS relies on as the standard for the breathwork portion of the modality. Instead, breathe gently, and only as deeply as is comfortable. If the body begins to charge too much, or if any light-headedness results, simply reduce the quantity of air and the speed of the breaths; try inhaling through the nose. Then, depending on the body's physical abilities, pay close attention to what the breath is activating in the body, and where. If gentle movement begins, allow it, as long as it is comfortable and does not push any injured areas. Work within available modality, but no further. Apply the remaining steps to the level possible, and also consider augmenting each BBTRS session with meditation and/or other mindfulness exercises.

Step 8: Pendulation

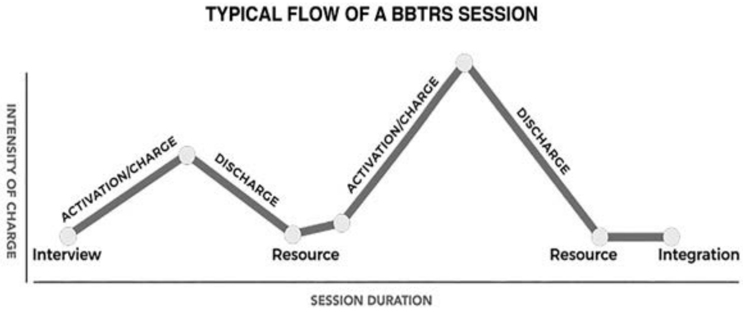
Throughout the session, the rhythm of energetic flow moves from activation to resource, and back again. A usual session involves two to three "rounds" of activation, followed by resourcing. How many rounds is determined by the session, the comfort level of the breather, and the intuition of both breather and facilitator. Each round reaches deeper levels of tension, so it's usually most effective to have at least two rounds. However, if the process feels too stimulating, then it's important to know when to stop. Beginning with even just one round is fine, with the hope that the breather will return for future sessions.

During activation phases, release of tension occurs through the natural, organic movement described as unwinding or tremoring. In these phases, it's possible that the breather might express something verbally or through an emotional display. Crying or laughing is not unusual, and a few words here or there can be useful. The facilitator can use these words, along with the breather's resource words, to assist in the session's flow. Sometimes the releasing process causes the breather to cough, and even expel phlegm. This is one handy use for tissues, and facilitators should have easy access and quick reflexes.

When the first round completes, the breather rests a little bit, allowing the nervous system to settle. The facilitator reminds the breather of his or her resource(s) and supports the slower breath cycle. The breather senses how the body is responding to the wave of energy, as it ebbs and flows. He or she focuses on internal or external resources—places, people, or sensations that are safe and calming. This part of the process reminds the entire system that the person is safe, and that the process is healing. The change in tempo of movement, breathing, and activation is immediately obvious to both participants.

After relaxing the breath for a short time, then the breather begins active breath cycle again, thus recharging the body. This time activation is quicker, because the body is already oxygenated. Within a minute or two, the activation is fully charged, and usually the peak of expression is higher. With this process, the breather can reach yet another layer of tension release. Again the movement comes, again the expression comes. This phase is maintained for several minutes, and possibly as long as twenty or thirty minutes.

The session progresses through its natural structure. The more intense the activation—without overstimulation—the deeper the layers of tension that are released. The deeper the layers of tension, the higher the peak of engagement and deeper the valley of resting. Please keep in mind that deep activation is only possible without overstimulation. The facilitator must be careful not to overstimulate the breather or to allow him or her to remain in a highly stimulated state for an extended period.


TYPICAL FLOW OF A BBTRS SESSION
INTENSITY OF CHARGE
ACTIVATION/CHARGE
DISCHARGE
ACTIVATION/CHARGE
DISCHARGE
Interview
Resource
Resource
Integration
SESSION DURATION

Multiple shorter periods of stimulation are more beneficial than one longer period of stimulation.

Additional elements—usually focusing on specific belts of tension—can be added to the active parts of a session, or can form the structure for a series of sessions. These additions are outlined in the Appendix.

Step 9: Closure

Usually at the end of the final round of activation, when the breather relaxes back into resource and rest, it's clear that the system's experience is complete. The charge that was accessible on this particular day has been released, and the breather is grounded and resourced. A facilitator cannot leave a breather in an activated state. Once the breather has activated something, both breather and facilitator must see it to the end. Sometimes a charge has been stuck for years, and so the awareness that release has occurred is a gentle effective conclusion. By returning to resource, and having time to return to normal activation levels, the breather can safely return to daily life.

The actual process of reintegration might take many minutes, and requires that the facilitator sit nearby. While the breather might even appear to be asleep, the support provided by the facilitator at this time is as essential as all other support during the session. Together, the pair simply "is," until the breather is ready to move out of the session. If the breather does fall asleep, or seems happy to lie in the neverland of peaceful relaxation, then after about ten or fifteen minutes, the facilitator can gently lift the breather's attention to the room, his or her body, and to the awareness that it's time to shift.

The process of activating and returning to resource is an essential skill for the breather. The process teaches a person how to self-regulate—not just in a BBTRS session, but also in day-to-day life. By learning it in the safety of the session, the breather receives another wonderful, long-term resource.

Step 10: Reflection

After reintegrating for a moment, the breather and facilitator resume their seated postures across from each other. They might hold hands or otherwise make contact, if it is a natural outgrowth of the experience. In this moment, the breather is invited to express in words anything relevant about the experience. The breather can say nothing at all, or speak to physical, emotional, or spiritual experiences that occurred. The facilitator can respond with reflections about the process, what he or she witnessed, and support of the breather's account. This portion of the session likely will take about five minutes or so, and should not linger onward. The breather has just experienced a deep and physical release; diving into the mind—with attempts to describe the experience in detail, or to analyze it—are not truly helpful at this time. Reflection allows for the last piece of closure, along with an acknowledgement of the partnership experienced between breather and facilitator at this moment, in this place.

Integrating belts of tension

Integrating work with belts of tension can occur in many ways in and around a BBTRS session. First, sometimes a session simply focuses attention on a particular part of the body. If tension has been sensed there, then attention and resourcing can engage that area. Second, the Appendix outlines belt-specific exercises. These can be done early in a session—during the "Diving In" phase—or even as "homework" on the morning of a day when a session is planned. Many of these can be done solo or with the oversight of a facilitator. Third, during the session itself, a facilitator can engage particular pressure points or support particular movements that target a belt of tension and adjacent areas.

Considering the body as an integrated energy system with particular, concentrated intersections is universal in human thought. How proficient an individual is with integrating belts of tension is related to his or her depth of understanding of Chinese medicine, acupuncture, the chakra system, and even massage therapy or other bodywork techniques. In a BBTRS session, an experienced facilitator might apply pressure to particular body segments, organs, joints, connective tissue, or pressure points. Since the application of these techniques is practitioner-specific, and requires hands-on training, they will not be explained in detail here.

However, the idea is clear. If tension is felt in the forehead, or the belly, or the shoulders, then a trained facilitator can use that information to choose specific interventions. A facilitator who is new to the modality can integrate safe exercises, draw attention to the impacted areas, and generally support the release of tension in them.

Resonance between breather and facilitator

A key to the success of sessions that are conducted with a facilitator is the rapport and resonance between the parties. Practically, to provide support, the facilitator watches the breather throughout the session. The attention is unflagging, continuous, and responsive. It is wide-ranging, moving from reminding the breather to stay in his or her breath, to holding a hand if it's extended, to providing gentle, reflective, encouraging feedback as the movement unfolds. It also can include the more physical interventions, bodywork, or supports, depending on the knowledge base of the facilitator.

On being a good vessel

> “I believe that good facilitation for people in the sessions we create involves a planned, cautious, and generous use of my own body. I step into their space, while remaining myself. I use my sense of resonance to tune into them. Maybe because of my relationship with music, I always view the breather as a transmitter, the string on a stringed instrument that releases energy when it is plucked. I am the

resonating chamber, amplifying the breather's energy, based on what I feel in my body. I strive to be fully empty, so that I can receive the other person.

My technique involves observing the breather's body and sensing where to apply my words, my presence, and my touch. I ask myself, "What do I say to them to help them go deeper?" Maybe I see some kind of emotional expression. I support that. I remind them to use felt sense, to connect to resource—and based on that, they allow physical expression to happen. Movement begins to happen, and I ask them to consider how it's going to spread.

Sometimes a person doesn't allow the body to move. I look for the slightest sign, even twitching in a single toe or finger. I apply bodywork techniques that activate their external and superficial muscular layers. I might ask them to push or pull against my resistance. Relaxing the outer musculature then allows the energy to flow down into the core. Then, at the peak of the session, expression happens naturally.

I also judge where tension is stuck in the body, and apply the touch there to support the opening. When this happens, movement comes through the entire system. Tremoring then becomes bigger. The system was already prepared and relaxed from last round. Tension is now dissolving, and the tremor can move to the periphery, and from superficial layers to the core.

It’s usually very easy to sense if the person is checking out or fully present in the session. If I can’t tell, I ask. Sometimes the physical expression is quite grand, and breathers might even “lose themselves” and dissociate. In this case, I bring them back to experience their felt sense and to ground back in the body. However, most of the time the breath simply begins to flow freely, the mind stops, and the process takes over. Sometimes “facilitating” simply means stepping back and allowing. No talking, no touching, just observing. Sometimes all that is needed is your presence.

The point is that in BBTRS, we activate, and we stay present with activation, avoiding overstimulation. I keep breathers present to activation, resource, and felt sense. I support them to release and discharge, as long as the process is not overwhelming. How to tell if the person is overwhelmed does come with experience. I have worked with so many people over the years. Once a session starts, I can sense how much a person can handle. For those newer to the modality, it’s better to protect yourself—whether you’re the breather or the facilitator—and stop it before the stimulation gets too much. With every successful release, we are encouraged for the next time.

Regardless of the level of expertise, it's important for the facilitator not to engage his or her own nervous system while assisting someone else. Yes, the facilitator can model the connected breathing, and even breathe with the other person for several moments. But too much active breathing or over-identifying with the breather removes the facilitator's focus and can cross boundaries. The focus is on the breather, from a distance that allows the facilitator to assist.

Shantam Lanz, the founder of Fluid Presence, relies heavily on resonance in her work, much of which happens in water in the form of Watsu® and other therapies. Requiring "experiential and sensory awareness" on the part of both the breather and practitioner, Shantam's work allows her to "connect profoundly with each client on a physical, energetic, emotional, and spiritual level."[178] In materials she developed for a course called "Presence of Being™," she outlines the characteristics of appropriate resonance. It includes comparing a facilitator's role to that of a musical instrument that is expected to resonate with the vibrations it receives. For a human being to be able to resonate with another, that person must be conscious of his or her own projections, and be willing to leave them behind. BBTRS facilitators likewise approach a breather from a posture of not knowing, of innocence, of curiosity, and of openness to that other person's state of being—and of activation. It is through this ability of sensing that the facilitator can assist the breather in crucial decisions, such as when to speed up or slow down, when to stay in the breath, and how or whether to touch or speak. Good resonance is equal parts intuition, observation, openness to receive, trust in the process, and distance. Good resonance involves feeling the other person's vibration, reflecting it, using the information it provides, and remaining separate from it, all at once.

Reflection

Thank you for reading this chapter. Now, please take a moment to bring your attention to yourself. Please read the following paragraphs, and then close your eyes and scan your physical body. Notice how what you read reflects in your body. What is happening in your body—is it relaxed? Perhaps start with the breath, and consider whether it is flowing freely. Then consider your posture, whether you are sitting or lying down. Do you feel balanced, or does your physical body have tightness, pain, or discomfort? Look at different aspects of your body, including inner and outer tension. Take a deep, full breath, and sigh out your mouth. Next, consider your thoughts—are they generally centered outside of you? If something external is consuming your attention, can you let it go for now?

Now, consider if the material in this chapter resonates in you, and whether the resonance involves you or someone you know. If there is some triggering—positive, negative, emotional, physical, spiritual, or some other experience—then what does it feel like? Maybe it feels like you're reading something intimate or personal or true about yourself—or maybe you didn't like something you read. What was it? Was the response because the information was revealing, challenging, nurturing, or something else? It's possible that something that's gone unnoticed within you, or something you've neglected or forgotten about, might really be valid

and making itself known. It's possible that this material might have awakened it in you. Or not. You be the judge.

After reading these things, and reflecting on them with your eyes closed, then open your eyes and bring your attention to your surroundings. Listen to the sounds. What do you see? Between chapters, perhaps take a brief walk, or dance to a couple of songs. Move around, reorient, and then settle in for the next chapter. Thank you.

Chapter 12

Solo practice

"I had no idea what was about to happen. I remember coming out of my first experience, and what felt like waking up—I couldn't believe the power of this healing thing that wasn't 'other' than me. I could do it. I was in this incredible awe and disbelief that I had the power all along."

—Claire Kearney, BBTRS Practitioner

Some people who are interested in BBTRS might not live near a facilitator, or prefer to try it first on their own. Practicing BBTRS without a facilitator or trainer has both challenges and benefits. To be fair, let's examine both. Solo sessions probably won't be as deep as facilitated sessions, which is actually good. That is, we generally require support to drop into the deepest layers of release—which means that we have a built-in, self-regulating "release valve." Most people won't let themselves go too far. They also can't stop themselves from "spacing out," which is a protection against going too far. A facilitator can support us to dive back into the body, and support us during any resulting challenges. Therefore, as with any significant practice, working with someone who has training and expertise deepens, enlivens, and expands the breather's access to release.

There are also polar aspects to the idea of "witnessing." One interpretation is the most benign aspect of having someone with us, which is simply to observe. Facilitators notice things we can't notice when we're in our process, because we are not able to see ourselves during a session, either literally or metaphorically. We cannot adequately and dispassionately access the visual experience of our own unwinding process. For example, consider a neutral observer's noticing a "stuck" shoulder during another person's session, and then using intuition in deciding whether, when, and how to facilitate movement in that shoulder. A solo breather can't surrender to the felt sense and at the same time remain in the mind to analyze the experience. Therefore, watching yourself in a mirror would be completely distracting. And asking, "Hey, how's my shoulder doing?" will send a person right out of the body and into the mind.

The deeper implications of "witnessing" can involve a second person who holds space, reminding us that we are not alone. This can be extremely therapeutic when working with deep release. However, witnessing can also happen when we are alone, and are forced to remain in a completely experiential level. Our resources might be internal or external, and they might include another human being, but that person is not physically in the room with us. When we release trauma alone, we connect with ourselves more deeply than in our day-to-day lives. To successfully activate the BBTRS experience, we connect with the breath, our bodily sensations, the movement of our bodies, and our inner witness. We see ourselves more completely.

Many practitioners who are experienced in BBTRS invite some tension release into their own personal or meditative practice, and then ask a facilitator for help when they want to go deeper. Therefore, it is recommended that anyone exploring a BBTRS journey work with a trained facilitator at some point. However, because of the incredible benefits possible during solo practice, let's explore how to structure a solo experience. We will attempt to circumvent as many procedural challenges as possible, and focus on richness and safety.

Activation levels

Often, when a person requests services from a practitioner of any health-related service, the person is suffering in some way. We seek out a massage therapist when we are sore, or a chiropractor when we need an adjustment. We seek out trauma release when we feel tension, and are therefore out of balance. We might be significantly activated by the time we walk through the practitioner's doorway. When conducting a solo BBTRS session, assessing the level of activation is crucial to creating a safe and productive experience. This assessment requires the conscious and conscientious use of the felt sense.

High activation

In cases of extreme activation—chronic nervousness, emotional upset, obvious tension throughout the body, extreme ups and downs—inviting further activation of the nervous system through deep, connected breath is not advised. Meditation is a better place to start. Or, simple body shaking—literally vibrating your physical body while standing, sitting, or lying down—can be used to release high-level activation. Shaking your body for fifteen to twenty minutes, followed by sitting silently, will do wonders in releasing extra tension and activation.

In moments of acute, high activation, immediate relief can be achieved through (1) knowing that this state will pass, like all other states, and (2) simply sitting still, breathing softly through the nose, with eyes closed. Release thoughts as they pass, and just be in your body. If you are in a place in your life where high activation is the norm, it's possible that BBTRS can be used in conjunction with other interventions or practices. Please consider a variety of options along with meditation, including counseling, massage, Reiki, and visiting your Chinese Medicine or

Western medical practitioner. In these periods of life, BBTRS or any deep-breathing practice should be done with a trained facilitator.

Medium or low activation

In other cases, on other days of just "normal life," how do we assess our activation level? You guessed it: by using the felt sense. If you are, right now, trying to determine if you feel "activated," just tune into the body and sense it. Initial concepts that flow through your awareness might include "unease" or "imbalance," but those are not physical. Consider what those concepts feel like. Words that might describe areas of activation include "tightness," "tingling," or something familiar, like "stomach ache." Then ask yourself, "Do I feel good about doing this on my own?" If your intuition signals hesitation, then stop. Ask a trusted friend or intimate to hold space for you. If your intuition signals calm and confidence, or you're even a little excited at the prospect, or hopeful that release will occur, then proceed.

Preparation & tips

Throughout this book, we have discussed processes and cautions. Before attempting a solo session, please read the entire book. Take extra care if you have limited movement, lung capacity, or muscle tone; if you know yourself to react strongly to stimulation; if you suffer from chronic injury or illness, or are recovering from an ailment; or if there are any other circumstances in which common sense would signal caution (see Chapter 10 for more on age, physical limitations, contraindications, and other considerations). When setting up your practice space, also use common sense. Set the room's temperature at a comfortable level; make any necessary adjustments so as to maintain this temperature for at least an hour. Arrange the space so that there is nothing you might knock into. There will be no one to scoot a cushion under your elbow, so make a padded area that's the size of at least a single bed, and even a double bed, if you can. Place a box of tissues and a bottle of water where you can reach them. Wear clothes you can move easily in, like yoga or exercise clothes; working without clothing is also fine, if this is natural for you. Schedule plenty of time for the session, and don't plan to immediately rush off to do anything important, like vote in Congress or operate machinery. Give yourself time to ground and feel "like yourself" before moving on.

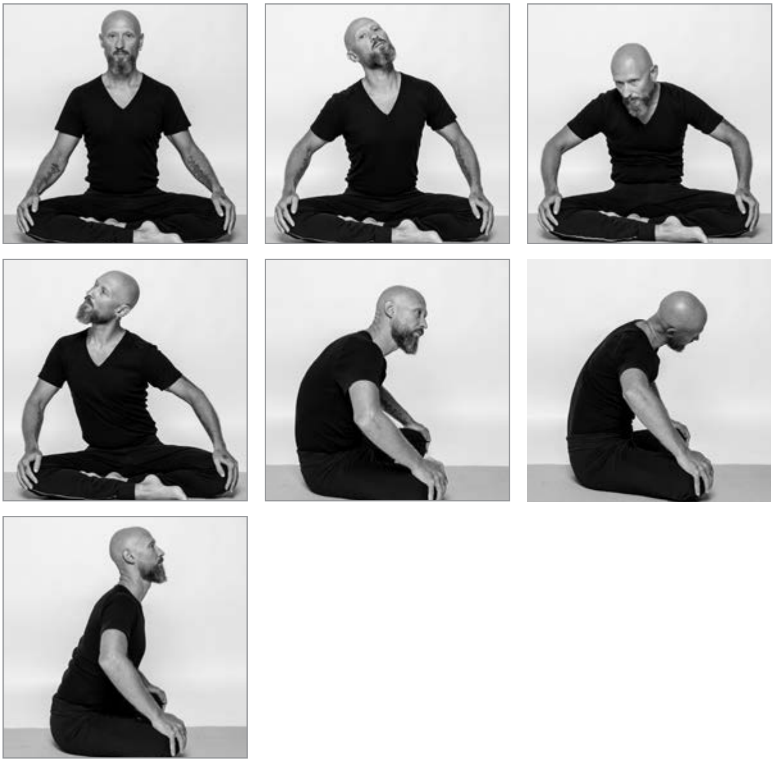
There are several demonstration videos on YouTube, which can be accessed through a link from the BBTRS website, or by searching "BioDynamic Breathwork & Trauma Release Institute." Viewing them can prepare a person for the overall experience. In addition, when setting up to work alone, sound can be very beneficial. You can use the same sort of music you might use in a massage session, or download BBTRS-specific soundtracks from Spotify. Just search "Giten Tonkov" on the Spotify app or website.

Finally, you can set timers, preferably using gentle bell sounds or something that doesn't jar you, to signal rounds. That is, have a bell signify when to go back to resource and another to stimulate another breathing segment.

Before launching into an actual session, let's begin by getting used to some of the BBTRS elements. *Assuming you have no physical injuries or limitations that contraindicate them,* try these short experiences:

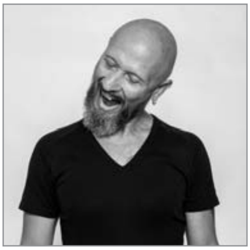
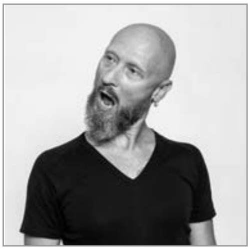
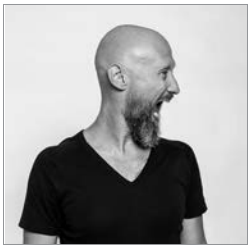
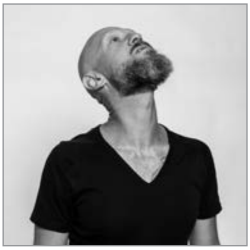
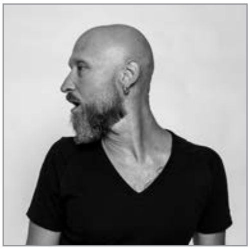
- Stop reading for a moment and simply tune into your breath.
- Put on some music and dance for a few minutes.
- Close your eyes, scan your body for tense areas and relaxed areas.
- Breathe into both areas and note what changes.

- Think about your internal resource, and write down all the physical sensations that make this place stand out.
- If you are familiar with toning, try toning a few sounds. Feel the vibrations they create within your body.
- Sit comfortably and experiment with gentle spinal movement. Just move side to side and in a circular motion from the hips upward.
- Intentionally shake your body for a few minutes. Just sit or stand and let it vibrate. Just shake it out.

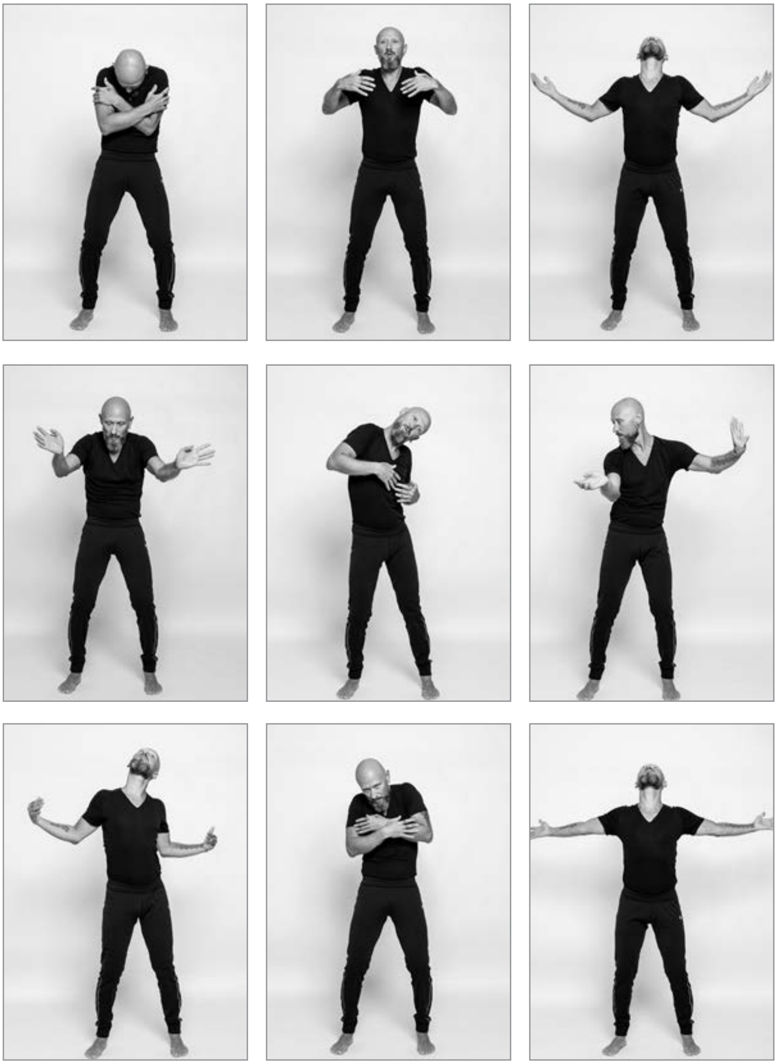


Next, try this "practice session," which should acclimate you further with the experiences in an actual session.

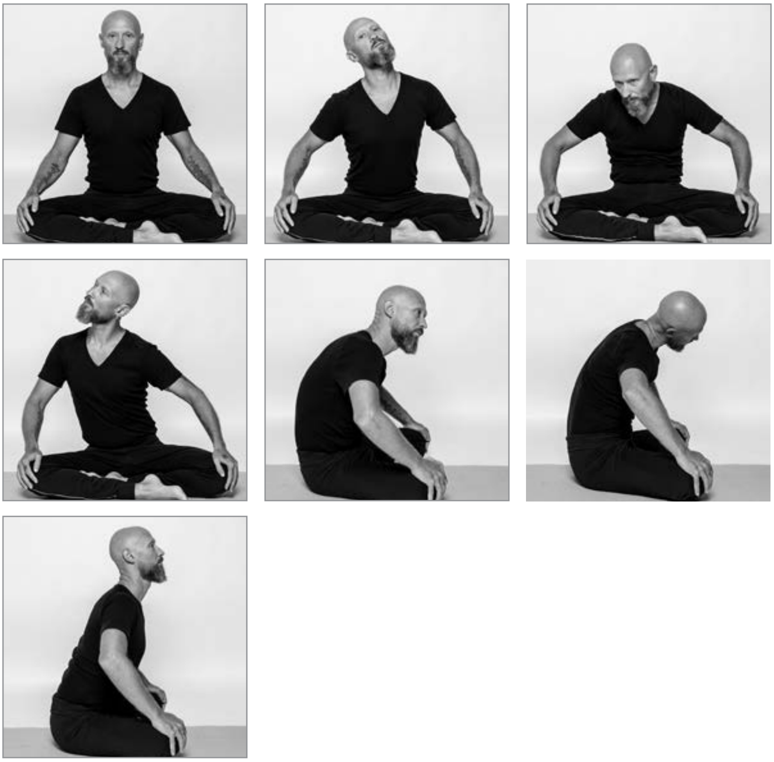
1. In a standing position, begin with an unwinding movement of neck. Unlock the jaw and face.

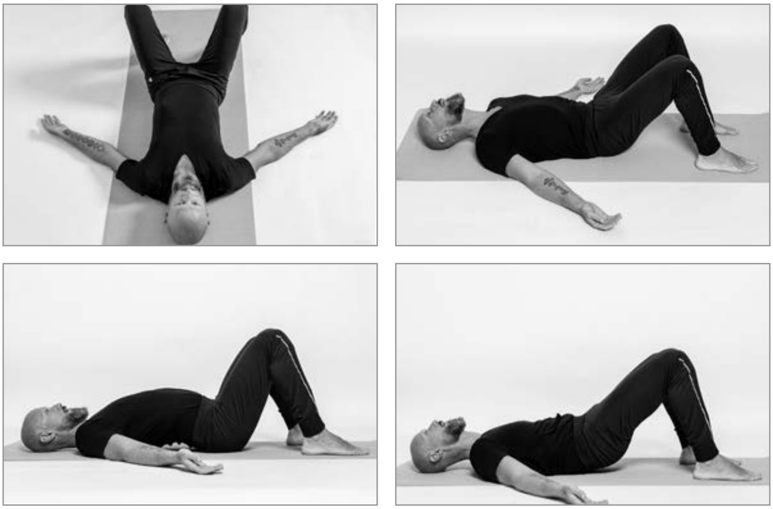
2. Next, involve the shoulders and arms, opening and closing them with the breath. That is, with inhalation, open the arms and shoulders wide; with exhalation, close them, or even hug the body. Gradually involve the face in the movement.



3. Sit down, rotate in spiral motion originating from the hips and waist. Isolate the movement of the spine and involve the neck, assuming that the neck is able to move freely.



4. Lie down with knees bent, feet on floor. Allow the pelvis to rotate slightly with breath, pushing down and away with inhalations and contracting with exhalations. Press the heels slightly into floor.



Please note: If you feel dizzy at any time while standing, squat down and touch the floor. In all cases, make sure there is plenty of padding under and around you in case you were to lose your balance.

Session structures

At first, you might set relatively short intervals, with perhaps ten or fifteen minutes of deep, connected breathing and shorter resourcing breaks. For those with lung and/or other limitations, breath intervals can begin at two to five minutes, and increase to approximately ten minutes. As comfort and experience grows, the length of time in each round can grow, and additional rounds can be added. Plus, once a breather becomes comfortable with BBTRS, smaller "refresher" sessions of just one breathing round can assist during particularly stressful days or just moments when tension needs to be released. These can even be done in a seated position, with unwinding generating from the waist, hips, shoulders, arms, and neck.

Regular, full sessions can begin in seated or reclined positions; unless a breather has physical restrictions, however, full expression of movement is greatest when reclined. Examples of how sessions can be structured follow. They all assume that the person has initiated deep, connected breathing.

Sample session 1:

Preparation and scanning: 5 minutes
Breathing round: 10 minutes
Resource round: 5 minutes
Breathing round: 10 minutes
Resource round: 15 minutes

Sample session 2:

Preparation and scanning: 5 minutes
Breathing round: 15 minutes
Resource round: 5 - 10 minutes, depending on activation
Breathing round: 15 minutes
Resource round: 20 minutes

Sample session 3:

Preparation and scanning: 5 minutes
Breathing round: 15 minutes
Resource round: 5 - 10 minutes
Breathing round: 15 minutes
Resource round: 10 minutes
Breathing round: 15-20 minutes
Resource round: 20 minutes

Sample session 4:

Preparation and scanning: 5 minutes

Breathing round: 15-20 minutes

Resource round: 5 - 10 minutes

Breathing round: 20 minutes

Resource round: 5-10 minutes

Breathing round: 20-30 minutes

Resource round: 20 minutes

For those with lung or movement limitations:

Modified sample session 1:

Preparation and scanning: 5 minutes

Breathing round: 2–5 minutes

Resource round: 5 minutes

Breathing round: 2–5 minutes

Resource round: 15 minutes

Modified sample session 2:

Preparation and scanning: 5 minutes

Breathing round: 5–10 minutes

Resource round: 5 - 10 minutes, depending on activation

Breathing round: 5–10 minutes

Resource round: 5 - 10 minutes, depending on activation

Breathing round: 5–10 minutes

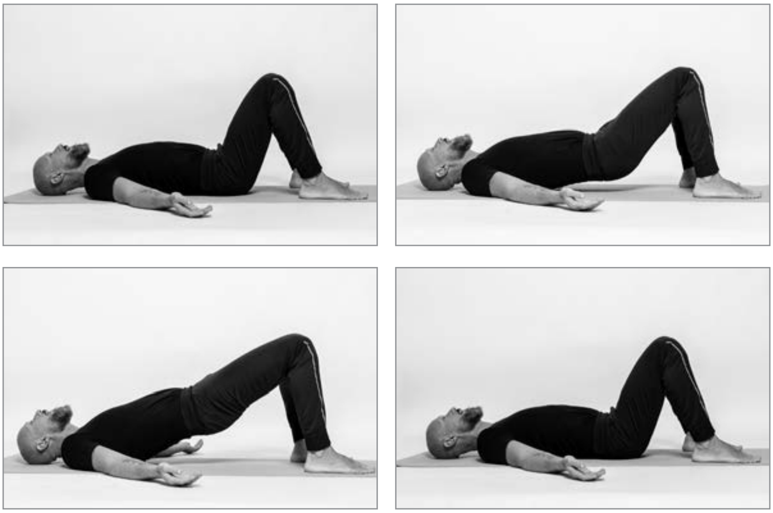
Resource round: 20 minutes

Getting started

- Review the space; make sure you have everything you need. Turn off the phone, unless you are using it for a timer.
- Stand naturally in front of a full-length mirror, and take stock of your physical status. How is your body holding itself? Is one shoulder higher than the other? Is your pelvis tilted? Is your head forward on your neck? In a moment, you'll use these details as starting points when scanning from the inside, with your felt sense.
- Sit cross-legged or in another comfortable fashion in the middle of your mat or padded area. Use a bolster if needed. Close your eyes and center yourself. Take a few full breaths, but do not begin activation.
- Bring attention to your emotional state. What emotions are at the surface, and which ones might be buried deeper, wishing to come forward? Notice how your emotional state is reflected in your physical body.
- Consider the depth of your breath. Is your breath naturally shallow? If so, take a couple of natural, but deeper breaths and sense if the body naturally relaxes. Make sure your lower jaw is relaxed, with mouth fully open, so that you're not breathing through pursed lips or clenched teeth.
- Scan the body systematically from the feet up, noticing areas that are more or less relaxed or tense. Does any place in your body feel particularly open or closed, full of "space" or "wound-up," overstretched or holding?
- Pay attention to the belts of tension, and see if your tense areas correspond to those.

- Identify resources. Hopefully, you will find something internal that feels especially inviting or helpful, but external resources are also excellent. Keep in mind that resources might change from day to day, and even while we are in a session. The idea is to be absolutely sure that you have helpful resources, and places you can go for strength. If you have difficulty finding a place of resource in your physical body, then bring your attention to the areas of the body that feel better than others. Notice what this difference is and name it to yourself.
- Begin deep, connected mouth breathing. Be sure to remain in the body, feeling the breath "go to" the areas of tension. After several moments (for some, activation occurs very quickly; for others, it might be five to ten minutes, especially at first), you will feel your body naturally begin moving. If seated, the movement likely will originate in a rocking or rotational movement in the hips. Arms and head are also areas that respond quickly. If reclining on your back, the hips and legs are often the first place to express.
- Allow the body to move organically. If seated, when the expression feels like it needs to expand, move to a position on the back. Continue to express, focusing on areas of tension.
- When the timer rings, or when activation feels high, then begin breathing through the nose and focus on areas of resource. Notice physical sensations that make these places of resource stand out and feel different from the rest of your body. Pay close attention to all other body sensations that begin to arise. These might include tingling in your arms and hands, and around your lips and face. Include all sensations into your practice. Observe yourself as a whole. You might feel yourself as a fully charged, energetic being.

- Repeat cycles. Be aware of deeper levels of activation and release.
- If emotional expression occurs—crying, laughing—allow it, but avoid going into the mind regarding its source or meaning. Remain in the body. If thoughts arise, allow them to pass through, considering only how they feel in your body. For example, "I'm angry at X for Y," translates to *what feeling* in the body? By continually returning to how emotions translate to the physical, emotional charges lose intensity. With each glance away from emotion, we feel our physicality.
- If you notice "stuck" places, feel free to touch them with your hands. Apply slight pressure or massaging actions to assist in release. Use your intuition to help.
- If at any point during breath cycles you realize that your body is not expressing, or you feel as if you're "losing contact" with the activation, then focus on connecting the breath, and perhaps breathe more deeply. Also, placing the legs in the "tremor position"



or bouncing the pelvis gently on the floor can activate the psoas and assist in connecting with the autonomic nervous system.

After the breath cycles are complete, allow a significant grounding/resourcing period. Most people come out of a session calm and peaceful, and it's okay if you fall asleep afterward. However, most people generally are not "tired" after a BBTRS session.

Supporting exercises

In the Appendix, a variety of exercises can be found to enhance the BBTRS breath sessions. These can be done before a session, either earlier in the day or immediately preceding a session. When planning sessions, consider using exercises in order—by starting from the ocular belt of tension and moving downward, perhaps one belt/session per week.

Remaining aware

Assuming that a person is physically healthy and otherwise prepared for the experience, it's a pleasure to experience BBTRS. Unlike a talk-therapy session in which you know you must "face your demons," or simply tackle a niggling issue, BBTRS should be a clearing, pleasant experience. Yes, a person might also recognize that deep-seated traumas are releasing, but the usual correlation with that process is relief. The more comfortable we are with the body, and the more open we are to positive change, the more organic and natural BBTRS becomes. With familiarity, we can just enjoy it, and surrender to the process.

For BBTRS to work, we need to feel. We need to feel the physical indications of where to find the tension, fear, contraction, hesitancy, or any other road sign giving us information. One small experience is worth a thousand words. Once we feel—both the originating tension and its release—we understand the power of the process. We might first think we understand it—the ideas might make perfect sense to our minds—but that's just information. The experience of release is readily available at any time.

It's important to recognize that traumatic experience comes packaged with our emotional responses. Therefore, as we play with and practice the BBTRS modality, we might notice the presence of emotions, especially fear, associated with a traumatic experience. This can occur whether we remember or are considering the experience itself. Fear can either stop us in our tracks or point to the exact directions we need to move. If you feel fear in the physical body or through arising emotions, go gently, cautiously into it. Bring your attention to the physicality of feeling fear. What parts of your body contract and shrink as the emotion of fear arises? Use fear as a tool—but at the same time, be very gentle with yourself. Don't go into a place of intense pain or emotional expression. Don't push yourself over any limits, but expand the container you think you're in. At each step of the way, you can expand these boundaries a little wider. At first, you may just touch the outside edge of the fear, and then return to your resource. Next time, stay slightly longer. The third time, it's usually not as scary as you thought it would be. Your resilience is more expansive than before. You know what might come up, and your ability to cope increases.

This is the general pattern for BBTRS as a whole. We become more resilient beings as we walk through the world. We strengthen our "resilience muscle." We strengthen our presence in our bodies, minds, and spirits. Our *xin* becomes more expansive, as does our relationship to others.

Further exploration

If the material in this volume resonates with you, then please explore the various ways you can experience it yourself. One way is simply to find a practitioner near you, someone to help you move to the next level in traumatic release. In addition, BBTRS offers two training tracks, an Experiential Route—which is open to anyone—and a Professional Certification Program. The professional program is open to healing artists with or without prior experience who are moved to help others. Both tracks are offered each year through BBTRS training workshops, in various countries around the world.

In whatever way you experience this work, thank you for your openness and willingness to heal. We hope you will include BBTRS in your own practice, and also share its possibilities. For more information on finding practitioners, attending workshops, or joining the BBTRS team, please explore biodynamicbreath.com.

Reflection

Thank you for reading this chapter. Now, please take a moment to bring your attention to yourself. Please read the following paragraphs, and then close your eyes and scan your physical body. Notice how what you read reflects in your body. What is happening in your body—is it relaxed? Perhaps start with the breath, and consider whether it is flowing freely. Then consider your posture, whether you are sitting or lying down. Do you feel balanced, or does your physical body have tightness, pain, or discomfort? Look at different aspects of your body, including inner and outer tension. Take a deep, full breath, and sigh out your mouth. Next, consider your thoughts—are they generally centered outside of you? If something external is consuming your attention, can you let it go for now?

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After reading these things, and reflecting on them with your eyes closed, then open your eyes and bring your attention to your surroundings. Listen to the sounds. What do you see? Between chapters, perhaps take a brief walk, or dance to a couple of songs. Move around, reorient—and then settle in for the Appendix and its sampling of exercises and meditations. Thank you.

Acknowledgments

With heartfelt gratitude to:

My parents for giving me life, believing in me, and supporting me on my path in everything I wanted to do. My son Sage and his mother Sandra Ferreira, for showing me how to flow with life and be ready for anything. For being a constant, loving, accepting presence in my life.

Chetana Barkan, who stood by me from the beginning and witnessed how BBTRS evolved into what it is today. A full gratitude to Chetana for continuing to be present fully and wholeheartedly. For believing in me and BBTRS and supporting BBTRS as it grew, and for keeping track of all the numerous details. For being fully present in the group room and selflessly showing up as a strong female presence in numerous workshops we lead together around the world.

Prema McKeever, for using her scientific mind as well as tireless desire to apply her knowledge base. For her input to the creation of BBTRS's educational training webinars.

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Bodhiray, Devapath, and Dwari of Osho Diamond Breath, for being my first breathwork teachers and supporting me to uncover my vulnerability and sharing their knowledge of breathwork and body oriented therapy. To Countless other teachers throughout my life who taught me everything I know.

To ALL music and sound—the biggest resource in my life.

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All my friends and lovers, past and present, for being my mirrors—and who supported me to keep my heart connection and teaching me what love, sexuality, honesty, vulnerability, and trust is all about.

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My personal clients, group participants, and trainees, for giving me their trust over the years. For believing in this work and having courage to dive in deeply into personal exploration process. For being a living proof of the wonderful healing that this work can bring to those who trust and are ready to take steps to self-healing.

To India, which over the years filled me with her beauty, embraced me, “pushed my buttons,” and taught me to surrender to and learn from anything that comes my way.

New York City for teaching me the street-smart ways that proved to be useful in many ways in my life.

To Bali, for sharing its beauty with me and being such an amazing home base for many years on the road.

To the Greek Island of Lesvos, and Osho Afroz Meditation Center, my summertime retreat for many years, and the place where I can recharge myself, drop into meditation, and share my work and my presence with countless friends from around the world.

Appendix

Exercises for the belts of tension

A breather can enhance the BBTRS experience with exercises that target each part of the body. After realizing where and why tension gathers in particular areas, it's easy to connect when one or another part of the body needs special attention. These exercises can be done on their own or in connection with BBTRS sessions. If a particular exercise is related to a BBTRS session, often it's best to do it on the morning of the session, so that your attention and energy are already focused. In this way, a breather can enhance, deepen, and connect the session with intention. Please note:

- While it's possible to do more than one exercise a day, any single exercise can bring up a lot of energy. Therefore, it is recommended to do just one exercise per session, or two at most. Allow sufficient time to process what has come up before moving further, whether into a full BBTRS session or into some other part of the day.
- Be careful not to allow the exercise to introduce an overcharged state, where there's a potential to reconnect with past trauma. For healing to happen, a breather needs to be fully present to their experience. When overcharged, the intensity disconnects the breather from feelings and his or her felt sense.

- Do not engage in any exercises if physical restrictions or injuries could be exacerbated by so doing. If there are any questions as to a breather's health and the suitability of these exercises, participants are asked to consult with their physicians. In addition, if facilitators assist in these exercises, they should be cautious regarding protecting the physical and emotional health of breathers.

The importance of movement

While there are countless exercises and preparations that a person could use to prepare for a BBTRS session, we have discovered that a good mixture of movement and stillness is usually best. "Active meditations" can be especially useful, since they take into account the fact that modern people are busy. Our minds are active; our days are full. This is why meditation is also a great counterbalance. It's important to move and to be still.

When our bodies are in motion, they provide an opportunity to check in, and to sense what is happening for us as living beings. Movement can come from the inside-out or the outside-in. The exercises we integrate into BBTRS training workshops generate movement in the body, help to open the breathing apparatus, and increase self-awareness.

We're used to sensing ourselves while we're sitting still. We know how to tune into the inner landscape, and we know what an emotional charge feels like. We're used to our body's inner movement, even if it's subtle. However, we're not used to paying attention to the inner functioning of our bodies when we are in motion. We don't notice that movement dissolves contracted fascial tissues; brings the mind into a silent place; activates the emotional body; and increases circulation and respiratory function, as well as the volume of breath. We don't notice that if we attend to our body’s sensations as we move around, we actually stay present to what we’re doing. The following exercises help us to cultivate the ability to be with the felt sense, to be with self while in movement, and to get to know ourselves better.

Kundalini Meditation

Although this meditation is focused on the pelvic belt, it's listed first because it is a general meditation that can be used by anyone at any time. It's a dance meditation, and can accompany a wide variety of dance music. Perhaps you'll want to create a soundtrack—or use the Spotify tracks available to you through BBTRS—and choose a different one each day. Choose a space with plenty of open area, as you'll do this meditation while wearing a blindfold.

First Stage, Blindfolded Free Dance: 30–40 minutes

After ensuring that the space doesn't contain anything you can bump into, start the music, place the blindfold, and explore movement with your body. Unless there's a real reason, do not remove the blindfold; its purpose is to allow the energy created by the movement to be held within the body. Otherwise, it can escape through the eyes.

Second Stage, Sit Quietly: 20 minutes

Maintaining the blindfold in place, sit in silence.

Ocular segment exercise

BBTRS Flow Meditation

This meditation lasts one hour and has four stages.

First Stage, Standing Unwinding: 15 minutes

With eyes closed, begin from the neck and move down the spine with an unwinding, undulating movement. Involve the entire face, jaw, and eyes—and then allow the unwinding to spread down to involve the pelvis. Since the purpose is to create flexibility in the spine, then reverse the movement. That is, once the unwinding has reached the pelvis, start sending upward "waves" back up the entire spine. Throughout the fifteen minutes, breathe deeply in through the nose and out through the mouth.

Second Stage, Seated Unwinding: 15 minutes

Sit down with eyes closed, mouth open and loose. Let your body sway from the waist, like a reed blowing in the wind, from side to side, around and around. Allow the movement to be organic, unfolding naturally, from moment to moment. Your awakened energies will gather at the navel center (*hara*). Next, let this movement become a spiraling motion, out from the center into large, circular movements. Throughout the fifteen minutes, breathe in through the nose and out though the mouth.

Third Stage, Reclined Unwinding: 15 minutes

Lie down comfortably on a mat, with open eyes. Start moving your eyes clockwise in a big circle, as if you are tracing a big clock on the ceiling. Your head remains still; only the eyes move. Maintain a gentle, slow breathing pattern, through the mouth. Try not to space out. Be aware of everything within your body that the eye movement and breath stimulate or cause to move. Allow your body to be free to express, in a slow, connected movement. Use your felt sense to observe all body sensations.

Fourth Stage, Silence: 15 minutes
With eyes closed, remain in the reclining position and simply become a witness to your body.

Oral segment exercise

We often hold the emotions of anger, rage, frustration, and resentment in our jaw and around the mouth. When people grind their teeth during sleep, tension in the jaw muscles can result. The tension can be painful and even lead to deterioration of the joint, or a condition called TMD or TMJD, temporomandibular joint dysfunction/disorder.

OSHO Dynamic Meditation

"This is a meditation in which you have to be continuously alert, conscious, aware, whatever you do. The first step, breathing; the second step, catharsis; the third step, the mantra, 'Hoo.' Remain a witness. Don't get lost. It is easy to get lost. While you are breathing you can forget; you can become one with the breathing so much that you can forget the witness. But then you miss the point. Breathe as fast, as deep as possible, bring your total energy to it, but still remain a witness. Observe what is happening as if you are just a spectator, as if the whole thing is happening to somebody else, as if the whole thing is happening in the body and the consciousness is just centered and looking. This witnessing has to be carried in all the three steps. And when everything stops, and in the fourth step you have become completely inactive, frozen, then this alertness will come to its peak." ~ Osho

This meditation is a quick and intense, and a powerful way to break old, ingrained, limiting patterns in the body-mind structure that keep one imprisoned in the past. The meditation helps one to experience freedom, the witness, stillness.

The Dynamic Meditation lasts one hour and has five stages. It is set to a specific musical soundtrack. Keep your eyes closed through the entire meditation or use a blindfold, if available. It can be practiced alone, but can be even more powerful if it is practiced with others.

First Stage: 10 minutes

First, blow and clear your nose. Then, while breathing chaotically through the nose, let the breath be intense, deep and fast, without rhythm, with no pattern, always concentrating on the exhalation. The body will take care of the inhalation. Breathe deeply into the lungs, as fast and hard as you possibly can until you literally become the breathing. Use your natural body movements to help you build up your energy. Feel it building up, focusing only on this type of breath in this stage.

Second Stage: 10 minutes

EXPLODE! Let go of everything that needs to be thrown out. Follow your body, giving it freedom to express whatever is there. Scream, shout, cry, jump, kick, shake, dance, sing, laugh, throw yourself around. Hold nothing back; keep your whole body moving. A little acting often helps to get you started. Never allow your mind to interfere with what is happening. DON'T HOLD BACK!! (One tip that assists in the release of tension around the mouth, jaw, and throat—assuming that teeth are strong and healthy: bite with molars onto a twisted towel, while pulling towel forward with hands. Caution: this can produce a gagging effect.)

Third Stage: 10 minutes

With arms raised high above your head, jump up and down shouting the mantra, "Hoo! Hoo! Hoo!" manifesting this mantra from deep down in your belly. Each time you land on the flats of your feet, let the sound hammer deeply into the sex center (base chakra). Give all you have; exhaust yourself completely. Keep your arms raised as best you can at all times. DO NOT GIVE UP ON YOURSELF.

Fourth Stage: 15 minutes

STOP! Freeze wherever you are, in whatever position you find yourself. Don't arrange the body in any way. A cough, a movement, anything, will dissipate the energy flow and the result will be lessened. Be a witness to everything that is happening to you. Don't think. Just observe, for the entire fifteen minutes.

Fifth Stage: 15 minutes

CELEBRATE! Once the music begins, dance and express whatever is there. Laugh, smile, move, and just be. Move in whatever way your soul wishes to celebrate.

Cervical segment exercise

The cervical belt of tension has to do with "swallowed" expression, and in particular, vocal expression. The free flow of energy through the throat gets interrupted and settles as a ring of tension just below the chin and at the bottom of the throat. This compromises the ability to speak up for yourself and express your truth freely.

Gibberish Meditation

This hour-long mediation comes to us from the Sufi tradition.

First Stage: 30 minutes

Speak a language you don't know. Make sounds, gestures and movements. Do not make any pauses. The gibberish has to be continuous for a minimum of 30 minutes. While speaking, be very dynamic with your body and use an expressive, assertive voice. Speak with your entire body movement. Move around the room. If you do this with other people in the same room, do not listen to anyone else's gibberish but your own.

Second Stage: 30 minutes

Sit or lie down in silence. Witness your mind, body and breath.

Thoracic segment exercise

The thoracic segment includes the shoulders, arms and chest. Often this area is related to the feelings of reaching out for pleasure and love. When this impulse to reach out is interrupted, we form armoring around the chest and shoulders. We literally protect our heart from being hurt, but feel the pain of separation. It is something like the iron breastplate of a medieval knight. However, this natural desire to reach out—to meet and be met in love—always remains in place no matter how long the impulse has been interrupted. When the impulse starts to flow once again from the heart toward the periphery, our touch takes on a different quality. The hands become charged with heart energy, and then the energy travels up to the head and down to the genitals, legs, and feet. This drastically changes how we relate to ourselves and to the world around us. Love begins to flow once again.

The act of reaching out can help to unlock tension and trauma in the thoracic region.

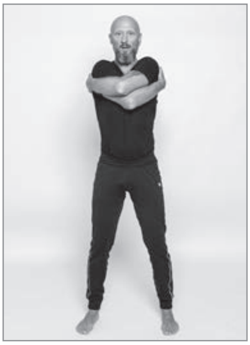
Heart–Breath Meditation

This exercise is done with a partner. You and your partner face each other and maintain eye contact throughout the meditation. Before starting this meditation, explore the following self-inquiry, and allow at least five to seven minutes for this question.

"What is a strategy I have developed in life to keep my heart protected?"

Explore this question thoroughly. Try to name any strategies you might have put in place to avoid the pain of separation and broken heart. These strategies can be:

- Spacing out
- Avoiding deeper connections with people
- Overeating
- Ignoring your urges
- Being a nice guy/girl
- Being outgoing
- Being lonely/alone
- Meaningless sexual connections
- Avoiding sexual connections
- And many more

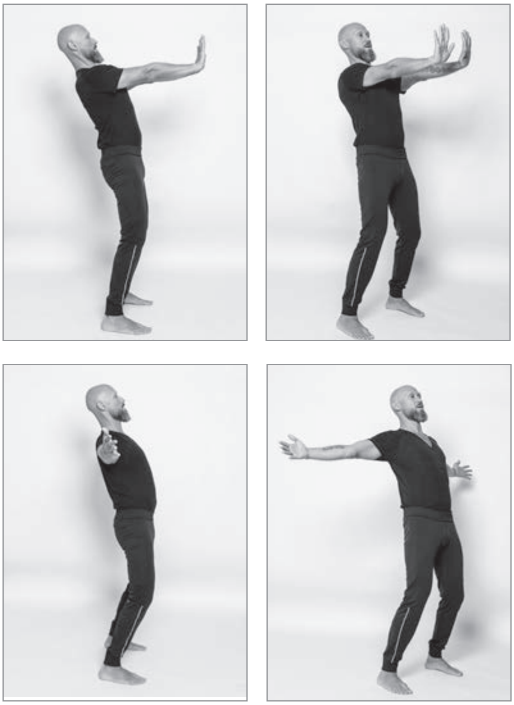


First Stage

Stand opposite your partner. Close your eyes and wrap your arms around yourself as in giving yourself a tight hug. Contract every muscle in your body. Allow yourself to experience the pain of carrying this protection. Stand in this position for a while.

Second Stage

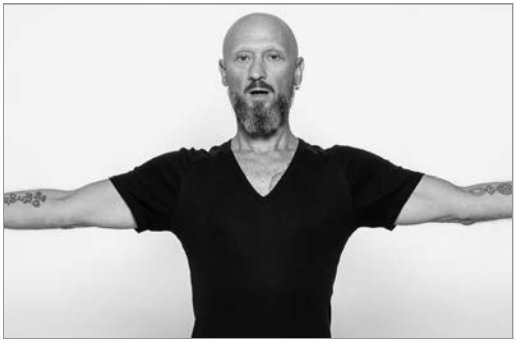
Slowly open your eyes and face your partner, maintaining eye contact. Raise your arms to shoulder level; hold them out, with elbows locked and hands in the type of protective position used to stop someone from

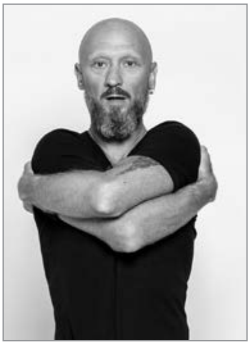


coming close to you. Connect with the feeling that your arms are an extension of the heart—and that the position protects the heart from others. Keep eye contact, as well as deep, connected breathing. As the pain starts to set in at the shoulder, maintain this position for a few more minutes. **Very slowly** start to open your arms toward each other, beginning to drop this protection. Let any emotions flow, maintaining eye contact. After a few minutes, slowly start to take steps toward each other until you come into embrace. Allow time for this to happen.

Third Stage

Sit down opposite each other. Place your hands on your heart and look into each other's eyes, breathing into your heart. Start to open your arms with each inhalation and bring them back to yourself with each exhalation. Come back to touching your heart with your hands and then again as you inhale, opening your arms as wide as possible—expanding in all directions and then bringing them back with each exhalation. With each out-breath come back to yourself and to the feeling of your heart. With each in-breath, expand toward each other and to the areas around you. Throughout this exercise, it is VERY IMPORTANT to maintain eye contact and to synchronize arm movements with the breath.





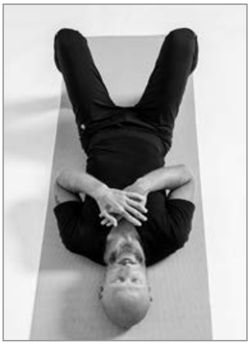
Fourth Stage

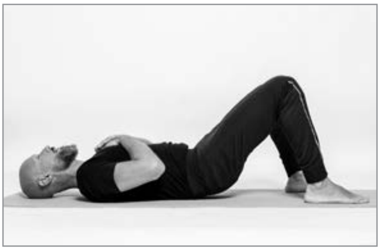
Bring your hands to your heart and breathe into your heart, by yourself, with closed eyes.

Allow yourself to feel the energy in your heart, and expand each breath fully into your chest. Come close to your partner and sit together, belly to belly and heart to heart. Stay in this position, holding each other and breathing into your belly and heart.

Fifth Stage

Lay down on your back with knees up and hands on your heart. Breathe while rotating the pelvis with each breath. Surrender to this movement and allow the energy to flow up into your heart, up to your neck and out through the top of the head. Completely surrender to the movement, allowing your body to move freely as you breathe.





Sixth Stage

Relaxing the breath and allowing yourself to fully surrender, relax into experiencing any physical sensations. Stay present and do not disconnect. It is vital to stay alert, feeling all the sensations and emotions that are present in the body. At the end, reach out to your partner and connect with him in the deep hug of surrender. Allow yourself to hold and be held by your partner.

Diaphragmatic segment exercises

The diaphragm is our main respiratory muscle. It is a "bridge" that connects and at the same time separates the lower and upper parts of our bodies. The actions of the diaphragm are very much affected by emotional stress and traumatic events. It reacts to fearful situations by contracting. When contraction becomes chronic, anxiety can occur. The word "anxiety" actually comes from the German word "angst," the Latin and Greek roots of which mean tightness, choking, and to strangle, sometimes related to a narrows.

The diaphragm sits above another passageway or "narrows"—the waist. This passage connects the thorax with the abdomen and pelvis. Impulses pass through this passage to the lower part of the body.

Tension in this area chokes the flow of energy and feelings to the genital area and to the legs, producing anxiety and a fear of falling. This results in the holding of the breath.

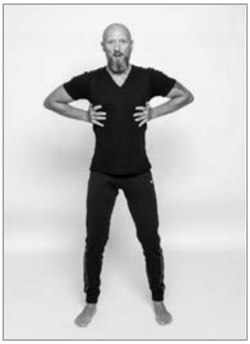
A "frozen" diaphragm and ribcage minimize the amount of air we can take in, which in turn causes the feeling of anxiety to arise.

Working with the belt of tension above the one you're addressing in a particular session supports the energy flow through the entire body. This sets the stage for the entire system to release.

Meditation to open the ribcage & diaphragm

First Stage

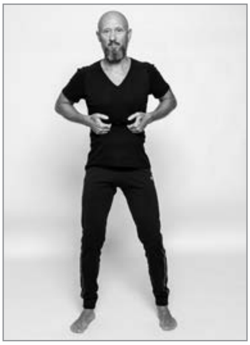
The breather stands fully grounded with feet apart and knees slightly bent. Observe how the natural breath is flowing through the entire body—including which parts are inflating with the inhalation, and how this creates movement in the body. Place your hands on those parts of the body where you sense movement. Imagine a rubber ball in the chest; imagine it inflating and returning to its original shape with every inhalation and exhalation. Become familiar with this sensation. For some people, it can be an entirely new experience to feel the body in this way. Place each hand on different areas of the body, encouraging the breath to these various locations while still imagining the rubber ball inflating and deflating with each breath.

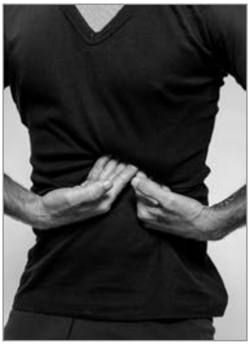


Then, while bringing the attention to the ribcage, put the hands on either side of the ribs, squeezing gently on exhalation and releasing on inhalation. This encourages movement of the ribcage and relaxation of the intercostal muscles. Stay in this area for some time, as it takes a while to open this section of the body. A facilitator can support the breather with touch on the back, and also apply more pressure on the ribcage by putting his or her hands on top of the breather's hands for more support.

Second Stage

Bring fingers to the midline of the body and hook fingertips under the ribcage, moving from the sternum toward the floating ribs, tracing the





line of the ribs downward with each exhalation. This self-palpation works the attachments of the diaphragm. Stay there for a few breaths.

Additional diaphragm, ribcage and throat opening exercises

1. **Sitting or standing**, use a "panting" breath, deeply inhaling using the diaphragm for two to three minutes. Feel the instinctive responses of the body. Usually this way of breathing is followed by a deep inhale and a deeper exhale afterward. Do this a couple of times, and put your hands on the diaphragm to feel the movement.
2. **Play at being a train**, by moving the arms forward and back (elbows bent) and engaging the breathing muscles. Let the sound vibrate from the diaphragm to the throat. The opening of the diaphragm and throat happens playfully. With each outbreath make the sound "choo," and really push the air out.
3. **Playfully imagine yourself as a dog**. Begin to bark. This type of playful breath deeply impacts the diaphragm.
4. **Stand and bend down on the in-breath**, arms in front of chest. On the out-breath, explode with a cathartic sound, such as a sigh or groan. Do this a few times. You can also crouch down into a small

ball and then stand and open arms powerfully, with a big sound on out-breath.

5. **After exercise 4, if working with a facilitator, look into the facilitator's eyes.** While standing still, breathing, and anchoring the feet firmly into the ground, use sound—such as growling, or saying NO strongly. When the whole body is vibrating with energy, lie down and continue with the deep connected breathing.

Allow any emotional expression that arises, including kicking, punching, hitting, and any other expression. Whether working alone or with another person, just make sure to be careful not to overcharge and not to dissociate!

Abdominal segment exercises

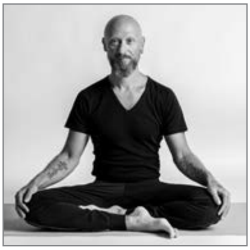
In our bellies, we store the emotions of fear and trust. It is by far one the most vulnerable places in our bodies. Our scull protects our brains, the ribcage protects our upper organs, but there's only a thin layer of muscles and skin that protects our digestive organs. Due to our fear of injuring these body parts, many of us have learned to keep the abdominal muscles in constant contraction. In addition, "health-related" advertising conditions us to want "six-pack" abs, creating a false ideal that we desperately try to fit into.

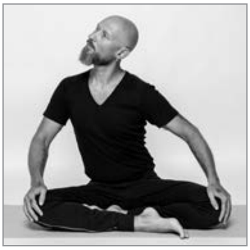
Abdominal-opening meditation

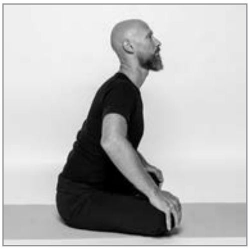
This mediation can be done alone, with a facilitator, as part of a BBTRS session, or as an entire session in itself. It can be a bit confusing, so read carefully and review the illustrations.

First Stage

Start by sitting with the spine straight, hands on knees. Begin with soft, connected breathing through the mouth. Bring your attention to the second chakra, just below the navel. Begin to rotate the body clockwise from this center point in an unwinding spiral movement for about five minutes, and then spiral back to center in the same direction for the same length of time. Throughout, the direction of the unwinding and "rewinding" back to original point remains the same. That is, "spiral back to center" does not mean to change direction; instead, it means to reduce circumference.

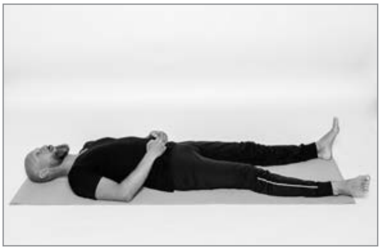


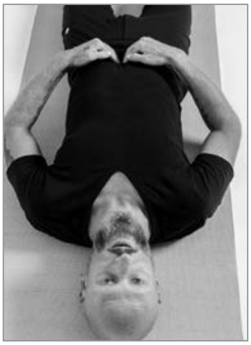




Second Stage

Lie down on your back, pressing down with the fingertips into the second chakra area on each exhalation. Use firm pressure but do not dig very deeply into the abdomen.





Release the pressure on inhalation so the area can open up and expand more with each breath. Explore the rest of the abdomen in a similar way, moving in a clockwise direction. Look for tight or sensitive areas where the breath does not move easily. Remain there for a few breaths to allow any restricted areas to open. Allow time to work under the ribcage and move under the diaphragm. A facilitator can add pressure if needed.

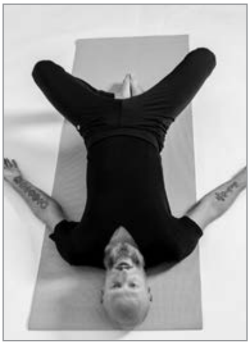
Pelvic segment exercises

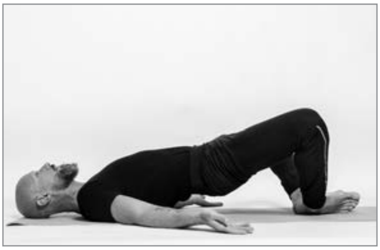
From personal experience, many of us know that pelvic tension is directly connected with being cut off from our sexuality or controlling sexual urges. Various aspects of social conditioning create guilt and shame around the most natural of the human conditions. Our sexual energy can become compromised, and therefore stuck in our pelvic region. This causes its upward flow to become constricted. When sexuality becomes internalized, it is not expressed freely or in connection with the heart; a tight belly and diaphragm further restrict the upward flow of sexual impulse to the heart. As we create an opening in the upper belts of tension, and clear the pathway for this energy to rise upward, we connect sex and sexuality to the heart.

Trauma Release Exercise (TRE) session

First Stage

To unlock tension in the pelvis, **lie on the back** with knees up and soles of the feet together. Bring the heels close to the buttocks. Allow the knees to fall out to the sides as much as possible, without pushing them. If you are super flexible, support the knees with the hands. Raise the pelvis slightly from the floor.





Second Stage

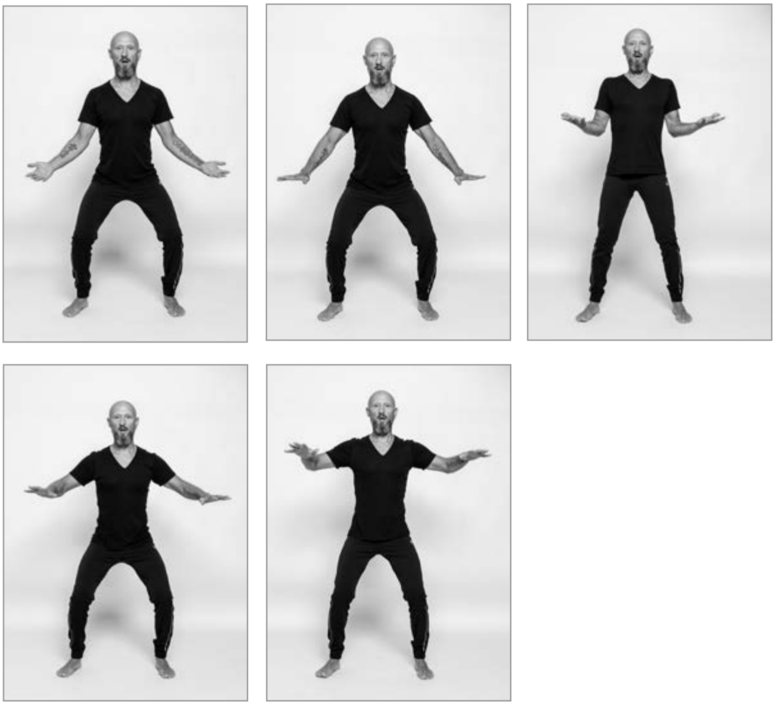
Slowly start to bring the knees closer together until they encounter slight trembling in the inner thighs. Stop there and breathe fully into this trembling until it starts to grow. The tremor will move up into the pelvis and lower belly. Allow the lower jaw to drop; breathe fully and deeply into whatever physical sensations arise.

Third Stage

Let the entire body follow this movement. Release control and surrender into the vibration that arises from the pelvis and travels through the entire body. Surrender into full, deep, connected breathing.

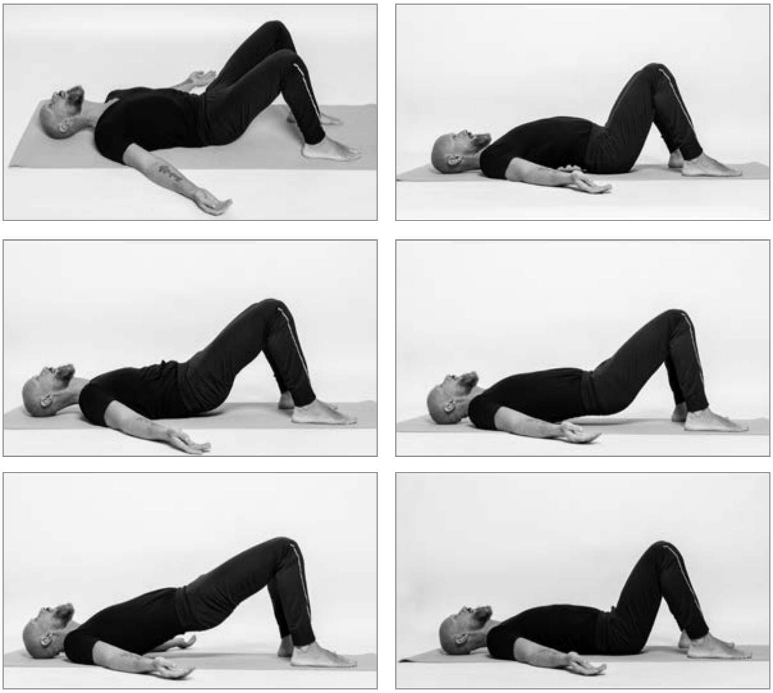
Simple charging exercises

1. **Standing feet parallel shoulder-width apart.** On the in-breath, sink into the knees and feet coming into a half-squatting position. With the out-breath, rise up so the energy can be charged from the ground through the legs and into the body. To strengthen that movement and the flow of energy through the body, also use your hands: in-breath palms down, out-breath palms up. Give it some time, so the stored tension in the muscles can release gently.



2. **Lying down, with feet on floor and knees up,** begin to lift the pelvis. Slowly exhale on the upward movement and inhale on the downward movement. Invite a constant movement, lifting the pelvis as high as possible, and then return slowly again to the floor. Do this for five to ten minutes, so the energy can charge through the legs. Often the energy can be observed as a slight vibration or shakiness.

(Images on following page.)



Pelvic/spinal unwinding exercise

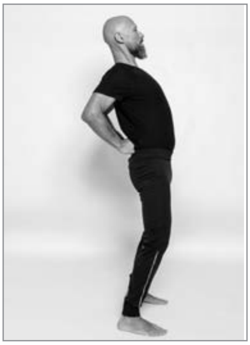
Stand with the feet firmly planted on the floor, hip-width apart. Shift the weight onto the right side of the body. Begin a soft connected breath. Notice the sensations on the right side of the body while focusing the breath there. Allow spontaneous movements to come from the right side of the body. Take your time and eventually repeat the same action on the left side. After a completed cycle, shift your weight to the center and feel the weight evenly distributed with support from the center.

Try to move only your pelvis in a figure-eight or infinity pattern. Bring the breath directly into this movement. Start with a very slow movement and allow it to continue to unwind. The movement might transform into a circular or spiral rotation of the pelvis or a tilting of the pelvis back and forth, as in lovemaking. Keep the movement of the pelvis isolated, so the rest of body is still but relaxed. Allow this movement to gradually move up the spine vertebrae by vertebrae, coming up to the neck and out toward the shoulders and arms. It is very important to keep the connected breathing steady throughout this exercise.

Bioenergetics, grounding, and stress exercises

The Bioenergetic Bow

This exercise is very effective in grounding and charging.



First Stage

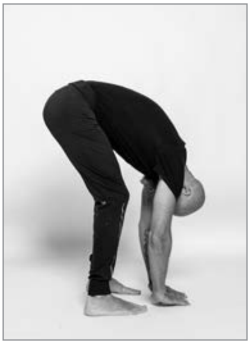
Stand with feet parallel, shoulder-width apart. As you sink into the legs, unlock the knees as if you want to sit down, and notice that the pelvis moves forward. The hands can support the pelvis from the sacrum (fists on sacrum, elbows back, shoulders down). The head is straight and eyes are open—looking straight ahead. If working with a facilitator, look into his or her eyes in a relaxed way, so as to maintain contact. Move eyebrows up and down, and breathe deeply through the mouth. **Take time for this exercise,** hold for 3-5 minutes.

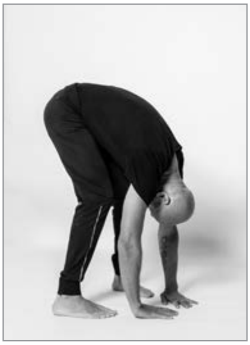
Second Stage

Next, bend over with the head down, arms hanging, allowing the back of the legs to become energized and to tremble. Finally, roll up slowly, shake, make sounds, and kick to release the energetic build-up. Repeat the whole sequence once or twice. If overwhelmed, make sounds and breathe deeper, but try to keep going, using the position of the bow.

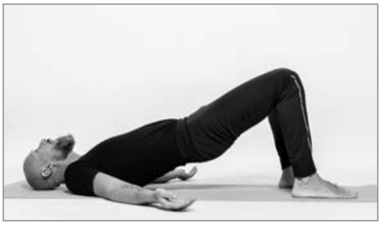
Third Stage

Bend down from the waist and allow your whole body to hang down. With knees slightly bent, gently and slowly bend and straighten your knees until you feel a slight tremor arising in the legs and knees. Let the tremor grow and spread into both legs. Keep this position for some time. To come out of this position, raise the torso slowly as if stacking each vertebra one on top of another, from the pelvis up to the ear. The head comes up last.





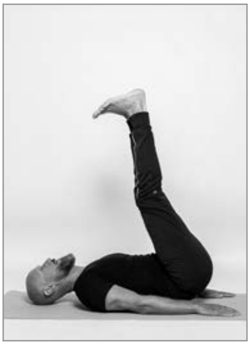
Bridge

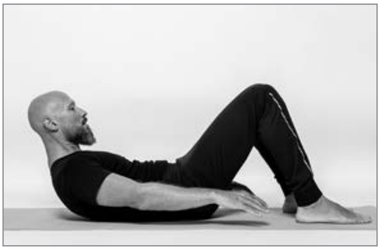


Lying on the back, with feet on the floor and knees up, push the pelvis up with the strength of the legs. Keep the pelvis up for five to ten minutes until some shaking starts in the legs. Throughout the exercise, always breathe deeply, and, if needed, allow sounds to release. Even if it is challenging, stay in the bridge position as long as possible (up to ten minutes).

Crunch

Lying down on the back, with legs straight and perpendicular to the floor, flex feet so they remain parallel to the ceiling. All muscles in the legs are engaged. Both the abdominal and lower back muscles are involved in this movement. This is a strong charging exercise. If you bend the neck and the head back strongly in order to hold the legs up, release the spine afterward by doing the opposite movement: feet and pelvis stay on the floor, and the head lifts as if to gaze through the knees towards the feet. This stretches the legs and the spine. If the neck is being strained, support the head.





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About the Author

BBTRS creator Giten Tonkov shares a mastery of body-oriented therapies that has evolved from more than 25 years of learning, exploration, and work with thousands of clients around the world. Giten leads practitioner trainings and experiential workshops several times per year in the Americas, Europe, and Asia—translating into several languages his “six-element” approach to healing, which is outlined in detail in this volume. He is a Licensed Massage Therapist and certified Breath & Body Oriented Therapist; both of these areas of expertise have assisted in the creation of his own modality.

A native of Ukraine, Giten has made his home in the Santa Cruz mountains in California, USA. In the past, he lived and practiced in New York City for 24 years, India and Bali, Indonesia, for more than ten years, and the Greek Island of Lesvos.